



SELF-STUDY

CLINICAL PSYCHOLOGICAL SCIENCE
THE UNIVERSITY OF TEXAS AT DALLAS
800 W. Campbell Road / Richardson TX 75080



SUBMISSION CYCLE: JANUARY 2025

Table of Contents

Overview	1
Mission and Training Philosophy	1
Goals and Objectives	1
Integration of Science and Practice	1
Structure and Leadership	2
Institutional Setting and Support	2
Leadership and Administration	3
Faculty Roles and Responsibilities	5
Resources and Support	10
Facilities and Equipment Access	10
Financial Support for Students	13
Additional Student Support Resources	14
Curriculum Design	14
Core Curriculum Overview	14
Integration of Research and Clinical Training	16
Student Recruitment and Mentorship	18
Recruitment Strategies	18
Admissions Procedures	19
Mentoring Model and Faculty Support	20
Training and Supervision	22
Training Experiences	22
Integration of Practice and Research	25
Quality of Clinical Supervision	26
Performance Evaluation and Feedback	27
Student Progress Monitoring and Feedback Mechanisms	27
Insufficient Progress and Grievance Handling	28
Quality Improvement and Outcomes	29
Data Collection and Program Evaluation	29
Continuous Improvement Strategies	30
Expected Outcomes for Graduates	31

List of Figures

Figure 1. Organization of concentration leadership.	4
Figure 2. Keyword co-occurrence network for UTD clinical science faculty publications.....	5
Figure 3. UTD faculty’s collaborations with institutions around the world based on their publications (collaborations shown are not exhaustive).	6
Figure 4. Cost of Living in Major Cities housing PCSAS-accredited programs or NIH.	13
Figure 5. Summary of Goals, Student Learning Outcomes, and Indicators.	16
Figure 6. Summary of major benchmarks. Apart from internship, these milestones are described more fully on pages 14-17 of the Psychology PhD Student Guide.....	17
Figure 7. Task-specific committees associated with key research benchmarks.....	21
Figure 8. Intensity of supervision interacts with volume of patient care to predict internship application competitiveness.	24
Figure 9. Research productivity and clinical supervision quality interact to predict internship application competitiveness.	25

List of Appendices

Appendix A: Concentration website materials (draft; not public; REDACTED)

Appendix B: PSEC website materials (draft; not public; REDACTED)

Appendix C: Psychology PhD Student Guide (REDACTED)

Appendix D: Attestation Letters from Clinically Trained Faculty (REDACTED)

Appendix E: Faculty Curriculum Vitae (CVs; REDACTED)

Appendix F: Building Realty Brochure (REDACTED)

Appendix G: Curriculum Map

Appendix H: Syllabi (SEE RESPONSE TO REVIEW INSTEAD)

Appendix I: Research Project Rubric (REDACTED)

Appendix J: Pre-dissertation Portfolio Rubric (REDACTED)

Appendix K: Dissertation Final Defense Rubric (REDACTED)

Appendix L: Concentration Handbook and PSEC Manual (draft)

Appendix M: PWC Codebook

Appendix N: Holistic Admission Rubric (REDACTED)

Appendix O: APCS Structured Interview for Admissions (REDACTED)

Appendix P: External Practicum Site Letters (REDACTED)

Appendix Q: Internship-level Competency Codebook

Appendix R: UT Dallas Policy on Student Grievances (REDACTED)

Appendix S: Alumni Survey

Appendix T: Alumni CVs (REDACTED)

Overview

Mission and Training Philosophy

The following excerpt from the mission statement of the Academy of Clinical Psychological Science captures the philosophical underpinnings of the clinical science doctoral concentration¹ in the Department of Psychology at UT Dallas:

Clinical science is a psychological science directed at the assessment, understanding, and amelioration of human problems in behavior, affect, cognition, or health, and at the application of knowledge to such problems in ways consistent with scientific evidence. The emphasis on the term "science" underscores a commitment to empirical approaches to advancing knowledge.

Our mission is to provide all students with high-quality, science-centered education and training to empower them with the knowledge and skills required for successful careers as scientists in clinical psychology. In alignment with this mission, the UT Dallas clinical science concentration is firmly grounded in the core tenets of the clinical science model and the Psychological Clinical Science Accreditation System (PCSAS). Rather than using the term "program," we employ "concentration" to emphasize our grounding of clinical psychology within the broader discipline of psychological science. Our approach aims to dispel the notion of clinical psychology as a separate entity and underscores its fundamental connection to the scientific principles of psychology.

Goals and Objectives

Our goal is to prepare graduates to be leaders and innovators in modern clinical psychology by equipping them with the necessary knowledge, skills, and abilities. Our proximal and distal objectives aim to produce graduates who are competent and successful in two key areas: (1) Conducting research relevant to the assessment, prevention, treatment, and understanding of health and mental health disorders. This includes developing skills necessary for immediate application in research and clinical practice. (2) Utilizing scientific methods and evidence to design, develop, select, evaluate, implement, deliver, supervise, and disseminate empirically based clinical assessments, interventions, and prevention strategies.

Integration of Science and Practice

Our concentration in Clinical Psychological Science is grounded in the principle that research and clinical service delivery are interconnected components of a unified scientific framework. This commitment to the clinical science model permeates all aspects of the concentration. From initial curriculum design and faculty mentorship to the multifaceted student evaluation practices, we strive to cultivate graduates who not only possess a deep understanding of clinical science but integrate it as a core tenet in their professional identity and activities. Throughout this document and on our planned (not yet visible) public facing website materials (Appendix A), we aim to underscore how our conceptual foundations influence the design, operation, climate, delivery, and anticipated student outcomes as well as the activities of concentration faculty, students, and eventual graduates.

¹ Although the term "program" is commonly used in academic vernacular to denote both comprehensive doctoral degree programs (i.e., Psychology Ph.D.) and specific areas of specialization within them (Clinical Psychology, Counseling Psychology, etc.) this usage can be technically inaccurate. To emphasize our grounding of clinical psychology within the broader discipline of psychological science, we intentionally use the term "concentration."

Structure and Leadership

Institutional Setting and Support

Founded in 1961, UT Dallas is a non-profit university fully recognized by the regional accrediting body, the Southern Association of Colleges and Schools Commission on Colleges (SACSCOC). Currently enrolling over 31,000 students, UT Dallas is a rapidly growing Carnegie Tier One (R1) research university. Its commitment to research excellence is reflected in its eight schools, housing 31 doctoral programs, with a strategic focus on growth in areas like medicine, health, and life sciences, all of which hold significant relevance to public health initiatives. In 2023 alone, UT Dallas boasted a research expenditure of nearly \$150 million. The UT Dallas Human Subjects Research Office safeguards ethical research conduct by providing regulatory oversight and training in human subject protection and good clinical practice.

The Department of Psychology, where the doctoral concentration in clinical science resides, is situated within the School of Behavioral and Brain Sciences (BBS). Focused on the intersection of mind, brain and behavior, BBS is committed to translating the latest research into treatment and interventions. BBS explores the biological and psychological underpinnings of thought, language, development, aging, pain, perception, and social interaction in both healthy individuals and those with atypical development or illness. BBS houses three academic departments: Neuroscience, Psychology, and Speech, Language and Hearing. BBS faculty are at the forefront of clinical science research, with \$31 million in new awards and a total research expenditure of \$34 million in the past year.

In addition to the academic departments, BBS fosters interdisciplinary collaboration by housing seven major research centers. Six of these are well-established: Callier Center for Communication Disorders, Center for Advanced Pain Studies, Center for BrainHealth, Center for Children and Families, Center for Vital Longevity, and Texas Biomedical Device Center. In addition, in coordination with the Office of Research and Innovation, BBS recently developed the Clinical and Translational Research Center (CTRC) to establish a centralized location and source of support for human trials of therapeutic drugs and devices. The new CTRC facilitates the assessment of novel therapies for human disease, focusing initially on disorders and conditions of behavior and the brain. The CTRC broadly supports research assessing clinical interventions by providing access to necessary expertise and necessary facilities and infrastructure, to include the following: Help with proposal development, including guidance on Good Clinical Practice (GCP) protocol development, staffing, budgeting, FDA interactions, FDA compliant data management, medical oversight, clinicaltrials.gov, space needs, and any other aspect of clinical trial design and development.

We have secured congressional funding to establish a new Psychological Services and Education Center (PSEC). The PSEC will provide internal practicum training as well as facilitate research opportunities that inform clinical science (See Appendix B for draft website materials). Under the guidance of faculty members with specialized expertise, students will engage in evidence-based interventions, gaining invaluable hands-on experience in a supportive environment. While leveraging specialized knowledge to provide high-quality clinical services to the community, the PSEC also serves as a research incubator by gathering data to inform clinical science research in faculty areas of interest. This model fosters a dynamic environment where service and scholarship seamlessly integrate. This approach is exemplified by the Specialty Clinic experience, where students actively select placements within ongoing programs of clinical research to deliver evidence-based interventions or conduct clinical assessments. This integrated experience compels students to harmonize, in real-time, individualized service delivery with the broader scientific objectives of the research program. By design, clinical service provision and research skills are

not treated as isolated entities; rather, they are integrated from the foundation, equipping graduates for success in public health-oriented clinical science careers.

The Department of Psychology at UT Dallas employs a dedicated team of staff members to support success. This team approach ensures critical tasks are covered by multiple individuals, fostering continuity and support for students and faculty. The department employs several Academic Support Coordinators, each specializing in specific graduate programs. These coordinators serve as the primary point of contact for students navigating crucial academic milestones, provide guidance and support with admissions processes, degree plan development, graduation requirements, and registration. An Administrative Services Officer manages purchasing requests, travel arrangements/reimbursements, and account reconciliation, ensuring smooth financial operations. The Department is further supported by an Administrative Assistant II who focuses on tasks related to the research management platform (i.e., Sona System) and provides support for faculty searches, the Psychology Lecture Series, and other departmental special events.

BBS offers a robust infrastructure staffed by well-qualified professionals. A team of IT Support Specialists provides comprehensive technical assistance. They address hardware and software issues, managing hardware inventory, and offering IT consultations to ensure smooth operation of essential technology for research and academic pursuits. In addition to IT support, administrative support is provided to coordinate new faculty setups, office moves, and facility projects. A Business Manager oversees essential administrative functions, including student, staff, and faculty hiring, timesheet administration, and coordination of HR transactions. A Manager of Research Administration and two Fiscal Operations Coordinators manage grant cost centers, assist with grant budget requests and approvals, and oversee Research Assistantship (RA) funding and appointments.

With respect to the concentration's visibility and outreach, BBS has numerous sources of support. A Communications Manager helps with research narratives, secures media coverage for significant research findings, and provides media training to faculty and researchers. The Marketing Manager offers consultation services, coordinates the creation of promotional items, and provides support for special events organized by the BBS. A dedicated Web Developer is available to assist with building lab websites and further promote online visibility (e.g., via the Comet Calendar and Lecture page, HTML emails and newsletters) of concentration information to students, faculty, and the broader scientific community. BBS also has a dedicated Recruiter to help get the best applicants, and who will also work to interface with the local community. An Administrative Assistant III is also available to assist with arrangements for VIP guests. The concentration plans to work closely with the Director and Senior Director of Development and Alumni Relations to identify and cultivate relationships with donors to support the concentration's continued growth and success.

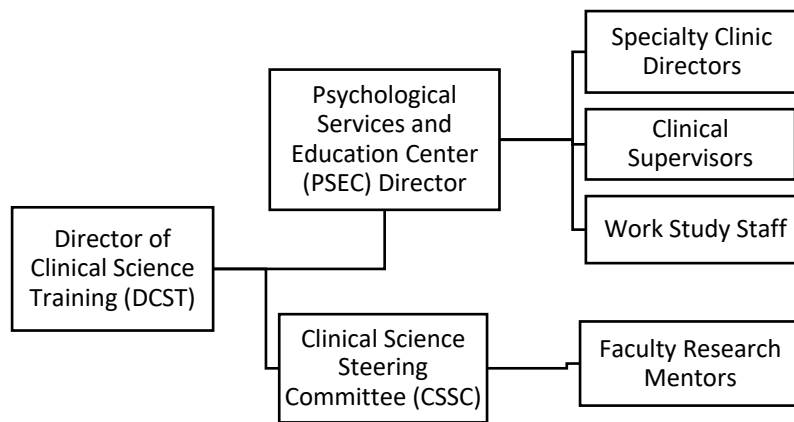
Leadership and Administration

The Director of Clinical Science Training (DCST) plays a crucial role in supporting the overall operations of the clinical psychology concentration. This position facilitates effective communication among faculty, students, and administrative bodies, enhancing the concentration's effectiveness. By bridging the gap between various components of the concentration, the DCST ensures that both academic and administrative functions are aligned, fostering an environment conducive to the development of future clinical scientists. The COGDOP Clinical Psychology Doctoral Program Leadership Structure Survey report (May 24, 2023), emphasizes the critical role of tenure-track faculty in leading clinical psychology training programs. A significant 88.9% were tenured or tenure-track faculty. This strong preference for experienced research faculty in leadership positions underscores the importance of academic stability

and expertise to effectiveness in maintaining high standards of education and training in the field. In keeping with established bylaws of BBS, the Department Chair, with the approval of the Dean, appoints the Director of Clinical Science Training (DCST) for a renewable five-year term. Dr. Callahan has agreed to serve as the inaugural Director of Clinical Science Training (DCST). She will be supported by an Academic Support Coordinator and a student worker.

The PSEC will be led by a full-time non-tenure system faculty member. In addition to providing administrative oversight of all aspects of the PSEC (e.g., staff, budget, infrastructure and technology). The PSEC Director will assure via policy, procedures, and standards of care, that delivery of appropriate evidence-based treatments meets standards of care and best practices to varied patient populations under close, feedback-rich supervision. Faculty-led specialty clinics within the PSEC will be led by a designated Director who is a faculty member with expertise in a specific population or clinical disorder (e.g., Psychosis, Bipolar, Neurocognitive Disorders). While the Specialty Clinic Directors hold leadership positions within their respective specialty clinic, their authority does not supersede that of the PSEC Director in the organizational structure (shown below) to ensure a cohesive and stable structure and environment.

Figure 1. Organization of concentration leadership.



Beyond the administrative DCST and PSEC Director appointments, leadership for the concentration is provided through the Clinical Science Steering Committee (CSSC). The CSSC acts as the concentration-specific graduate steering committee (GSC) for any GSC identified responsibility in the overarching Psychology PhD Student Guide (Appendix C). The CSSC will be composed of a mixture of core clinical science faculty and research mentors to prevent an insular approach to managing the clinical psychological science concentration. The CSSC will be chaired by the Director of Clinical Science Training and will include the Department Chair as well as the Director of the Psychological Services and Education Center (PSEC) as ex-officio members. All other members ($n = 4$; at least 2 of which must be clinically trained) will rotate off CSSC every 3 years. Upon activation of the CSSC, half of the members will be appointed for only 2 years to allow for staggered terms thereafter. This structure will ensure that the committee remains aligned with the department's goals and priorities while also prioritizing clinical science.

The committee will be responsible for promoting research, mentorship, and clinical practice within the concentration, and it will work to create a supportive and inclusive environment for all members. By adopting a flexible and inclusive approach to membership, the CSSC can effectively address the needs of

the concentration and contribute to the overall success of the department. The CSSC will meet at least monthly during the academic year to review, evaluate, and revise the concentration. This purview encompasses, but is not limited to, the concentration's mission, aims, core competencies, policies, curriculum development, research training, clinical training experiences, and admissions procedures. Additionally, the CSSC will consider briefings on research advancements and departmental, university, regional, and national/international developments that may impact the clinical science concentration and will deliberate potential concentration modifications in response to those updates.

Faculty Roles and Responsibilities

The faculty of the UT Dallas Department of Psychology have a rich history of producing cutting-edge science and infusing this expertise across teaching, research, and service activities. Our faculty, who serve as doctoral student mentors, provide rigorous scientific training. Expertise spans critical domains (e.g., clinical, cognitive, developmental, social and personality), methods (e.g., neuroscience, public health), diagnostic areas (e.g., autism, mood disorders, schizophrenia), behavioral health disparities, dissemination and implementation, as well as systems of care. Additionally, faculty hold expertise in advanced statistical techniques essential for psychological research, offering courses on dyadic group analyses, longitudinal growth curve modeling, multilevel modeling, and advanced multivariate analyses. Figure 2 displays major nodes from a network analysis (i.e., bibliometrix; Aria & Cuccurullo, 2017) of author keywords in UTD psychology faculty publications (nodes shown are not exhaustive).

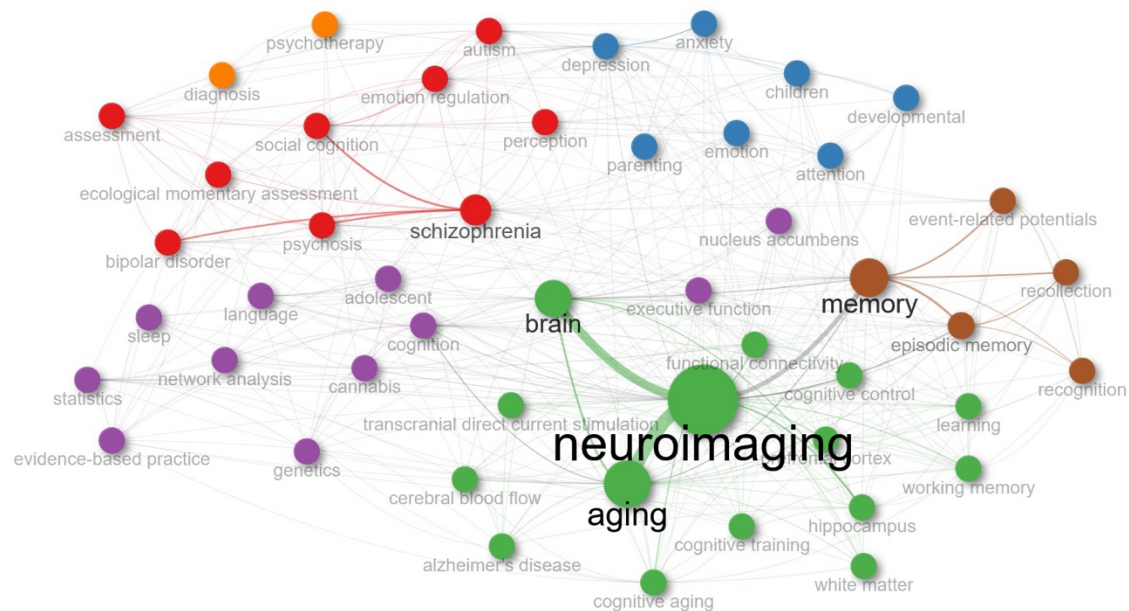


Figure 2. Keyword co-occurrence network for UTD clinical science faculty publications.

Consistent with the clinical science model, our faculty employ integrative strategies and produce high quality and scientifically informed products. Space limitations preclude an exhaustive list of all faculty scientific accomplishments, but prospective faculty mentors can be broadly described as successfully directing active research labs (typically funded by federal and non-federal research grants), producing peer-reviewed publications, and mentoring graduate students. The 36 faculty research mentors have a mean h-index of 38 (median = 31, range = 6 to 117). Figure 3 displays a network analysis (i.e., bibliometrix; Aria & Cuccurullo, 2017) of faculty's collaborations with institutions around the world based on their publications (collaborations shown are not exhaustive).

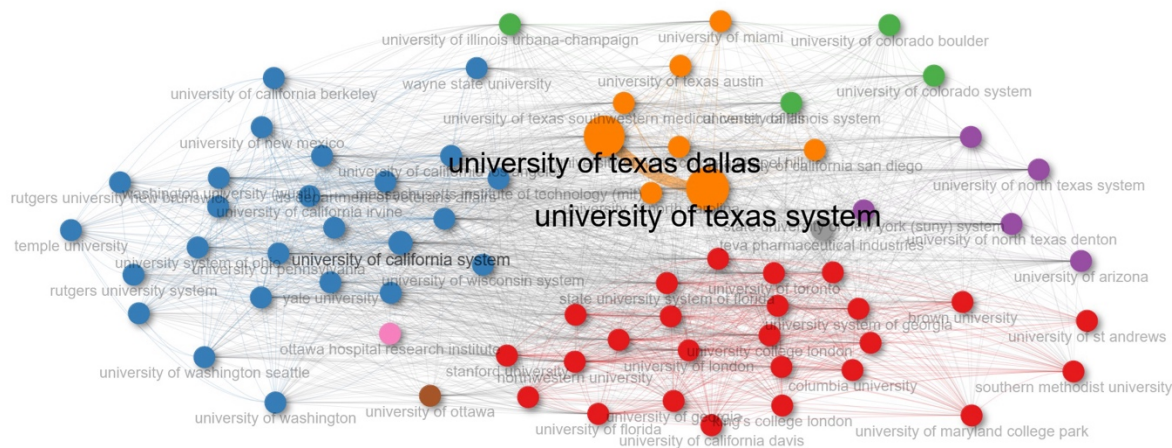


Figure 3. Psychology faculty collaborations with institutions around the world based on their publications (collaborations shown are not exhaustive).

The Department of Psychology currently has six faculty who are also clinically trained: Drs. Callahan (Professor), Kent (Assistant Professor), Pinkham (Professor), Ruggero (Professor), Warren (Associate Professor), and Ybarra (Professor of Instruction). Please see Appendix D for letters attesting to their commitments in support of the concentration. In addition, the department is currently conducting two open-rank tenure-track faculty searches in clinical science. The concentration is supported by a larger pool of faculty research mentors, from faculty across the Department of Psychology. The student-faculty ratio for any given research mentor is anticipated to be no more than 4:1 to permit high quality, intensive supervision of students. All faculty carry a teaching load. Tenure-track faculty typically teach 1-3 courses, depending on grant funding or administrative roles. Faculty supporting a graduate research assistant through a grant without a significant administrative role teach on a 1-1 schedule.

In recent years, many of the faculty research mentors have served as Editor-in-Chief, Senior Editor, or Associate Editor for peer reviewed journals, including Ackerman (*Personal Relationships*), Callahan (*Journal of Psychotherapy Integration; Training and Education in Professional Psychology*), Nelson (*Journal of Child and Family Studies*), Kennedy (*Imaging Neuroscience*), O'Toole (*Psychological Science; British Journal of Psychology; Journal of Vision; IEEE Transactions on Biometrics, Behavior, and Identity Science*), Park (*The Journal of Gerontology: Psychological Sciences*), Pinkham (*Journal of Experimental Psychopathology*), Rodrigue (*Psychological Science*), Rypma (*Brain Connectivity*), and Sasson (*Autism in Adulthood*).

In addition, many of the prospective faculty mentors have been appointed or elected to leadership positions in national or international organizations, including Callahan (President of APA Division 29; President of the Society for the Exploration of Psychotherapy; Board member of the Council of University Directors of Clinical Psychology; Fellow of APA Division 29), Golden (Secretary/Treasurer of Society for Mathematical Psychology), Kennedy (Executive Committee of International Society for Behavioral Neuroscience), Mills (Secretary of Cognitive Development Society), O'Toole (Scientific Advisory Board for Max Planck Institute for Biological Cybernetics), Owen (Scientific Advisory Boards for CDC and NICHD; Board Member for Greenhill School), Park (Member of APA Publications and Communications Board; Member of Board of Directors for American Psychological Society), Pinkham (International Consortium on Paranoia Research), Rugg (Member of National Steering Committee,

Institute of Gerontology; Member of Advisory Committee, Conte Center on the Neurobiology of Social Decision-Making, Chair NIMH BRAIN Initiative, Fellow of American Association for the Advancement of Science, Fellow of Association for Psychological Science, Fellow of Royal Society of Edinburgh), Ruggero (Executive board and founding member, Hierarchical Taxonomy of Psychopathology Consortium), Spence (Fellow of Association for Psychological Science), and Wig (Elected Member of Memory Disorder Research Society).

Further underscoring their achievements, several faculty hold esteemed academic appointments including Abdi (Senior Fellow, EURIAS Institute of Advanced Studies of Paris), Chapman (Dee Wylie Distinguished University Chair), D'Mello (Jon Heighen Scholar in Autism Research), Krawczyk (Jim Francis Chair in Behavioral Brain Sciences), O'Toole (Aage and Margareta Møller Professor), Owen (Robinson Family Professor), Rugg (Distinguished Chair in Brain and Behavioral Sciences), and Rypma (Meadows Foundation Chair in Behavioral Brain Sciences). Faculty have also received numerous awards and honors for their work in psychology, including prestigious fellowships and recognitions from major organizations such as the American Psychological Association, Association for Psychological Science, the Maltz Prize for Innovative and Promising Schizophrenia Research from the Brain and Behavior Research Foundation (Pinkham), and the Lewis Alan Opler Prize awarded for contributions in measurement by the International Society for CNS Clinical Trials and Methodology (Pinkham).

Faculty research mentors are listed below, with a brief description of their research interests. Please see Appendix E for a compilation of faculty curriculum vitae (CVs), history of research mentorship and current CVs for prior graduates.

Hervé Abdi: Psychometrics/Sensometrics/Advanced Multivariate Statistics/Qualitative Data Analysis/Genomics & Brain Imaging Methodology/Individual Differences.

Robert Ackerman: Narcissism and the impact of individual differences on interpersonal relationships.

Chandramallika Basak: Attentional control and working memory, cognitive training strategies, neural and cognitive predictors of complex skill acquisition, and aging.

Ryan Boyd: Computational social science, verbal behavior, personality and individual differences, social interaction, mental health, natural language processing, text analysis, machine learning, AI in psychology.

Jennifer Callahan: Identification and development of mechanisms for improvement of patient care and behavioral health outcomes among under-served and disadvantaged populations.

Sandi Chapman: Enhancing cognitive capacity and underlying brain systems across the lifespan.

Katherine Damme: How adolescent brain, pubertal, and cognitive development translates into pathways of risk for psychopathology; promoting healthy neural and cognitive development;

Anila D'Mello: Cognitive neuroscience of language and social cognition across development and in autism; Cerebellar contributions to cognition and disorders.

Francesca Filbey: Addiction as a model to understand reward system dysfunction in humans using multimodal brain imaging techniques.

Richard Golden: Computational psychometrics, learning sciences, and statistical machine learning.

Jiahui Guo: Cognitive and computational neuroscience of high-level perception and cognition (e.g., face recognition, social perception) in typical and neuropsychological populations; Naturalistic stimuli; Neuroimaging methods; Artificial intelligence; Deep neural networks.

Shayla Holub: Development and socialization of children's healthy eating, body image, and weight-bias.

Heidi Kane: Examine how close relationships are related to health; social support process, stress, coping and sleep.

Kristen Kennedy: Normal and pathological (AD) aging of brain and cognition; environmental/genetic/health/lifestyle modifiers; MRI brain function-structure association.

Jerillyn Kent: Motor abnormalities in psychopathology, with a particular interest in cerebellar abnormalities in individuals with psychotic disorders.

Daniel Krawczyk: Understanding reasoning and decision-making via behavioral studies, pupillometry, and brain stimulation.

Candice Mills: Cognitive development, with a primary focus on how children learn to take a critical stance when learning from others.

Jackie Nelson: Studies emotions in family relationships, particularly parent-child interaction, family stress and conflict, and child social-emotional development.

Alice O'Toole: Face recognition by humans and machines; neural processing of faces and bodies.

Noa Ofen: Developmental cognitive neuroscience across the lifespan; human memory; spatiotemporal dynamics of brain function; intracranial EEG; functional MRI; high-res imaging of the human hippocampus; genetic & environmental influences on cognitive & brain development.

Margaret Owen: Contextual influences of risk and resilience affecting children's development, addressing family relationships, poverty, and culture.

BoKyung Park: Social, cultural, and emotional factors that shape individuals' moral judgments and subsequent social decisions; underlying neural mechanisms.

Denise Park: Cognitive neuroscience of aging; preclinical Alzheimer's disease; effect of an engaged lifestyle on cognition; cultural neuroscience.

Amy Pinkham: Identify mechanisms of social dysfunction in schizophrenia and related mental illnesses in order to improve outcomes.

Karen Rodrigue: Adult lifespan studies of brain and cognitive aging utilizing structural, functional and PET imaging, genetic and environmental risk factors.

Michael Rugg: Cognitive neuroscience of memory, aging and neurological disease; neurobiological bases of fMRI and EEG signals.

Camilo Ruggero: Assessment and modeling of psychopathology; leveraging AI to improve assessment and improve access for underserved communities.

Bart Rypma: The cognitive and neurobiological mechanisms of human memory and experimental methodology of functional magnetic resonance aging.

Noah Sasson: Social cognition and interaction in autism, with a focus on bi-directional breakdowns in understanding between autistic and non-autistic people.

Kendra Seaman: Understanding the intersections of learning, motivation, decision-making, and adult development.

Melanie Spence: Studies how and when young infants process communicative cues in faces and voices; mechanisms of face perception.

Alva Tang: Developmental changes in social-emotional development; Individual differences and social-contextual factors; Mental and physical health.

Stacie L. Warren: Internalizing and externalizing psychopathology development and progression, executive function, emotion regulation, cognitive and affective neuroscience, cognitive training, computational modeling, clinical neuropsychology.

Gagan Wig: Brain network connectivity, healthy and pathological aging, brain health disparities across the lifespan.

Adam Woods: Primary prevention trials, cognitive aging remediation, non-invasive interventions, Alzheimer's disease, dementia prevention, neuroplasticity/neuromodulation, non-invasive brain stimulation, clinical translational neuroscience

Leehyun Yoon: Mechanistic understanding of the development of self-esteem, social motivation, and emotion regulation using neuroimaging techniques and behavioral experiments; preventing and treating depression and interpersonal difficulties.

Review of the provided CVs quickly indicates that faculty members have demonstrated remarkable success in securing grants from a variety of sources, including federal agencies such as the National Institutes of Health and the National Science Foundation, as well as state organizations and private foundations. This funding supports a wide range of research projects, from basic science to applied clinical studies. Many grants are directed toward innovative research initiatives that tackle pressing psychological issues, such as the development of evidence-based interventions and the exploration of behavioral health disparities. Furthermore, the faculty's commitment to interdisciplinary research is evident in the collaborative nature of many funded projects, which often bridge psychology with fields such as neuroscience, public health, and education.

Our approach to faculty recruitment and retention, for both tenure/tenure track and non-tenure system faculty, is multifaceted, encompassing not only attracting top talent but also fostering their professional growth and well-being within a supportive and inclusive environment. The School of Behavioral and Brain Sciences (BBS), the Department of Psychology, and the clinical science concentration are all committed to fostering a faculty community that reflects the richness of human experience and intellectual perspectives. Our recruitment strategy actively seeks candidates whose backgrounds enrich the intellectual discourse and create a stimulating environment. As an equal opportunity employer, faculty searches cast a wide net, attracting a wide pool of top candidates through professional networks. The selection process is rigorous, prioritizing individuals who will thrive within our collaborative and inclusive environment given its existing areas of strength and opportunities for growth. Recognizing the vital role faculty play in shaping the intellectual climate and future careers of students, competitive compensation and start-up packages are tailored to faculty research needs. Importantly, our commitment to these qualities is not restricted to only the tenure/tenure-track faculty positions.

Faculty satisfaction and well-being are crucial for long-term success (Kristof-Brown et al., 2005; Duffy et al., 2023). We promote faculty retention beginning with the recruitment and hiring processes. During faculty searches, we clearly articulate our mission and values to ensure a good fit between new faculty

and the available support structures. Upon hiring, faculty are encouraged to pursue their professional callings within a supportive and collaborative environment. As noted in the 2022 UT Dallas Climate Survey Summary, 7 out of 10 people (both faculty and staff) reported that they are happy working at UT Dallas and would recommend UT Dallas as a great place to work. The top six strengths of UT Dallas at engaging faculty, in order, were identified as: Feeling cared about by one's supervisor, fair performance evaluations, fair and objective decisions, accommodation of caregiving responsibilities, respectful treatment, and non-discrimination.

Resources and Support

Facilities and Equipment Access

As a rapidly growing Carnegie Tier One (R1) research institution, UT Dallas demonstrates a strong commitment to research, evidenced by its research expenditures exceeding \$145 million in 2023 and over \$147 million in 2024. This investment has fostered an infrastructure supporting numerous research-intensive facilities and initiatives (e.g., 50 research centers). For clarity, in this self-study we focus on resources specifically associated with the School of Behavioral and Brain Sciences (BBS). While recognizing that many resources blur the lines between research and clinical application, this section first overviews facilities and equipment that are heavily focused on research, before describing those that include a strong emphasis on the application of clinical science in partner settings.

In addition to lab facilities for each tenure/tenure-track faculty member in Psychology, BBS oversees the newly developed Clinical and Translational Research Center. Several centers, including the Center for BrainHealth (established 2000), Center for Vital Longevity (established 2010), and the Center for Children and Families (established in 2008), are led by faculty from the Psychology Department. The primary research facilities for Clinical Psychological Science concentration students and faculty are the Center for BrainHealth (CBH, established 2000), the Center for Vital Longevity (CVL, established 2010), and the faculty research labs in the interconnected Green and Jonsson buildings on the main campus.

In both core facilities and salient individual faculty labs, there are electrical wave recording and neuromodulation devices, such as EEG, ERP, TMS, TCDS, photobiomodulation, fNIRS, eye-tracking, and biophysiology equipment. Human neuroimaging research occurs at the UT Dallas Sammons BrainHealth Imaging Center, which features two MRI scanners (3T Siemens Prisma) and other equipment optimized for the acquisition and interpretation of data about the brain's structure and function. The MRI suites are designed and constructed to ensure optimal radio frequency shielding and fiber optic lighting systems. The MRI suites feature video monitoring, fMRI stimulus delivery and eye-tracking systems as well as physiological monitoring and recording equipment. Two full-time research technologists operate the scanner and facilitate data collection, storage, and transfer. In addition, UT Dallas collaborates closely with UTSW to utilize the shared Advanced Imaging Research Center (AIRC) facilities. The AIRC is a collaborative center that interacts with local universities (including UT Dallas, Southern Methodist University, and University of Texas at Arlington). It was established in September of 2005 as a formal academic and administrative entity to support the work of neuroscientists, radiologists, neurologists, neuropsychologists, psychiatrists, engineers, chemists, biomedical engineers and computer scientists in the development and application of burgeoning imaging technologies. The Center contains several human and animal MR scanners for tissue structure, function, and metabolism properties, including a high-field (7T Phillips) human scanner (e.g., for MR spectroscopy). There are also CT/PET scanners and a cyclotron for radioligand production and imaging (e.g., for assessment of Alzheimer's disease proteinopathies: beta amyloid and tau uptake).

The CVL houses more than 60 individuals, occupies approximately 36,000 square feet. It comprises two floors of experimental testing space, classrooms, offices, a server room with independent cooling and emergency power supply, and several conference rooms. It currently accommodates nine research groups along with visiting faculty and scientists. Full-time support staff include an associate director, a financial analyst, a grants specialist, and an IT specialist, with additional IT support contracted from the UT Dallas Office of Information Technology. The center is equipped throughout with high-speed internet that connects directly to the UT Dallas network, allowing efficient data-transfer and back-up. It is the focus of a vibrant and highly interactive scientific community, drawing from UT Dallas, UTSW, UT Arlington and Southern Methodist University, with strong interests in lifespan cognitive neuroscience and the psychology and neurobiology of aging and its disorders. The center organizes a biennial 'Dallas Aging and Cognition Conference', the next of which is scheduled for February 2025. It hosts weekly scientific colloquia with speakers drawn from the local and national scientific communities and organizes two annual public lectures which are free and open to members of the surrounding community.

The newly established Clinical and Translational Research Center (CTRC) is dedicated to accelerating the translation of UT Dallas innovations into effective health solutions. It currently offers approximately 4,400 square feet of combined waiting room, office, and laboratory space, including 1,200 square feet dedicated solely to the CTRC. The center provides comprehensive support for clinical trials, including administrative, clinical, medical, and regulatory assistance. The CTRC maintains six procedure rooms. Three of the rooms are set up as interview rooms, two rooms include audiological test suites with cutting edge equipment (updated summer 2024), one room is equipped with eye-tracking equipment, one room is equipped with an EEG system, one room is equipped with electromagnetic articulography, and one room houses an ultrasound WAVE system. Multiple rooms are equipped with Noldus technology that enables video and audio data capture for post-session coding, and a Noldus-control room is available for those collected Noldus data. Students also have access to two computer labs, each with 8 computer stations. A large conference room is available for reservation. Current plans are to relocate the CTRC to be on the same floor as the PSEC in a newly purchased building.

UT Dallas has a High Performance Computing Facility that is accessible to Clinical Psychological Science concentration students and faculty. The UT Dallas Cyberinfrastructure and Research Center (CIRC) is a central campus facility established in 2015 to support research and teaching by offering a suite of cyberinfrastructure services to faculty, staff, and students. Their mission is to facilitate research and education on campus by providing computing resources, support and training that are synergistic with national resources. The UT Dallas CIRC department provides free services to faculty, staff and students that includes the following: Access to High Performance Computing (HPC) resources; full system lifecycle support including design, purchase, installation, administration and consulting; software consulting including installation and debugging / optimization support; workflow optimization and parallelization assistance; and Cloud computing support for commercial cloud services. Among CIRC's resources is Ganymede, a heterogeneous local HPC cluster. Ganymede is a ~6000 core cluster based on CentOS and OpenHPC. It has a 10 Gigabit Ethernet network and a QDR (40 Gbps) InfiniBand interconnect configured in a fat-tree topology. It has two distributed file systems, one for the home directories over the 10 Gigabit Ethernet network available via NFS and one for the work file system that is a 1.2 PB high-performance parallel file system (GPFS) accessible over the InfiniBand network. The GPFS file system uses Data Direct Networks (DDN) directly attached to InfiniBand storage for the back-end hardware. Compute nodes are all dual processor, many core systems with a variety of Intel architectures including Cascade Lake, Sky Lake, Sandy Bridge, Haswell and Broadwell. Additionally, several nodes in the system have Nvidia Tesla accelerators based on the Nvidia Pascal and Volta architectures. In addition to Ganymede, CIRC supports ~10 (FIXME) other small clusters and several research group file and compute

servers for individual research groups. CIRC also provides campus support for both on-premises Linux desktops, Linux clusters, servers, and workstations, as well as providing full support for cloud computing resources including database and website support via Amazon Web Services. Most recently, they deployed ~150 NVIDIA H100 GPUs in the fall of 2024. These are the first dedicated GPUs on campus and represent the top processors of this type in existence.

Supporting the work of research labs as well as Centers, the UT Dallas library system provides a world-class foundation for doctoral research in clinical science. Central to this support system is the Eugene McDermott Library, housing a major collection of print and electronic materials. A Behavioral and Brain Sciences “PowerSearch” has been developed and is promoted on the database landing page to encourage simultaneously searching across salient databases (i.e., RIC, MEDLINE, APA PsycArticles, Psychology and Behavioral Sciences Collection, APA PsycInfo, SocINDEX). This is particularly helpful for interdisciplinary research like clinical science, which may require information from psychology, neuroscience, and medical databases. The Callier Library, a satellite facility dedicated to communication disorders, offers access to specialized resources relevant to the neuropsychological aspects of clinical science. Additionally, the library system leverages interlibrary loan services, providing access to materials not held within the UT Dallas collection.

The McDermott Library also provides access to valuable clinical resources, such as APA PsycTests, which offers standardized psychological tests and assessment instruments. Subscriptions to online platforms, like the series of APA Handbooks in Psychology, offer immediate access to authoritative and comprehensive reference materials, fostering in-depth exploration of core concepts and theories. In addition to its extensive collection, the McDermott Library fosters a culture of research support. Librarians with specialized expertise in psychology are available to assist students in navigating the complexities of academic research. These librarians offer personalized consultations, guide students in developing effective search strategies, and provide training on advanced research tools and databases. Faculty and students also have access to advanced statistical software, psychometric testing tools, and other software and equipment to support both experimental and clinical research projects.

Drs. Warren and Small (former BBS Dean) secured \$1,498,850 from the U.S. Department of Health and Human Services to establish a Psychological Services and Education Center (PSEC) at UT Dallas. A building adjacent to campus was purchased in fall 2024 that will house the PSEC as well as several clinical science research labs. Monies to create the PSEC are being used for necessary space renovation, furnishing, and equipping (technology infrastructure as well as clinical equipment and test kits) the PSEC. As described more fully in the realtor’s sales description (Appendix F), the 5-story building that will house the PSEC is adjacent to the UT Dallas campus with 151,340 gross square feet, ample parking and access to public transit. In addition, the Provost has approved funding via a competitive UT Dallas strategic funding initiative that provides an additional \$1,375,701 in funding to support fees and costs associated with seeking preliminary accreditation and joining the PCSAS Founder’s Circle; a non-tenure track faculty position for a full-time PSEC Director on a 12-month appointment; a graduate student assistant to work with the PSEC Director; two tenure-track faculty and their associated start-up costs; and independent contractor costs, if needed.

A state-of-the-art PSEC facility will support high quality training in the application of clinical psychological science. The PSEC will be equipped with digital video and audio recording devices, as well as advanced assessment tools and psychotherapy resources to ensure students can practice skills in a closely supervised and controlled environment. Additionally, the PSEC will provide the necessary infrastructure to establish faculty-directed specialty clinics that will act as clinical research laboratories within the PSEC

where students can work closely with faculty. The PSEC will support students as they gain practical experience working with individuals from various cultural and socio-economic backgrounds. Lastly, the Texas Network for Advancing Behavioral Health Outcomes, Research, and Services (Texas NABORS) was recently launched by two of our clinical science faculty (Ruggero; Callahan). Texas NABORS facilitates community engagement and collaboration among researchers, clinicians, and policymakers in the region to redress local behavioral health needs.

Financial Support for Students

The School of Behavioral and Brain Sciences (BBS) at UT Dallas recognizes the importance of financial support in attracting and retaining high-caliber graduate students. To minimize financial barriers and foster student success, BBS and the Psychology Department offer competitive financial support packages. These packages include a monthly stipend (for all 12 months, annually) and a university scholarship covering full-time tuition and fees (i.e., 9 hours of enrollment during the fall and spring semesters and up to 6 hours of enrollment during the summer). Award of either a TA or RA allows tuition to be assessed at the Texas resident rate. Students awarded assistantships who enroll in the UT Dallas Student Health Insurance Plan (UT Dallas SHIP) also receive 100% of the cost for UT Dallas SHIP individual coverage (student only) premiums as part of their compensation. The monthly stipend for research and teaching assistants is currently \$2,269 per month; some faculty pay RAs a higher stipend per agreement with their granting agency. Stipends are currently aligned with the National Institutes of Health (NIH) graduate salary benchmark. As shown in Figure 4, the cost of living in our area compares favorably when contrasted to either other major cities with PCSAS-accredited programs, or the metro area associated with NIH.

Figure 4. Cost of Living in Major Cities housing PCSAS-accredited programs or NIH.



Source: salary.com

Funding for both TAs and RAs can be renewed annually for up to five years, contingent upon maintaining satisfactory academic progress. While we strive to ensure continuity in research assistantships, the dependence on external funding may necessitate offering a teaching assistantship. Students are also welcome to directly apply for TA positions to gain teaching experience. By offering this comprehensive financial support structure, the Department aims to attract talented students and create an environment conducive to academic focus and research productivity. A written description of the financial package, in the form of a letter, is provided by the Psychology Department to each student who is selected for admission to the doctoral concentration.

Because students are expected to complete the clinical concentration in 6 years (including internship), funding for a sixth year is not an issue unless a student encounters unexpected delays during their dissertation or does not match with an internship. If a student does not match for internship and has

exhausted guaranteed funding, we will seek additional funding through external fellowships (e.g., NIH NRSA), research assistantships, and/or training grants. Students receiving financial support packages are encouraged to explore and apply for external funding opportunities to further alleviate financial burdens during their graduate studies. Department's minimum stipend commitment is generally reduced dollar-for-dollar if the student accepts funding from other sources (e.g., fellowships, training grants). In some cases, funding from other sources exceeds the minimum level set by the Department.

Additional Student Support Resources

Students can apply annually for \$1,000 travel awards to attend one or more conferences. After the dissertation proposal defense, students can apply for \$1,000 Psychology Department Dissertation Awards. Students can also apply for additional funding through the UT Dallas Office of Graduate Education (OGE); these awards include the Betty and Gifford Johnson Travel Awards (which provide an additional \$1,000 in travel funding beyond the departmental travel awards). OGE also has competitive awards for dissertation funding, including the Dean of Graduate Education Dissertation Research Award (\$5,000 award for dissertation work). This office also awards the Best Dissertation Award annually for one student from each Ph-D granting school, as well as the annual Outstanding Doctoral Student Award. Last, but not least, BBS Centers provide additional support for excellent doctoral research. The Center for Children and Families has a new Robinson Student Research Awards program, which allows students to apply for up to \$3,000 to support their dissertation research. Similarly, the Center for BrainHealth, supports emerging researchers at the master's, doctoral, and post-doc levels with project awards of ~\$20,000. Since 2008, Friends of BrainHealth donors have raised more than \$4.3 million and launched dozens of cutting-edge studies exploring ways to enhance, protect, and restore brain health across the lifespan.

The UT Dallas Office of Research and Innovation and the Office of Human Subjects Protections ensure that all research adheres to ethical standards and regulatory requirements. Departmental resources provide information and training that students may use to learn how to efficiently navigate funding opportunities, proposal development, post-award fiscal and effort advising, as well as guidance in facilitating commercial partnerships. Inventors and entrepreneurs benefit from commercialization support with industry knowledge to assist with startup incubation, negotiations, and licensing. The Office of Sponsored Projects can help students in two areas: Sponsored Projects and Post Award Management. The Sponsored Projects (OSP) team provides support in obtaining and managing sponsored awards, as well as review and approval of proposals sent to external sponsors. The Post Award Management (OPM) team provides support in managing sponsored awards and assuring proper stewardship of funding. While providing central oversight for the post-award fiscal activities of all sponsored projects, including student RA positions, OPM establishes sponsored projects cost centers and budgets, reviews and validates transactions to support research, and acts as a liaison between Research and Billing/Accounts Receivable office.

Curriculum Design

Core Curriculum Overview

Doctoral training in clinical psychological science at UT Dallas will be guided by a comprehensive curriculum map. Our COMETS, or Competency-Oriented Methods of Education, Training, and Supervision, emphasize developing essential knowledge, skills, and abilities for students to succeed as clinical scientists. This innovative approach is grounded in adult learning research, enhancing functional competency, knowledge retention, and student confidence (DiBartolomeo, Shukla, Westra, & Shekarak Ghashghaei, 2021; Hecimovich & Volet, 2011; Lewis, Rebesch, & Hunt, 2022; Yamin, et al., 2023). COMETS are built on a hierarchical framework that includes five goals encapsulating our expectation that

graduates will emerge into their careers as leaders in clinical psychological science. Within Appendix G, Table 1 provides an overview of curriculum linkages, Table 2 provides rank order information on the ease of attainment associated with the competencies expected of students, and Table 3 provides granular information. Draft syllabi are provided in Appendix H.

As summarized in Figure 5, student learning outcomes (SLOs) are nested within each goal. Each SLO is further delineated by observable indicators of student's attainment of each SLO. Behaviors indicative of need for remediation are also delineated for each SLO. Our curriculum map utilizes a developmental scaffold. We believe the successful student for our training will have begun their preparation well before matriculation. Informed by the current APA Guidelines for the Undergraduate Psychology Major (Version 3.0; August 2023), indicators of readiness for matriculation are considered during the admission process. Table 3 of Appendix G identifies methods and measures used to assess readiness for admission. These indicators can be feasibly attained by exceptional candidates for admission through completion of a bachelor's degree at most institutions offering a major in psychology, even among institutions without active faculty research labs. Focusing on these indicators supports our plans for broad outreach and recruitment of applicants for admission across a wide range of institutional settings (as detailed more fully in the section entitled, "Recruitment Strategies").

Table 3 of Appendix G also differentiates between indicators of pre-candidacy and candidacy level competency attainment. The pre-candidacy indicators emphasize an expansion of foundational knowledge, which is primarily addressed in courses such as CPSY 7300 (Clinical Psychological Science I) and CPSY 7310 (Clinical Psychological Science II), as well as core departmental courses (HCS 6302, PSYC 6312, PSYC 6313, PSYC 6317). Candidacy indicators reflect mastery of the advanced knowledge, skills, and competencies expected of doctoral graduates in clinical psychological science at UT Dallas. These competencies are primarily evaluated through the pre-dissertation portfolio and clinical competency assessments conducted by expert supervisors. Successful completion of the predoctoral internship and

oral dissertation defense indicates readiness for an independent career. The concentration is designed to be completed in six years. This timeline acknowledges the rigor of the concentration while ensuring adequate time for research training, clinical skill development, dissertation completion, and the predoctoral internship.

Integration of Research and Clinical Training

The curriculum fosters a bidirectional relationship between clinical practice and research. Throughout training students and faculty engage in research addressing challenges with clinical implications, applying evidence-based practices while contributing to the field. Core clinical courses encourage students to use empirical evidence to inform treatment and interventions, apply research findings to case studies, and link theory to practice, while their clinical training raises research questions that contribute to the knowledge base addressing clinical challenges. The concentration will incorporate an evidence-based practice model, training students to integrate clinical expertise, patient values, and research evidence. Students will learn to critically evaluate and apply research to their clinical decisions, ensuring that their treatment approaches are grounded in the most current and relevant findings. Clinical benchmarks are more fully described later in this document, within the section entitled, "Training Experiences".

Figure 5. Summary of Goals, Student Learning Outcomes, and Indicators.

Goal 1: Mastery of Foundational Principles of Clinical Science

- 1.1 Scientific Mindedness
- 1.2 Scientific Foundations

Goal 2: Advanced Research Competence in Clinical Science

- 2.1 Research Skills Development
- 2.2 Innovative Inquiry

Goal 3: Ethical and Culturally Competent Practice

- 3.1 Knowledge of Ethical, Legal, and Professional Standards and Guidelines
- 3.2 Awareness and Application of Ethical Decision-making
- 3.3 Ethical Conduct
- 3.4 Self as Shaped by ICD and Context
- 3.5 Others as Shaped by ICD and Context
- 3.6 Interaction of Self and Others as Shaped by ICD and Context
- 3.7 ICD Applications Based on Individual and Cultural Context
- 3.8 Empowerment

Goal 4: Clinical Expertise and Intervention Skills

- 4.1 Concern for the Welfare of Others
- 4.2 Reflective Practice
- 4.3 Self-Assessment
- 4.4 Self-Care
- 4.5 Clinical Skills
- 4.6 Diagnosis
- 4.7 Measurement and Psychometrics
- 4.8 Assessment Evaluation Methods
- 4.9 Application of Assessment Methods
- 4.10 Diagnostic Conceptualizations and Recommendations
- 4.11 Intervention Planning
- 4.12 Intervention Implementation
- 4.13 Knowledge of Interventions
- 4.14 Scientific Foundations of Professional Practice
- 4.15 Treatment Monitoring

Goal 5: Leadership and Professional Development

- 5.1 Integrity
- 5.2 Deportment
- 5.3 Professional Identity
- 5.4 Accountability
- 5.5 Interpersonal Relationships
- 5.6 Expressive Skills
- 5.7 Affective Skills
- 5.8 Supervision Skills Development
- 5.9 Supervision Expectations and Roles
- 5.10 Supervision Processes and Procedures
- 5.11 Awareness of Factors Affecting Supervision Quality
- 5.12 Participation in Supervision Process
- 5.13 Ethical and Legal Issues in Supervision
- 5.14 Knowledge of the Shared and Distinctive Contributions of Other Professions
- 5.15 Functioning in Multidisciplinary and Interdisciplinary Contexts
- 5.16 Understands how Participation in Interdisciplinary Collaboration / Consultation Enhances Outcomes
- 5.17 Respectful and Productive Relationships with Individuals and Other Professions
- 5.18 Role of Consultant
- 5.19 Addressing Consultation Referral Question
- 5.20 Application of Consultation Methods

Figure 6. Summary of major benchmarks. Apart from internship, these milestones are described more fully on pages 14-17 of the Psychology PhD Student Guide.

1st Year
Research Project Proposal, due 2/15
2nd Year
Project Manuscript, due 5/1
Grant Proposal OR Integrated Lit Review, due 5/1
3rd Year
Project Presentation, due prior to dissertation proposal
Dissertation Prospectus, due fall semester
Dissertation Proposal, due spring semester
Public Defense of Dissertation Proposal, due 5/15
5th Year
Internship Application, due fall semester
Oral Defense of Dissertation, due spring semester
6th Year
Internship 6th year (must be APPIC member program)

Although not a strict requirement, our curriculum and foci are structured to encourage at least two first-author publications and around eight total peer-reviewed publications. We believe our emphasis on research activity is well-aligned with the PCSAS Internship Report (March 2023; *m* articles = 8 and median first authored = 2). While a strong publication record is not the sole determinant of internship placement, it can significantly enhance applicant competitiveness. The PCSAS Internship Report also indicated that approximately 21.2% of students from PCSAS programs successfully secured federal fellowships. While obtaining a fellowship is not required, actively applying for and securing such funding is encouraged. Our inclusion of grant proposals aligns with leading programs nationally and supports students in their professional development, preparing them to become successful independent investigators and possibly strengthening their candidacy for competitive internship placements.

Ethical Standards and Legal Requirements

Foundational knowledge pertaining to ethics and legal requirements will be acquired via a combination of research-intensive core courses (e.g., HCS 6302, PSYC 6312, PSYC 6313, PSYC 6317) and clinical science specific core courses (e.g., CPSY 7300; CPSY 7310). Students will be required to complete Human Subjects Protection (HSP) training modules through the CITI Program every three years. Additionally, those involved in clinical trials must complete the CITI Program's Good Clinical Practice (GCP) training and renew it every three years thereafter. This comprehensive educational framework lays the groundwork for students to develop functional competencies related to ethical standards and legal requirements throughout their research and clinical training. To further enhance this learning, experiential opportunities are a vital component of our curriculum, fostering active engagement and competency development in ethical standards and legal requirements.

As described more fully in the section entitled, "Nature and Format of Students' Research Requirements" students are expected to be actively engaged in research throughout their doctoral training and required to develop research proposals that adhere to ethical guidelines. Primary research mentors will provide frequent formative feedback and prepare students for summative evaluations of the pre-dissertation portfolio and dissertation (as described in the sections on "Mentoring Model and Faculty Support" and "Training Experiences"). In addition, the Post-Approval Monitoring Program (PMP) at UT Dallas performs comprehensive evaluations of ongoing studies to ensure they align with approved protocols and regulatory standards. The primary objectives of the PMP are to protect the well-being of participants, offer educational resources to researchers, and pinpoint opportunities for enhancement. Reviews can be triggered randomly, upon the request of the Principal Investigator (PI), or in response to concerns regarding participant safety. Competency in ethical standards and legal requirements pertaining to research is evaluated via the research project (Appendix I, Research Project Rubric items 9-

12), pre-dissertation portfolio (Appendix J, Pre-dissertation Portfolio Rubric items 20-24), and dissertation oral defense (Appendix K, Dissertation Final Defense Rubric items 20-24).

Students will also engage in internal and external clinical practicum placements to gain experience under the close supervision of seasoned professionals who are committed to ethical practice. The PSEC Manual (Appendix L) provides guidance to support trainees in applying foundational knowledge to the clinical setting, particularly in the areas of Ethics and Confidentiality, Technology and Records Management, Communication and Social Media, and HIPAA Procedures and Practices. Clinical supervisors will serve as mentors, imparting insights into ethical decision-making frameworks and facilitating discussions that enhance students' ability to analyze and resolve complex ethical issues encountered in clinical settings. For both internal and external practicum placements, competency in navigating real-world ethical dilemmas will be documented via items 2A, 2B, and 2C of the Profession-wide Competencies (PWC) Rating Form (Appendix L, Addendum B) using the PWC Codebook (Appendix M).

Student Recruitment and Mentorship

Recruitment Strategies

Following attainment of provisional accreditation, we plan to employ a multifaceted approach to nationwide recruitment of applicants. We are committed to cultivating a supportive and inclusive learning environment that recognizes the unique experiences and perspectives of each student. We aim to create a supportive and inclusive learning community that empowers all students. For many potential applicants, the website for our concentration (Appendix A) will provide their first contact with our training. Following attainment of preliminary accreditation, we will launch the planned website with google analytics installed so we can evaluate how potential applicants are interacting with our public facing materials (particularly the section of the site entitled, "Student Recruitment and Selection"). However, we will not rely on search engines for recruitment.

Locally, we will work within UT Dallas to recruit from the large number of National Merit Scholars who attend UT Dallas. UT Dallas has consistently ranked among the top 5 universities for matriculation of National Merit Scholars. According to institutional data, more than half of those Scholars are from outside of Texas. In speaking with one such scholar who is graduating with a major in psychology, she characterized many such scholars as coming from disadvantaged economic backgrounds.

Regionally, we will focus our recruitment efforts by engaging with institutions that are not part of the UT System. According to the most recent (2024-2025) NASA Minority Serving Institution (MSI) List, Texas is home to 76 public 4-year institutions of higher education. Drs. Ruggero and Callahan have been collaborating with the largest MSI in our area (University of North Texas) on HRSA-funded grants to foster supportive relationships between MSIs and major public universities in our region. In addition to stimulating faculty research via seed grant awards, they have screened the MSIs to determine which offer a bachelor's degree with a major in psychology and have assembled a contact list to engage in outreach to those psychology departments. We will use that contact list to reach out to these departments and alert them of our concentration upon attainment of provisional accreditation to ensure that we reach qualified students from those institutions.

Nationally, we will advertise the concentration via prominent psychology networks and online platforms frequented by prospective graduate students and attend recruitment events held by national organizations (e.g., CUDCP). Program faculty will also attend and participate in conferences relevant to clinical science, showcasing the concentration's strengths and connecting with potential applicants.

Admissions Procedures

The Department of Psychology prioritizes holistic admissions processes that value varied backgrounds and experiences. This multifaceted evaluation considers not only academic records but also personal statements, recommendation letters, and relevant experiences, promoting fairness and mitigating biases in selection. In addition to an online application form and transcript(s), applicants submit essays to highlight their experiences and interests. Strong recommendation letters from psychology professors and research mentors are also highly valued. GRE test scores were temporarily waived due to the recent pandemic but may be reinstated in 2025 due to state legislation. As part of the holistic review process, each GRE score submitted will be recoded to indicate which quartile the score falls within, relative to other applicants in the Department. In this way, the relative strength of academic achievement can be preserved for consideration of admission (i.e., by identifying whether the student was in the top quartile, middle two quartiles, or lowest quartile), while risks to those impacted by known issues of test bias are minimized (i.e., the margin of error associated with demographic variables).

Designated faculty members review each application using two standardized rubrics (Matrix 1 and Matrix 2 in Appendix N). The admissions committee convenes to discuss applicants in descending order of their combined rubric scoring. Significant discrepancies between raters are discussed to reach consensus. Top-scoring applicants typically demonstrate exceptional qualifications and are often unanimously recommended to be invited to interview ("green"). Further review becomes more nuanced for second tier applicants. These candidates may be marked "yellow" for potential invitation to interview (contingent on department capacity, strong faculty advocacy, or the availability of research assistantships). Conversely, applicants deemed unqualified, lacking necessary skills, or lacking a research fit with available faculty mentors are designated "red" and are deemed unacceptable for admission. Finally, to ensure a comprehensive evaluation, the committee may re-sort applicants based on scores associated with traditional academic metrics versus holistic review as a final verification check that no standout strengths or weaknesses have been overlooked.

Applicants seeking to enter the clinical science concentration who are deemed green or yellow will advance to consideration by the CSSC. The CSSC will conduct structured interviews using materials made available by the Academy of Psychological Clinical Science (APCS; Appendix O). The remote video-based interview will seek to elicit behavioral samples associated with the following ten qualities: Critical Thinking; Scientific Curiosity; Good interpersonal skills; Analytical thinking; Open-minded; Problem solving skills; Independence and self-guided inquiry; Takes initiative; Works well within a team; and Evidence-based thinking. A standardized rubric will be applied to responses given with two faculty providing scores for each applicant. Depending upon the volume of applicants advanced for their consideration, the CSSC may elect not to interview applicants coded yellow to keep the process moving forward in a timeline manner. Following completion of interviews, the concentration committee convenes for a consensus meeting to review applicant rankings. Applicants who are deemed acceptable will be routed to potential faculty mentors for consideration.

To remain under consideration following faculty review, two conditions must be met: (1) at least one faculty member must express interest in serving as the applicant's research mentor, (2) at least one clinically trained faculty member must express interest in serving on the applicant's personalized training committee. Applicants meeting those conditions will be invited for an in person visit to UT Dallas for an open house and in person interviews. The typical visit has applicants arrive on a Thursday evening, with open house and interviewing on Friday that concludes with a dinner, before departing on Saturday morning. Applicants' travel expenses are covered by the Department of Psychology. If the applicant resides within 3 hours of driving time from UT Dallas, mileage and hotel expenses are covered. For

applicants who reside more than 3 hours from UT Dallas, the Department additionally pays the cost of airfare. Food and local transportation are provided.

Mentoring Model and Faculty Support

We recognize the unique needs of each student and understand that no single faculty member can meet all their needs. We believe a more robust culture of support that includes a range of mentoring durations and sources is necessary to provide appropriate support to our students. Our mentoring community is comprised of long-term relationships (e.g., primary research mentor, personalized training committee, peer cohort), task-specific relationships (e.g., course instructors, practicum supervisors, and evaluative committees), and short-term exposures (e.g., speakers, conferences, workshops).

As previously described (in the section entitled, “Admissions Procedures”), primary research mentors are identified during the admissions process. Faculty research mentors must be appointed in a tenured or tenure-track position and demonstrate a consistent record of producing high-quality, scientifically rigorous research. Mentorship matches consider a combination of factors. Shared research interests provide a strong foundation for collaboration. Additionally, methodological expertise of the faculty member should align with the student's research goals. While applicants may express interest in specific mentors, not all have had access to prior research mentoring experience. To address this, the admissions review committee considers each applicant's research interests and identifies potentially compatible mentors as part of the review process. Faculty are approached to gauge their interest in serving as a mentor for these students before extending an invitation for an in-person visit. Applicants for whom no faculty express interest are not invited to interview, regardless of academic qualifications. This ensures a strong match between student research aspirations and faculty expertise. Beyond research interests and methodology, the on-site visit provides an opportunity for applicants and faculty to evaluate compatibility. The concentration prioritizes a low faculty to clinical science student ratio, ideally 1:4 or below, to ensure close mentorship and personalized attention throughout doctoral training. This approach fosters a supportive environment where students can thrive under the guidance of experienced faculty mentors.

Our admissions processes also require that at least one clinically trained faculty member is willing to serve on each student's personalized training committee. A three-faculty personalized training committee, including the primary research mentor and at least one clinically trained faculty member, is formed during the first semester. This committee provides mentorship throughout the training program. Meeting at least once per semester, their collective role is to provide expert guidance that encourages each student to take full advantage of the opportunities available to them as they prepare for their career.

In addition to task-specific mentoring during courses and practicum, three task-specific committees are formed prior to key research benchmarks (summarized in Figure 7). These committees serve a mentoring role during the student's task work before taking on responsibility for provision of summative evaluation upon task completion. Additional information on these committees (e.g., student input, formation timeline and process, faculty composition) is found in Appendix C. Members of the personalized training committee may or may not additionally serve on these task-specific committees. Key task-specific committees, their composition, purpose, and membership are summarized below. In addition to these committees, the CSSC also holds some task specific roles with students (e.g., reviewing the research paper). However, in contrast to the personalized training committee, the CSSC is not formed for any given student. While the CSSC can be an important source of feedback, their primary role is to ensure

standards *across* students. It is the role of the personalized training committee to address CSSC feedback during their mentorship of the student.

Figure 7. Task-specific committees associated with key research benchmarks.

Pre-dissertation Portfolio Evaluation Committee Composed of the research mentor and two UT Dallas faculty (one from the Psychology Department), this committee evaluates the pre-dissertation portfolio.	Supervising Dissertation Committee Four faculty members from the Psychology Department, nominated by the Director of Clinical Science Training and approved by the Dean of Graduate Education, oversee the dissertation.	Dissertation Oral Examination Committee The Dean of Graduate Education appoints a neutral Chair to work with the Supervising Dissertation Committee for the oral dissertation defense.
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Short-term exposures to intellectual leaders in one's area can also have lasting effects. Students can be awarded \$1,000 annually to attend professional conferences and network with experts in their field. One of the expectations of the personalized mentoring committee is that they will assist the student in learning how to use conferences for networking and engaging with leading scholars in their area. Students who are unable to travel may still benefit from such encounters via attendance and participation at local events. Across BBS, faculty routinely engage with prominent clinical scientists, inviting them to present their cutting-edge research to the program community. Organized by BBS centers and programs, [external speakers](#) appear weekly, if not more, on campus across the year. The Department also provides seminars, talks, and workshops on topics such as grant writing, research ethics, and career development opportunities to further support professional growth.

Additional time-limited advising and/or mentorship is available on an as needed basis for students needing support in areas that go beyond the bounds of specific faculty member's primary expertise or role expectations. The [Office of Graduate Education](#) offers a range of services, including academic advising, career counseling, and support for international students. Additionally, the [University Career Center](#) provides career counseling, job search assistance, and resume and cover letter writing workshops. To foster a sense of community and support, the [Graduate Student Assembly](#) organizes social and professional events. For students with disabilities, the [AccessAbility Resource Center](#) offers a variety of support services. The [Counseling Center](#), provides mental health services to all students, including graduate students, but is not a practicum site for our concentration. Furthermore, UT Dallas offers a variety of academic support services, such as a [writing center](#), [oral presentation tutoring](#), and [library resources](#). [Financial aid advising](#) is also available to help students navigate scholarships, fellowships, loans, and other financial aid options.

Finally, peer learning and support are integral to our mentoring culture. Structured group activities, such as study groups, peer supervision, and collaborative projects, enable students to share knowledge, discuss challenges, and support one another in their academic and clinical endeavors. The Psychology and Neuroscience Departments host a two-day off-site retreat at the start of the academic year to foster peer and faculty connections. In addition, students in the doctoral concentration can volunteer to be on one of several department committees (Recruitment and Admissions; Social; Professional Development)

which provides students with an opportunity to plan events that support their peers' professional and personal development.

Our concentration is committed to upholding the highest standards of respect, fairness, and ethical consideration in all interactions involving students, graduates, faculty, staff, and the public. Fostering a sense of belonging among all students and faculty is important to retention. The Psychology Department appreciates this importance and provides workshops and seminars, as well as supporting student-led initiatives to promote understanding and respect for others. Faculty mentors are also expected to continually develop their cultural awareness and consider students' cultural backgrounds and experiences when providing guidance. Policies addressing discrimination and harassment further contribute to an environment where all students feel safe and valued. Mechanisms for student feedback on the concentration's climate and inclusivity (described in the section entitled, "Quality Improvement and Outcomes") will further support continuous improvement in the concentration's climate and culture.

Training and Supervision

Training Experiences

We believe high research activity under close faculty supervision is required to develop as a first-rate clinical scientist. Research is a cornerstone of the curriculum, and students are expected to work closely with faculty mentors to develop research proposals, conduct studies, and disseminate their findings through presentations and publications across the entirety of their doctoral training. Students, depending on their funding source (TA/RA), are expected to dedicate 10-20 hours per week to research activities, fostering engagement and enabling them to develop their own projects. Based on degree plan flexibility needs, the personal training committee may advise a student to enroll/not enroll in elective research credit hours. Credit hours are not used to determine competency attainment. All research benchmarks must be met, regardless of elective credit hour enrollment.

Regular weekly or biweekly meetings with the primary research mentor will facilitate feedback on research progress, allowing students to refine their work and prepare for presentations and publications. Students present their research at departmental colloquia and other academic events to receive feedback from a wide range of sources. Students are also strongly encouraged to present their research at national and international conferences, enhancing their visibility in the field and providing networking opportunities with leading figures in psychology. Finally, the concentration places significant emphasis on the importance of publishing research findings, with students expected to earn lead authorship on manuscripts submitted for peer review.

In addition to research training, all students are required to engage in clinical training within the PSEC and external practicum settings. Attainment of all specific clinical benchmarks is mandatory for demonstration of sufficient competency and readiness to advance in training, but enrollment in course credit is elective to allow for greater flexibility in degree plans. As with research, the personal training committee may advise students on elective credit hours based on their career aspirations and training needs.

With respect to required clinical benchmarks, all students are required to complete three comprehensive integrated psychological assessment reports during internal practicum training within the PSEC. Students may begin working towards this benchmark after completion of CPSY 7310 (Clinical Psychological Science II). Each report must include a review of history, an interview, and at least two psychological tests from one or more of the following categories: Personality, intellectual, cognitive, or neuropsychological tests.

A report written from an interview that is only history-taking, a clinical interview, and/or only the completion of behavioral rating forms, where no additional tests are administered, is not an integrated report.

With respect to competency in psychotherapy, a determination of readiness of external practicum training will be determined based careful review of student's functional clinical competencies. The entire PSEC Manual (Appendix L) outlines clinical expectations within the PSEC internal practicum; the manual section entitled, "Supervision and Feedback Mechanisms" and the corresponding Addendum B within that manual are particularly notable.

External clinical practicum training sites are important resources for students as they learn to apply their skills in real-world settings and gain additional exposure to underserved patients in our area. Each of these external practicum sites has an established history of training doctoral students in clinical psychology and has committed to maintaining close association with the us to ensure students engage in high-quality clinical experiences aligned with the clinical science model (see letters of commitment in Appendix P). These sites include:

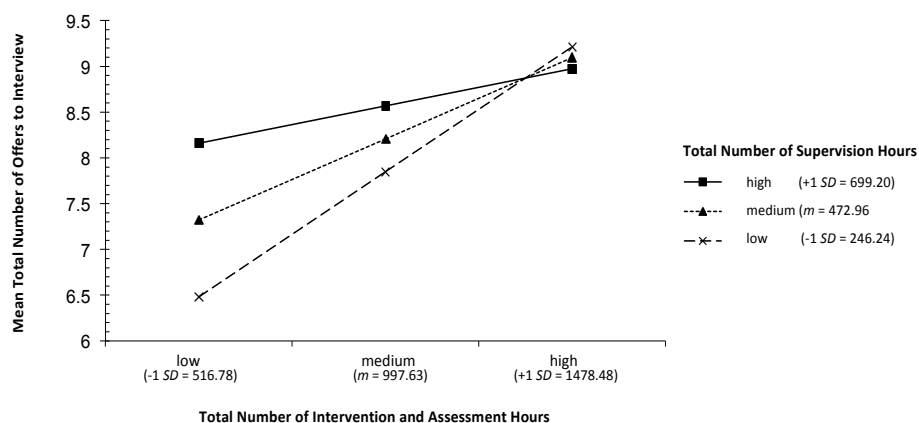
1. The Steven A. Cohen Military Family Clinic at Metrocare: The Cohen Clinic is part of Metrocare, the largest provider of mental health services in North Texas. Located in a federally designated health professional shortage area for mental health, the Cohen Clinic provides integrated, interdisciplinary behavioral health services within a community-based primary care setting. The clinic serves military families, with 22% identifying as Hispanic, 14% as African American/Black/African-origin, and 10% as Asian. Trainees work as members of interdisciplinary care teams that include social workers, psychiatrists, licensed chemical dependency counselors, and licensed marriage and family therapists, gaining valuable experience in collaborative care.
2. John Peter Smith Hospital (JPS): JPS is in a Health Professional Shortage Area (HPSA) for mental health and houses the only psychiatric emergency center in Tarrant County. As a public hospital, JPS provides both inpatient and outpatient interdisciplinary services, including in-person and tele-behavioral health, to a high unmet need population, including homeless and indigent residents. The hospital serves a community that is 28.9% Hispanic and 17.1% African American/Black/African origin. JPS is home to the largest Family Medicine residency program in the country and serves as the main teaching hospital for the Texas College of Osteopathic Medicine. With accredited residency programs in psychiatry and emergency medicine, JPS offers an ideal environment for interdisciplinary training on integrated care teams, allowing trainees to collaborate with a wide range of healthcare professionals.
3. VA North Texas Health Care System (VANTHCS): The VANTHCS is home to the largest mental health service in the VA Health Care System, with over 34,000 mental health outpatient visits annually. It is also the second largest VA in the nation, with over 1.4 million annual outpatient visits. VANTHCS encompasses multiple facilities and community-based outpatient clinics, providing extensive tele-behavioral health linkages to serve 38 counties across predominantly rural parts of North Texas and two counties in southern Oklahoma. The complexity and volume of patients seen at VANTHCS create an enriched training environment. Trainees work closely with providers and trainees in other disciplines, including medicine, psychiatry, nursing, social work, and other allied health professionals.
4. Jewish Family Service of Greater Dallas (JFS): As a nonsectarian interdisciplinary mental health and social services agency, JFS offers a wide range of services to individuals and families in need, regardless of their ability to pay. JFS serves a clientele that reflects the rich tapestry of Dallas, including individuals in West Dallas, a federally designated health professional shortage area for mental health (HPSA #7489988627). This HPSA designation underscores the critical need for

mental health services in the community, particularly given that the surrounding population is 40.2% Hispanic and 24.3% African American/Black. The agency's interdisciplinary care teams, comprised of social workers, therapists, and counselors, offer a comprehensive approach to mental health treatment, providing trainees with valuable experience in a multicultural environment.

5. **Baylor Scott & White Health:** The largest not-for-profit health care system in Texas and one of the largest in the United States, Baylor Scott & White consists of an extensive network of 51 hospitals and more than 800 patient care sites. Baylor Scott & White is committed to building healthier communities and strengthening lives in Texas by investing in valuable programs and services for the uninsured, underserved and most at-risk populations that focus on quality, compassionate care. The Baylor Scott & White Research Institute conducts more than 2,000 active research protocols, spanning more than 58 specialties. In addition, Baylor Scott & White offers a comprehensive range of mental health services delivered by a multidisciplinary team of psychiatrists, psychologists, and licensed professional counselors. These services address a variety of mental health needs, using individual, group, couples, and family therapy modalities as well as comprehensive psychological and neuropsychological assessments.

To prioritize quality supervision, we will cap weekly direct contact hours at 8 per week. This cap is intended to prevent students from becoming excessively focused on accumulating clinical hours at the expense of quality supervision (Callahan & Watkins, 2018). As shown in Figure 8, high quality supervision plays a critical role in ensuring that trainees are not only gaining experience but are also developing the necessary skills and competencies effectively (Callahan et al., 2014). Thus, trainees will be required to maintain a direct contact to supervision hours ratio of no more than 4:1 during clinical practicum training (consistent with a resolution on quality supervision by the Council of University Directors of Clinical Psychology; CUDCP). In other words, a student who accrues 8 direct contact hours should also accrue a minimum of 2 hours of supervision. We will monitor the ratio of patient care hours to supervision monthly to make sure that each trainee maintains a manageable caseload that allows for the focused guidance and thorough supervision need to refine clinical skills and develop competencies.

Figure 8. Intensity of supervision interacts with volume of patient care to predict internship application competitiveness.



According to the PCSAS Internship Report (March 2023), students in PCSAS-accredited programs are competitive for high quality clinical internships if they accumulate between 688.5 (bottom quartile threshold) and 802 (median) direct-contact hours across internal and external practicum training experiences. To determine if students can accumulate sufficient clinical hours within four years while

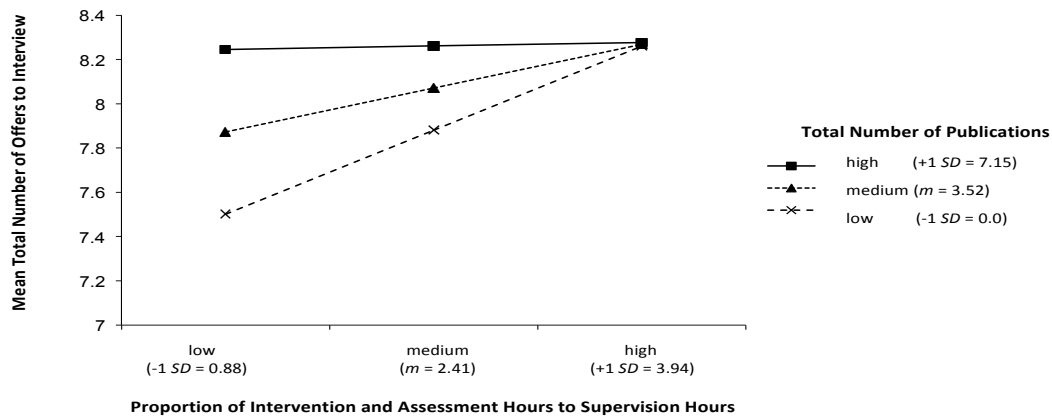
capping weekly direct contact hours, we analyzed a typical academic calendar. To be conservative, we excluded summer sessions and between semester breaks. At UT Dallas, an academic year generally consists of about 30 instructional weeks, across both fall and spring semesters. Over the course of four years, this totals approximately 120 weeks of potential practicum engagement (30 weeks per year multiplied by 4 years) prior to applying for internship at the start of training year five. Based on these figures, a trainee could amass up to 960 hours of direct contact hours with a minimum of 240 hours of supervision.

These estimates are based on a conservative, fixed training schedule. However, students can flexibly accrue hours throughout the year, including summer months.² As described in the Training Overview section of the PSEC Manual (Appendix L), we expect students to begin accruing hours early in training, using a heavily supervised titration of responsibility based on individual student's competencies. Thus, it is anticipated that students will be able to comfortably meet the range of hours appropriate to be competitive at top internship sites. The flexibility of the concentration allows for a more accommodating and supportive environment for students to fulfill their clinical training requirements. By limiting total hours and ensuring close supervision, trainees will be encouraged to prioritize a balanced experience, reducing risk of burnout and promoting deeper understanding and skill development, leading to stronger applications and successful internship matches.

Integration of Practice and Research

A nationwide study of internship applicants revealed a significant interaction between research achievements and quality practicum experiences (Figure 9), strongly indicating that neither research productivity nor clinical training is expendable (Callahan et al., 2014). We believe the reciprocal interaction among each area of training is essential to developing a broad base of sound competencies that allow one to be competitively prepared to emerge into the workforce.

Figure 9. Research productivity and clinical supervision quality interact to predict internship application competitiveness.



Our curriculum emphasizes the connection between research and clinical practice. Research courses emphasize the practical utility of scholarly inquiry, while core clinical courses compel students to apply empirical evidence in treatment decisions and intervention strategies. This dual focus fosters the integration of theory and practice, as clinical training generates novel research questions that contribute

² Regardless of their enrollment, trainees are expected to be engaged full-time in their training throughout the summer session (see page 18 as well as the section entitled, "Summer and Intersession Travel" within the Psychology PhD Student Guide; Appendix C).

to the evolving body of knowledge aimed at addressing critical clinical challenges. Students build on this foundation through advanced practical training in research and clinical settings. In fact, the reciprocal interplay between clinical practice and scholarly inquiry is a cornerstone of our concentration's culture. Students and faculty engage in research that directly addresses pressing questions salient to clinical science. Faculty-led laboratories and specialized Centers (detailed in the sections “Faculty Roles and Responsibilities” and “Facilities and Equipment Access”) offer students collaborative opportunities with experienced researchers, while immersive clinical training in the PSEC and external practicums encourages hypothesis formulation informed by direct clinical exposure.

Rigorous training cultivates critical thinking skills essential for evaluating and applying research findings to clinical practice. To facilitate evidence-based practice, we mandate the use of standardized measures (e.g., OQ45.2, WAI) to systematically assess patient progress and treatment processes. This systematic assessment equips students with the skills to monitor changes over time and evaluate the efficacy of their interventions and the therapeutic alliance. Regular outcome assessments also empower trainee clinicians to make informed treatment adjustments based on empirical evidence. Our commitment to evidence-based practice and ongoing evaluation will enhance the quality of patient care and prepare students to become reflective practitioners. They learn to critically analyze the impact of their interventions, seek supervisory feedback, and adapt strategies as necessary. This iterative process of evaluation and adjustment is essential for developing competent clinicians capable of navigating the complexities of clinical psychology and delivering high-quality, effective psychological services. Research findings associated with training clinics are systematically integrated into PSEC procedures (see Appendix L), reinforcing our commitment to a research-informed clinical practice.

Quality of Clinical Supervision

Supervision is a fundamental aspect of practicum training, serving as the primary mechanism for providing formative feedback that shapes trainees' competencies and influences patient outcomes. Empirical evidence underscores the critical role of effective supervision in enhancing trainee skills, with research indicating that supervisory quality can account for a significant percentage of variance in patient symptom distress—up to 16.4% of treatment outcome variance among patients during practicum training (Callahan et al., 2009). Supervisory effectiveness varies, with some supervisors achieving better patient outcomes than others. Research suggests that supervisory effectiveness is not necessarily linked to faculty status or years of supervision experience (Callahan & Watkins, 2018). In short, quality of supervision cannot be assumed based on solely on a review of qualifications. The PSEC Director and DCST will work together to prioritize the quality and appropriateness of training at practicum sites through a systematic evaluation process, ensuring that students receive exemplary clinical training aligned with the concentration's educational objectives.

While practicum sites may use a combination of one-on-one and group supervision models, personalized feedback and support is essential. We expect clinical training and supervision to be provided by doctoral-level psychologists and, when appropriate, experienced clinicians from various healthcare disciplines. In interdisciplinary settings, the licensed psychologist is expected to serve as the primary mentor, guiding students through their external training experiences. Supervision sessions will be expected to occur at least weekly, allowing for ongoing discussions of clinical cases and the integration of research findings into practice. Students will be expected to work closely with their expert supervisors in a safe environment to discuss clinical experiences, challenges, and successes, promoting professional growth and the development of essential clinical skills.

PSEC trainees will review digital recordings of patient encounters, tagging recordings with reflective comments prior to supervision. This process enhances critical thinking and self-reflection, allowing

students to integrate theoretical knowledge with practical application. For those trainees who are at external practicum sites, we similarly expect weekly individual supervision sessions will enable students to review cases, discuss treatment plans, and reflect on therapeutic techniques. Across internal and external placements, group supervision sessions are encouraged to foster community and enrich the learning experience by encouraging students to share experiences and discuss diverse perspectives. Further, regardless of setting, students are expected to receive training in culturally sensitive, evidence-based interventions, preparing them to apply their research knowledge effectively in clinical contexts. This focus on culturally competent care is essential for developing practitioners who can meet the needs of patients from varied backgrounds, ultimately enhancing the quality of care provided.

Weekly individual, group, and grand rounds sessions are designed to align with the demands of clinical placements and competency development. We will require clinical supervisors to provide formative feedback based on direct observation of student's work. As students advance in their training, the supervision model adapts to their evolving needs, providing more intensive support initially while fostering greater independence as they build confidence. Scaffolding supports continuous improvement and clinical skill refinement and also cultivates a lifelong dedication to engaging in the professional growth that will be necessary across their careers as ethical and responsible professionals. All students, regardless of their placement site, will be required to provide monthly reports to the PSEC Director for on-going monitoring. The PSEC Director will solicit regular evaluations from all supervisors to improve training practices based on student and supervisor feedback. Student feedback regarding supervisors will be solicited no less than semesterly by the PSEC Director. To encourage constructive feedback, student responses will be aggregated by the PSEC Director on a rolling 3-year cycle before sharing with supervisors.

Performance Evaluation and Feedback

Student Progress Monitoring and Feedback Mechanisms

The concentration emphasizes clear communication and multiple perspectives. Feedback is ongoing throughout completion of the concentration, beyond formal evaluations. Students are encouraged to actively engage in deliberate and reflective practice, using feedback from all those they encounter (e.g., supervisors, staff, peers, patients) to inform their self-assessments and enhance their self-awareness concerning their skill development and professional identity. To support students throughout their training, regular assessments and feedback will be given across three key areas: Academic performance, clinical competencies, and research progress.

With respect to academic performance, faculty instructors in the Psychology Department are encouraged to provide timely and specific formative feedback on student performance in coursework throughout the semester, to augment summative course grades at the conclusion of each term. Academic performance is primarily evaluated through course grades, with a minimum grade of B required in all courses. This grading standard serves as a fundamental measure of academic success, providing a clear benchmark for assessing students' comprehension of core concepts in clinical psychology. Regular assessments and feedback sessions are integral to this process, allowing faculty to identify students who are excelling as well as those who may be encountering difficulties.

In addition to weekly formative feedback (refer to the section entitled, "Supervision and Feedback Mechanisms" in the Psychological Services and Education Center Manual, Appendix L) summative clinical competency evaluations will assess functional competencies at the end of each semester. These evaluations will assess students' clinical competencies based on established criteria and utilize validated

measures (Price et al., 2017). Evaluation of functional competencies at the internship level will be requested during the final semester of each student's internship placement, though their completion cannot be required (in keeping with APPIC policy). At every level, students are provided a copy of evaluative feedback. Appendices include both the pre-internship (Appendix M) and the internship level evaluative codebooks (Appendix Q).

Research progress is assessed by time to degree and evaluation of the pre-dissertation portfolio and dissertation (see pages 13 thru 17 of the Psychology PhD Student Guide in Appendix C). Across these experiences, feedback is provided in both formative and summative formats. Formative feedback offers ongoing guidance for improvement, while summative feedback assesses overall progress towards concentration goals. As described throughout this document, faculty instructors and mentors, clinical supervisors, the personalized training committee, and task-specific committees will engage in both formative and summative evaluations of students' progress, providing tailored feedback that allows for necessary adjustments to their individual training plans. Each student will also be provided an annual review letter by the CSSC to summarize their progress. Any student making insufficient progress will be reviewed on a semesterly basis by the CSSC.

Insufficient Progress and Grievance Handling

Insufficient progress may be identified via faculty, supervisors, staff, peers, patients, or public behavior. Although it is not an exhaustive listing of problems of competency, Table 3 in Appendix G provides exemplar behavioral indicators associated with need for remediation, parsed by each goal and student learning outcome. When necessary, a remediation plan will be developed collaboratively with the student to identify specific steps for improvement, such as supplemental coursework, delayed placements, or retaking courses. The personalized training committee will be involved in all remediation planning. The PSEC Director will also be involved if the remediation needed pertains to clinical competencies. Processes are detailed in the "Insufficient Progress" and "Remediation" sections of the Psychological Clinical Science PhD Handbook (Appendix C) to ensure that students are fully informed about the expectations and support available to them. Remediation plans are designed to be supportive and promote progress towards successful concentration completion. However, in some cases, dismissal from the concentration may be necessary. This could occur due to:

- **Professional Competency Issues:** Students demonstrating professionalism challenges that significantly compromise safe and ethical practice, despite appropriate levels of support.
- **Insufficient Progress Despite Remediation:** Persistent shortcomings in core competencies, despite multiple remediation attempts.
- **Insufficient Development of Research Competencies:** Student consistently underperforming in research, raising concerns about their ability to contribute original knowledge to the field.
- **Ethical Violations or Harmful Behavior:** Serious ethical violations, demonstrably harmful behavior towards patients or colleagues, or posing a safety risk can lead to dismissal.
- **Lack of Self-Awareness or Motivation:** Displays of poor self-insight or unwillingness to engage in professional development suggest further attempts at remediation may be unproductive.
- **Pattern of Non-Compliance:** Repeated disregard for concentration requirements, resistance to supervision, or a pattern of non-compliance may necessitate dismissal.

The CSSC will carefully consider all factors and consult the student's personalized training committee before making a dismissal decision. The CSSC is committed to prioritizing fairness to the student while also upholding the safety and ethical obligations of the concentration, institution, and profession.

In addition to UT Dallas policy on student grievances (Appendix R), the CSSC provides a non-adversarial platform for resolving concerns not addressed by existing university policies (e.g., grade changes, anti-discrimination, sexual harassment). Informal exploration of concerns is encouraged before initiating the formal grievance process. Students are encouraged to consult with their personalized training committee for advice and support. If informal attempts at resolution are unsuccessful, a formal written grievance can be submitted to the DCST for consideration by the CSSC.

The CSSC will then assemble a grievance committee composed of three faculty members. One member will be chosen by the student, another by the implicated party(ies), and the third will be a neutral party chosen by the other two members of the ad hoc grievance committee. If neither the student nor the implicated party identifies a faculty member to serve on the grievance committee within seven days of the CSSC's request, the Department Chair will appoint a faculty member to fill the vacant seat. The 3-member committee reviews the grievance, gathers information, and deliberates with sensitivity to all parties involved. Recommendations for resolution are submitted to the CSSC within 45 days. The CSSC can then implement the recommendations or escalate them to the Department, BBS, or appropriate university authorities, if necessary.

Quality Improvement and Outcomes

Data Collection and Program Evaluation

Our curriculum map (Appendix G) details the expected timeline and measures used to assess proximal outcomes associated with each of our Goals and Student Learning Outcomes. These data will be aggregated to identify the relative strengths and weaknesses of our concentration. Data insights will be enriched by student feedback (via course evaluations, supervisor and site evaluations), as well as feedback from faculty and supervisors. Aggregated information concerning patient progress, as measured through routine outcome monitoring with standardized measures (e.g., OQ45.2, WAI) in the PSEC will also be considered in our self-assessments.

Beyond course evaluation feedback, we will further ensure student's voices are considered by annually survey them on their perceptions across four areas (curriculum and supervision; climate and comfort; honesty in recruitment; research) using a validated measure intended for use in clinical psychology training (Peters et al., 2011). Anonymity will encourage candid responses, and the results will be analyzed to identify trends and areas for improvement. We will use available national norms for comparison purposes (Gregus et al., 2020). Use of these data may offer timely insights into student learning and concentration delivery, allowing for responsive adjustments and improvements. They may also be used to inform strategic decisions (e.g., by the CSSC, PSEC Director, DCST, Department Chair, or BBS Dean) regarding resource allocation and concentration enhancements that ensure continued alignment of training with the concentration's conceptual foundations.

The PSEC Director will review the effectiveness of supervisors on at least an annual basis. This evaluation will include student feedback on supervisory experiences (e.g., guidance quality, supervision frequency, and overall supervisory relationship). Supervisors will be encouraged to engage in self-evaluation, reflecting on their practices and identifying opportunities for growth. In addition, for those providing supervision within the PSEC³, patient outcomes across students will be aggregated by supervisor and examined to further inform evaluations of supervisory effectiveness. These iterative processes will allow

³ Supervisors working within Specialty Clinics are exempt from this requirement.

us to identify and address site and/or supervisor issues promptly, maintaining high training quality standards.

Annual evaluations of practicum sites will provide a comprehensive understanding of each site's training capabilities and student experiences. In addition to collecting feedback from students regarding their practicum experiences (e.g., supervision quality, breadth and depth of clinical experiences, and overall support), the PSEC Director will conduct site visits to observe the training environment firsthand and assess supervisor engagement, review training materials and resources, and evaluate the extent to which practicum sites provide appropriate experiences that adhere to evidence-based practices.

To evaluate the distal outcomes of graduates over their early career years (i.e., the ten-year period following their graduation), we will implement a robust alumni tracking system that collects data on graduates' career trajectories, professional achievements (including licensure and NPI numbers), and contributions to their respective fields. Annual alumni surveys will assess various distal outcomes, including employment status, job satisfaction, professional development, and engagement in continuing education. Our survey will capture both quantitative data, such as publications, grants and professional contributions, and qualitative feedback regarding the relevance of the concentration's training to their current roles (see Appendix S). This data will be compiled and analyzed to evaluate the concentration's effectiveness in producing graduates who are actively contributing to the field.

Continuous Improvement Strategies

A key aspect of the curriculum is its emphasis on continuous review and adaptation. The concentration is committed to undergoing regular assessments to ensure that its content remains aligned with current research, clinical practices, and the evolving needs of the workforce. This iterative process involves gathering feedback from faculty, students, and stakeholders to make necessary curriculum adjustments. Such responsiveness to external input not only enhances the relevance of the educational offerings but also ensures that graduates are equipped with the most up-to-date knowledge and skills required in the field.

Communication will serve as a vital strategy for continuous improvement of the new program, encouraging both collaboration and responsiveness. The concentration will engage in targeted inquiries to address specific training issues and adapt to contemporary challenges, such as assessing holistic review procedures and the determinants of readiness for doctoral training to evaluate admissions effectiveness. We will host periodic focus groups, town hall meetings, and forums to encourage faculty and student feedback. Additionally, a mentorship feedback mechanism will allow students to evaluate their mentorship experiences, promoting a culture of open communication and continuous improvement. To facilitate this engagement, the concentration will implement communication channels that include regular updates through email, scheduled meetings, and a sharing of resources and/or feedback. Periodic workshops and training sessions will further ensure that all supervisors are informed and aligned with the concentration's goals and expectations.

Another important strategy is the use of a shared decision-making model by the CSSC. Input will be welcome from other research mentors, clinical supervisors, students, and relevant stakeholders. These groups bring expertise and understanding to the decision-making process. They will be involved in evaluating student performance data, course evaluations, and feedback from various sources, which will inform discussions about potential policy changes. Faculty input will be essential in ensuring that the concentration maintains high academic standards and aligns with best practices in the field of psychology.

The PSEC Director will meet regularly with clinical supervisors to discuss student progress, addressing concerns, and sharing insights on the training experience. By fostering open lines of communication, the concentration will keep supervisors informed about the curriculum, concentration expectations, and any changes that may impact their supervision of students. This proactive approach enhances supervisors' understanding and strengthens their ability to guide students effectively. Additionally, a dedicated MS Teams communication channel will disseminate important information and resources to clinical supervisors. This platform will include updates on concentration policies, upcoming events, and professional development opportunities, while also serving as a repository for best practices and guidelines related to supervision. This will help standardize expectations and improve the quality of training provided to students.

Finally, transparency will serve as a key strategy for continuous improvement. Annual reports will be generated to highlight both areas of success and those requiring improvement to foster a culture of accountability and growth. We believe a commitment to transparency, collaboration, and continuous improvement is essential for the success of any educational initiative and expect it to foster a sense of ownership and engagement among all involved.

Expected Outcomes for Graduates

As the concentration has not yet matriculated students, we are currently unable to provide specific outcomes with respect to this concentration's graduates. However, review of alumni trained by our faculty mentors within the past 10 years provides strong evidence in training effectiveness (see Appendix T). Following admission of our first cohort, we will enact processes for gathering both proximal and distal data to ensure we are producing graduates who make high-quality contributions that mirror expectations for psychological clinical scientists.

APPENDIX G

Curriculum Map

Table 1: Curriculum Map Overview

Table 2: Curriculum Map for Mastery of Competencies in Clinical Psychological Science

Table 3: Rank Order of Students' Competency Attainment Ease for Goals 1, 3, 4, and 5 in a Sample of Clinical Psychology Doctoral Trainees (source data does not include items that align with Goal 2)

Table 1.

Curriculum Map Overview

Goal #	Student Learning Outcome #	Competency	Admissions Process	CPSY 5300	CPSY 7160	CPSY 7270	CPSY 7180	CPSY7181	CPSY 7190	CPSY 7300	CPSY 7310	CPSY7320	CPSY 7330	CPSY7355	CPSY7340	CPSY8100	HCS6302	HCS6312	HCS6313	HCS6317	HCS 7355	CPSY 8v87	8v97
1	1.1	Scientific Mindedness	Introduced		Mastered					Expanded	Expanded			Mastered							Mastered	Expanded	Mastered
1	1.2	Scientific Foundations	Introduced		Mastered					Expanded	Expanded								Expanded		Mastered	Expanded	Mastered
2	2.1	Research Skills Development	Introduced		Expanded									Expanded			Expanded	Expanded	Expanded	Expanded		Expanded	Mastered
2	2.2	Innovative Inquiry	Introduced		Expanded									Expanded			Expanded	Expanded	Expanded	Expanded		Expanded	Mastered
		Knowledge of Ethical, Legal and	Introduced			Expanded	Expanded		Mastered	Expanded	Expanded	Expanded	Expanded		Expanded	Mastered		Expanded	Expanded				
3	3.1	Professional Standards and Guidelines																					
		Awareness and																					
3	3.2	Application of Ethical Decision Making	Introduced			Expanded	Expanded		Mastered	Expanded	Expanded	Expanded				Mastered							
3	3.3	Ethical Conduct	Introduced			Expanded	Expanded		Mastered							Mastered							
		Self as Shaped by Individual and	Introduced			Expanded	Expanded	Expanded	Mastered							Mastered							
3	3.4	Cultural Diversity and Context																					
		Others as Shaped by Individual and	Introduced			Expanded	Expanded	Expanded	Mastered			Expanded				Mastered							
3	3.5	Cultural Diversity and Context																					
		Interaction of Self and Others as Shaped by	Introduced			Expanded	Expanded	Expanded	Mastered							Mastered							
3	3.6	Individual and Cultural Diversity and Context																					
		ICD Applications	Introduced			Expanded	Expanded	Expanded	Mastered			Expanded			Expanded	Mastered							
3	3.7	Based on Individual and Cultural Context																					
		Empowerment	Introduced						Mastered	Expanded	Expanded					Mastered							
4	4.1	Concern for the Welfare of Others	Introduced			Expanded	Expanded	Mastered	Mastered							Mastered							
4	4.2	Reflective Practice	Introduced			Expanded	Expanded	Mastered	Mastered			Expanded				Mastered							
4	4.3	Self-Assessment	Introduced			Expanded	Expanded	Mastered	Mastered			Expanded				Mastered							
4	4.4	Self-Care	Introduced	Expanded		Expanded	Expanded	Mastered	Mastered							Mastered							
4	4.5	Clinical Skills	Introduced			Expanded	Expanded	Mastered	Mastered			Expanded				Mastered							
4	4.6	Diagnosis	Introduced			Expanded	Expanded	Mastered	Mastered	Expanded	Expanded	Expanded	Expanded			Mastered							
4	4.7	Measurement and Psychometrics	Introduced			Expanded			Mastered	Expanded	Expanded					Mastered							
		Assessment																					
4	4.8	Evaluation Methods	Introduced			Expanded			Mastered	Expanded	Expanded		Expanded			Mastered							
4	4.9	Assessment Methods																					
		Diagnostic	Introduced			Expanded		Mastered	Mastered	Expanded	Expanded	Expanded	Expanded			Mastered							
4	4.10	Conceptualization and Recommendations																					
		Intervention Planning	Introduced			Expanded	Expanded	Mastered	Mastered	Expanded	Expanded	Expanded			Expanded	Mastered							
4	4.12	Intervention Implementation	Introduced				Expanded	Mastered	Mastered		Expanded	Expanded				Mastered							
4	4.13	Knowledge of Interventions	Introduced				Expanded	Mastered	Mastered		Expanded	Expanded				Mastered							
		Scientific Foundation of Professional Practice	Introduced					Mastered	Mastered	Expanded	Expanded	Expanded			Expanded	Mastered							
4	4.14																						
4	4.15	Treatment Monitoring	Introduced				Expanded	Mastered	Mastered	Expanded	Expanded	Expanded				Mastered							
5	5.1	Integrity	Introduced	Expanded		Expanded	Expanded	Expanded	Mastered							Mastered							Mastered
5	5.2	Depotment	Introduced	Expanded		Expanded	Expanded	Expanded	Mastered							Mastered							Mastered
5	5.3	Professional Identity	Introduced	Expanded	Mastered		Expanded	Mastered		Expanded	Expanded			Expanded		Mastered							Mastered
5	5.4	Accountability	Introduced	Expanded	Expanded	Expanded	Expanded	Expanded	Mastered	Expanded	Expanded					Mastered							Mastered
5	5.5	Interpersonal Relationships	Introduced	Expanded	Expanded	Expanded	Expanded	Expanded	Mastered	Expanded	Expanded					Mastered							Mastered
		Expressive Skills	Introduced	Expanded	Expanded	Expanded	Expanded	Expanded	Mastered	Expanded	Expanded			Mastered		Mastered							Mastered
5	5.7	Affective Skills	Introduced	Expanded	Expanded	Expanded	Expanded	Expanded	Mastered	Expanded	Expanded			Mastered		Mastered							Mastered
5	5.8	Supervision Skills Development	Introduced	Mastered			Expanded	Expanded		Expanded	Expanded					Mastered							Mastered
		Supervision																					
5	5.9	Expectations and Roles	Introduced			Expanded	Expanded		Mastered							Mastered							Mastered
		Supervision																					
5	5.10	Processes and Procedures	Introduced	Expanded					Mastered							Mastered							Mastered
		Awareness of Factors																					
5	5.11	Affecting Supervision Quality	Introduced			Expanded	Expanded	Mastered	Mastered							Mastered							
		Participation in Supervision Process	Introduced			Expanded	Expanded		Mastered							Mastered							Expanded

Table 1.

Curriculum Map Overview

5.13	Ethical and Legal Issues in Supervision Knowledge of the Shared and	Introduced	Expanded	Expanded	Expanded							Mastered		Mastered
5.14	Distinctive Contributions of Other Professions	Introduced										Mastered		
5.15	Functioning in Multidisciplinary and Interdisciplinary Contexts	Introduced										Mastered		
5.16	Understands How Participation in Interdisciplinary Collaboration/ Consultation Enhances Outcomes	Introduced		Expanded	Expanded		Mastered	Expanded	Expanded	Expanded		Mastered		Mastered
5.17	Respectful and Productive Relationships with Individuals from Other Professions	Introduced		Expanded								Mastered		
5.18	Role of Consultant Addressing	Introduced	Expanded	Expanded	Expanded		Mastered			Expanded		Mastered		Mastered
5.19	Consultation Referral Questions	Introduced					Mastered					Mastered		
5.20	Application of Consultation Methods	Introduced		Expanded			Mastered			Expanded		Mastered		Mastered

Table 2

Curriculum Map for Mastery of Competencies in Clinical Psychological Science

Goal 1. Mastery of Foundational Principles of Clinical Science.			
SLO 1.1. Scientific Mindedness			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 7300 / 7310 / 8v87	CPSY 7160; HCS 7355; CPSY 8v97
Measure	Interview	Course Tasks & PWC Codebook 1A Ratings	Course Tasks & Candidacy Codebook 6A Ratings
Attainment Indicators	- Exhibits foundational knowledge of key psychological concepts and theories. - Articulates the relevance of psychological principles to contemporary issues.	- Displays critical scientific thinking. - Demonstrates awareness of need for evidence to support assertions. - Questions assumptions of knowledge. - Evaluates study methodology and scientific basis of findings. - Presents own work for the scrutiny of others.	- Demonstrates value of the scientific method and applies it to professional practice. - Articulates, in supervision and case conference, support for issues derived from the literature. - Formulates appropriate questions regarding case conceptualization. - Generates hypotheses regarding own contribution to therapeutic process and outcome. - Performs scientific critique of literature.
		Remediation Indicators - Evidences passive acceptance of published literature as truth. - Unable/Unwilling to present own research in multiple	
SLO 1.2. Scientific Foundations			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Application Review	HCS 6317; CPSY 7300 / 7310 / 8v87	CPSY 7160; HCS 7355; CPSY 8v97
Measure	Admission Rubrics	ACAT Scores; Observational Ratings; Course Tasks & PWC Codebook 1B Ratings	Observational Ratings & Course Tasks & Competencies Codebook 6C Ratings
Attainment Indicators	- Identify major psychological theories and their applications. - Discuss the impact of sociocultural factors on psychological research.	- Illustrates understanding of psychology as a science. - Demonstrates understanding of core scientific conceptualizations of human behavior. - Demonstrates understanding of psychology as a science, including basic knowledge of the breadth of scientific psychology. - Evaluates scholarly literature on a topic.	- Demonstrates in-depth knowledge of core science. - Displays intermediate level knowledge of and respect for scientific bases of behavior.
		Remediation Indicators - Unable to articulate the scientific method. - Applies illogical steps in place of scientific reasoning and evaluation.	

Goal 2. Advanced Research Competence in Clinical Science			
SLO 2.1. Research Skills Development – Sophisticated research knowledge, skills, and abilities including experimental design, data analysis, and interpretation.			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	HCS 6302 / 6312 / 6313 / 6317; CPSY 7160 / 7340 / 8v87	CPSY 8v97
Measure	APCS Interview, Q7a.	Course Tasks	Annual Review of CV
Attainment Indicators	- Demonstrates basic research skills, including data collection and analysis. - Addresses ethical considerations in psychological research.	- Demonstrates proficiency in planning, conducting, and evaluating research projects, including experimental design and data analysis. - Writes comprehensive research proposals that reflect a deep understanding of experimental design and data interpretation. - Communicates research findings clearly and effectively, ensuring that data analysis and interpretations are accessible to diverse audiences. - Engages in collaborative learning and peer review, enhancing understanding of research methodologies and promoting critical evaluation of experimental designs.	Leads collaborative research initiatives and mentors peers in research practices.
		Remediation Indicators - Demonstrates inability to design and implement a research study effectively. - Shows lack of understanding of data analysis techniques and their application. - Struggles to articulate the steps involved in conducting research.	
SLO 2.2. Innovative Inquiry – Engages in innovative inquiry that addresses pressing questions in clinical psychological science.			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Application Review; Interview	HCS 6302 / 6312 / 6313 / 6317; CPSY 7160 / 7340 / 8v87	CPSY 8v97
Measure	Admission Rubrics; APCS Interview, Q6a.	Course Tasks	Annual Review of CV

Attainment Indicators	- Identifies current issues and trends in clinical psychology research. - Formulates research questions that reflect contemporary challenges. - Identifies several points they want further clarity on, at least most of which are related to scientific evaluation of the project	- Actively engages in innovative inquiry by developing research skills and critically evaluating existing literature to address significant questions in clinical psychology. - Demonstrates a thorough understanding of research ethics, ensuring responsible conduct in all research activities. - Participates actively in discussions about pressing questions in the field, fostering an environment of innovative inquiry and encouraging critical thinking about current issues.	- Secures funding for research projects through grant writing and proposal submissions. - Publishes innovative research that contributes to the advancement of clinical practices. - Presents research findings at conferences, fostering discussions on innovative approaches.
		Remediation Indicators - Demonstrates limited ability to critically evaluate existing research and identify gaps in knowledge. - Demonstrates lack of awareness regarding ethical considerations in research practices. - Participates minimally in discussions regarding significant questions in the field.	
Goal 3. Ethical and Culturally Competent Practice			
SLO 3.1. Knowledge of Ethical, Legal and Professional Standards and Guidelines			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 7170 / 7180 / 7300 / 7310 / 7330 / 7340	CPSY 7190 / 8100
Measure	APCS Interview, Q8a.	Observational Ratings; Course Tasks & PWC Codebook 2A Ratings	Observational Ratings & Course Tasks & Competencies Codebook 3A Ratings
Attainment Indicators	- Recognizes the importance of ethical guidelines and cultural diversity in psychology. - Articulates personal values and how they relate to professional ethics. - Demonstrates commitment to pursuing scholarly activities that advance the interests of marginalized and/or underrepresented groups.	- Provides evidence of the basic knowledge and principles of the Belmont Report, APA Ethical Principles and Code of Conduct [ethical practice and basic skills in ethical decision making]. - Demonstrates beginning knowledge of legal and regulatory issues in the practice of psychology that apply to clinical practicum (e.g., APA Ethics Code and principles, Ethical decision-making models). - Demonstrates knowledge of typical legal issues (e.g., child and elder abuse reporting, HIPAA, Confidentiality, Informed Consent).	- Illustrates intermediate level knowledge and understanding of the Belmont Report, APA Ethical Principles and Code of Conduct and other relevant ethical/ professional codes, standards and guidelines; laws, statutes, rules, regulations. - Identifies ethical dilemmas effectively. - Actively consults with supervisor to act upon ethical and legal aspects of practice. - Addresses ethical and legal aspects within the case conceptualization. - Discusses ethical implications of professional work. - Recognizes and discusses limits of own ethical and legal knowledge.

		Remediation Indicators • Unable to locate the Belmont Report and/or APA Ethical Code. • Unable to identify additional codes that are salient to clinical research or professional practice (e.g, child custody, forensic, and so on) • Unable to identify which resources are applicable to context.	
SLO 3.2. Awareness and Application of Ethical Decision-Making			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 7300 / 7310 / 7170 / 7180	CPSY 7190 / 8100
Measure	APCS Interview, Q8a.	Observational Ratings & Course Tasks & PWC Codebook 2B Ratings	Observational Ratings & Course Tasks & Competencies Codebook 3B Ratings
Attainment Indicators	• Identifies personal strengths and areas for growth in professional practice. • Reflects on experiences to inform future ethical decision-making. • Demonstrates commitment to marginalized and/or underrepresented groups in research and other scholarly pursuits.	• Demonstrates the importance of an ethical decision model applied to practice. • Recognizes the importance of basic ethical concepts applicable in initial practice (e.g. child abuse reporting, Informed consent, confidentiality, multiple relationships, and competence). • Identifies potential conflicts between personal belief systems, APA ethics code and legal issues in practice.	• Knows and applies an ethical decision-making model and can apply relevant elements of ethical decision making to a dilemma. • Uses an ethical decision-making model when discussing cases in supervision. • Readily identifies ethical implications in cases and to understand the ethical elements in any present ethical dilemma or question. • Discusses ethical dilemmas and decision-making in supervision, staffing, presentations, & practicum settings.
		Remediation Indicators • Unable to identify, with the narrative code available, applicable sections to an ethical decision-making dilemma. • Failure to identify appropriate means and resources for obtaining consultation and/or supervision when facing an ethical decision-making dilemma.	
SLO 3.3. Ethical Conduct			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 7170 / 7180	CPSY 7190 / 8100
Measure	APCS Interview, Q8a.	Observational Ratings & Course Tasks & PWC Codebook 2C Ratings	Observational Ratings & Course Tasks & Competencies Codebook 3C Ratings

Attainment Indicators	- Recognizes the importance of ethical guidelines and cultural diversity in psychology. - Articulates personal values and how they relate to professional ethics. - Understands marginalized and/or underrepresented groups from either personal experience or education.	- Ethical attitudes and values evident in conduct. - Demonstrates desire to help others. - Demonstrates openness to new ideas. - Shows honesty/integrity/values ethical behavior. - Demonstrates personal courage consistent with ethical values of psychologists. - Displays a capacity for appropriate boundary management. - Implements ethical concepts into professional behavior.	- Provides evidences that knowledge of own moral principles/ethical values is integrated in professional conduct. - Articulates knowledge of own moral principles and ethical values in discussions with supervisors and peers about ethical issues. - Spontaneously discusses intersection of personal and professional ethical and moral issues.
		Remediation Indicators - Fails to seek consultation before responding to an ethical decision-making dilemma. - Enacts conduct that is inconsistent with supervisor mandates regarding ethical conduct, without first seeking consultation from Clinic Director.	
SLO 3.4. Self as Shaped by Individual and Cultural Diversity and Context			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 7170 / 7180 / 7181	CPSY 7190 / 8100
Measure	APCS Interview, Q8a.	Observational Ratings & Course Tasks & PWC Codebook 3A Ratings	Observational Ratings & Course Tasks & Competencies Codebook 2A Ratings
Attainment Indicators	- Demonstrates understanding of marginalized and/or underrepresented groups from either personal experience or education. - Demonstrates commitment to marginalized and/or underrepresented groups in research and other scholarly pursuits.	- Illustrates Knowledge, awareness, and understanding of one's own dimensions of diversity and attitudes towards diverse others. - Demonstrates this self-knowledge, awareness, and understanding (e.g., articulates how ethnic group values influence who one is and how one relates to other people.	- Monitors and applies knowledge of self as a cultural being in assessment, treatment, and consultation. - Demonstrates understanding and monitors own cultural identities in relation to work with others. - Uses knowledge of self to monitor effectiveness as a professional. - Critically evaluates feedback and initiates supervision regularly about diversity issues.
		Remediation Indicators Has difficulty recognizing biases.	
SLO 3.5. Others as Shaped by Individual and Cultural Diversity and Context			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Application Review	CPSY 7170 / 7180 / 7181 / 7320	CPSY 7190 / 8100
Measure	APCS Interview, Q8a.	Observational Ratings & Course Tasks & PWC Codebook 3B Ratings	Observational Ratings & Course Tasks & Competencies Codebook 2B Ratings

Attainment Indicators	Specifically expresses understanding of members of other groups.	- Provides evidence of knowledge, awareness, and understanding of other individuals as cultural beings. - Demonstrates knowledge, awareness and understanding of the way culture and context shape the behavior of other individuals. - Critically evaluates feedback and initiates supervision regularly about diversity issues with others.	- Demonstrates deep understanding and application of cultural competence. - Shows consistent, proactive engagement with diversity issues across a broad range of cultural and individual contexts (e.g., age, gender, gender identity, race, ethnicity, sexual orientation, socioeconomic status, etc.). - Skillfully integrates diverse perspectives into practice. - Actively promotes inclusion and equity. - Seeks continuous growth and reflection. - Collaborates effectively across differences. - Demonstrates a strong ability to build trust and rapport across cultural lines, while encouraging open dialogue about diversity issues.
		Remediation Indicators Does not talk about inexperience or lack of exposure to individuals/groups from different backgrounds.	
SLO 3.6. Interaction of Self and Others as Shaped by Individual and Cultural Diversity and Context			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 7170 / 7180 / 7181	CPSY 7190 / 8100
Measure	APCS Interview, Q8a.	Observational Ratings & Course Tasks & PWC Codebook 3C Ratings	Observational Ratings & Course Tasks & Competencies Codebook 2C Ratings
Attainment Indicators	Demonstrates commitment to marginalized and/or underrepresented groups in research and other scholarly pursuits.	- Provides evidence of knowledge, awareness, and understanding of interactions between self and diverse others. - Demonstrates knowledge, awareness and understanding of the way culture and context may shape interactions between and among individuals. - Demonstrate awareness of how intersecting identities (e.g., gender, sexuality, disability) influence individuals' experiences. - Applicants can describe situations where they considered multiple dimensions of diversity when interacting with or providing support to others.	- Applies knowledge of the role of culture in interactions in assessment, treatment, and consultation of diverse others. - Demonstrates understanding of the role of multiple cultural identities in interactions among individuals. - Uses knowledge of the role of culture in interactions in work as a professional. - Critically evaluates feedback and initiates supervision regularly about diversity issues with others.
		Remediation Indicators Demonstrates that holding some beliefs limit effectiveness with certain clients.	
SLO 3.7. ICD Applications Based on Individual and Cultural Context			
Timeline	Admission Readiness	Pre-candidacy	Candidacy

Method	Application Review; Interview	CPSY 7170 / 7180 / 7181 / 7340	CPSY 7190 / 8100
Measure	Admission Rubric Matrix 2	Observational Ratings & Course Tasks & PWC Codebook 3D Ratings	Observational Ratings & Course Tasks & Competencies Codebook 2D Ratings
Attainment Indicators	Applicants will display the capacity to adjust communication styles and approaches based on the social, cultural, and contextual differences of others, promoting effective and respectful engagement in diverse environments.	- Demonstrates basic knowledge of and sensitivity to the scientific, theoretical, and contextual issues related to ICD (as defined by APA policy) as they apply to professional psychology. - Shows understanding of the need to consider ICD issues in all aspects of professional psychology work (e.g., assessment, treatment, research, relationships with colleagues). - Demonstrates basic knowledge of literatures on individual and cultural differences and engages in respectful interactions that reflects this knowledge. - Demonstrates understanding of the need to consider ICD issues in all aspects of professional psychology work through respectful interactions.	- Demonstrates knowledge of ICD literature, including guidelines for practice with diverse individuals, groups and communities. - Demonstrates ability to address ICD issues across professional settings and activities. - Works effectively with diverse others in professional activities. - Demonstrates awareness of effects of oppression and privilege on self and others.
		Remediation Indicators Lacks insight into impact of personal experiences on case conceptualization, choice of treatment, etc.	
SLO 3.8. Empowerment			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 7300 / 7310	CPSY 7190 / 8100
Measure	APCS Interview, Q1b., Q8a.	Observational Ratings & Course Tasks & PWC Codebook 3E Ratings	Observational Ratings & Course Tasks & Competencies Codebook 16A Ratings
Attainment Indicators	- Shows recognition of the importance of ethical guidelines and cultural diversity in psychology. - Provides multiple appropriate examples in which they acted independently to set goals or objectives, with a productive result.	- Demonstrates Awareness of social, political, economic and cultural factors that impact individuals, institutions and systems, in addition to other factors that may lead them to seek intervention. - Articulates social, political, economic or cultural factors that may impact on human development and functioning.	- Identifies specific barriers to client improvement, e.g., lack of access to resources. - Assists client in development of self-advocacy plans.
		Remediation Indicators - Signals lack of awareness that social, political, economic, or cultural factors may impact human development. - Signals lack of awareness that service provision must be sensitive to social, political, economic, or cultural factors.	

Goal 4. Clinical Expertise and Intervention Skills			
SLO 4.1. Concern for the Welfare of Others - Consistently acts to understand and safeguard the welfare of others.			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 7170 / 7180	CPSY 7181 / 7190 / 8100
Measure	APCS Interview, Q4a.	Observational Ratings & Course Tasks & PWC Codebook 4E Ratings	Observational Ratings & Course Tasks & Competencies Codebook 1D Ratings
Attainment Indicators	• Demonstrates knowledge of basic assessment tools and intervention strategies. • Shows understanding of the principles of evidence-based practice in clinical settings. • Consistently describes considering other perspectives and options	• Displays initiative to help others • Articulates importance of concepts of confidentiality, privacy, informed consent. • Demonstrates compassion.	• Regularly demonstrates compassion. • Displays respect in interpersonal interactions with others including those from divergent perspectives or backgrounds. • Determines when response to client needs takes precedence over personal needs.
		Remediation Indicators • Engages in behaviors that compromise a safe environment emotionally or physically. • Demonstrates abandonment of client(s).	
SLO 4.2. Reflective Practice - Broadened self-awareness; self- monitoring; reflectivity regarding professional practice (reflection-on-action); use of resources to enhance reflectivity; elements of reflection-in-action.			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 7170 / 7180	CPSY 7181 / 7190 / 8100
Measure	APCS Interview, Q4a.	Observational Ratings & Course Tasks & PWC Codebook 4F Ratings	Observational Ratings & Course Tasks & Competencies Codebook 4A Ratings

Attainment Indicators	• Describes the application of psychological theories to clinical practice. • Recognizes the importance of client-centered approaches in interventions. • Acknowledges difficulty in approaching conflict, but description of behavior consistently indicates goal-focused management of own affect	Displays: • basic mindfulness and self-awareness. • basic reflectivity regarding professional practice (reflection-on-action). • problem solving skills (critical thinking). • organized reasoning. • intellectual curiosity and flexibility. Demonstrates openness to: • considering own personal concerns & issues. • recognizing impact of self on others. • articulating attitudes, values, and beliefs toward diverse others. • self-identifying multiple individual and cultural identities. • systematically reviewing own professional performance with supervisors/teachers.	• Articulates attitudes, values, and beliefs toward diverse others. • Recognizes impact of self on others. • Self-identifies multiple individual and cultural identities. • Describes how others experience him/her and identifies roles one might play within a group. • Responsively utilizes supervision to enhance reflectivity. • Systematically and effectively reviews own professional performance via videotape or other technology with supervisors. • Displays initial indicators of monitoring and adjusting professional performance in action as situation requires.
		Remediation Indicators • Does not articulate attitudes, values, and beliefs that guide behavior and decision-making. • Fails to acknowledge impact of self on others. • Does not use introspection to monitor the effect of personal issues on professional work and relationships. • Fails to foster development of individual and cultural identities. • Does not use supervision as an opportunity to facilitate self-awareness as related to professional performance, roles, and responsibilities.	
SLO 4.3. Self-Assessment - Broadly accurate self-assessment of competence; consistent monitoring and evaluation of practice activities.			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Application Review; Interview	CPSY 7170 / 7180	CPSY 7181 / 7190 / 8100
Measure	Admission Rubric Matrix 2	Observational Ratings & Course Tasks & PWC Codebook 4G Ratings	Observational Ratings & Course Tasks & Competencies Codebook 4B Ratings

Attainment Indicators	Applicants will provide examples where they critically evaluated their own performance in professional or academic tasks, identifying strengths and areas for improvement.	- Demonstrates knowledge of core competencies; emerging self- assessment re: competencies. - Demonstrates awareness of clinical competencies for professional training. - Develops initial competency goals for early training (with input from faculty).	- Self-assessment comes close to congruence with assessment by peers and supervisors. - Identifies areas requiring further professional growth. - Writes a personal statement of professional goals. - Identifies learning objectives for overall training plan. - Systemically and effectively reviews own professional performance via videotape or other technology.
		Remediation Indicators - Fails to recognize professional limitations & mistakes. - Fails to recognize strengths. - Despite input from others, does not develop specific and realistic professional goals. - Cannot describe an adequate subset of core competencies for scientist-practitioners. - Does not devote adequate time to self-evaluation and review of professional performance.	
SLO 4.4. Self-Care - Attention to personal health and well-being to assure effective professional functioning.			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 5300 / 7170 / 7180	CPSY 7181 / 7190 / 8100
Measure	APCS Interview, Q2a	Observational Ratings & Course Tasks & PWC Codebook 4H Ratings	Observational Ratings & Course Tasks & Competencies Codebook 4C Ratings
Attainment Indicators	- Recognizes the importance of client-centered approaches in interventions. - Consistently uses some combination of self-reflection, creativity, and practical knowledge to solve interpersonal problems, and is aware of this process	- Displays understanding of the importance of self-care in effective practice. - Illustrates knowledge of self-care methods. - Attends to self-care. - Demonstrates basic awareness and attention to self-care.	- Monitors issues related to self-care with supervisor. - Demonstrates understanding of the central role of self-care to effective practice. - Works with supervisor to monitor issues related to self-care. - Acts as recommended by supervisor for self-care to ensure effective training.
		Remediation Indicators - Lacks awareness of personal & professional values and priorities. - Fails to balance personal & professional commitments in a manner consistent with one's values & priorities. - Relies on ineffective &/or maladaptive coping (e.g., puts physical health or important relationships at risk). - Denies personal vulnerabilities. - Fails to seek help/advice for personal difficulties with the potential to impair professional functioning.	

SLO 4.5. Clinical Skills			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 7170 / 7180	CPSY 7190 / 8100
Measure	APCS Interview, Q3a	Observational Ratings & Course Tasks & PWC Codebook 5D Ratings	Observational Ratings & Course Tasks & Competencies Codebook 10B Ratings
Attainment Indicators	- Describes the application of psychological theories to clinical practice. - Recognizes the importance of client-centered approaches in interventions. - Remains positive and easy to work with even in difficult situations, and is pleasant in current interview even while discussing such topics	Demonstrates basic helping skills, such as empathic listening, framing problems.	- Develops rapport with most clients. - Develops therapeutic relationships. - Demonstrates appropriate judgment about when to consult supervisor.
		Remediation Indicators - Has difficulty establishing rapport. - Frequently shows lack of confidence. - Misses or misperceives important information/themes presented by client. - Fails to identify any goals. - Lacks awareness of underlying problems. - Unable to demonstrate empathy and caring such that most clients become willing to trust and utilize therapeutic assistance.	
SLO 4.6. Diagnosis			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Application Review	CPSY 7170 / 7180 / 7300 / 7310 / 7320 / 7330	CPSY 7190 / 8100
Measure	Admission Rubric Matrix 1	Observational Ratings; Course Tasks & PWC Codebook 6A Ratings	Observational Ratings & Course Tasks & Competencies Codebook 9D Ratings
Attainment Indicators	Provides evidence from Coursework or other exposure (work experience) to basic psychopathology	- Displays basic knowledge regarding the range of normal and abnormal behavior in the context of stages of human development and diversity. - Identifies DSM criteria. - Describes normal development consistent with broad area of training.	- Applies concepts of normal/abnormal behavior to case formulation and diagnosis in the context of stages of human development and diversity. - Articulates relevant developmental features and clinical symptoms as applied to presenting question. - Demonstrates ability to identify problem areas and to use concepts of differential diagnosis.

		Remediation Indicators • Lack of basic knowledge of range of normal and abnormal behavior. • Lack of, or misunderstanding of, DSM criteria and system.	
SLO 4.7. Measurement and Psychometrics			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 7170 / 7300 / 7310	CPSY 7190 / 8100
Measure	APCS Interview, Q7a	Observational Ratings & Course Tasks & PWC Codebook 6B Ratings	Observational Ratings & Course Tasks & Competencies Codebook 9A Ratings
Attainment Indicators	• Discussion repeatedly and consistently refers to evidence within the abstract, empirical evidence in the literature, or observations of reality	• Shows basic knowledge of the scientific, theoretical, and contextual basis of test construction and interviewing. • Demonstrates awareness of the benefits of standardized assessment. • Demonstrates knowledge of the construct(s) being assessed. • Shows understanding of basic psychometric constructs such as validity, reliability, and test construction.	• Selects assessment measures with attention to issues of reliability and validity. • Identifies appropriate assessment measures for cases seen at practice site. • Routinely consults with supervisor regarding selection of assessment measures.
		Remediation Indicators • Lack of, or misunderstanding of, the scientific, theoretical, and contextual basis of test construction. • Lack of, or misunderstanding of, basic psychometric constructs.	
SLO 4.8. Assessment Evaluation Methods			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 7170 / 7330	CPSY 7190 / 8100
Measure	APCS Interview, Q7a	Observational Ratings & Course Tasks & PWC Codebook 6C Ratings	Observational Ratings & Course Tasks & Competencies Codebook 9B Ratings

Attainment Indicators	Demonstrate knowledge of basic assessment tools and intervention strategies.	- Demonstrates basic knowledge of administration and scoring of traditional assessment measures, models and techniques, including clinical interviewing and mental status exam. - Accurately and consistently administers and scores various assessment measures. - Demonstrates knowledge of initial interviewing (both structured and semi-structured interviews, mini-mental status exam).	- Demonstrates awareness of the strengths and limitations of administration, scoring and interpretation of traditional assessment measures as well as related technological advances. - Demonstrates intermediate level ability to accurately and consistently select, administer, score and interpret assessment tools with client populations. - Collects accurate and relevant data from structured and semi-structured interviews and mini-mental status exams.
		Remediation Indicators - Displays a lack of or misunderstanding of the scientific, theoretical, and contextual basis of evaluation methodologies. - Fails to check out successfully (with supervisor or their designee) on standardized assessment measures within two attempts.	
SLO 4.9. Application of Assessment Methods			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Application Review	CPSY 7170 / 7300 / 7310 / 7330	CPSY 7190 / 8100
Measure	Admission Rubric Matrix 1	Observational Ratings & Course Tasks & PWC Codebook 6D Ratings	Observational Ratings & Course Tasks & Competencies Codebook 9C Ratings
Attainment Indicators	Academic achievement is commensurate with readiness for graduate level training	- Demonstrates knowledge of measurement across domains of functioning and practice settings. - Demonstrates awareness of need to base diagnosis and assessment on multiple sources of information. - Demonstrates awareness of need for selection of assessment measures appropriate to population/problem.	- Selects appropriate assessment measures to answer diagnostic question. - Selects assessment tools that reflect awareness of patient population served at a given practice site. - Regularly selects and uses appropriate methods of evaluation. - Demonstrates ability to adapt environment and materials according to client needs (e.g., lighting, privacy, ambient noise).

		Remediation Indicators - Displays lack of, or misunderstanding of, the scientific, theoretical, and contextual basis of the correct application of evaluation methodologies. - Cannot articulate to supervisor the multi-trait, multi-method matrix approach to assessment. - Proposes to an assessment battery that attends only to confirming hypotheses (e.g., hypothesizes ADHD and proposes a battery that fails to consider other possible causes for problems with attention/concentration).	
SLO 4.10. Diagnostic Conceptualization and Recommendations			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Application Review	CPSY 7170 / 7300 / 7310 / 7320 / 7330	CPSY 7181 / 7190 / 8100
Measure	Admission Rubric Matrix 1	Observational Ratings & Course Tasks & PWC Codebook 6E Ratings	Observational Ratings & Course Tasks & Competencies Codebook 9E Ratings
Attainment Indicators	Provides evidence that academic achievement is commensurate with readiness for graduate level training	- Displays basic knowledge of formulating diagnosis and case conceptualization. - Demonstrates the ability to discuss diagnostic formulation and case conceptualization. - Prepares basic reports which articulate theoretical material applied to case conceptualization.	- Utilizes systematic approaches of gathering data to inform clinical decision-making. - Presents cases and reports demonstrating how diagnosis is based on case material.
		Remediation Indicators - Lack of or misunderstanding of diagnostic formulation/case conceptualization. - Inability to prepare basic reports which articulate theoretical material applied to case conceptualization.	
SLO 4.11. Intervention Planning - Formulates and conceptualizes cases and plan interventions utilizing at least one consistent theoretical orientation.			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 7170 / 7180 / 7300 / 7310 / 7340	CPSY 7181 / 7190 / 8100
Measure	APCS Interview, Q2a	Observational Ratings & Course Tasks & PWC Codebook 7A Ratings	Observational Ratings & Course Tasks & Competencies Codebook 10A Ratings

Attainment Indicators	Consistently uses some combination of self-reflection, creativity, and practical knowledge to solve interpersonal problems, and is aware of this process	- Displays basic understanding of the relationship between assessment and intervention. - Articulates a basic understanding of how intervention choices are informed by assessment.	- Formulates and conceptualizes cases and plan interventions utilizing at least one consistent theoretical orientation. - Articulates a theory of change and identifies interventions to implement change. - Writes understandable case conceptualization reports and collaborative treatment plans incorporating evidence-based practices.
		Remediation Indicators - Responses to clients indicate significant inadequacies in their theoretical understanding and case formulation. - Chooses interventions without regard to their scientific status or their alignment. - Chooses interventions without regard to their acceptability within the client's values. - Chooses interventions without regard to own relevant preparation/expertise.	
SLO 4.12. Intervention Implementation			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 7180 / 7310	CPSY 7181 / 7190 / 8100
Measure	APCS Interview, Q7a	Observational Ratings & Course Tasks & PWC Codebook 7B Ratings	Observational Ratings & Course Tasks & Competencies Codebook 10C Ratings
Attainment Indicators	Evidences necessary micro-skills for treatment fidelity; their discussion keeps track of details and uses of those details to effectively evaluate strengths and weaknesses	- Displays basic knowledge of intervention strategies. - Articulates awareness of theoretical basis of intervention and some general strategies.	- Implements evidence-based interventions that consider empirical support, clinical judgment, and client diversity (e.g., client characteristics, values, and context). - Applies specific evidence-based interventions. - Presents case that documents application of evidence-based practice.

		Remediation Indicators • Fails to target interventions to client's level of understanding and motivation. • Demonstrates inability to apply interventions that are technically consistent with supervisory and literature-based guidance. • Applies interventions without regard to their scientific status. • Applies interventions without regard to their acceptability within the client's values. • Applies interventions without regard to own relevant preparation/expertise.	
SLO 4.13. Knowledge of Interventions - Knowledge of scientific, theoretical, empirical and contextual bases of intervention, including theory, research, and practice.			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 7180 / 7310	CPSY 7181 / 7190 / 8100
Measure	APCS Interview, Q5a	Observational Ratings & Course Tasks & PWC Codebook 7C Ratings	Observational Ratings & Course Tasks & Competencies Codebook 8A Ratings
Attainment Indicators	• Provides evidence of necessary micro-skills in scientific curiosity, mentions several points they want further clarity on, at least most of which are related to scientific evaluation (of the presented project)	• Demonstrates basic knowledge of scientific, theoretical, and contextual bases of intervention and basic knowledge of the value of evidence-based practice and its role in scientific psychology. • Articulates the relationship of EBP to the science of psychology. • Identifies basic strengths and weaknesses of intervention approaches for different problems and populations.	• Demonstrates knowledge of interventions and explanations for their use based on EBP. • Demonstrates the ability to select interventions for different problems and populations related to the practice setting. • Investigates existing literature related to problems and client issues. • Writes a statement of one's own theoretical perspective regarding intervention strategies.
		Remediation Indicators • Lacks self-initiative to request readings or resources used to inform treatment. • Fails to complete assigned readings associated. • Unable to articulate basic knowledge of scientific, theoretical, and contextual bases of evidence-based practice.	
SLO 4.14. Scientific Foundation of Professional Practice - Knowledge, understanding, and application of the concept of evidence-based practice.			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 7300 / 7310 / 7340	CPSY 7181 / 7190 / 8100
Measure	APCS Interview, Q7a	Observational Ratings & Course Tasks & PWC Codebook 7D Ratings	Observational Ratings & Course Tasks & Competencies Codebook 6C Ratings

Attainment Indicators	• Discussion (of presented project) repeatedly and consistently refers to evidence within the abstract, empirical evidence in the literature, or observations of reality	• Demonstrates understanding of the scientific foundation of professional practice. • Demonstrates understanding of the development of evidence-based practice in psychology (EBP). • Displays understanding of the scientific foundations of the functional competencies. • Cites scientific literature to support an argument. • Evaluates scholarly literature on a practice-related topic.	• Illustrates knowledge, understanding, and application of the concept of evidence-based practice. • Applies EBP concepts in case conceptualization, treatment planning, and interventions. • Compares and contrasts EBP approaches with other theoretical perspectives and interventions in the context of case conceptualization and treatment planning.
		Remediation Indicators • Fails to learn the evidence base associated with interventions and assessments. • Considers interventions and assessments without regard to their scientific status. • Considers interventions and assessments without regard to their acceptability within the client's values. • Considers interventions and assessments without regard to the clinician's preparation/expertise.	
SLO 4.15 Treatment Monitoring			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 7300 / 7310	CPSY 7180 / 7190 / 8100
Measure	APCS Interview, Q7a	Observational Ratings & Course Tasks & PWC Codebook 7E Ratings	Observational Ratings & Course Tasks & Competencies Codebook 10D Ratings
Attainment Indicators	• Provides evidence of necessary micro-skills for treatment monitoring; discussion repeatedly balances limitations and weaknesses of the work against the strengths of the work to form a balanced critique.	• Demonstrates basic knowledge of the assessment of intervention progress and outcome. • Demonstrates basic knowledge of methods to examine intervention outcomes.	• Evaluates treatment progress and modifies treatment planning as indicated, utilizing established outcome measures. • Assesses and documents treatment progress and outcomes. • Alters treatment plan accordingly. • Describes instances of lack progress and actions taken in response.

		Remediation Indicators • Fails to appreciate the need for ongoing evaluation of working alliance and treatment effectiveness. • Shows unawareness of methods to examine intervention outcomes. • Unable/Unwilling to perform assessments of intervention outcomes. • Persistently applies interventions that appear to be having deleterious effects.	
Goal 5. Leadership and Professional Development			
SLO 5.1. Integrity - Honesty, personal responsibility and adherence to professional values.			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Application Review	CPSY 5300 / 7170 / 7180 / 7181	CPSY 7190 / 8100 / 8v97
Measure	Admission Rubric Matrix 1	Observational Ratings & Course Tasks & PWC Codebook 4A Ratings	Observational Ratings & Course Tasks & Competencies Codebook 1A Ratings
Attainment Indicators	• Identifies personal and professional goals related to a career in clinical psychology. • Engages in networking opportunities and professional organizations.	• Demonstrates honesty, even in difficult situations. • Takes responsibility for own actions. • Displays basic understanding of core professional values. • Demonstrates ethical behavior.	• Demonstrates knowledge of professional values. • Demonstrates adherence to professional values. • Identifies situations that challenge professional values and seeks faculty/supervisor guidance as needed. • Demonstrates ability to share, discuss and address failures and lapses in adherence to professional values with supervisors/faculty as appropriate.
		Remediation Indicators • Fails to complete assigned or routine clinical tasks on time. • Fails to complete required clinic trainings (e.g., HIPPA) and orientations on time.	
SLO 5.2. Deportment - Understands how to conduct oneself in a professional manner (appropriate communication and physical conduct, including attire, across different settings).			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Application Review; Interview	CPSY 5300 / 7170 / 7180 / 7181	CPSY 7190 / 8100 / 8v97
Measure	Admission Rubric Matrix 1	Observational Ratings & Course Tasks & PWC Codebook 4B Ratings	Observational Ratings & Course Tasks & Competencies Codebook 1B Ratings

Attainment Indicators	- Recognizes the importance of mentorship in professional development. - Identifies qualities of effective mentors and supervisors.	- Demonstrates appropriate personal hygiene and attire. - Distinguishes between appropriate and inappropriate language and demeanor in professional contexts.	- Demonstrates awareness of the impact behavior has on client, public and profession. - Utilizes appropriate language and demeanor in professional communications. - Demonstrates appropriate physical conduct, including attire, consistent with context.
		Remediation Indicators - Not presenting to clinic in professional attire and appropriate hygiene. - Discusses patient(s) information in public areas of the clinic or elsewhere. - Discussion of patient(s) in unprofessional terms or with disrespect for individual differences.	
SLO 5.3. Professional Identity - Emerging professional identity as psychologist; uses resources (e.g., supervision, literature) for professional development			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Application Review; Interview	CPSY 5300 / 7300 / 7310 / 7355	CPSY 7160 / 7181 / 8100
Measure	Admission Rubric Matrix 1; APCS Interview, Q1a. Rubric Matrix 1; APCS Interview, Q1a.	Observational Ratings & Course Tasks & PWC Codebook 4C Ratings	Observational Ratings & Course Tasks & Competencies Codebook 1E Ratings
Attainment Indicators	- Identifies personal and professional goals related to a career in clinical psychology. - Displays multiple appropriate examples in which they acted independently to set goals or objectives, with a productive result.	- Has membership in salient professional organizations. - Demonstrates knowledge of the program and profession (training model, core competencies). - Demonstrates knowledge about practicing within one's competence. - Demonstrates understanding that knowledge goes beyond formal training.	- Attends broader Department and BBS colloquia, as well as regional, national and/or international conferences. - Consults literature relevant to client care and clinical science.
		Remediation Indicators - Cannot conduct an appropriate literature search. - Fails to seek needed supervision when appropriate. - Does not integrate supervision into care of client(s). - Fails to attend schedule supervision (individually or on team).	
SLO 5.4. Accountability - Accountable and reliable (consistently reliable; consistently accepts responsibility for own actions).			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Application Review	CPSY 5300 / 7160 / 7170 / 7180 / 7181 / 7300 / 7310	CPSY 7190 / 8100 / 8v97
Measure	Admission Rubric Matrix 2	Observational Ratings & Course Tasks & PWC Codebook 4D Ratings	Observational Ratings & Course Tasks & Competencies Codebook 1C Ratings

Attainment Indicators	Shows leadership in academic, non-academic, or personal situations. Evidence of managing a project or team.	- Turns in assignments in accordance with established deadlines. - Demonstrates personal organization skills. - Plans and organizes own workload. - Is aware of and follows policies and procedures of institution.	- Completes required case documentation promptly and accurately. - Accepts responsibility for meeting deadlines. - Is available when “on-call”. - Acknowledges errors. - Utilizes supervision to strengthen effectiveness of practice.
		Remediation Indicators - Fails to complete required observation or client contact hours and notes or reports in a timely manner. - Fails to maintain responsibility in scheduling rooms and/or resources both when initiating scheduling and when changes are needed.	
SLO 5.5. Interpersonal Relationships			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 5300 / 7160 / 7170 / 7180 / 7181 / 7300 / 7310	CPSY 7190 / 8100 / 8v97
Measure	APCS Interview, Q2a.	Observational Ratings & Course Tasks & PWC Codebook 5A Ratings	Observational Ratings & Course Tasks & Competencies Codebook 5A Ratings
Attainment Indicators	- Provides multiple appropriate examples in which they managed difficult conflict to lead to a positive outcome, potentially including ending interactions with people who are not willing to negotiate. - Consistently uses some combination of self-reflection, creativity, and practical knowledge to solve interpersonal problems, and is aware of this process.	- Displays mature interpersonal skills. - Listens and is empathic with others. - Respects and shows interest in others’ cultures, experiences, values, points of view, goals and desires, fears, etc. - Demonstrates skills verbally and non-verbally. - Receives feedback well.	- Forms/maintains productive, respectful relationships with clients, peers, supervisors and professionals from other disciplines. - Forms effective working alliance with clients. - Engages with supervisors to work effectively. - Works cooperatively with peers. - Is involved in departmental, institutional, and/or professional activities or governance. - Demonstrates respectful and collegial interactions with those who have different professional models or perspectives.

		Remediation Indicators • Demonstrates limited or poor interpersonal relationships. • Fails to listen / attend to others. • Is unable to see or dismisses alternative views & values. • Is unreliable / inconsistent. • Blames others; does not take responsibility for own errors. • Repeated boundary violations or excessive distance in therapeutic / supervisory relationship. • Does not prepare for supervision or seek assistance.	
SLO 5.6. Expressive Skills – Ability to clearly and articulately express oneself.			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 5300 / 7160 / 7170 / 7180 / 7181 / 7300 / 7310	CPSY 7190 / 7355 / 8100 / 8v97
Measure	APCS Interview, Q3a and b.	Observational Ratings & Course Tasks & PWC Codebook 5B Ratings	Observational Ratings & Course Tasks & Competencies Codebook 5C Ratings
Attainment Indicators	• Expresses thoughts clearly and is able to communicate both academically and emotionally	• Demonstrates coherent expressive skills. • Communicates ideas, feelings and information verbally and non-verbally.	• Utilizes clear and articulate expression. • Communicates clearly using verbal, nonverbal, and written skills. • Demonstrates understanding of professional language.
		Remediation Indicators • Uses language that is vague or contains too much jargon. • Is unable to articulate e.g. stumbles, incomplete sentences/ideas, rambles too much or overly brief.	
SLO 5.7. Affective Skills - Negotiates differences and handles conflict satisfactorily; provides effective feedback to others and receives feedback non-defensively.			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 5300 / 7160 / 7170 / 7180 / 7181 / 7300 / 7310	CPSY 7190 / 7355 / 8100 / 8v97
Measure	APCS Interview, Q4a.	Observational Ratings & Course Tasks & PWC Codebook 5C Ratings	Observational Ratings & Course Tasks & Competencies Codebook 5B Ratings

Attainment Indicators	Acknowledges difficulty in approaching conflict, but description of behavior consistently indicates goal-focused management of own affect	- Demonstrates affective skills. - Demonstrates affect tolerance. - Tolerates and understands interpersonal conflict. - Tolerates ambiguity and uncertainty. - Demonstrates awareness of inner emotional experience. - Demonstrates emotional maturity. - Listens to and acknowledges feedback from others.	- Negotiates differences and handles conflict satisfactorily; provides effective feedback to others and receives feedback non-defensively. - Works collaboratively. - Demonstrates active problem-solving. - Makes appropriate disclosures regarding problematic interpersonal situations. - Acknowledges own role in difficult interactions. - Provides feedback to supervisor regarding supervisory process. - Provides feedback to peers regarding peers' clinical work in context of group supervision or rounds. - Accepts and implements supervisory feedback non-defensively.
		Remediation Indicators - Displays frequent conflict with others OR avoids conflict. - Doesn't challenge or confront clients when appropriate. - Forms immature, superficial or distant relationships. - Forms poor working alliance with clients. - Becomes angry or overly upset by negative feedback. - Provides insensitive, overly critical feedback or is resistant to providing constructive criticism to peers.	
SLO 5.8. Supervision Skills Development - Knowledge of the supervision literature and how clinicians develop to be skilled professionals.			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 7170 / 7180 / 7300 / 7310	CPSY 5300 / 8100 / 8v97
Measure	APCS Interview, Q3a.	Observational Ratings & Course Tasks & PWC Codebook 41 Ratings	Observational Ratings & Course Tasks & Competencies Codebook 13C Ratings
Attainment Indicators	- Demonstrates knowledge about mentorship. - Displays understanding of the relationship between faculty and student. Evidence of low difficulty receiving feedback, or ability to resolve this difficulty internally, along with thoughts about valuing or asking for feedback.	- Displays interpersonal skills of communication and openness to feedback. - Completes self-assessment (e.g., Hatcher & Lassiter, 2006). - Integrates faculty/supervisor feedback into self-assessment.	- Successfully completes coursework on supervision. - Demonstrates formation of supervisory relationship integrating theory and skills including knowledge of development, educational praxis.

		Remediation Indicators • Cannot describe different models of supervision. • Fails to complete self-assessment related to supervisory skills.	
SLO 5.9. Supervision Expectations and Roles – Knowledge of purpose for roles in supervision.			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 7170 / 7180	CPSY 5300 / 7190 / 8100 / 8v97
Measure	APCS Interview, Q3a.	Observational Ratings & Course Tasks & PWC Codebook 8A Ratings	Observational Ratings & Course Tasks & Competencies Codebook 13A Ratings
Attainment Indicators	• Displays basic understanding of what a supervisor position entails. • Is agreeable to work with.	• Demonstrates basic knowledge of expectations for supervision. • Demonstrates knowledge of the process of	• Identifies roles and responsibilities of the supervisor and supervisee in the supervision process.
		Remediation Indicators • As a supervisee, is routinely unprepared for supervision (e.g., unsure what to discuss, fails to bring clinical materials for review). • As a supervisee, fails to disclose information that supervisor needs to understand clinical situations and/or accurately assess supervisee's training needs and performance.	
SLO 5.10. Supervision Processes and Procedures - Knowledge of procedures and processes of supervision.			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 5300	CPSY 7190 / 8100 / 8v97
Measure	APCS Interview, Q3a.	Observational Ratings & Course Tasks & PWC Codebook 8B Ratings	Observational Ratings & Course Tasks & Competencies Codebook 13B Ratings
Attainment Indicators	• Remains positive and easy to work with even in difficult situations, and is pleasant in current interview even while discussing such topics	• Demonstrates basic knowledge of supervision models and practice.	• Identifies goals and tasks of supervision related to developmental progression. • Tracks progress achieving goals and setting new goals.
		Remediation Indicators • Does not demonstrate a basic understanding of supervision models and practice.	
SLO 5.11. Awareness of Factors Affecting Supervision Quality – Knowledge about the impact of diversity on all professional settings and supervision Participants including self as defined by APA policy.			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 7170 / 7180	CPSY 7181 / 7190 / 8100
Measure	APCS Interview, Q4a.	Observational Ratings & Course Tasks & PWC Codebook 8C Ratings	Course Tasks

Attainment Indicators	Consistently describes considering other perspectives and options.	- Demonstrates basic knowledge of and sensitivity to issues related to individual and cultural differences (i.e., the APA definition) as they apply to the supervision process and relationships. - Demonstrates basic knowledge of literature on individual and cultural differences and engages in respectful interactions that reflect that knowledge.	- Demonstrates beginning knowledge of personal contribution to therapy and to supervision. - Demonstrates knowledge of Individual and Cultural Differences (I&CD) literature and APA guidelines in supervision practice. - Demonstrates awareness of role of oppression and privilege on supervision process.
		Remediation Indicators - Cannot differentiate appropriate and inappropriate responses to individual and cultural differences in supervision. - Lacks insight into how one's cultural identity may differentially affect supervisees.	
SLO 5.12. Participation in Supervision Process - Observation of and participation in supervisory process (e.g., peer supervision).			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 7170 / 7180 / 8v97	CPSY 7190 / 8100
Measure	APCS Interview, Q3a and b.	Observational Ratings & Course Tasks & PWC Codebook 8D Ratings	Observational Ratings & Course Tasks & Competencies Codebook 4D Ratings
Attainment Indicators	Report of low difficulty receiving feedback, or ability to resolve this difficulty internally, along with thoughts about valuing or asking for feedback.	- Shows awareness of need for straightforward, truthful, and respectful communication in supervisory relationship. - Demonstrates willingness to admit errors, accept feedback.	- Reflects on supervision process, areas of strength and those needing improvement. - Seeks supervision to improve performance, presenting work for feedback, and integrating feedback into performance.
		Remediation Indicators - Fails to provide candid and sensitive critical feedback to when needed. - Is a passive observer rather than actively engaged in group supervision. - Discounts supervisee's perspective or opinion. - Does not seek supervision of supervision &/or supervisory consultation when needed. - Fails to adopt a genuine openness to feedback from supervisees as evident from developmental stagnation as a supervisor.	
SLO 5.13. Ethical and Legal Issues in Supervision - Knowledge of and compliance with ethical/professional codes, standards and guidelines; institutional policies; laws, statutes, rules, regulations, and case law relevant to the practice of psychology and its supervision.			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 5300 / 7170 / 7180 / 7190	CPSY 8100 / 8v97

Measure	APCS Interview, Q2a.	Observational Ratings & Course Tasks & PWC Codebook 8E Ratings	Course Tasks
Attainment Indicators	Provides evidence from multiple appropriate examples in which they managed difficult conflict to lead to a positive outcome, potentially including ending interactions with people who are not willing to negotiate. <u>Or</u> a single example from a particularly challenging situation that led to a positive outcome, potentially including ending interactions with people who are not willing to negotiate. <u>Or</u> spontaneous description of working style that effectively manages conflict before it arises.	- Demonstrates knowledge of principles of ethical practice and basic skills in supervisory ethical decision making. - Demonstrates knowledge of legal and regulatory issues in supervision. - Demonstrates understanding of this knowledge (e.g., APA 2002 ethical principles).	- Behaves ethically. - Recognizes ethical and legal issues in clinical practice and supervision.
		Remediation Indicators - Fails to recognize ethical issues in supervision. - Fails to recognize legal issues in supervision. - Cannot articulate institutional &/or APA principles as related to supervision. - Behaves unethically toward supervisee (e.g., verbally abusive, sexually objectifying).	
SLO 5.14. Knowledge of the Shared and Distinctive Contributions of Other Professions			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Application Review	CPSY 7190	CPSY 8100
Measure	Admission Rubric Matrix 1	Observational Ratings & Course Tasks & PWC Codebook 9A Ratings	Observational Ratings & Course Tasks & Competencies Codebook 14A Ratings
Attainment Indicators	Has had exposure to behavioral health field.	- Demonstrates beginning, basic knowledge of the viewpoints and contributions of other professions/ professionals. - Demonstrates knowledge, respect, and valuing of roles, functions and service delivery systems of other professions.	- Shows awareness of multiple and differing worldviews, roles, professional standards, and contributions across contexts and systems. - Demonstrates intermediate level knowledge of common and distinctive roles of other professionals. - Reports observations of commonality and differences among professional roles, values, and standards.
		Remediation Indicators Signals lack of awareness or understanding of basic contributions of other professions such as social work, psychiatry, primary care, etc.	
SLO 5.15. Functioning in Multidisciplinary and Interdisciplinary Contexts			
Timeline	Admission Readiness	Pre-candidacy	Candidacy

Method	Interview	CPSY 7190	CPSY 8100
Measure	ChatGPT; APCS Interview, Q2a.	Observational Ratings & Course Tasks & PWC Codebook 9B Ratings	Observational Ratings & Course Tasks & Competencies Codebook 14B Ratings
Attainment Indicators	- Shows willingness to engage in teamwork. - Describes one or more appropriate examples in which they productively handled conflict.	- Demonstrates cooperation. - Demonstrates ability to cooperate with others in task completion.	- Demonstrates beginning knowledge of strategies that promote interdisciplinary collaboration vs. multidisciplinary functioning. - Demonstrates knowledge of the nature of interdisciplinary vs. multidisciplinary function and the skills that support interdisciplinary process.
		Remediation Indicators Displays defensiveness when interacting with others.	
SLO 5.16. Understands How Participation in Interdisciplinary Collaboration/ Consultation Enhances Outcomes			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Application Review	CPSY 7170 / 7180 / 7300 / 7310	CPSY 7190 / 7355 / 8100
Measure	Admission Rubric Matrix 2	Observational Ratings & Course Tasks & PWC Codebook 9C Ratings	Observational Ratings & Course Tasks & Competencies Codebook 14C Ratings
Attainment Indicators	Identifies team collaboration as enhancing a project.	- Shows knowledge of how participating in interdisciplinary collaboration/ consultation can be directed toward shared goals. - Demonstrates understanding of consultation concept.	- Participates in and initiates interdisciplinary collaboration/ consultation directed toward shared goals. - Consults with and cooperates with other disciplines in service of clients.
		Remediation Indicators Does not practice reflective listening skills or dominates discussion with own ideas.	
SLO 5.17. Respectful and Productive Relationships with Individuals from Other Professions			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Interview	CPSY 7190	CPSY 8100
Measure	APCS Interview, Q4a.	Observational Ratings & Course Tasks & PWC Codebook 8D Ratings	Observational Ratings & Course Tasks & Competencies Codebook 14D Ratings
Attainment Indicators	- Recognizes the differences in others and where assumptions can be harmful. - Acknowledges difficulty in approaching conflict, but description of behavior consistently indicates goal-focused management of own affect.	- Shows awareness of the benefits of forming collaborative relationships with other professionals. - Expresses interest in developing collaborative relationships and respect for other professionals.	- Develops and maintains collaborative relationships and respect for other professionals. - Communicates effectively with individuals from other profession.
		Remediation Indicators Makes dismissive or disparaging comments of other disciplines.	

SLO 5.18. Role of Consultant			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Application Review	CPSY 5300 / 7170 / 7180	CPSY 7190 / 8100
Measure	Admission Rubric Matrix 1	Observational Ratings & Course Tasks & PWC Codebook 9E Ratings	Observational Ratings & Course Tasks & Competencies Codebook 11A Ratings
Attainment Indicators	The personal statement evidences micro-skills necessary to successful consultation, by aligning ambition and future goals to faculty and program expertise.	- Understands the basic concept of the consultant's role - Identifies and differentiates the consultant's role from other professional roles (e.g., therapist, supervisor), though not always with full clarity or depth. - Engages in discussions with the consultee about the consultant's role. - Begins to articulate the common and distinctive roles of a consultant but may require guidance to effectively compare and contrast consultation with clinical and supervision roles. - Demonstrates emerging competence in applying consultation frameworks but needs further experience to integrate these frameworks effectively into real-world situations.	- Displays knowledge of the consultant's role and its unique features as distinguished from other professional roles (such as therapist, supervisor, teacher). - Articulates common and distinctive roles of consultant. - Compares and contrast consultation, clinical and supervision roles.
		Remediation Indicators - Does not recognize multiple professional roles such as consultant. - Cannot distinguish among consultant roles. - Does not discuss with or inform consultee of clinician's role in consultation.	
SLO 5.19. Addressing Consultation Referral Question			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Application Review	CPSY 7170 / 7330	CPSY 7190 / 8100
Measure	Admission Rubric Matrix 2	Observational Ratings & Course Tasks & PWC Codebook 9F Ratings	Observational Ratings & Course Tasks & Competencies Codebook 11B Ratings

Attainment Indicators	The personal statement evidences micro-skills necessary to successful problem solving and planning.	- Demonstrates a foundational understanding of common psychological assessment tools, though may need supervision in selecting the most appropriate tools for specific referral questions. - Can collect data using standardized tools - Shows awareness of various sources of assessment tools but may lack experience in identifying the most effective ones for different consultative situations. - Demonstrates capability in making preliminary decisions about assessment selection with supervision - Demonstrates understanding of the basics of a consultative role but is still developing the ability to independently carry out a full consultation, including the selection of assessment tools.	- Knowledge of and ability to select appropriate means of assessment to answer referral questions. - Implements systematic approach to data collection in a consultative role. - Identifies sources and types of assessment tools.
		Remediation Indicators Inappropriate or incorrect selection of assessment tools.	
SLO 5.20. Application of Consultation Methods			
Timeline	Admission Readiness	Pre-candidacy	Candidacy
Method	Application Review	CPSY 7170 / 7330	CPSY 7190 / 8100 / 8v97
Measure	Admission Rubric Matrix 2	Observational Ratings & Course Tasks & PWC Codebook 9G Ratings	Observational Ratings & Course Tasks & Competencies Codebook 11D Ratings
Attainment Indicators	Letters of recommendation includes examples of applicant seeking or providing appropriate consultation.	- Demonstrates emerging ability to identify literature relevant to consultation methods. - Shows an understanding of basic consultation assessment methods and can apply them with supervision. - Begins to integrate assessment findings into decision-making, though requires refinement. - Can identify appropriate interventions based on consultation assessment findings. - Demonstrates awareness of the system in which consultation occurs and is beginning to consider broader factors that affect outcomes. - Indicates understanding of ethical guidelines and cultural competence in consultation. - Clearly communicates findings and recommendations.	- Identifies and acquires literature relevant to unique consultation methods (assessment & intervention) within systems, clients or settings. - Identifies appropriate interventions based on consultation assessment findings.

	Remediation Indicators • Struggles to identify and locate relevant literature pertaining to unique consultation methods, assessment, and intervention across systems, clients, or settings. • Has difficulty understanding or applying basic consultation assessment methods. Fails to appropriately integrate assessment findings into decision-making processes. • Makes inconsistent or inappropriate selection of interventions based on assessment findings. Lacks understanding of how to tailor interventions to specific consultation needs. • Shows limited comprehension of the broader system dynamics within which the consultation is occurring (e.g., organizational culture, team dynamics, or external factors). • Fails to adequately consider ethical standards, cultural competence, or diversity factors in consultation scenarios. • Struggles to effectively communicate assessment findings and intervention recommendations to clients or system stakeholders.	
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Table 3

Rank Order of Students' Competency Attainment Ease for Goals, 1, 3, 4, and 5 in a Sample of Clinical Psychology Doctoral Trainees (source data does not includes items that align with Goal 2).

Goal #	Student Learning Outcome #	Competency	Pre-candidacy Indicators	Candidacy Indicators	Ease of Attainment
4	4.1	Concern for the Welfare of Others	CPSY 7170 / 7180	CPSY 7181 / 7190 / 8100	.67
5	5.5	Interpersonal Relationships	CPSY 5300 / 7160 / 7170 / 7180 / 7181 / 7300 / 7310	CPSY 7181 / 7190 / 8100 / 8v97	.46
5	5.1	Integrity	CPSY 5300 / 7170 / 7180 / 7181	CPSY 7190 / 8100 / 8v97	.45
5	5.2	Deporment	CPSY 5300 / 7170 / 7180 / 7181	CPSY 7190 / 8100 / 8v97	.45
5	5.4	Accountability	CPSY 5300 / 7160 / 7170 / 7180 / 7181 / 7300 / 7310	CPSY 7181 / 7190 / 8100 / 8v97	.40
5	5.12	Participation in Supervision Process	CPSY 7170 / 7180 / 8v97	CPSY 7190 / 8100	.36
5	5.7	Affective Skills	CPSY 5300 / 7160 / 7170 / 7180 / 7181 / 7300 / 7310	CPSY 7181 / 7190 / 7355 / 8100 / 8v97	.28
3	3.3	Ethical Conduct	CPSY 7170 / 7180	CPSY 7190 / 8100	.24
5	5.9	Supervision Expectations and Roles	CPSY 7170 / 7180	CPSY 5300 / 7190 / 8100 / 8v97	.24
4	4.5	Clinical Skills	CPSY 7170 / 7180	CPSY 7190 / 8100	.21
5	5.6	Expressive Skills	CPSY 5300 / 7160 / 7170 / 7180 / 7181 / 7300 / 7310	CPSY 7181 / 7190 / 7355 / 8100 / 8v97	.16
4	4.8	Assessment Evaluation Methods	CPSY 7170 / 7330	CPSY 7190 / 8100	.15
4	4.9	Application of Assessment Methods	CPSY 7300 / 7310 / 7330	CPSY 7170 / 7190 / 8100	.15
5	5.10	Supervision Processes and Procedures	CPSY 5300	CPSY 7190 / 8100 / 8v97	.13
5	5.17	Respectful and Productive Relationships with Individuals from Other Professions	CPSY 7190	CPSY 8100	.13
4	4.7	Measurement and Psychometrics	CPSY 7170 / 7300 / 7310	CPSY 7190 / 8100	.12
5	5.13	Ethical and Legal Issues in Supervision	CPSY 5300 / 7170 / 7180 / 7190	CPSY 8100 / 8v97	.03
4	4.4	Self-Care	CPSY 5300 / 7170 / 7180	CPSY 7181 / 7190 / 8100	.00
5	5.19	Addressing Consultation Referral Question	CPSY 7170 / 7330	CPSY 7190 / 8100	-.03

4	4.6	Diagnosis	CPSY 7170 / 7300 / 7310 / 7320 / 7330 /	CPSY 7180 / 7190 / 8100	-.04
3	3.2	Awareness and Application of Ethical Decision Making	CPSY 7300 / 7310 / 7170 / 7180	CPSY 7190 / 8100	-.04
4	4.10	Diagnostic Conceptualization and Recommendations	CPSY 7170 / 7300 / 7310 / 7320 / 7330	CPSY 7181 / 7190 / 8100	-.04
3	3.1	Knowledge of Ethical, Legal and Professional Standards and Guidelines	CPSY 7300 / 7310 / 7330 / 7340	CPSY 7170 / 7180 / 7190/ 8100	-.04
4	4.2	Reflective Practice	CPSY 7170 / 7180	CPSY 7181 / 7190 / 8100	-.05
3	3.5	Others as Shaped by Individual and Cultural Diversity and Context	CPSY 7170 / 7180 7181 / 7320	CPSY 7190 / 8100	-.05
5	5.3	Professional Identity	CPSY 5300 / 7300 / 7310 / 7355	CPSY 7160 / 7181 / 8100	-.07
4	4.12	Intervention Implementation	CPSY 7180 / 7310	CPSY 7181 / 7190 / 8100	-.07
1	1.1	Scientific Mindedness	CPSY 7300 / 7310 / 8v87	CPSY 7160; HCS 7355; CPSY 8v97	-.08
3	3.8	Empowerment	CPSY 7300 / 7310	CPSY 7190 / 8100	-.08
4	4.3	Self-Assessment	CPSY 7170 / 7180	CPSY 7181 / 7190 / 8100	-.09
5	5.20	Application of Consultation Methods	CPSY 7170 / 7330	CPSY 7190 / 8100 / 8v97	-.10
3	3.6	Interaction of Self and Others as Shaped by Individual and Cultural Diversity and Context	CPSY 7170 / 7180 / 7181	CPSY 7190 / 8100	-.11
3	3.4	Self as Shaped by Individual and Cultural Diversity and Context	CPSY 7170 / 7180 / 7181	CPSY 7190 / 8100	-.12
5	5.16	Understands How Participation in Interdisciplinary Collaboration/ Consultation Enhances Outcomes	CPSY 7170 / 7180 / 7300 / 7310	CPSY 7190 / 7355 / 8100	-.16
3	3.7	ICD Applications Based on Individual and Cultural Context	CPSY 7170 / 7180 / 7181 / 7340	CPSY 7190 / 8100	-.16
1	1.2	Scientific Foundations	HCS 6317; CPSY 7300 / 7310 / 8v87	CPSY 7160; HCS 7355; CPSY 8v97	-.19
4	4.15	Treatment Monitoring	CPSY 7300 / 7310	CPSY 7180 / 7190 / 8100	-.22
4	4.14	Scientific Foundation of Professional Practice	CPSY 7300 / 7310 / 7340	CPSY 7181 / 7190 / 8100	-.22
4	4.11	Intervention Planning	CPSY 7170 / 7180 / 7300 / 7310 7340	CPSY 7181 / 7190 / 8100	-.23
5	5.18	Role of Consultant	CPSY 5300 / 7170 / 7180	CPSY 7190 / 8100	-.24
5	5.15	Functioning in Multidisciplinary and Interdisciplinary Contexts	CPSY 7190	CPSY 8100	-.25

5	5.14	Knowledge of the Shared and Distinctive Contributions of Other Professions	CPSY 7190	CPSY 8100	-.31
5	5.11	Awareness of Factors Affecting Supervision	CPSY 7170 / 7180	CPSY 7181 / 7190 / 8100	-.32
4	4.13	Knowledge of Interventions	CPSY 7180 / 7310	CPSY 7181 / 7190 / 8100	-.36
5	5.8	Supervision Skills Development	CPSY 7170 / 7180 / 7300 / 7310	CPSY 5300 / 8100 / 8v97	-.37

Note. Attainment difficulty statistics are listed in logits, with 0 representing the SLO that falls at the mean difficulty point. Items listed in the table above the bolded item are attained increasingly quickly, while items below the bolded item are increasingly more difficult to attain. Table adapted from Price, S. D., Callahan, J. L., & Cox, R. J. (2017). Psychometric investigation of competency benchmarks. *Training and Education in Professional Psychology, 11*, 128-139. <https://doi.org/10.1037/tep0000133>.

APPENDIX L

Concentration Handbook and PSEC Manual



2026-2027

CLINICAL PSYCHOLOGICAL SCIENCE PHD CONCENTRATION HANDBOOK

THE UNIVERSITY OF TEXAS AT DALLAS



Table of Contents

Training Goal and Principles	3
Accreditation and Implications	4
Concentration Overview	4
Concentration Advising and Mentorship	4
Concentration Commitment to Clinical Psychology as a Profession	5
Personalized Training.....	6
Course, Research, and Clinical Requirements	6
Course Descriptions	7
Research Requirements.....	11
Clinical Requirements	14
Psychological Services and Education Center	14
Externship	15
Internship.....	17
Patient-Trainee Clinician Value Conflicts	18
Patient Care and Vacation Time	18
Completion of Practicum Supervisor Evaluations.....	19
Professional Development Requirements	19
Instructor of Record.....	19
Developing Expertise via Professional Development	19
Digital Discourse	19
Optional Pathways.....	20
Documentations for Professional Practice	20
Streamlined Entrance to the Academy or Research Industry.....	21
Insufficient Progress	22
Remediation	22
Student Grievances.....	23
BBS Research Centers	24
UT Dallas Resources.....	25
Resources Beyond UT Dallas.....	26
National Organizations	26
Grants and Fellowships	27
Mental Health Resources.....	27

Psychological Clinical Science PhD Concentration Handbook
ClinicalPsychology@utdallas.edu

This handbook is your guide to the Psychological Clinical Science PhD concentration at UT Dallas. It outlines the concentration's unique requirements and procedures, complementing but not superseding the Psychology PhD Student Guide or the Psychological Services and Education Center (PSEC) Manual.

The policies and procedures presented here represent the concentration's expectations for doctoral students as of their matriculation. These guiding principles will remain in effect throughout your tenure, except in the case of deliberate and cohort-wide implementation of new substantive policies.

For detailed regulations governing student conduct and academic standards, please refer to the relevant documents of the Office of Graduate Education, School of Behavioral and Brain Sciences, and the Department of Psychology.

Tailored Training, Grounded in Rigor

We champion scientific rigor, but we also celebrate the spark of individual brilliance that can fuel groundbreaking discoveries. You are encouraged to explore, experiment, and evolve to forge your own path while adhering to the bedrock principles of evidence-based research.

Key Takeaways:

- **Concentration Requirements:** Understand the essential building blocks including coursework, research experiences, and clinical training.
- **Decision Points:** Navigate key crossroads in your training, such as choosing research areas, selecting mentors, and planning your dissertation.
- **Faculty Insights:** Access practical tips and valuable advice.
- **Resources and Support:** Discover the wealth of resources available to you, from funding opportunities to academic support services.

Remember, as you navigate this guide:

- **Take Responsibility:** This handbook provides directions, but the steering wheel is in your hands.
- **Seek Guidance, Not Just Rules:** We offer clear guidelines, but your journey is best charted with the combined insights of your expert mentors.
- **Embrace Exploration:** Innovate, connect seemingly disparate dots, and let your scientific curiosity lead the way.

Training Goal and Principles

The following excerpt from the mission statement of the Academy of Clinical Psychological Science captures the philosophical underpinnings of your concentration:

Clinical science is a psychological science directed at the assessment, understanding, and amelioration of human problems in behavior, affect, cognition, or health, and at the application of knowledge to such problems in ways consistent with scientific evidence. The emphasis on the term "science" underscores a commitment to empirical approaches to advancing knowledge.

In keeping with that philosophy, our concentration's goal is to launch you into the forefront of modern clinical psychology. We will equip you to become a leader and innovator, contributing significantly to diverse careers as clinical psychological scientists.

You will achieve this by:

- **Conducting and sharing impactful research:** We will train you in basic, applied, and translational research, focusing on mental illness and well-being. You will learn to uncover new knowledge, develop cutting-edge assessments and treatments, and share your findings effectively.
- **Providing top-notch care:** You will master evidence-based interventions for diverse populations, tailoring approaches to meet individual needs while upholding ethical and legal standards. We will also build your skills in consulting with multidisciplinary teams and navigating different care contexts.
- **Solving important problems:** We emphasize problem-solving, teaching you to identify and tackle novel clinical and research challenges. You will learn to design, implement, and evaluate new solutions, becoming an adaptable and resourceful expert.
- **Embracing diversity:** We believe diverse perspectives fuel both research and practice. You will learn from and alongside faculty, students, and collaborators from various backgrounds, enriching your understanding of mental health and its complexities.
- **Bridging the gap:** Translational research – applying basic science to real-world problems – is crucial in alleviating mental illness. We will equip you with a deep understanding of key psychological processes and the skills to translate discoveries into effective assessments, interventions, and prevention strategies.
- **Mastering the cutting edge:** Dive into emerging fields and methodologies relevant to your interests. Work closely with your advisors to tailor your training, pursuing specialized coursework, collaborating with experts, and exploring workshops or lab rotations.
- **Seamless integration:** Forget compartmentalized learning. We seamlessly integrate theory, research, and clinical practice, allowing you to wear one "clinical science" hat throughout your concentration, ready to excel in any environment.

Across your training you will master the skills and competencies necessary for an impactful career, whether that career is focused on uncovering scientific insights, shaping innovative treatments, or promoting equitable access to care.

Accreditation and Implications

The Clinical Psychological Science concentration is provisionally accredited by the Psychological Clinical Science Accreditation System (PCSAS). We stand firmly behind PCSAS's mission to cultivate high-impact, science-driven education and training in clinical psychology and share in PCSAS's unwavering commitment to excellence. To learn more about PCSAS accreditation and its rigorous standards, please visit <https://www.pcsas.org/>.

It's important to note that PCSAS-accredited programs are currently navigating an evolving landscape of internship and post-doctoral placement acceptance. Students are able participate in the Association of Psychology Postdoctoral and Internship Centers (APPIC) matching service. Additionally, career opportunities within the U.S. Department of Veterans Affairs and the U.S. Public Health Service offer full parity for PCSAS graduates interested in those specific settings. However, full parity with APA accreditation for university, college, and medical center positions is still in development.

Licensure eligibility also varies by state, though there is a growing list of states that now explicitly recognize PCSAS-accreditation for licensing purposes. The concentration is committed to providing a rigorous and impactful clinical psychological science education, and we are dedicated to supporting graduates in securing diverse career opportunities. However, professional licensure is not a primary outcome goal for our concentration. Aligned with our vision for the future of doctoral level clinical psychology as a field, we prioritize research and academic training to prepare students for leadership roles. Therefore, if your primary focus is on traditional clinical practice, we encourage you to carefully consider all options and ensure your chosen path aligns with your professional goals.

Concentration Overview

The UTD Clinical Psychological Science PhD concentration follows a structured yet flexible pace. The academic year spans August to May, divided into two semesters. Summer typically offers research, special studies, or additional clinical experiences. You will typically take 9-12 units per semester, completing coursework before embarking on your required full-time clinical internship (normally in year 6). Dissertation work usually begins in year 3 and should be completed by the end of year 5. Expect to spend 5 years in residence at UT Dallas, plus one internship year, with graduation contingent on completed internship hours, even if your dissertation finishes earlier.

Concentration Advising and Mentorship

Our concentration boasts a vibrant research ecosystem woven tightly with the larger Department's leading investigators in diverse fields like cognitive neuroscience, human development, and social psychology. This interdisciplinary ethos is reflected in our admissions process, where we seek applicants whose scholarly and empirical interests resonate with two or more faculty members. This ensures you will find the ideal research mentor to guide your journey. Across your training years, you will likely collaborate on research with multiple researchers, within and beyond the Department, to develop diverse skills and perspectives.

Your primary research mentor plays a crucial role in your training, helping you craft a personalized training plan that is aligned with your goals and training needs while also meeting concentration requirements. Expect to dedicate 10-20 hours per week to ongoing research activities with your primary research mentor, honing your investigative skills and building a body of research relevant to your passion. Remember, good research mentorship is an intensive and continuous process that thrives on communication, so come prepared to your meetings with specific goals and questions.

Beyond your primary mentor, a personalized training committee will support and guide your progress. Composed of at least two faculty members (at least one of which has been clinically trained), this committee will help you refine your training plan, review your progress across all domains, and act as a valuable resource for guidance throughout your doctoral journey. You will meet with them at least once each semester, and changes to the committee composition can be requested (by either you or faculty) as your interests and plans evolve. The personalized training committee is formed in the fall semester of the first year. Students are asked to submit their preferences for the second member of the committee, and the Director of Clinical Science Training (DCST) will coordinate with faculty members to finalize this assignment. Any subsequent changes should be submitted to the DCST.

Remember, the personalized training committee is there to support you. Keep them informed of your accomplishments and any challenges you face. Seek their input and utilize their expertise. To facilitate fruitful discussions, submit your CV and a brief progress survey before each meeting with your personalized training committee, and follow up with a memo summarizing the key points afterwards. Formal feedback mechanisms further complement the ongoing guidance you receive. Each year, you will receive a detailed letter from your personalized training committee highlighting your progress and areas for growth.

Beyond the comprehensive mentorship and support of your personalized training committee, you will also receive summative evaluations of your clinical activities every semester. These evaluations, along with other concentration records, are stored in your student file and used to gauge your progress and support your internship applications. Records are retained indefinitely; you can request access to your file through the DCST.

While most advisor relationships are positive, remember you have options if faced with any difficulties. First, talk to your personalized training committee – open communication is key. If issues persist, you can reach out to the Director of Clinical Science Training, faculty on the Clinical Science Steering Committee (CSSC), the [Psychology PhD Program Head](#), or even the Department Chair or the UT Dallas [Office of Graduate Education](#) and/or [Dean of Students](#). It's important to follow an escalation process within the concentration before seeking external resolution. You have the right to understand the parameters of any interaction regarding a concern you raise. Ask about confidentiality, potential actions, and the information you will receive during the process.

Concentration Commitment to Clinical Psychology as a Profession

Our concentration holds a deep responsibility to the public trust and professional conduct. We strive to ensure that our graduates possess the knowledge, skills, and ethical commitment necessary to deliver responsible scientific and service activities. This responsibility holds true throughout your enrollment, including periods of temporary leave.

As graduate students, you are expected to uphold the highest standards of professional and ethical conduct. For your reference, we have listed key publications detailing accepted professional and ethical guidelines:

- [Ethical Principles of Psychologists](#)
- [APS Code of Conduct for Conventions and Events](#)
- [Ethical Principles in the Conduct of Research with Human Participants](#)
- [Standards for Education and Psychological Tests](#)
- [Texas Law and the Practice of Psychology](#)

Beyond these broad standards, each research and service delivery setting has its own set of local practices and procedures. Upon joining any professional activity site, it is your responsibility to thoroughly familiarize yourself with these specific regulations and ensure all your actions adhere to them. By diligently upholding these expectations, you contribute to the concentration's mission of fostering responsible and ethical psychologists who serve the public with distinction.

Personalized Training

Your training needs and career goals are as unique as you are. Across your training, a personalized training committee will monitor your training and use their expertise to guide you so that you take full advantage of the opportunities available during your training.

At key benchmarks required of all PhD students (regardless of concentration), task-specific committees will be formed to provide summative evaluations of your work. A summary of these committees, their composition, purpose, and membership is provided below. Please consult the Psychology PhD Student Guide for more detailed information (e.g., how to work with your research advisor to identify potential committee members from among BBS faculty).

Key Committees

- 1 — **Personalized Training Committee**
This committee consists of your research mentor and two additional faculty members, with at least one being clinically trained. They guide you throughout your training years, ensuring your specific needs and career goals are met optimally.
- 2 — **Pre-dissertation Portfolio Evaluation Committee**
Composed of your research mentor and two UT Dallas faculty (one from the Psychology Department), this committee evaluates your pre-dissertation portfolio.
- 3 — **Supervising Dissertation Committee**
Four faculty members from the Psychology Department, nominated by the Director of Clinical Science Training and approved by the Dean of Graduate Education, oversee your dissertation.
- 4 — **Dissertation Oral Examination Committee**
The Dean of Graduate Education appoints a neutral Chair to work with your Supervising Dissertation Committee for your dissertation's oral defense.

Course, Research, and Clinical Requirements

The Psychology PhD concentration in clinical psychological science requires a minimum of 75 semester hours beyond the bachelor's degree. By keeping concentration-required credit hours minimal for degree attainment, you are free to enhance your training by taking a variety of available elective coursework and research opportunities. Students typically take more electives than would be needed to meet minimum requirements.

Qualified and accepted students may enter the doctoral concentration holding either a bachelor's or master's degree. If the coursework is sufficient to meet doctoral program requirements the doctoral

course requirement may be waived. That decision is made by the department and not the concentration. Waiver of requirements does not reduce the required number of credit hours, but it does create more flexibility in your degree plan. Core Courses with a CPSY prefix cannot be waived (i.e., CPSY 7300 and CPSY 7310). Students must earn a grade of B or better (i.e., a B- is not considered a B) in all Core Courses listed below regardless of prefix. For all students enrolled in the clinical concentration specifically, any course grade of B- or lower in a Core Course results in the student being required to repeat the course. Required core courses and elective choices that are commonly of interest to students in this concentration are summarized below:

Course Number	Title	Credit Hours
Core Courses		
HCS 6302	Issues in Behavioral and Brain Sciences	3
HCS/ACN/PSYC 6312	Research Methods in Behavioral and Brain Sciences - Part I	3
HCS/ACN/PSYC 6313	Research Methods in Behavioral and Brain Sciences - Part II	3
HCS/PSYC 6317	Research Methods in Psychology	3
HCS XXXX	Two core courses from any one of the following Department of Psychology concentrations: Developmental Psychology (includes HCS/ACN/PSYC 6331; HCS/PSYC 6350; HCS/ACN/PSYC 6368); Cognitive Psychology (includes HCS/ACN/PSYC 6330; HCS/ACN/PSYC 6333; HCS/ACN/PSYC 6395); Social/Personality Psychology (includes HCS/PSYC 6327; HCS/PSYC 6376); Neuroscience (includes HCS/ACN/PSYC 6338; HCS/ACN/PSYC 6346)	6
CPSY 7300	Clinical Psychological Science I	3
CPSY 7310	Clinical Psychological Science II	3
Typical Electives		
PSYC 7308	Psychopathology	3
CPSY 7330	Advanced Neuropsychological Assessment – From Bench to Bedside	3
CPSY 7340	Advanced Clinical Trials Design and Analysis	3
HCS 7355	Seminar in Psychology – Clinical Psychological Science	3
CPSY 7160	Clinical Psychological Science Research Colloquium	8
CPSY 8v87	Research in Psychology	6
CPSY 7270	Assessment Clinic	2
CPSY 7180	Intervention Clinic	5
CPSY 7181	Clinical Psychological Science Grand Rounds	5
CPSY 7190	External Practicum	2
CPSY 8100	Internship	2
CPSY 8v97	Dissertation	9
CPSY 5300	Effective Teaching and Supervision in Psychology	3

Course Descriptions

HCS 6302 Issues in Behavioral and Brain Sciences (3 credits)

Doctoral proseminar on current theory and research in cognition and neuroscience; speech, language, and hearing sciences; and psychology. Pass/Fail only. Prerequisite: BBS doctoral students only or instructor consent required

HCS 6312 (ACN 6312 / PSYC 6312) Research Methods in Behavioral and Brain Sciences - Part I (3 credits)

This course focuses on applying, understanding, and interpreting various ANOVA-related statistical techniques in a behavioral science context. Students learn the frameworks for hypothesis testing and effect size estimation. The course provides students with an understanding of the interrelationships among statistical techniques, and computer skills required for data analyses. Students without the necessary background knowledge of basic statistics and experimental design will be required to take PSY

3392 before registering for ACN 6312. Prerequisites: BBSC majors only and department consent required.

HCS 6313 (ACN 6313 / PSYC 6313) Research Methods in Behavioral and Brain Sciences - Part II (3 credits)

This course focuses on applying, understanding, and interpreting regression and analysis of variance - related statistical techniques in a behavioral and brain science context. The course provides students with increased conceptual understanding of topics within regression and analysis of variance (e.g., hierarchical regression analysis, multiple regression with continuous and categorical predictors, regression diagnostics, fixed, random, and mixed effect models), along with computer skills required to interpret data analyses. Prerequisites: (ACN 6312 or HCS 6312 or PSYC 6312) and department consent required.

HCS 6317 (PSYC 6317) Research Methods in Psychology (3 credits)

This course overviews research methods in psychological science. Students learn to design, conduct, and evaluate psychological research. Students will learn to critically evaluate the methodology and conclusions of existing and proposed research. Students will develop a formal research proposal and will learn about the process of grant submission and peer review. Students will also learn about issues related to professionalism, diversity, and ethics in the conduct and publication of research in psychology. Prerequisite: BBSC majors only and department consent required.

CSPY 7300 Clinical Psychological Science I (3 credits)

This foundational course equips students with critical thinking skills for navigating mental health research and practice. Through lectures, discussions of classic texts, and exploration of contemporary topics, students gain a deep understanding of scientific theory, knowledge acquisition, and the distinction between science and pseudoscience. Topics include scientific reasoning, theory formulation, evidence interpretation, pseudoscience critique, classic papers, and contemporary issues (e.g., diagnostic limitations, research challenges). Additionally, students begin developing essential clinical skills through mastery-based learning experiences focusing on core microskills (rapport building, active listening) and ethical considerations within the framework of informed consent, risk assessment, confidentiality, and mandated reporting. Prerequisites: PSYC 7308. Department consent required.

CSPY 7310 Clinical Psychological Science II (3 credits)

Building upon the established foundation in CPSY 7300, this course delves deeper into assessment and treatment of mental health concerns. Students explore the ethical and cultural considerations in psychological assessment, case formulation, and integrative report writing. Key topics include adult assessment of intellectual, cognitive, emotional, and personality functioning; psychometric principles (reliability, validity, norms, standardized scores); ethical and cultural considerations; case formulation; and integrative report writing. Additionally, a pro-seminar format addresses the crucial role of context, culture, and diversity in influencing psychopathology prevalence, presentation, and development across diverse populations. The course culminates by exploring how scientific knowledge on these issues informs the development, adaptation, and dissemination of culturally-sensitive evidence-based mental health interventions. Prerequisites: CPSY 7300. Department consent required.

PSYC 7308 Psychopathology (3 credits)

Comprehensive overview of psychopathology as a clinical science discipline. The course emphasizes empirical and theoretical models of mental disorders, including their epidemiology, genetic and neurobiological underpinnings, cognitive and affective mechanisms, and environmental and cultural risk factors. Major disorder domains (e.g., mood, anxiety, trauma-related, psychotic, substance use, externalizing, and personality pathology) are examined primarily in adulthood, with developmental and

lifespan considerations integrated throughout. Readings focus on primary research literature, with attention to comorbidity, heterogeneity, and dimensional nosology frameworks (e.g., HiTOP, RDoC) alongside traditional DSM/ICD classification systems. Historical and cultural perspectives on psychopathology are also addressed. Prerequisites: Department consent required.

CPSY 7330 Advanced Neuropsychological Assessment – From Bench to Bedside (3 credits)

Integrates theoretical foundations with practical applications in clinical contexts. Covers fundamental neuroanatomy concepts as well as advanced assessment methodologies to facilitate a nuanced understanding of brain function and dysfunction. Utilizing case studies and contemporary assessment tools, students engage in experiential learning to refine diagnostic skills and develop strategies for translating research insights into clinical practice. Prerequisites: CPSY 7300 and CPSY 7310. Instructor consent required.

CPSY 7340 Advanced Clinical Trials Design and Analysis (3 credits)

In-depth examination of the principles and practices underlying the design, implementation, and analysis of clinical trials in psychological science. Through a combination of theoretical instruction and practical application, students develop expertise in advanced statistical techniques and research methodologies pertinent to clinical trials, including randomized controlled trials, quasi-experimental designs, and longitudinal studies. Emphasizes critical evaluation and ethical considerations in the design of rigorous research protocols, analysis of complex data sets, and interpretation of findings with precision. Prerequisites: HCS 6302, HCS 6312 or ACN 6312 or PSYC 6312, HCS 6313 or ACN 6312 or PSYC 6313, and HCS 6317 or PSYC 6317.

HCS 7355 Seminar in Psychology – Clinical Psychological Science (3 credits)

This course goes in-depth on a number of advanced scientific topics related to research and practice in clinical psychology. Topics covered include critical thinking, philosophy of science, ontological, nosological, and other (often intersecting) issues that make research in psychology and especially psychopathology difficult, clinical versus statistical prediction, and the importance of evidence-supported treatments and the practice of a scientific clinical psychology. Prerequisites: BBSC majors only and department consent required.

CPSY 7320 Advances in Nosology, Clinical Formulation, and Treatment of Psychopathology (3 credits)

Comprehensive examination of evidence-based practices, focusing on the formulation and treatment of psychopathology, in the context of contemporary research. Prerequisites: Completion of CPSY 7310. Only open to students completing the Clinical Psychological Science concentration. Department consent required.

CPSY 7160 Clinical Psychological Science Research Colloquium (1 credit)

Evidence-based conversations at the graduate level focused on original research in clinical psychological science. Course may be repeated for credit. Prerequisites: Departmental consent required.

CPSY 8v87 Research in Psychology (3 credits)

Supervised research experience. Pass/Fail only. May be repeated for credit. Prerequisites: BBSC majors only and instructor consent required.

CPSY 5300 Effective Teaching and Supervision in Psychology (3 credits)

Course provides training in a variety of teaching techniques, reviews relevant pedagogical issues, and assists graduate students in mastering their initial teaching and supervision experiences. Prerequisites: Departmental consent required.

CPSY 7270 Assessment Clinic (2 credits)

Focus is on psychological assessment of an adult seeking services through the UTD Psychology Services and Education Center with corresponding completion of three (3) comprehensive integrated psychological assessment reports. Evaluation must be completed by the enrolled student and the resultant report must include a review of history, results of an interview and at least two psychological tests from one or more of the following categories: Personality measures, intellectual tests, cognitive tests, and/or neuropsychological tests. A report written from an interview that is only history-taking, a clinical interview, and/or only the completion of behavioral rating forms, where no additional tests are administered, is not an integrated report. Corequisite or Prerequisite: Completion of CPSY 7310. Only open to students completing the Clinical Psychological Science concentration. Pass/Fail only. May be repeated for credit.

CPSY 7180 Intervention Clinic (1 credit)

Combines didactics and hands-on clinical work in the instructor's area of clinical expertise. Includes review of empirical literature, delivery of interventions, and evaluation of intervention effectiveness. Select readings are included in the course and class discussion is a central part of the course. Written products are also a part of the course, either in the form of a presentation or publication of findings from the clinic. Includes weekly case conference and individual supervision with a caseload of two cases at a time (no more than 3 patient contact hours per week). Prerequisites: Only open to students completing the Clinical Psychological Science concentration. Department consent required. Pass/Fail only. May be repeated for credit.

CPSY 7181 Clinical Psychological Science Grand Rounds (1 credit)

Fosters professional development with a focus on identifying solutions to common challenges in the clinical environment, identify strategies to elicit effective supervision, disseminate information, and change behavior of clinicians in training to improve patient outcomes. Includes discussion with faculty affiliated with the concentration in Clinical Psychological Science regarding their particular program of research. Additional rotating topics are announced at the beginning of the semester, but examples include planning a research program, choosing a dissertation committee, identifying career options, presenting work at conferences and in journals, preparing grant proposals, preparing for job interviews, juggling professional and personal life, and recognizing obstacles in career development. Corequisites: CPSY 7170 or CPSY 7180 or CPSY 7190. Prerequisites: Only open to students completing the Clinical Psychological Science concentration. Department consent required. May be repeated for credit.

CPSY 7190 External Practicum (1 credit)

Supervised experience in the practice of clinical psychology in a professional setting that has been approved for that purpose by the faculty of the clinical psychological science concentration. Prepares students for highly competitive internships. Prerequisites: Open only to students enrolled in the Clinical Psychological Science concentration who have completed CPSY 7170 and CPSY 7180. Pass/Fail only. May be repeated for credit.

CPSY 8v97 Dissertation (3 credits)

Supervised research experience. Pass/Fail only. May be repeated for credit. Prerequisites: Clinical Psychological Science concentration and instructor consent required.

CPSY 8100 Internship (1 credit)

Full-time (12 month) field placement in clinical psychology at an approved internship site. Internship is consistent with objectives of the student's program and current professional practices in a variety of

agencies, hospitals, medical schools, or other internship sites that are members of the Association of Psychology Postdoctoral and Internship Centers (APPIC). Prerequisites: Only open to students completing the Clinical Psychological Science concentration. Attainment of doctoral candidacy. Pass/Fail only. May be repeated for credit.

Note: If a student decides that participation in the one-year full-time clinical internship is not necessary for their career trajectory, they may change from the Clinical Psychological Science concentration to one of the other Psychology PhD concentrations (Developmental Psychology, Cognitive Psychology, Social/Personality Psychology, or Neuroscience), if they have met the requirements of that concentration.

Electives.

All students in the Clinical Psychological Science concentration will take PSYC 7308 (Psychopathology) as one of their electives. This course serves as the foundational preparation for the concentration's core sequence (CPSY 7300 and CPSY 7310) and is therefore a pre-requisite for enrollment in CPSY 7300. Although it appears as an elective on the degree plan, it functions as an essential part of the training sequence, ensuring that all students enter the concentration with a shared grounding in psychopathology scholarship. Beyond PSYC 7308, you should work with your training committee to determine which additional electives are most appropriate for your needs and goals.

Research Requirements

We believe high research activity under the close faculty supervision is required for you to develop as a first-rate clinical scientist. Research is a cornerstone of the curriculum, and you are expected to work closely with faculty mentors to develop research proposals, conduct studies, and disseminate their findings through presentations and publications across the entirety of your doctoral training. Depending upon your funding source (TA/RA), you are expected to dedicate 10-20 hours per week to research activities. Based on degree plan flexibility needs, your personalized training committee may advise you to enroll/not enroll in elective research credit hours. *Credit hours are not used to determine competency attainment.* All research benchmarks must be met, regardless of elective credit hour enrollment.

Regular weekly or biweekly meetings with your primary research mentor will facilitate feedback on research progress, allowing you to refine your work and prepare for presentations and publications. You will be required to present your research at departmental colloquia and other academic events to receive feedback from a wide range of sources. You are also strongly encouraged to present your research at national and international conferences to enhance your visibility in the field and provide networking opportunities with leading figures in psychology. Finally, the concentration places significant emphasis on the importance of publishing research findings and you are expected to earn lead authorship on manuscripts submitted for peer review. Across your training years, we suggest you target accumulation of 8 peer reviewed publications, with at least 2 first authored.

All research requirements specified in the Psychology PhD Student Guide must be met. Regardless of whether you have earned a Master's degree from another institution, you will embark on a Master's-level research project. Your research project is your chance to delve deep into a topic that excites you and contributes to the field. Across BBS, faculty routinely engage with prominent clinical scientists, inviting them to present their cutting-edge research to the program community. Organized by BBS centers and programs, [external speakers](#) appear weekly, if not more, on campus across the year. These speakers serve as valuable models for how to present findings and engage with the academic community, while also providing ample opportunity to broaden the scope of your knowledge. While attendance is expected, you may elect whether or not to enroll in credit hours (on the advice of your

personalized training committee). The Department also provides seminars, talks, and workshops on topics such as grant writing, research ethics, and career development opportunities to further support your professional growth.

Some internship sites may prioritize applicants with Master's degrees. To overcome this potential barrier, non-Master's students in our concentration are strongly advised to consult the Psychology PhD Student Guide (see the "Master's Degree en Route" section) for more information and file an application for awarding of en route Master's degree. To be awarded the Master's degree, your mentor and two additional faculty members must sign off on your thesis.

By February 1st of your first year, you will submit a concise proposal outlining your plan for completion of your research project. Here's how you take the lead and get a strong start on your program of research:

- Work closely with your primary research mentor: They're your guide and sounding board.
- Craft a compelling proposal: Clearly define your research question, methodology, and expected outcomes. Remember, no cover page, appendices, or attachments – just your concise vision.
- Hit the deadline: February 15th is the target. Good academic standing hinges on submitting your proposal to the Clinical Science Steering Committee (CSSC) on time. The CSSC will review your proposal to assess the feasibility of your project and provide you with feedback.

Formatting tips:

- Center your proposal title at the top of page 1. Center your byline immediately below the title.
- Keep it concise: 2-5 pages to outline the rationale and methodology, 11-point font, 0.5-inch margins.
- References welcome: Dedicate a separate page for them.

As described fully in the Psychology PhD Student Guide, you will dive deep into your research, gathering data, analyzing results, and crafting your paper/thesis while also preparing a grant proposal or integrated literature review across your second year. In addition to task-specific mentoring during courses and practicum during those same years, three task-specific committees are formed prior to key research benchmarks (summarized in Figure 1). These committees serve a mentoring role throughout the process and hold responsibility for evaluation upon task completion. Members of your personalized training committee may or may not additionally serve on these task-specific committees. Key task-specific committees, their composition, purpose, and membership are summarized below. In addition to these committees, the CSSC also holds some task specific roles (e.g., reviewing your research paper). However, in contrast to the personalized training committee, the CSSC is not formed for any given student. While the CSSC can be an important source of feedback, their primary role is to ensure standards *across* students. It is the role of the personalized training committee to address CSSC feedback during their mentorship of the student.

Pre-dissertation Portfolio Evaluation Committee Composed of the research mentor and two UT Dallas faculty (one from the Psychology Department), this committee evaluates the pre-dissertation portfolio.	Supervising Dissertation Committee Four faculty members from the Psychology Department, nominated by the Director of Clinical Science Training and approved by the Dean of Graduate Education, oversee the dissertation.	Dissertation Oral Examination Committee The Dean of Graduate Education appoints a neutral Chair to work with the Supervising Dissertation Committee for the oral dissertation defense.
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Figure 1. Task-specific committees associated with key research benchmarks.

Remember, your primary research mentor's deadlines and progress checks with your personalized training committee are meant to help you keep on track. *Across the arc of your graduate training, a slight majority of your time should be at the doctoral candidacy stage* (i.e., not at the pre-candidacy stage associated with master's level work). Completion of the pre-doctoral portfolio review signals your timely entry to training at the doctoral level.

The clinical psychological science concentration, specifically, requires that your dissertation take a bundled / integrated approach to foster development of a strong programmatic line of research, mastery of advanced analytical and theoretical skills, and demonstration of your potential for research productivity that is on par with expectations for leaders in the field.

Your dissertation will consist of three *empirical* manuscripts, each a distinct and original research study conducted during your doctoral training at UT Dallas. The three manuscripts must be conceptually related to each other, ideally demonstrating a programmatic line of research. These manuscripts will be intricately interwoven in the final dissertation through two dedicated sections:

- Integrated Introduction: Provides conceptual cohesion by introducing your research program, including literature review, and providing brief manuscript summaries.
- Comprehensive Discussion: Synthesizes and connects findings across all studies, explores broader implications, and suggests future directions.

The proposal document consists of the introduction, including an integrative review of the literature, as well as a brief overview of each of the three (3) manuscripts that will be included in the dissertation. Manuscripts can be submitted for peer review before the oral defense but only with the prior review and approval of all dissertation committee members.

It is important to keep the following in mind after your proposal is accepted and you begin working on your dissertation document.

- Non-Significant Results: Even if a study doesn't yield significant results, the analyses must still be in "ready to be submitted" manuscript format.
- Authorship: You must be the first author (i.e., the major contributor) on all manuscripts.
- Manuscript Types: One paper can be a meta-analysis or systematic review. Book chapters, conference presentations, encyclopedia entries, book reviews, etc., are not included.
- Pre-dissertation Exclusion: Only one of the papers may be something submitted for the pre-dissertation portfolio.

In some cases, students may write a more traditional dissertation instead of a bundled/integrated dissertation consisting of three papers. This decision must be made prospectively (i.e., before writing the proposal document) via shared decision making that involves the dissertation committee and DCST. *The traditional format will only be considered when the research question inherently necessitates a protracted and large scope of data collection.* Insufficient research competency development or progress, change in career aspirations (pursuit of a practice-only career), and/or life events are not acceptable reasons for a change in format. *The decision must be based on the science.* The dissertation committee holds full responsibility for evaluating the research design necessity. The DCST is charged only with ensuring that the difficulty level of the project remains commensurate with those associated with the bundled/integrated approach. Consult the Psychology PhD Student Guide for additional information concerning Dissertation milestones and procedures.

Clinical Requirements

Attainment of all specific clinical benchmarks is mandatory for demonstration of sufficient competency and readiness to advance in training, but enrollment in course credit is elective to allow for greater flexibility in degree plans. As with research, the personal training committee may advise students on elective credit hours based on their career aspirations and training needs.

Psychological Services and Education Center

You must complete a background check and a training provided by the State of Texas Health and Human Services Commission on [Recognizing, Reporting, and Preventing Child Abuse](#) prior to beginning training in the Psychological Services and Education Center. Additional trainings specified by the Psychological Services and Education Center must also be completed as required.

All students in the Clinical Psychological Science concentration are required to use the Process-Based Psychotherapy training platform (powered by COMETS) as an integral component of their clinical training. The platform delivers a structured, semester-sequenced curriculum comprising educational modules, knowledge checks with mastery gating, AI-powered virtual patient practice coaches, worked examples with faded scaffolding, transfer challenges, and spaced review. Students must complete assigned platform activities and demonstrate mastery of knowledge check domains as part of their clinical training requirements. Competency estimates generated by the platform are formative only and are clearly distinguished from supervisor-administered summative evaluations. The platform supplements—but does not replace—supervised clinical experiences with real patients in the PSEC and at externship sites.

All Psychological Clinical Science students in-residence are required to purchase, and renew annually, 1 million/3 million Student Liability Insurance for clinical work. You can apply through [The Trust](#) or [American Professional Agency](#) before starting in the Psychological Services and Education Center. Remember to verify "tail" coverage for your cases if you changed insurance companies later. The concentration does not reimburse the cost of the insurance coverage for clinical work conducted in the Psychology Clinic. Copies of your coverage verification must be on file in the PSEC Director's office.

During PSEC orientation, students will be onboarded to the Process-Based Psychotherapy training platform, including account creation, completion of the welcome tour, and acknowledgment of the Academic Integrity and Data Use Notice. Students may also be invited to provide voluntary consent for archival research use of de-identified platform data. Platform access credentials and technical support information will be provided during orientation. Contact the PSEC Director for further instructions.

With respect to required clinical benchmarks, you are required to complete three comprehensive integrated psychological assessment reports during internal practicum training within the PSEC. You may begin working towards this benchmark after completion of CPSY 7310 (Clinical Psychological Science II). Each report must include a review of history, an interview, and at least two psychological tests from one or more of the following categories: Personality, intellectual, cognitive, or neuropsychological tests. A report written from an interview that is only history-taking, a clinical interview, and/or only the completion of behavioral rating forms, where no additional tests are administered, is not an integrated report. While completing 3 assessments in the Psychology Clinic fulfills the minimum requirement for the Assessment Clinic, many internship programs now prefer applicants with at least 100 assessment hours. To bolster your competitiveness, you can:

- Take on additional cases in the training clinic (dependent on supervisor availability).
- Seek an off-site externship with a clinical assessment component.

With respect to competency in psychotherapy, your readiness of external practicum training will be determined based on careful review of your functional clinical competencies. The entire PSEC Manual (Appendix L) outlines clinical expectations within the PSEC internal practicum; the manual section entitled, “Supervision and Feedback Mechanisms” and the corresponding Addendum B within that manual are particularly notable.

Within the PSEC you will typically have a caseload of 2 patients as the primary clinician at any given time though you are likely to be working secondarily on other cases. You must review all digital recordings of your patient encounters and tag those recordings with reflective comments prior to supervision. This requirement is in place to enhance your critical thinking and self-reflection.

Weekly individual, group, and grand rounds sessions are designed to align with the demands of clinical placements and competency development. Your clinical supervisors are required to provide you with formative feedback based on direct observation of your work. As you advance in your competencies, you can expect supervision to adapt to your evolving needs (e.g., more intensive initial support will abate to foster greater independence as you build confidence). Supervisor scaffolding will support your continuous improvement and clinical skill refinement. We hope it will also cultivate a lifelong dedication to engaging in your professional growth across your career.

The intensive training within the PSEC will not result in a sufficient number of intervention hours to be competitive for the internship matching process. A substantial portion of your intervention training will occur while on externship.

Externship

Off-site externship training will diversify your clinical experience and expose you to new populations, issues, and settings. To participate, you should be in good academic standing and demonstrate research progress (e.g., publications, presentations). Externships usually occur in Years 4 or 5, though exceptional cases may be approved in Year 3.

Placements are **limited to 10 hours or less per week** and require prior approval from your personalized training committee, the PSEC Director, and DCST. Contact the PSEC Director for necessary forms and a Memorandum of Understanding template.

Externships may be for 3 months (summer term only), 9 months (fall and spring semesters), or 12 months (summer term thru spring semester). You should review externship site information in early

January and notify the PSEC Director no later than February 15th if you are interested in pursuing an externship.

Submit your application materials (listed below) to the PSEC Director no later than April 15th. Be aware that some external sites may have earlier deadlines. You may not submit to those sites without first submitting to the PSEC Director.

Cover Letter - Introduction and opportunity to describe your interest in, and fit with, a particular site.

CV - be sure to include:

- Identifying information
- Education
- Training
- Clinical experiences; practicum; psychotherapy experiences
- Supervision experience
- Research experience
- Publications, grants, professional presentations
- Teaching experience
- University and professional service
- Related work experience
- Volunteer activities
- Awards/honors
- Professional members, leadership positions held

Interview - Optional & site-dependent

Completed Memorandum of Understanding – approval signatures from your training committee, and externship site supervisor should be solicited before you sign it yourself.

To prioritize quality supervision, your weekly direct contact hours are capped at no more than 8 per week. This cap is intended to discourage you from becoming excessively focused on accumulating clinical hours at the expense of quality supervision. Further, because high quality supervision plays a critical role in ensuring that you benefit from your practicum experiences by developing your skills and competencies, you are required to maintain a direct contact to supervision hours ratio of no more than 4:1 during clinical practicum training. In other words, if you accrue 8 direct contact hours should also accrue a minimum of 2 hours of supervision in the same week.

According to a PCSAS Internship Report (March 2023), students in PCSAS-accredited programs are competitive for high quality clinical internships if they accumulate between 688.5 (bottom quartile threshold) and 802 (median) direct-contact hours across internal and external practicum training experiences. To determine if students can accumulate sufficient clinical hours within four years while capping weekly direct contact hours, we analyzed a typical academic calendar. To be conservative, we excluded summer sessions and between semester breaks. At UT Dallas, an academic year generally consists of about 30 instructional weeks, across both fall and spring semesters. Over the course of four years, this totals approximately 120 weeks of practicum engagement (30 weeks per year multiplied by 4 years) prior to applying for internship at the start of training year five. Based on these figures, a trainee could amass up to 960 hours of direct contact hours with a minimum of 240 hours of supervision.

These estimates are based on a conservative, fixed training schedule. However, students can flexibly accrue hours throughout the year, including summer months.¹ As described in the Training Overview section of the PSEC Manual (Appendix L), we expect students to begin accruing hours early in training, using a heavily supervised titration of responsibility based on individual student's competencies. Thus, it is anticipated that students will be able to comfortably meet the range of hours appropriate to be competitive at top internship sites. The flexibility of the concentration allows for a more accommodating and supportive environment for students to fulfill their clinical training requirements. By limiting total hours and ensuring close supervision, we hope to encourage you to prioritize a balanced experience. Our hope is to also reduce risk of burnout and promote deeper understanding and skill development that leads you to having a stronger application and a successful internship match that fits your goals and needs.

While practicum sites may use a combination of one-on-one and group supervision models, personalized feedback and support is essential. We expect clinical training and supervision to be provided by doctoral-level psychologists and, when appropriate, experienced clinicians from various healthcare disciplines. In interdisciplinary settings, the licensed psychologist is expected to serve as the primary mentor, guiding students through their external training experiences. Supervision sessions will be expected to occur at least weekly, allowing for ongoing discussions of clinical cases and the integration of research findings into practice. Students will be expected to work closely with their expert supervisors in a safe environment to discuss clinical experiences, challenges, and successes, promoting professional growth and the development of essential clinical skills.

We expect weekly individual supervision sessions will enable you to review cases, discuss treatment plans, and reflect on therapeutic techniques. Across internal and external placements, group supervision sessions are encouraged to foster community and enrich the learning experience by encouraging students to share experiences and discuss diverse perspectives. Further, regardless of setting, students are expected to receive training in culturally sensitive, evidence-based interventions, preparing them to apply their research knowledge effectively in clinical contexts. This focus on culturally competent care is essential for developing practitioners who can meet the needs of patients from varied backgrounds, ultimately enhancing the quality of care provided.

Internship

You will complete a year-long, full-time accredited internship (or equivalent) at an APPIC-member site in Year 6. Applications are submitted at the start of Year 5. To apply for internship, the DCST is required to vet all aspects of your application and attest to your good standing across coursework, research, practicum training, and professionalism before you enter the nationwide internship match process. A failure to be in good standing in *any* of these areas will result in denial of your request to apply for internship and may require a remediation plan before approval is granted.

Broadly applying to internship sites across the country is crucial, as Dallas-Fort Worth internships are limited and highly competitive. Remember, only APPIC-member internships are accepted, except in rare situations. When applying for internship, you will be required to detail each of your clinical hours. Accurately tracking your clinical training experiences throughout the concentration is crucial, even during your first year.

¹ Regardless of their enrollment, trainees are expected to be engaged full-time in their training throughout the summer session (see page 18 as well as the section entitled, "Summer and Intersession Travel" within the Psychology PhD Student Guide; Appendix C).

We recommend using the free Public Access Tracking of Clinical Hours (PATCH) tool provided to you by the concentration. PATCH simplifies internship application by 1) providing a personal Qualtrics survey for logging hours, 2) exporting data to a transferable .csv file for AAPI applications, and 3) summarizing your hours in clear tables and charts for optimizing your clinical training path. If you prefer you may instead use Time2Track to log your clinical experiences at your own expense. To prepare well, consider aiming for 100 assessment hours, 500 intervention hours, and 5-7 integrated assessment reports, as suggested by informal surveys of PCSAS member programs. You will be required to submit monthly reports to the PSEC Director throughout your training, but it is your responsibility to accurately track accrual of hours.

Patient-Trainee Clinician Value Conflicts

Whether you are in practicum at the PSEC or training at an externship site, we encourage students to cultivate the knowledge, skills, and self-awareness needed to work effectively with a diverse public while acknowledging the influence of personal beliefs and values in professional interactions. When such influences create tensions, we believe in open communication and constructive interventions. Our faculty and supervisors are dedicated to partnering with students to explore these challenges, providing guidance and support to resolve potential biases and develop the necessary competencies to provide ethical and responsible care to all patients. While recognizing that navigating these complexities may require additional time and resources for some students, we remain committed to supporting their growth and ensuring that all graduates can fulfill their professional obligations towards any patient assigned, regardless of any personal considerations. Professional competencies are critical for safeguarding public well-being and ethical practice, and therefore, refusing to engage with specific populations or develop essential skills is not an acceptable option within our concentration.

Our commitment to your ethical development extends beyond formal practicums. As noted in the Psychology PhD Student Guide, the university does not permit students supported as TAs or RAs be employed elsewhere. Further, to ensure adherence to Texas state law, **any** external clinical work, including assessments, interventions, consultations, group facilitation, and/or counseling (**paid or volunteer**), requires prior approval from the concentration. Documentation of approval by the DCST must be submitted to your student file and detail the terms of the approval (e.g., volunteer-only). Please be aware that such experiences will not be considered sanctioned or supervised by the concentration and cannot be included among hours reported on the internship application.

Patient Care and Vacation Time

The Psychology PhD Student Guide specifies expectations for RA and TA supported students during semester breaks. Because you are in the clinical concentration, it is important to additionally consider the schedule of the PSEC. The PSEC observes a two-week winter break. During periods when the PSEC is open, one week of personal leave is permitted with scheduling considerations for uninterrupted patient support. You must discuss your plan and secure approval from your clinical supervisor at least one month in advance.

Prior to starting your externship, meet with your site supervisor(s) to clarify their leave and vacation policy. Keep in mind the potential discrepancies between UT Dallas and your externship site's schedules. Negotiate and document any agreed-upon time-off arrangements – just like in any professional setting, early and clear communication regarding leave requests is essential.

Completion of Practicum Supervisor Evaluations

To monitor training quality, we collect supervisor evaluations within the PSEC and among externships thrice yearly: December (Fall semester end), May (Spring semester end), and August (Summer semester end). Supervisor evaluations forms are sent electronically by the PSEC Director.

Professional Development Requirements

Instructor of Record

Success in the highly competitive academician job market typically necessitates submission of a teaching portfolio that includes a well-honed teaching philosophy and evidence of teaching effectiveness. Your personalized training committee can work with the Department Chair to identify undergraduate courses that are well suited to your developing areas of expertise. To equip you for success, you must complete the CPSY 5300 Effective Teaching and Supervision in Psychology course (3 credits). This course can be taken before or during your first teaching experience as the instructor of record. Your personalized training committee will attend at least one of your classes and provide formative feedback to supplement the summative student feedback gathered by the university. It is your responsibility to aggregate and retain formative feedback from experts, summative feedback from students, and/or records of pre/post knowledge change in a manner compliant with FERPA for use in your teaching portfolio.

Developing Expertise via Professional Development

Your academic journey extends far beyond the coursework you encounter in your classes. To fully equip yourself for professional success and intellectual growth, we encourage you to actively engage in the rich training opportunities offered by the Psychology Department, BBS Centers, and the wider UT Dallas academic community. These local opportunities for socializing you into the field also develop your expertise offer targeted knowledge, refine your skills, and expose you to diverse perspectives within your area of interest.

Continuous learning and professional development are crucial for thriving in your chosen field. Make the most of your time by actively seeking out valuable training opportunities at the national level. Don't hesitate to consult your primary research mentor and personalized training committee for guidance on navigating the available options of high-quality conferences and workshops to tailor your training journey to your specific goals and interests. National and international events provide invaluable networking opportunities, connect you with leading figures in your field, and keep you abreast of the latest advancements.

Digital Discourse

The internet offers an expansive platform for sharing ideas and connecting with others. However, within the sphere of clinical psychology, navigating these avenues requires cautious consideration. Students in the concentration who engage in social media are expected to do so with utmost professionalism and respect, both towards others and the sensitive nature of the field. While refraining from sharing patient information seems intuitive, responsible online conduct extends beyond this basic principle. Simply mentioning you're discussing a patient, even with complete anonymity, can impact public perception of mental health services. The concentration and the profession, therefore, uphold a high standard for online conduct, regardless of platform or audience.

Regardless of the chosen platform, you are accountable for the content you post or promote. This includes potential repercussions by the concentration, Psychology Department, BBS, or UT Dallas. Unprofessional online behavior, ranging from disruptive posts to academic misconduct, can result in consequences as severe as dismissal from the concentration or University. Engaging in unwise or inappropriate social media use can jeopardize one's educational and professional opportunities indefinitely. To avoid such scenarios, consider these points:

- Limited access does not guarantee privacy: Online content can reach far beyond intended audiences. Posts may offend others, including colleagues, students, or even patients who stumble upon them.
- Public Scrutiny: Training sites, potential employers, and patients increasingly utilize social media for evaluation. Maintain a professional online presence with this in mind.
- The Internet Never Forgets: Posted content becomes virtually impossible to retract completely. Once uploaded, it can be copied, saved, and shared indefinitely.
- Accountability for Hosted Content: Be mindful not only of what you post but also comments left by others on your pages. The content you host reflects on you.
- Transparency and Boundaries: If posting about the concentration, Department, or UT Dallas, explicitly clarify that you do not represent those entities and your views are personal.
- Ethical Alignment: Ensure your online conduct aligns with ethical and professional guidelines and relevant state authorities.

Remember, the virtual world is an extension of our professional realm. Navigate it with the same integrity and responsibility expected in clinical practice.

Optional Pathways

Documentations for Professional Practice

Licensure eligibility varies by state, though there is a growing list of states that now explicitly recognize PCSAS accreditation for licensing purposes. The concentration is committed to providing a rigorous and impactful clinical psychological science education, and we are dedicated to supporting graduates in securing diverse career opportunities. However, professional licensure is not a primary outcome goal for our concentration. Aligned with our vision for the future of doctoral level clinical psychology as a field, we prioritize research and academic training to prepare students for leadership roles. Therefore, if your primary focus is on traditional clinical practice, we encourage you to carefully consider all concentration options and ensure your chosen path aligns with your professional goals.

If you think you might want to pursue licensure in the future, you can complete optional activities that may aide you in demonstrating equivalency to an implementing regulation regarding curriculum (known as "IR C-7 D") that is required of all doctoral programs that are accredited by the APA Commission on Accreditation. These are not required of students as this concentration is not APA-accredited, but completion of these activities may have implications for licensure in *some* jurisdictions.

- *Category 1 DSK*: Take and pass an undergraduate level exam on the History of Psychology. If you have already taken an undergraduate course in the history of psychology you have already met this category and no exam is necessary. If you are banking your credentials, be sure you upload a transcript to show the course completion.
- *Category 2 DSK*: Take and pass a graduate level knowledge exam that covers the following basic content areas in scientific psychology (Affective, Biological, Cognitive, Developmental, and Social Aspects of Behavior).
- *Category 3 DSK*: This necessitates advanced Integrative Knowledge in Scientific Psychology [entails integration of multiple basic discipline-specific content areas identified in Category 2

(i.e., integration of at least two of: affective, biological, cognitive, social, or developmental aspects of behavior). Advanced integrative knowledge in Category 2 areas can be acquired in either of two ways: 1) an evaluated educational experience* that integrates at least two Category 2 content areas that have been previously covered through other methods; or 2) an evaluated educational experience* that provides basic coverage in two or more areas and integration across those areas.]

- *Category 4 DSK*: To ensure your department core courses are interpreted as having met the requirements of this category (Research Methods, Quantitative Methods, Psychometrics), take and pass a graduate level knowledge exam.

You are encouraged to [bank your credentials](#) with the Association of State and Provincial Licensing Boards (ASPPB) if you are interested in possibly pursuing licensure in the future. The ASPPB credentials bank provides primary source verification of all necessary information about your training now (while your supervisors and faculty are alive) and can relay those verifications across the long arc of your career. All verifications and documents are maintained on a secure and continuously updated server, so you will have what you need for licensure applications, insurance verifications, and/or other practice-related needs across your career.

Streamlined Entrance to the Academy or Research Industry

If you decide that participation in the one-year full-time clinical internship is not necessary for your career trajectory, you may change from the clinical psychological science concentration to one of the other Psychology PhD concentrations (Developmental Psychology, Cognitive Psychology, Social/Personality Psychology, or Neuroscience). To graduate, you must meet the requirements of the new concentration.

You may be moved out of the Psychological Clinical Science concentration if you do not demonstrate mastery of the knowledge, skills, and attitudes marking clinical competency. After admission, your focused, high-quality training experiences are designed to promote rapid ascent in competencies. You will be regularly assessed via experts and provided with both formative and summative feedback on your competency development. In keeping with the standards of this concentration as producing leaders in the field, “competent” attainment of clinical knowledge, skills, and attitudes is not considered sufficient for retention in the Psychological Clinical Science concentration. Throughout your training we will work with you to evaluate your likely trajectory for obtaining board certification, which is a distinction attained by approximately 5% of psychologist following rigorous peer review of practice competencies, typically with an oral defense. If you are not demonstrating the rapid ascent necessary for fully benefiting from the focused, intensive training of the UT Dallas concentration and showing probable attainment of board certification level competency, you may be moved out of the concentration in Psychological Clinical Science. Whether you self-select out or are exited from the Psychological Clinical Science concentration will not be disclosed by the concentration.

If you move out of the Psychological Clinical Science concentration you will not need to reapply for admission to the Psychology PhD program. You will most likely retain your primary research mentor, but this should be confirmed prior to deciding to exit the concentration. You will not be required to retain a personalized training committee if you leave the concentration. Even with retention of the primary research mentor, it is important that you be mindful that your professional identity will be tied to the concentration you select. You will not be able to refer to yourself as a “clinical psychologist” and you will not be eligible for licensure without re-specializing (UT Dallas does not offer a re-specialization pathway).

Insufficient Progress

Insufficient progress is not to be confused with academic misconduct. Academic misconduct will not be tolerated and will be addressed as required by the UT Dallas Community Standards and Conduct. Please note that the Clinical Psychological Science concentration may implement additional sanctions beyond those outlined in the University's Community Standards and Conduct if the misconduct violates ethical standards for our discipline.

Students exhibiting insufficient progress in the Clinical Psychological Science concentration will be required to participate in a tailored remediation plan, developed by their training committee. Regular consultation with the concentration faculty and advisors is essential during this process. Indicators of insufficient progress include, but are not limited to, failure to successfully complete research milestones on time or earning a grade of C or below in any required course.

The Clinical Psychological Science Concentration is committed to ensuring its graduates are competent and ethical as practicing clinical psychologists. Therefore, we go beyond traditional assessments like coursework and exams to evaluate your fitness for practice in various areas:

- *Interpersonal and Professional Competence*: How you interact with clients, peers, faculty, and others with respect and professionalism.
- *Self-Awareness and Reflection*: Your ability to understand your biases and how they impact your work, and your willingness to learn and grow.
- *Openness to Supervision*: Your receptiveness to feedback and guidance from supervisors and faculty.
- *Problem-Solving and Remediation*: Your ability to address personal or professional challenges constructively, through therapy, remediation plans, or other means.

For a more comprehensive listing of specific clinical practice competencies, please see Table 1. Evaluation happens within the context of your formal training, including:

- *Coursework*: Your participation and engagement in class discussions and projects.
- *Practica*: Your performance with clients and interactions with supervisors.
- *Supervision*: Your openness to feedback and willingness to engage in self-reflection.

Your personal life outside of the academic and professional setting is not routinely considered as part of the evaluative process. However, if your personal conduct demonstrably affects your performance, raises ethical concerns, poses a public safety risk, or damages the concentration's reputation, it may be reviewed through appropriate channels.

We believe in clear communication and supportive evaluation. You can expect:

- *Clear information*: You will receive details about the evaluation processes, standards, and their purpose (e.g., to enhance your development and identify areas for improvement).
- *Multiple perspectives*: Your evaluation will consider information from multiple sources, such as different supervisors and settings.

Remediation

Our concentration is dedicated to graduating competent and ethical professionals. This evaluation process allows us to support your development and ensure you are well-equipped to serve clients and the public with integrity and skill. If appropriate, your personalized training committee will work with you to develop a remediation plan to address identified areas for improvement. Remediation plans are designed to support student success and ensure progress towards concentration completion. Potential

strategies may include but are not limited to: Enrolling in supplemental coursework to strengthen research skills or address knowledge gaps, delaying advancement in clinical placements until specific competency thresholds are met, and/or retaking courses where necessary.

In some cases, it may be determined that remediation is not appropriate. Here are some potential scenarios where remediation might not be appropriate:

- *Problems of professional competency*: If a student struggles with professionalism challenges that significantly interfere with their ability to practice safely and ethically, even with intensive support, remediation may not be feasible. The safety of patients and the student themselves takes paramount importance.
- *Insufficient progress despite multiple attempts*: When a student exhibits persistent shortcomings in core competencies like interpersonal skills, professional conduct, or clinical judgment, and fails to respond to multiple remediation plans and interventions, attempting further remediation might not yield acceptable results.
- *Insufficient development of research competencies*: When a student is consistently underperforming in research, it becomes increasingly less likely that the student will contribute original knowledge to the field, as required, within the amount of time allowed for degree attainment.
- *Ethical violations or harmful behavior*: If a student engages in serious ethical violations, exhibits demonstrably harmful behavior towards clients or colleagues, or poses a safety risk, dismissal from the concentration may be necessary to protect the public and uphold professional standards.
- *Lack of self-awareness and motivation*: For remediation to be successful, the student needs to be self-aware of their deficits and genuinely motivated to improve. If a student displays a persistent lack of self-insight or unwillingness to engage in personal and professional development, further attempts might be futile.
- *Pattern of non-compliance and resistance*: If a student repeatedly disregards concentration requirements, resists supervision, or demonstrates a pattern of non-compliance despite support and guidance, continuing remediation may not be productive.

It's crucial to remember that these are just some potential scenarios. The personalized training committee will consider all relevant factors and evidence and is charged with deciding with utmost care and compassion, ensuring fairness to the student while prioritizing the safety and ethical obligations of the concentration and the profession. Ultimately, not attempting remediation is a serious decision with substantial consequences for the student and the concentration. It is only undertaken after thorough consideration and with clear evidence that significant concerns remain unaddressed despite extensive efforts to support the student's development. If remediation is not appropriate or unsuccessful, you may be dismissed from the concentration.

Student Grievances

The concentration's grievance process exists to offer a non-adversarial platform for resolution of concerns. Its scope encompasses issues like discrepancies in research credit allocation, student-faculty relationship dynamics, and navigating the intricacies of personalized training committee decisions. In essence, it provides a forum for concerns that are not of a criminal nature, and not already encompassed by the University's grade change policies, anti-discrimination policies, and/or sexual harassment policies.

Before embarking on the formal path, informal exploration is expected. A candid dialogue with the involved party might ease the tension. Your personalized training committee can offer advice and support, as well as grounding in the reasonableness of your concerns. If concerns remain after exhausting informal mechanisms, you may file a formal grievance in writing with the DCST.

The DCST will constitute a committee, composed of three faculty, to review your grievance. One faculty member will be chosen by you to represent your perspective. Another faculty member, selected by the implicated party, will be chosen to provide counterpoint. If neither the student nor the implicated party identifies a faculty member to serve on the grievance committee within seven days of the DCST request, the Department Chair will appoint a faculty member to fill the vacant seat. Finally, a neutral third faculty member, chosen jointly by the other two faculty, ensures balanced evaluation. The resulting 3-member grievance committee will seek information and deliberate with sensitivity to safeguarding the dignity of all participants. Within 45 days the committee will submit their recommendation(s) to the DCST for resolution of the grievance. The DCST may escalate recommendations to higher levels of authority if the recommended actions are not within the scope of concentration authority.

BBS Research Centers

[Callier Center for Communication Disorders](#): Focuses on the causes, prevention, assessment and treatment of communication disorders. Callier's two locations, in downtown Dallas and on the UT Dallas Richardson campus, house sophisticated clinical and neuroscience facilities for research. The center provides excellent opportunities to study a broad range of communication disorders, including outpatient clinics for people with speech, language and hearing disorders, as well as infant and preschool programs serving hearing, hearing-impaired and language-impaired children.

[Center for Advanced Pain Studies](#): Researches the fundamental mechanisms underlying chronic pain and migraine, seeking novel therapeutics via cross-disciplinary investigations. Areas of expertise include neuroscience, pharmacology, molecular biology, bioengineering and chemistry. The center advances discoveries, translates findings, and fosters the next generation of multidisciplinary scientists through research leadership, entrepreneurship and innovation with the goal of creating new, non-opioid pain therapies.

[Center for BrainHealth](#): Located in downtown Dallas, BrainHealth is focused on understanding, protecting and healing the human brain. The center works through research programs to understand the brain's ability to restore or protect healthy brain function, protect the brain from unnecessary mental decline, and heal the brain through treatments that regenerate brain function. A key part of the center is the Brain Performance Institute, which provides scientifically validated programs and assessments that enhance individual brain performance and health.

[Center for Children and Families](#): Focuses on all aspects of child development from birth through adolescence. The center promotes optimal child development by enhancing research, practice and outreach. CCF offers an array of clinical and community outreach activities organized around three initiatives: parenting healthy families, strengthening interpersonal relationships and enhancing learning and thinking.

[Center for Vital Longevity](#): Located in downtown Dallas, CVL is focused on understanding and expanding the capacity of the aging mind. Center researchers use cutting-edge brain imaging technologies and advances in cognitive science to understand how the brain changes from young to old adulthood, the consequences of neural aging for everyday function and what interventions show promise for slowing cognitive aging.

[Texas Biomedical Device Center](#): Consists of scientists, engineers, medical doctors, regulatory specialists and clinicians committed to the development of affordable and innovative therapies to improve the quality of life for individuals suffering from neurological disorders. Center researchers are developing new medical devices to detect and treat these disorders by harnessing the brain's natural ability to adapt and reorganize.

[Clinical and Translational Research Center](#): Aims to accelerate the translation of UT Dallas innovation and discoveries into effective health solutions. Helps investigators bridge the gap between cutting-edge research and clinical discovery, accelerating the journey from scientific discovery to improved health solutions by offering comprehensive support, internal and external collaboration, and training.

UT Dallas Resources

[Office of Information Technology](#): Empowers doctoral students with the resources and expertise to manage their research data, access advanced computing power, and navigate the university's technological infrastructure, ensuring a smooth and efficient research experience.

[International Center](#): Feeling overwhelmed by the cultural shift? The International Center provides support systems for international students, including cultural events, English language assistance, and local trips. They also offer programs for spouses and families and help with acclimating to American customs.

[Orbit & New Graduate Student LAUNCH](#): New to UT Dallas? Orbit offers pre-orientation modules and events specifically geared towards graduate students. LAUNCH, a new program, provides information about campus resources, faculty insights, and tips for academic and professional success.

[The Office of Research and Innovation](#): Looking to immerse yourself in the culture of research at UTD? The Office of Research and Innovation features advancements across UTD and offers resources for graduate students as they delve into their own research.

[Blackstone LaunchPad](#): Doing innovative research and need entrepreneurial support? Blackstone LaunchPad provides resources and guidance on developing ideas, overcoming challenges, connecting with mentors and peers, and accessing online support networks.

[Housing](#): Need someplace to live? UT Dallas offers on-campus housing options for graduate students, including furnished and unfurnished apartments in [University Village](#), [Canyon Creek Heights](#), and [Northside](#). All options provide a convenient living space within walking distance to classes.

[University Recreation](#): Need a break from studies? A variety of fitness amenities, including weight and cardio equipment, racquetball courts, a climbing wall, and a swimming pool are available. Outdoor courts and fields for basketball, cricket, sand volleyball, soccer, tennis, softball, and baseball are also available.

[The Office of Graduate Education](#): Want to take full advantage of a UTD graduate education? Workshops on writing, finding fellowships, and navigating your graduate career, from assistantships to dissertations and theses are available. They also collaborate with faculty to enhance the overall graduate student experience.

[Student Counseling Center](#): Need personal support? In addition to in counseling services for students, the Student Counseling Center maintains [extensive resources](#) in various modalities to support the many and various roles that are associated with graduate training (e.g., TA with a student in distress, support for self-care, professional development materials).

[AccessAbility Resource Center](#): Have documented need for accommodations? The AccessAbility Resource Center supports equitable experiences and opportunities for UTD community members with disabilities while maintaining the integrity and standards required.

[Center for Teaching and Learning](#): Want to improve your competencies in teaching? The Center for Teaching and learning provides campus-wide leadership and coordination of activities aimed at supporting excellence in teaching.

[Eugene McDermott Library](#): Need an article? In addition to special collections, the library houses extensive physical and digital resources to support your research and learning.

Graduate Student Assembly: Want more? A student-led organization, advocates for graduate students and organizes social and professional events. You can find them on social media (i.e., Facebook, Instagram) for more information.

Resources Beyond UT Dallas

For **learning about human genetics** and associations with psychological traits a workshop run by the CU Boulder institute for behavior genetics that happens the first week of March. See here:

<https://www.colorado.edu/ibg/workshop>

Neuroanatomical dissection of the human brain and spinal cord is available each summer through Marquette University, but requires registration well in advance: <https://www.marquette.edu/health-sciences/continuing-education/neuroanatomical.php>

ABCT has a lot of great workshops for **learning about empirically supported treatments**. At least in the past, those who volunteered to help with the local arrangements can sometimes earn a free workshop attendance. See here: www.abct.org

ABCT compiles an **event to help understand the internship process and preparation** each year, and they also keep a list of handy information on their website here: <https://www.abct.org/for-students/internship-process/>

SSCP has lots of good **resources for students** on their website <https://societyforscienceofclinicalpsychology.wildapricot.org/page-18231>

Don't forget that there is a growing set of resources available online for learning! One class that is taught by highly respected experts on **the basics of fMRI** is available here: Principles of fMRI: <https://www.coursera.org/learn/functional-mri/>

National Organizations

Psychological Clinical Science Accreditation System (PCSAS): <https://www.pcsas.org/>

American Psychological Association Insurance Trust:
<https://www.trustinsurance.com/Insurance-Programs/Student-Liability>

Association for Psychological Science: <http://www.psychologicalscience.org/members/apssc>

Society for a Science of Clinical Psychology: <http://www.sscpweb.org/>

Grants and Fellowships

National Institute of Mental health (NIMH): <https://www.nimh.nih.gov/funding/training>

National Science Foundation (NSF) Graduate Research Fellowship Program:
<https://beta.nsf.gov/funding/opportunities/nsf-graduate-research-fellowship-program-grfp>

AAUW (American Association of Women in Universities) Fellowship Program:
<https://www.aauw.org/resources/programs/fellowships-grants/>

Mental Health Resources

Therapeutic services can be costly and may not be easily accessible due to strong workforce shortages across the Metroplex. Given this, online mental health resources can help make therapy more accessible to individuals in need. Below is a list of online, telephone, and app-based resources for mental health problems.

Mental Health Broadly

- [iCBT App](#) - This low-cost app uses CBT strategies to provide users a platform to identify, appraise and reappraise negative thoughts.
- Medisafe - An app that keeps tracks of your medications and sends reminders on when to take them.
- Get more [free mental health and self-help resources](#).

Depression:

- [DBSA \(Depression and Bipolar Support Alliance\) Wellness Tracker](#) is used to track your moods, symptoms, medications, and lifestyle every day to give a visual representation of wellness for the month
- [Centre for Interactive Mental Health Solutions](#) is an interactive program used for depression. The program is based on cognitive-behavioral therapy and is composed of 8 sessions with illustrated examples, mood monitors, and worksheets.
- [Virtual Hope Box](#) is a free app to manage symptoms of depression by incorporating coping and emotion regulation skills.
- [Moodgym](#) is a self-help program based in cognitive-behavioral therapy that is used to manage symptoms of depression and anxiety.

Anxiety:

- [ThisWayUp](#) offers a course on social anxiety that is based on cognitive-behavioral therapy. This course consists of six lessons that provide psychoeducation on social anxiety and related symptoms; teach individuals exposure, behavioral techniques, and social skills; address and challenge unhelpful thought patterns; and provide relapse prevention.

- [SAM \(Self-help for Anxiety Management\)](#) is a free app that helps you monitor anxious thoughts, track behavior over time, and use guided self-help exercises to discourage stress.
- [Breathe2Relax](#) is a mobile app that helps individuals with stress management using breathing techniques. The app uses videos, text, and audio to teach breathing exercises and allows individuals to track their progress.
- [Anxiety Coach](#) compiles strategies for managing and overcoming symptoms of anxiety and offers resources on different types of anxiety disorders from a licensed psychologist.
- For a list of more self-help apps to help with anxiety, click [here](#)

National Telephone Crisis Lines:

- [National Suicide Prevention Lifeline](#): 1-800-273-8255
- [National Sexual Assault Hotline and Online Hotline](#): 1-800-656-HOPE (4673)
- [The Trevor Project](#): Crisis and Suicide Prevention Helpline for LGBTQ Youth: 1-866-488-7386 or text START to 678678
- [Veteran's Crisis Line](#): 1-800-273-8255 (press 1) or text HOME to 838255
- [National Domestic Violence Hotline](#): 1-800-799-SAFE (7233)
- [SAMSHA National Helpline for Substance Abuse](#): 1-800-662-4357
- [National Crisis Text Line](#): Text "HOME" to 741741
- [Disaster Distress Helpline](#): 1-800-985-5990 or text "TalkWithUs" to 66746

Table 1

Specific Competencies with Timeline for Attainment, Presented in Order of Attainment Ease (easiest to hardest)

Specific Competency	Pre-candidacy Level	Candidacy Level
<i>Concern for the Welfare of Others—Consistently acts to understand and safeguard the welfare of others</i>	✓	✓
<i>Interpersonal Relationships</i>	✓	✓
<i>Integrity – Honesty, personal responsibility, and adherence to professional values</i>	✓	✓
<i>Deportment - Understands how to conduct oneself in a professional manner (appropriate communication and physical conduct, including attire, across different settings)</i>	✓	✓
<i>Accountability - Accountable and reliable (consistently reliable; consistently accepts responsibility for own actions)</i>	✓	✓
<i>Participation in Supervision Process -- Observation of and participation in supervisory process (e.g., peer supervision)</i>	✓	✓
<i>Affective Skills - Negotiates differences and handles conflict satisfactorily; provides effective feedback to others and receives feedback non-defensively</i>	✓	✓
<i>Ethical Conduct - Knowledge of own moral principles/ethical values integrated in professional conduct</i>	✓	✓
<i>Expectations and Roles -- Knowledge of purpose for roles in supervision</i>	✓	✓
<i>Intervention Implementation: Common Skills</i>	✓	✓
<i>Expressive Skills – Ability to clearly and articulately express oneself</i>	✓	✓
<i>Assessment: Evaluation Methods</i>	✓	✓
<i>Assessment: Application of Methods</i>	✓	✓
<i>Knowledge of procedures and processes of supervision</i>	✓	✓
<i>Respectful and productive relationships with individuals from other professions</i>	✓	✓
<i>Assessment: Application of Measurement and Psychometrics</i>	✓	✓
<i>Administration -- Knowledge of and ability to effectively function within professional settings and organizations, including compliance with policies and procedures</i>	✓	✓
<i>Ethical and Legal Issues – Act in compliance with ethical/professional codes, standards, and guidelines; institutional policies; laws, statutes, rules, regulations, and case law relevant to the practice of psychology and its supervision</i>	✓	✓
<i>Self-Care – Attention to personal health and well-being to assure effective professional functioning</i>	✓	✓
<i>Addressing Referral Question</i>		✓
<i>Diagnosis</i>		✓
<i>Awareness and Application of Ethical Decision Making - Knows and applies an ethical decision-making model and can apply relevant elements of ethical decision making to a dilemma</i>		✓
<i>Case Conceptualization and Recommendations</i>		✓
<i>Knowledge and understanding of the APA Ethical Principles and Code of Conduct and other relevant ethical/ professional codes, standards, and guidelines; laws, statutes, rules, regulations</i>		✓
<i>Reflective Practice - Broadened self-awareness; self- monitoring; reflectivity regarding professional practice (reflection-on-action); use of resources to enhance reflectivity; elements of reflection-in-action</i>		✓

<i>Understanding of others as shaped by individual and cultural diversity (e.g., cultural, individual, and role differences, including those based on age, gender, gender identity, race, ethnicity, culture, national origin, religion, sexual orientation, disability, language, and socioeconomic status) and context.</i>		✓
<i>Professional Identity - uses resources (e.g., supervision, literature) for professional development</i>		✓
<i>Management -- Participates in management of direct delivery of professional services; responds appropriately in management hierarchy</i>		✓
<i>Intervention-Specific Implementation Skills</i>		✓
<i>Scientific Mindedness</i>		✓
<i>Empowerment -- Uses awareness of the social, political, economic, or cultural factors that may impact human development in the context of service provision</i>		✓
<i>Self-Assessment - Broadly accurate self-assessment of competence; consistent monitoring and evaluation of practice activities</i>		✓
<i>Application of Consultation Methods</i>		✓
<i>Understanding of interaction of self and others as shaped by individual and cultural diversity (e.g., cultural, individual, and role differences, including those based on age, gender, gender identity, race, ethnicity, culture, national origin, religion, sexual orientation, disability, language, and socioeconomic status) and context</i>		✓
<i>Leadership -- Recognition of own role in creating policy, participation in system change, and management structure</i>		✓
<i>Understanding of self as shaped by individual and cultural diversity (e.g., cultural, individual, and role differences, including those based on age, gender, gender identity, race, ethnicity, culture, national origin, religion, sexual orientation, disability, language, and socioeconomic status) and context</i>		✓
<i>Understands how participation in interdisciplinary collaboration/consultation enhances patient outcomes</i>		✓
<i>Applies knowledge, sensitivity, and understanding regarding individual and cultural diversity to work effectively with diverse others in assessment, treatment, and consultation</i>		✓
<i>Knowledge of core scientific foundation of psychology</i>		✓
<i>Systems Change -- Promotes change to enhance the functioning of individuals</i>		✓
<i>Patient Progress Evaluation</i>		✓
<i>Knowledge, understanding, and application of the concept of evidence-based practice</i>		✓
<i>Intervention Planning -- Formulates and conceptualizes cases and plan interventions utilizing at least one consistent theoretical orientation</i>		✓
<i>Functional competency in role of consultant</i>		✓
<i>Able to develop and prepared to offer constructive criticism and suggestions regarding management and leadership of organization</i>		✓
<i>Understanding of multidisciplinary and interdisciplinary contexts for patient care</i>		✓
<i>Knowledge of the shared and distinctive contributions of other professions</i>		✓
<i>Awareness of Factors Affecting Quality -- Knowledge about the impact of diversity on all professional settings and supervision process including self</i>		✓
<i>Knowledge of Interventions – Knowledge of scientific, theoretical, empirical, and contextual bases of intervention, including theory, research, and practice</i>		✓

<i>Supervision Skills Development -- Knowledge of the supervision literature and how clinicians develop to be skilled professionals</i>		✓
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Note. “✓” indicates the specific competency is benchmarked for attainment, with appropriate independence, at the identified level. Table adapted from Price, S. D., Callahan, J. L., Cox, R. J. (2017). Psychometric investigation of competency benchmarks. *Training and Education in Professional Psychology, 11*, 128-139.

<https://doi.org/10.1037/tep0000133>



2026-2027

PSYCHOLOGICAL SERVICES AND EDUCATION CENTER MANUAL

THE UNIVERSITY OF TEXAS AT DALLAS



Table of Contents

INTRODUCTION.....	3
Purpose of the Manual	3
Mission and Goals.....	3
Overview of Clinic Operations	4
CLINIC STRUCTURE.....	5
Leadership Roles and Organizational Structure	5
Training and Supervision Procedures.....	6
ETHICS AND CONFIDENTIALITY	10
Ethical Guidelines	10
Confidentiality Policies.....	12
HIPAA and FERPA Compliance Practices	12
CLINICAL PRACTICES	13
Client Intake Procedures.....	13
Assessment and Treatment Protocols.....	15
Documentation and Reporting Standards	16
Report Templates.....	17
Finalizing Reports.....	18
TECHNOLOGY AND RECORDS MANAGEMENT	18
Use of Electronic Health Records (EHR)	18
Data Security and Privacy Measures.....	19
Guidelines for De-identified Information.....	19
CLINIC OPERATIONS.....	20
Appointment Scheduling	20
Cancellation and No-Show Policies.....	20
Emergency Procedures	21
COMMUNICATION AND SOCIAL MEDIA.....	22
Internal Communication Protocols	22
Social Media Guidelines for Staff	22
Social Media Guidelines for Patients	23
TRAINING AND PROFESSIONAL DEVELOPMENT	24
Orientation.....	24
Training Overview	24
Continuing Education Opportunities	26
Supervision and Feedback Mechanisms	27
ADDENDUMS	32
A. HIPAA Procedures and Practices.....	32
B. Profession-wide Competencies Rating Form	40

List of Figures

Figure 1. PSEC Organizational Chart.....	6
Figure 2. Example instructions for a beginning trainee on taking notes during a CBT session in the PSEC.	7
Figure 3.Farnsworth and Callahan (2013) model for addressing trainee clinician-patient value conflict.	9
Figure 4. The Detect-Articulate-Respond (DAR) model for navigating value conflicts between trainees and supervisors (Dunn et al., 2017).	10
Figure 5. Example instructions for a beginning trainee on taking notes during a CBT session in the PSEC.	25
Figure 6. Exemplars of figures that trainees create and submit to supervisors as part of the summative evaluation process.....	29
Figure 7. Exemplar of figure that trainees create and submit to supervisors as part of the summative evaluation process.....	31
Figure 8. Sample competency rating from the Profession-wide Competencies Form.	31

INTRODUCTION

Purpose of the Manual

The purpose of this manual is to provide a comprehensive guide for the operations, policies, and procedures of the Psychological Services and Education Center (PSEC) at The University of Texas at Dallas. This document serves as a foundational resource that standardizes practices to ensure consistency in service delivery, thereby promoting high-quality care for patients. By outlining clear expectations for staff and trainees regarding their roles and responsibilities, the manual facilitates a cohesive approach to clinical practice.

In addition to standardization, this manual is designed to be an essential training resource for new staff and trainee clinicians. It offers guidance on clinic operations, ethical practices, and clinical procedures, thereby supporting a smooth onboarding process and fostering ongoing professional development. The emphasis on compliance and accountability is paramount, as the manual details PSEC policies and procedures to ensure adherence to legal and ethical standards, including HIPAA regulations as well as university policies. This framework upholds accountability among staff and trainee clinicians.

Moreover, the manual promotes ethical standards by emphasizing the importance of confidentiality and professionalism in all clinic activities. It provides a narrative for decision-making that aligns with the mission and values of the PSEC, which seeks to integrate exceptional service delivery with scholarly research. This integration not only enhances the educational experience for students but also contributes to the advancement of psychological science.

Finally, this document is intended to be a living resource that evolves with the PSEC. It encourages feedback from patients, faculty, staff, trainees, and the larger community to identify areas for improvement and to incorporate best practices in clinical service delivery. Ultimately, the aim of this manual is to foster a collaborative and professional environment that enhances the training experience for students while ensuring the highest quality of care for patients at the Psychological Services and Education Center.

Mission and Goals

The Psychological Services and Education Center (PSEC) at The University of Texas at Dallas is dedicated to advancing scientific clinical psychology by promoting the highest standards of education and training in psychological clinical science. Our mission is to provide high-quality, evidence-based psychological services to patients while fostering a learning environment for students, staff, and faculty, with a strong emphasis on ethical standards and cultural competence.

We are committed to delivering compassionate care that respects the dignity and individuality of each patient. Recognizing that psychological well-being is influenced by social, cultural, and environmental factors, we strive to create an inclusive atmosphere where patients feel valued.

Our training programs encourage students to engage in discussions about individual differences and diverse contexts.

Our mission includes increasing the number of well-trained psychological clinical scientists who contribute to the advancement of knowledge related to the diagnosis, treatment, and prevention of mental and behavioral health problems. We aim to produce integrative and trans-disciplinary scientists who employ diverse scientific methods to address public health challenges.

By involving students in clinical research, we enhance their understanding of evidence-based interventions and the importance of empirical data in treatment decisions. This focus on service and scholarship enriches the educational experience and ensures that patients receive care informed by the latest scientific findings. We are dedicated to enhancing the quality, availability, and reliability of evidence-based, cost-effective, and equitable mental health services for individuals from all backgrounds.

The PSEC's mission and goals complement those of the Clinical Psychological Science Concentration at UT Dallas, which emphasizes a science-centered education aimed at the assessment, understanding, and amelioration of human problems in behavior, affect, cognition, and health. Both the PSEC and the Clinical Psychological Science Concentration are grounded in the core tenets of the clinical science model and the Psychological Clinical Science Accreditation System (PCSAS). Together, we aim to prepare graduates who are not only competent in conducting relevant research but also skilled in utilizing scientific methods to design, implement, and evaluate clinical assessments and interventions.

Ultimately, PSEC's mission is to contribute to the development of knowledge and services that reduce the incidence of mental health problems, promote adaptive functioning, and improve the overall quality and impact of mental health care delivery. Through our collaborative learning environment, we prepare students to become competent and compassionate clinicians, ready to meet the diverse needs of our patients and advance the field of clinical psychological science in alignment with the overarching goals of the Clinical Psychological Science Concentration.

Overview of Clinic Operations

The Psychological Services and Education Center (PSEC) at The University of Texas at Dallas serves as a comprehensive provider of psychological services while functioning as a critical training hub for aspiring trainee clinicians. Our clinic is dedicated to delivering high-quality care to patients, ensuring that their diverse needs are met through evidence-based therapeutic interventions.

Service delivery at the PSEC begins with a thorough intake process, where potential patients are assessed to determine their suitability for the clinic's services. This careful evaluation aligns patient needs with appropriate therapeutic approaches, ensuring that each patient receives tailored care. Importantly, before trainee clinicians are permitted to see patients, they must demonstrate sufficient competency in the requisite knowledge, skills, and abilities. Our tailored

approach to training emphasizes that completion of coursework alone does not indicate readiness; instead, trainees must demonstrate proficiency and understanding through rigorous supervisory assessments and evaluations.

Once accepted for services, patients engage in assessment and/or treatment facilitated by student trainee clinicians, who operate under the close supervision of experienced clinical supervisors. This mentorship model not only enhances the quality of care provided but also fosters an environment conducive to learning and professional growth. Supervisors play a crucial role in evaluating trainee competencies, ensuring that only those who meet the established standards of readiness are involved in direct patient care.

The PSEC is equipped with modern facilities and resources that support both clinical practice and research initiatives. Advanced assessment tools and digital recording devices create a controlled environment where students can practice and refine their skills. Regular supervision and constructive feedback are integral to the training process, allowing students to critically evaluate their clinical performance and make necessary adjustments. This emphasis on ongoing evaluation ensures that trainees are continuously developing their competencies while effectively addressing the challenges they encounter in their clinical work.

In addition to direct patient care, the PSEC actively integrates faculty research into its operations. This integration not only enhances the educational experience but also ensures that patients benefit from the latest advancements in clinical psychological science, reinforcing our commitment to evidence-based practice and the continuous improvement of clinical services.

CLINIC STRUCTURE

Leadership Roles and Organizational Structure

The Director of the PSEC is a full-time non-tenure system faculty member who works collaboratively as an ex-officio member of the Clinical Science Steering Committee (CSSC) to execute the goals of the clinical psychological science concentration and provide instrumental support to faculty-led specialty clinics. In addition to administrative oversight of staff, budget, infrastructure, and technology, the PSEC Director ensures that policies, procedures, and standards of care are in place to deliver appropriate evidence-based treatments that meet best practice standards for diverse patient populations.

Specialty clinics within the PSEC serve as clinically-focused research incubators, designed to advance the understanding and treatment of specific populations or clinical disorders (e.g., Psychosis, Bipolar Disorder, Neurocognitive Disorders). Each Specialty Clinic is overseen by a designated "Director" who possesses extensive expertise in their respective area, ensuring that the clinic operates with a high level of clinical and research integrity. By creating a structured environment where evidence-based practice can be tested and refined, Specialty Clinics contribute to the development of innovative treatment approaches that are informed by the latest scientific findings. This dual focus on service delivery and research allows trainee

clinicians to engage in hands-on learning experiences that enhance their clinical skills while also contributing to the broader field of clinical psychological science.

While the Specialty Clinic Directors hold leadership positions within their respective clinics, their authority is designed to complement, rather than supersede, that of the PSEC Director. This collaborative structure ensures a cohesive and stable environment, fostering effective communication and coordination across all clinical and research activities. By maintaining this balance of leadership, the PSEC can uphold its commitment to high-quality training and patient care while advancing the frontiers of psychological research.

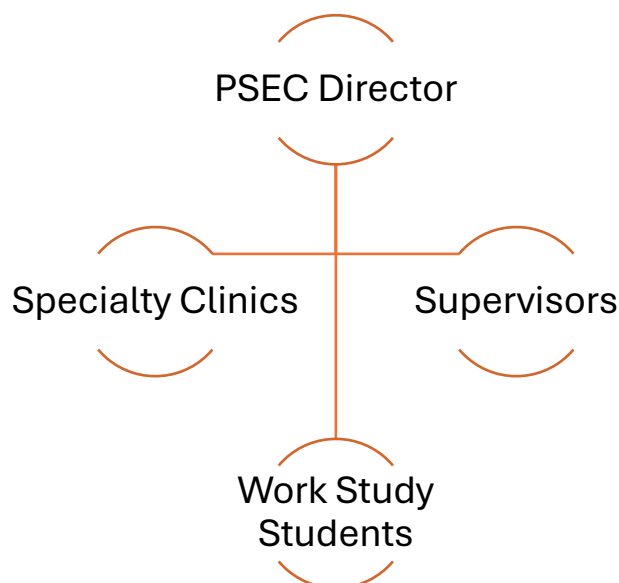


Figure 1. PSEC Organizational Chart

Work study students play an important role in supporting the overarching mission of the PSEC, including its embedded Specialty Clinics, while gaining invaluable exposure to behavioral health care. Work study students work under the close supervision of the PSEC Director to implement policies and procedures that align with the strategic objectives of the entire clinic. In performing their tasks well, these students help cultivate a professional environment that prioritizes high-quality care and contribute to the overall integrity of the clinic's operations.

Training and Supervision Procedures

Training for clinical practice is designed to provide students with comprehensive, hands-on experiences that seamlessly integrate theoretical knowledge with practical skills through structured, sequential, and progressively complex training. This approach ensures the robust development of clinical competencies. Trainees are expected to enter the PSEC evidencing already well-developed competence in foundational skills pertaining to professional integrity, deportment, and a genuine concern for the welfare of others, thereby ensuring their readiness to actively engage in the PSEC's clinical training.

Most trainees begin a two-year practicum sequence in the PSEC in their first semester. In the initial stages of this training, trainees focus on developing micro-skills and observational abilities under close supervision. Direct patient contact experiences are introduced gradually, based on trainee readiness as assessed by supervisors. Early involvement may include attending established evidence-based treatment sessions, where trainees participate in nonverbal communication, note-taking, and documenting treatment fidelity for later discussion in supervision. As trainees progress through their first semester, they are expected to demonstrate readiness to take on discrete roles within sessions, such as conducting mood checks, assisting in formulating SMART goals, performing treatment fidelity checks and summarizing sessions. This gradual increase in responsibility helps build confidence and competence. By the end of their first semester, trainees are typically able to show proficiency in micro-skills through role plays and simulations, acquiring the foundational knowledge necessary to co-conduct intake interviews and provide psychoeducation to patients. Our gradual approach in the PSEC allows trainees to begin gradually accruing patient contact and supervision hours early in their training and prepares them for more advanced training and supervision.

1. **Focus on key elements:** Pay attention to the therapist's techniques and the patient's responses. Note down the specific CBT strategies used, such as cognitive restructuring, exposure therapy, or behavioral activation.
2. **Identify cognitive distortions:** Observe if the patient exhibits any common cognitive distortions, such as catastrophizing, overgeneralization, or all-or-nothing thinking.
3. **Track progress:** Note the patient's progress in identifying and challenging negative thoughts, developing coping strategies, and implementing behavioral changes.
4. **Observe therapeutic alliance:** Pay attention to the quality of the therapeutic relationship and how it contributes to the patient's progress.
5. **Note any challenges or obstacles:** Identify any difficulties the patient may encounter in applying CBT techniques or achieving their goals.
6. **Ask clarifying questions:** If you're unsure about a particular technique or concept, feel free to ask the therapist for clarification after the session.
7. **Maintain confidentiality:** Remember to keep all patient information confidential, and avoid sharing any details outside of the training setting.

Figure 2. Example instructions for a beginning trainee on taking notes during a CBT session in the PSEC.

The PSEC setting provides a controlled environment for practicing evidence-based treatments with a range of conditions and populations, emphasizing mental health equity and the importance of multicultural competency in clinical practice. From the outset, trainees engage in discussions about individual differences and cultural competence, while also integrating scientific research with clinical practice. Trainees are also trained to utilize routine outcome monitoring — a key component of evidence-based practice — and are taught how to critically evaluate the fidelity of their treatments and the effectiveness of their interventions to determine not only whether a culturally adapted treatment is indicated but also where such adaptation(s) is needed for a given patient.

All application training within the PSEC is firmly grounded in evidence-based practices, ensuring that trainees are equipped with the most effective and scientifically supported methods for delivering psychological services. Throughout their training, trainees engage in ongoing evaluation of their interventions and outcomes, fostering a culture of continuous improvement and accountability in clinical practice. To facilitate this process, trainee clinicians utilize standardized measures to systematically assess patient progress (e.g., OQ45.2, BDI-II, WAI-SF).

Objective data on various aspects of patient functioning allow trainees and supervisors to monitor changes over time and evaluate the effectiveness of interventions and the quality of the therapeutic alliance.

By regularly assessing patient outcomes, students are empowered to make informed treatment adjustments based on empirical evidence, ensuring that their clinical approaches remain responsive to the unique needs of each patient. The commitment of the PSEC to evidence-based practice and ongoing evaluation not only enhances the quality of care provided to patients but also prepares trainee clinicians to become reflective practitioners. Trainees are supported in their learning to critically analyze the impact of their interventions, seek feedback from supervisors, and adapt their strategies as necessary. This iterative process of evaluation and adjustment is essential for developing competent clinicians who can navigate the complexities of clinical practice and deliver high-quality, effective psychological services.

Supervisors play a vital role in this mentorship approach, providing guidance and oversight throughout the clinical process. Supervisors work closely with the PSEC Director to tailor training experiences based on each student's readiness and competencies, ensuring personalized development. Weekly formative feedback is provided, alongside summative evaluations for each patient's treatment course and each trainee's semester performance (described more fully in the section entitled, **Supervision and Feedback Mechanisms**).

To encourage habitual reflective practice, prior to supervision, trainees are required to review every digitally recorded patient interaction. Trainees are also expected to draft evidence-based treatment plans and progress notes that include routine outcome tracking data prior to supervision. Individual and group supervision sessions facilitate open dialogue, allowing trainee clinicians to discuss cases, share insights, and address challenges, ultimately enhancing their professional competencies and commitment to ethical practice.

In addition to direct supervision, the PSEC supervisors and Director emphasize and model the importance of ongoing professional development throughout the career lifespan. To inculcate lifelong learning into their professional identity, trainee clinicians are encouraged to participate in workshops, seminars, and training sessions that address emerging trends in clinical practice and research. If not already attained, supervisors are (a) encouraged and supported in securing board certification and (b) encouraged to include supervision as part of their practice demonstration. Based on the availability of funds, fees to attain initial board certification may be reimbursed by the PSEC.

Even with expert close supervision, it is expected that challenging situations will arise in the patient-clinician and/or clinician-supervisor relationship(s). Although historical theories of psychotherapy emphasized the importance of value neutrality to development of a strong working alliance, value neutrality is increasingly recognized as an unrealistic ideal in the training clinic setting. Research conducted with trainee clinicians found it was nearly inevitable that trainees would bring their own beliefs and values into the therapeutic relationship (Hogan et al., 2016). The ability to navigate patient-clinician value conflicts with skill and sensitivity is

paramount. It not only influences the effectiveness of treatment but also fosters a respectful and inclusive environment for patients and diminishes likelihood of either premature termination (by the patient) or patient abandonment (by the clinician). Understanding and addressing the interplay between a trainee clinician's values and those of their patients is essential for promoting ethical practice and enhancing the therapeutic alliance (Hogan et al., 2016).

Within the Psychological Services and Education Center (PSEC), we employ a structured approach to support trainees and supervisors in navigating value conflicts that may arise between patients and clinicians, as well as between clinicians and their supervisors. This framework is essential for addressing the complexities that can occur when the beliefs and values of different parties diverge, potentially impacting the therapeutic relationship and the effectiveness of treatment.

To address value conflicts between patients and trainee clinicians, we utilize the Farnsworth and Callahan (2013) model which takes an educative approach and was developed for use in training clinics. In their model, supervisors hold the responsibility for guiding trainees through the process of navigating patient value conflicts. Supervisors are expected to provide support and facilitate discussions that help trainees reflect on their values and understand the perspectives of their patients. However, onus of responsibility is with the trainee for disclosing to their supervisor any value conflicts they experience with their patients. Only by openly communicating these conflicts, can trainees enable the supervisory relationship to address and navigate the issues effectively, ensuring that the therapeutic alliance remains intact, and that patient care is provided in an ethically sound manner.

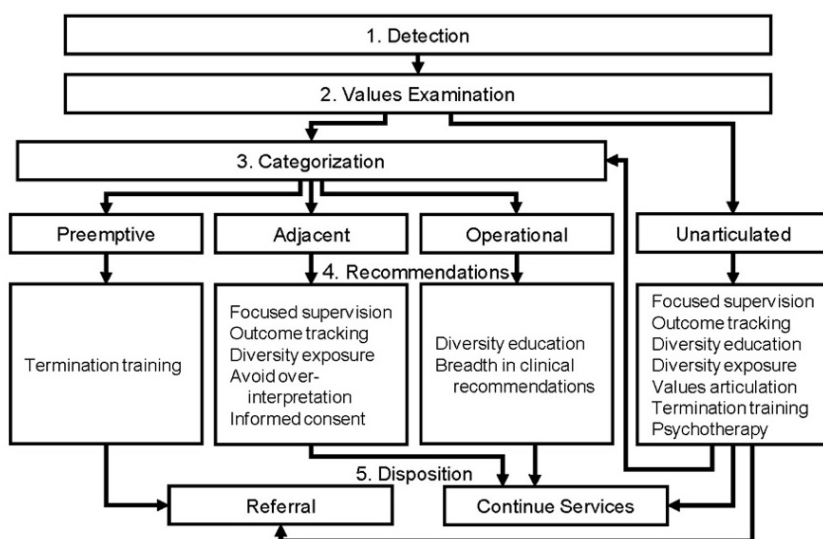


Figure 3. Farnsworth and Callahan (2013) model for addressing trainee clinician-patient value conflict.

In the context of trainee clinician-supervisor relationships, we instead apply the Dunn and colleagues (2017) model. Like the Farnsworth and Callahan model, the responsibility for guiding the process of working through clinician-supervisor value conflicts rests primarily with the supervisor. Supervisors are tasked with creating a supportive environment that encourages open communication and mutual respect. However, the success of navigating this model inherently depends on the trainee's full participation and sincere openness to discussion with the supervisor. Trainees are encouraged to voice their concerns and perspectives, which allows

for a collaborative process that can foster trust and learning. By actively engaging in this dialogue, trainees can explore their values in relation to their professional development, ultimately enhancing the supervisory relationship and equipping them with the skills necessary to navigate similar challenges in their future clinical practice.

Together, these models highlight the critical role of ethical reflection and communication in fostering a respectful and effective clinical training environment in the PSEC. Importantly, value congruence is not an expected outcome in either of these models. Rather, by integrating these approaches into our training, we aim to cultivate clinicians who are not only aware of their own values but also adept at navigating the complexities of value conflicts in clinical practice and hierarchical professional relationships. To that end, trainee clinicians are expected to routinely engage in self-reflection regarding their own values and how these may influence their clinical practice. We believe this proactive engagement normalizes discussions about values and enhances trainees' clinical competence while prioritizing patient care.

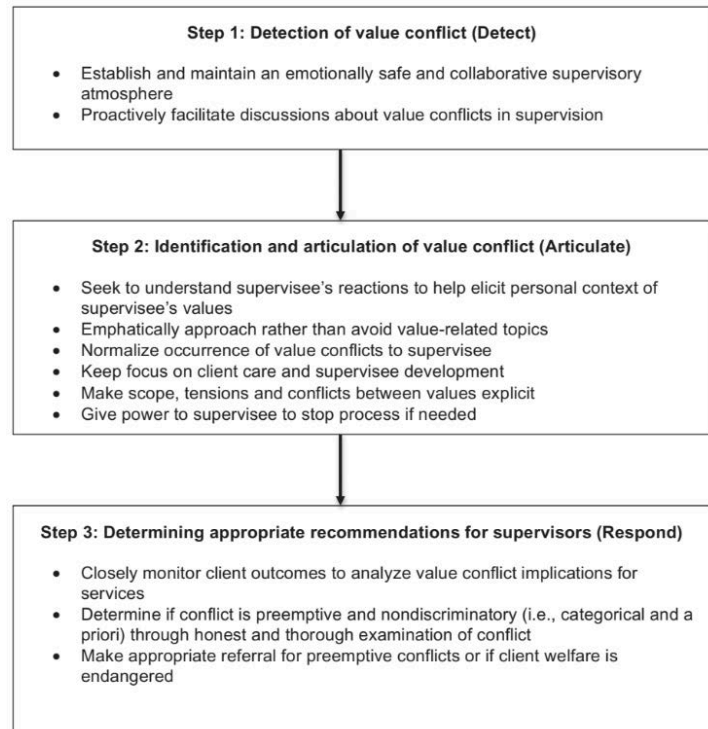


Figure 4. The Detect-Articulate-Respond (DAR) model for navigating value conflicts between trainees and supervisors (Dunn et al., 2017).

Respectful values-based discussions are an important part of demonstrating multicultural competency. In learning to navigate value conflicts with respect and sensitivity, trainees are equipped with the skills necessary to improve patient treatment outcomes and foster inclusive therapeutic environments. Recent data from the Our commitment to ethical practice and cultural competence ensures that trainees are well-prepared to meet the challenges of modern clinical practice, enhancing their ability to provide high-quality care to all clients.

ETHICS AND CONFIDENTIALITY

Ethical Guidelines

The PSEC upholds the highest ethical standards in all aspects of clinical practice, research, and education. These guidelines provide a framework for trainee clinicians, faculty, and staff, ensuring that ethical considerations are integral to their professional conduct. While adherence to these principles is essential for fostering a culture of integrity, respect, and accountability within the PSEC, it is important to acknowledge that this framework does not encompass every

potential ethical issue that may arise in practice. The absence of specific guidelines for certain situations does not diminish the ethical obligation of all members of the PSEC community to act with professionalism and integrity. Ethical practice requires a commitment to continuous reflection and vigilance, ensuring that all actions align with the highest standards of ethical conduct, even in circumstances not explicitly addressed by these principles.

Integrity and honesty are fundamental to all interactions within the PSEC. Trainee clinicians must conduct themselves with transparency, avoiding any misrepresentation of qualifications, experience, or the status of evidence informing patient care. It is essential to maintain clear boundaries and avoid dual relationships that could impair professional judgment or create conflicts of interest, thereby protecting the therapeutic relationship and ensuring a professional environment.

The importance of informed consent cannot be overstated. Patients must be fully informed about the nature of the services provided, including potential risks and benefits, before obtaining consent for evaluation, treatment, and/or participation in research. Trainee clinicians are also responsible for ensuring that their patients have been fully informed about how patient information will be used and shared. This process includes obtaining explicit consent for any treatment or research participation, which must be documented and revisited as necessary throughout the course of care, especially if there are changes in treatment plans or research protocols.

Trainee clinicians are expected to practice within the boundaries of their competence, seeking supervision or consultation when faced with cases outside their expertise. Beyond discipline-specific knowledge, skills, and abilities, cultural competence is also a critical aspect of ethical practice; trainee clinicians must be aware of and sensitive to the diverse backgrounds, values, and beliefs of patients. Efforts should be made to understand and respect the cultural contexts of patients, ensuring that treatment approaches are appropriate and effective.

All trainee clinicians and staff are expected to adhere to the ethical principles outlined in the most recent American Psychological Association (APA) Ethical Principles of Psychologists and Code of Conduct, as well as any relevant state and federal regulations (e.g., FERPA, in some contexts). Regular training and discussions on ethical issues will be provided to ensure that all members of the PSEC community are aware of and understand these ethical standards. Research conducted within the PSEC must adhere to ethical standards, including obtaining appropriate approvals from the institutional review boards (IRB) and ensuring the welfare of research participants. Researchers are responsible for the integrity of their work, which includes accurate reporting of findings and acknowledgment of contributions from others.

Any suspected ethical violations or breaches of these guidelines should be reported to the Clinic Director (for clinical breaches) or the UT Dallas Office of Research and Innovation (for research ethics breaches). The PSEC encourages a culture of accountability where ethical concerns can be addressed without fear of retaliation. By embracing these ethical guidelines, all members of the PSEC community contribute to a safe, respectful, and effective environment for clinical practice.

and research. Continuous reflection on these ethical principles is essential for the growth and development of trainee clinicians and the overall integrity of the PSEC.

Confidentiality Policies

The confidentiality of patient information is of paramount importance and is a fundamental aspect of ethical clinical practice. All trainee clinicians are required to uphold strict confidentiality standards to protect the privacy of patients and ensure that their sensitive information is handled with the utmost care. This commitment to confidentiality extends to all forms of patient data in the PSEC, including verbal communications, written records, and electronic files.

All patient-related information, including but not limited to clinical notes, assessment data, and personal identifiers, is confidential and should only be shared with authorized personnel within the PSEC. Discussions regarding patient cases should occur in designated areas that ensure privacy, and any communication about patients outside of the clinical setting must be approached with caution to prevent unauthorized disclosure.

PSEC has established protocols for the secure handling of patient information. Trainee clinicians and supervisors are trained at least annually to utilize secure methods for storing and transmitting patient data, to ensure that identifying information is never saved on unsecured devices or shared through unprotected channels. When preparing reports or documentation, trainee clinicians must use de-identified information and follow established procedures for finalizing documents to prevent any breach of confidentiality.

Maintaining confidentiality is not only a legal obligation but also a critical component of building trust in the therapeutic relationship. PSEC trainee clinicians, supervisors, and staff must remain vigilant in their adherence to these policies, recognizing that the ethical obligation to maintain confidentiality extends beyond compliance with regulations; it is a core value that underpins the integrity of clinical practice at PSEC. Trainee clinicians and their supervisors are encouraged to engage in ongoing discussions about ethical dilemmas related to confidentiality, fostering an environment where questions and concerns can be addressed openly and trainees are prepared to navigate the complexities of confidentiality while prioritizing the well-being and rights of patients.

HIPAA and FERPA Compliance Practices

The commitment to maintaining the privacy and security of both patient and trainee clinician information necessitates strict adherence to the Health Insurance Portability and Accountability Act (HIPAA) and the Family Educational Rights and Privacy Act (FERPA). These regulations guide the PSEC in its efforts to protect sensitive information while fostering an environment of trust and professionalism.

HIPAA compliance safeguards the privacy of patient health information. Trainee clinicians and supervisors are trained no less than annually to understand the critical importance of handling

Protected Health Information (PHI) with the utmost care. This training encompasses the appropriate use and disclosure of PHI, ensuring that access is limited to those who require it for treatment, payment, or PSEC operations. The PSEC employs robust security measures to protect electronic records, including encryption and secure communication channels, thereby minimizing the risk of unauthorized access. HIPAA procedures and practices are detailed more fully in Addendum A.

In parallel, FERPA compliance is equally vital within the university-based training environment of PSEC. As trainee clinicians are also students, their educational records, which include evaluations and clinical training experiences, are protected under FERPA. The clinic is dedicated to ensuring that these records are kept confidential, and that any disclosure of personally identifiable information is conducted in accordance with FERPA regulations. This means that trainee clinicians' performance and participation in the PSEC are shared only with authorized personnel and only with the necessary consent.

The intersection of HIPAA and FERPA presents unique challenges, particularly when dealing with patients who are also students. PSEC recognizes the importance of navigating these regulations carefully to ensure that both health information and educational records are managed appropriately. This dual compliance not only protects the rights of patients and trainee clinicians but also reinforces the clinic's commitment to ethical practice. Regular workshops and training sessions are proactively conducted to keep everyone informed about updates to HIPAA and FERPA regulations, as well as best practices for maintaining compliance.

CLINICAL PRACTICES

To ensure that all patients receive the appropriate level of care and support, individuals exhibiting suicidal ideation (SI) or homicidal ideation (HI) are not admitted for treatment at PSEC. Additionally, individuals presenting with symptoms associated with thought disorders will only be considered for services in the context of an appropriate Specialty Clinic or if they have established treatment with a prescribing provider.

The PSEC does not provide services to individuals who are not demonstrating clinically significant distress and/or functional impairment due to mental health issues. The intake process is used to identify which individuals soliciting services will be admitted for treatment, based on their presenting concerns and PSEC clinician training needs.

Client Intake Procedures

The intake process is a critical first step in establishing a therapeutic relationship and ensuring that each patient receives appropriate care tailored to their individual needs. This process begins with the initial contact, where prospective patients are welcomed and provided with essential information about the clinic's services, fees, and the importance of confidentiality.

Upon scheduling an appointment, patients are informed about the necessary documentation required for their intake session. This typically includes a comprehensive intake form that gathers pertinent information regarding their personal history, presenting issues, any previous mental health treatment, and patient preferences. The intake form is designed to capture a holistic view of the patient's background, including medical history, family dynamics, and social factors that may influence their mental health. Patients whose preferences cannot be met within the PSEC will be referred elsewhere for services instead of being scheduled for an intake appointment.

Trainee clinicians are expected to review the screening documentation prior to the intake session to facilitate a more informed and engaging conversation. During the intake appointment, trainee clinicians conduct a thorough assessment, utilizing active listening and open-ended questions to encourage patients to express their thoughts and feelings, creating a supportive and empathic environment in which patients feel encouraged to share their concerns openly. This session is not only an opportunity to gather information but also to build rapport and trust. Prior to co-conducting intake appointments, trainees must be evaluated via simulation or role plays for their sensitivity and attunement to patient comfort. The trainee clinician's role is to guide the conversation while allowing the patient to share their narrative at their own pace, ensuring the patient feels safe and respected throughout the process.

Discussion of the PSEC's fee structure is a necessary part of the intake process. This includes a clear outline of the fees for services rendered, any potential sliding scale options based on income, and the PSEC policies regarding payment and billing. In compliance with federal regulations, such as the No Surprises Act, PSEC is committed to providing transparent pricing information to patients, allowing them to make informed decisions about their care. Patients are encouraged to ask questions about the fee structure, and trainee clinicians are expected to provide transparent and accurate information to avoid any misunderstandings. This discussion not only helps patients prepare for the financial aspects of their care but also reinforces the clinic's commitment to ethical practices.

In addition to gathering clinical information and discussing fees, the intake process includes a discussion of the clinic's policies regarding confidentiality and informed consent. Patients are informed about their rights concerning their health information, including how it will be used and disclosed in accordance with HIPAA regulations. This transparency is essential in fostering trust and ensuring that patients feel empowered in their treatment journey.

Importantly, the intake process at PSEC extends beyond merely evaluating the presence of psychopathology. It includes a micro-intervention designed to enhance treatment engagement among patients.

In psychology training clinics, patient dropout rates can significantly hinder the effectiveness of therapeutic interventions (Callahan, Aubuchon-Endsley, Borja, & Swift, 2009; Callahan et al., 2014; Swift, Callahan & Levine, 2009). Further, meta-analytic findings indicate patient attrition occurs at a much higher rate than found in other treatment settings (Swift & Greenberg, 2012).

To address this issue, an evidence-based micro-intervention is done as a standard of care in the PSEC intake process. Patients are provided with concise, research-backed information about the expected duration of therapy and its relationship to treatment outcomes that is grounded in the dose-effect literature, which indicates that sustained treatment of approximately 15-18 sessions is positively correlated with improved recovery rates. By setting realistic expectations regarding the number of sessions required for effective therapy, patients are more likely to remain engaged in the therapeutic process. Research has shown that this patient education communication significantly lessens the risk of attrition from training clinic services (Swift & Callahan, 2011).

Within 48 hours of the intake appointment, trainee clinicians are required to review the digital recording of the appointment and synthesize the information gathered to formulate an initial understanding of the patient's needs. Managing intake responsibilities in a timely manner allows for subsequent assessment and treatment planning to commence without delay.

Assessment and Treatment Protocols

In the PSEC, the assessment and treatment of patients are guided by a commitment to evidence-based practices and a thorough understanding of each patient's unique needs. The process begins with a comprehensive assessment that employs a multi-trait multi-method (MTMM) approach, designed to gather detailed information about the patient's psychological, emotional, and social functioning. Trainee clinicians are trained to utilize a variety of assessment tools and techniques, including standardized questionnaires, clinical interviews, and observational methods, to ensure a holistic understanding of the patient's presenting concerns. This MTMM approach not only enhances the reliability and validity of the assessment but also allows for a richer, more nuanced understanding of the patient's experiences.

The assessment phase is not merely a data-gathering exercise; it is an integral part of the therapeutic process that fosters a collaborative relationship between the trainee clinician and the patient. During this phase, clinicians engage patients in discussions about their experiences, symptoms, and goals for treatment. This dialogue is essential for building rapport and trust, allowing patients to feel comfortable sharing sensitive information. Trainee clinicians are encouraged to adopt a person-centered approach, actively listening to patients and validating their feelings while also guiding them through the assessment process.

Once the assessment is complete, trainee clinicians analyze the collected data to formulate a comprehensive understanding of the patient's needs. This analysis informs the development of individualized treatment plans that are tailored to address the specific challenges and goals identified during the assessment. In the PSEC, all treatments must be evidence-based. Our commitment to evidence-based practice is grounded in research indicating that patients treated in a psychology training clinic significantly fare better when evidence-based practices are implemented as a mandated clinic policy (Cukrowicz et al., 2011).

Treatment plans in the PSEC are designed to be dynamic and flexible, allowing for adjustments based on the patient's progress and feedback. Clinicians are trained to integrate evidence-based

interventions that have been shown to be effective for the patient's presenting issues, ensuring that the care provided is both relevant and effective.

Throughout the treatment process, ongoing evaluation is a critical component of clinical practice in the PSEC. Trainee clinicians are trained to regularly assess the effectiveness of the interventions being implemented, utilizing standardized measures to track patient progress and outcomes. This continuous feedback loop not only informs treatment adjustments but also empowers patients by involving them in their own care. Clinicians are encouraged to engage patients in discussions about their progress, celebrating successes and collaboratively addressing any challenges that may arise.

The commitment to high-quality care at the PSEC extends beyond the individual treatment sessions. Trainee clinicians are supported by experienced supervisors who provide guidance and mentorship throughout the assessment and treatment process. This collaborative model ensures that patients receive care that is not only informed by the latest research but also enriched by the insights and expertise of seasoned professionals. By fostering a culture of learning and growth, the PSEC aims to equip trainee clinicians with the skills and knowledge necessary to deliver effective, compassionate care to all patients.

Documentation and Reporting Standards

Accurate and thorough documentation serves multiple purposes: It facilitates effective communication among the members of the care team, provides a record of patient progress, and ensures compliance with legal and ethical standards. As such, trainee clinicians are expected to document all patient interactions comprehensively and promptly. The definition of promptly necessarily varies according to context and good judgment is expected. In general, situations involving a real or potential crisis (e.g., risk of harm to self or others), require immediate documentation. In non-urgent circumstances (e.g., following a treatment session), promptly may mean within 72 hours. When in doubt, trainees should err on the side of being more prompt than is strictly required. Documentation refers to appropriately detailed notes on assessments, treatment sessions, and any communications with patients or other professionals involved in the patient's care. Documentation should reflect the patient's presenting concerns, the clinician's observations, the interventions employed, and the patient's responses to treatment. By capturing this information, trainee clinicians contribute to a continuous narrative of the patient's journey, which is vital for ongoing evaluation and adjustment of treatment plans.

Standardized report templates for various clinical documents are designed to ensure consistency and completeness in documentation, making it easier for clinicians to capture all necessary information while adhering to best practices. Trainee clinicians are encouraged to familiarize themselves with these templates and to use them as a guide when preparing reports, including intake assessments, progress notes, and discharge summaries. Each report should be finalized with the appropriate signatures from both the trainee clinician and their supervisor, ensuring that all documentation meets the clinic's standards for quality and accountability.

Finalizing reports is a critical step in the documentation process. Once a report is drafted, it should undergo a review process that includes feedback from supervisors. This collaborative approach not only enhances the quality of the documentation but also serves as a valuable learning opportunity for trainee clinicians. Supervisors provide insights into the content and structure of the clinical record, helping trainees refine their documentation skills and understand the nuances of effective clinical writing. This includes individualized training on handling sensitive information responsibly in the clinical record.

By prioritizing thorough, accurate, and timely documentation, PSEC ensures that patients receive the highest quality of care while also preparing trainee clinicians for their future roles as competent and ethical professionals in the field of psychology. Ultimately, the documentation and reporting standards at PSEC are designed to support both patient care and the educational development of trainee clinicians.

Report Templates

Standardized report templates are provided in the PSEC's electronic health records system and are used as training tools to support trainee clinicians in their development in producing clear and organized clinical records. PSEC templates are specifically designed to guide trainee clinicians in capturing essential information relevant to patient care while maintaining consistency across various types of documentation. By utilizing these templates, trainee clinicians can focus on delivering high-quality care without the distraction of formatting concerns.

Each template serves a distinct purpose within the clinical workflow, such as intake assessments, progress notes, and discharge summaries. The intake report template, for example, is structured to gather comprehensive information about the patient's background, presenting issues, and initial clinical observations. This framework not only aids in the thorough assessment of the patient's needs but also sets the stage for effective treatment planning. The templates prompt clinicians to include both quantitative and qualitative data, ensuring a holistic view of the patient's progress. This approach enhances the clinician's ability to evaluate treatment effectiveness and make informed decisions regarding future interventions.

PSEC templates are regularly updated to reflect the latest evidence-based practices and feedback from clinical supervisors. As such, they are not included in this manual, as they undergo revisions on a more frequent basis to ensure their relevance and effectiveness in supporting the clinical work of trainee clinicians. While the templates provide a necessary structure, trainee clinicians are expected and encouraged to incorporate their clinical insights and observations into the reports, creating narrative documents that accurately represent the patient and/or treatment. This personalized approach enhances the quality of documentation and fosters a deeper connection between the clinician and the patient.

Finalizing Reports

After drafting a clinical record (of any kind), trainee clinicians must engage in a thorough review process that incorporates feedback from their supervisors. This collaborative effort enhances the quality of the documentation and provides a valuable opportunity for trainee clinicians to develop their clinical writing skills. This collaborative process allows supervisors an opportunity to offer insights into the content and structure of the reports that helps trainees refine their documentation practices, emphasizing the importance of clarity and precision in capturing patient information. This mentorship also ensures that PSEC documentation adheres to the necessary standards.

Once revisions are made, the finalized report must be signed by both the trainee clinician and their supervisor. This signature process signifies that the document has been reviewed and meets the PSEC's standards for quality and accountability. Timeliness in submitting finalized reports is expected of all trainees and supervisors. By prioritizing the finalization of reports, PSEC not only upholds high standards of clinical practice but also equips trainee clinicians with the skills necessary for their future roles as ethical and competent professionals in psychology.

TECHNOLOGY AND RECORDS MANAGEMENT

Use of Electronic Health Records (EHR)

The PSEC's Electronic Health Records (EHR) system facilitates the comprehensive recording of patient interactions, assessments, treatment plans, and progress notes, allowing trainee clinicians to maintain a continuous and organized narrative of each patient's health record. This digital approach not only enhances the accuracy of documentation but also streamlines communication among the care team, ensuring that all members have access to up-to-date patient information. At the same time, the PSEC EHR system is designed to protect sensitive data, ensuring that only authorized personnel have access to patient records. A commitment to data security is paramount in fostering trust between patients and the clinic, as well as in complying with legal and ethical standards governing patient information.

Trainee clinicians are trained to utilize the EHR system effectively before working with patients, to ensure data is entered in a timely manner and that all documentation adheres to the PSEC's standards for quality and completeness. The EHR system is designed to support the clinical workflow, enabling trainee clinicians to efficiently document patient encounters while minimizing the risk of errors associated with paper-based records. This technology also allows for easy retrieval of historical data, which is essential for ongoing evaluation and treatment adjustments.

In addition to improving documentation practices, the EHR system of the PSEC is equipped with features that enhance patient care. For instance, it allows for the tracking of patient outcomes over time, enabling trainee clinicians to assess the effectiveness of interventions and make informed decisions regarding treatment modifications. Furthermore, the EHR system supports

the integration of evidence-based practices by providing access to standardized assessment tools and treatment protocols, which are essential for delivering high-quality clinical services.

Data Security and Privacy Measures

The PSEC employs a multifaceted approach to data security that encompasses both technological safeguards and comprehensive training for trainee clinicians, supervisors, and staff. The clinic's EHR system is fortified with advanced security features, including encryption and strict access controls, which limit patient record access to authorized personnel only. This ensures that trainee clinicians and supervisors can engage with patient information relevant to their clinical duties while preventing unauthorized disclosures.

In addition to the technological framework, PSEC emphasizes the necessity of proper training in data security practices. Trainee clinicians and supervisors are required to attend annual trainings concerning PSEC protocols for handling both electronic and physical records, including the importance of secure communication methods when discussing patient information. This training also includes guidance on using secure platforms for transmitting sensitive documents, thereby reinforcing the clinic's commitment to confidentiality.

The PSEC also maintains stringent policies regarding the storage and disposal of patient records. All patient files are securely stored, with digital records regularly backed up to prevent data loss. When it comes to disposing of records, the clinic follows best practices to ensure that all patient information is irretrievably destroyed, thus minimizing the risk of accidental disclosures.

Regular audits and assessments of data security measures are conducted to identify potential vulnerabilities and implement necessary improvements. This proactive approach not only enhances the security of patient information but also fosters a culture of accountability. All are encouraged to report any concerns regarding data security or privacy breaches to the PSEC Director.

Guidelines for De-identified Information

De-identified information refers to data that has been stripped of all personal identifiers, making it impossible to link the information back to individual patients. The handling of de-identified information in the PSEC is governed by strict guidelines to ensure compliance with legal and ethical standards while promoting the responsible use of patient data for research and educational purposes. This process is essential for protecting patient privacy while allowing for the analysis of clinical outcomes and the advancement of psychological science.

When utilizing patient data for research or educational activities, all identifying information, such as names, addresses, and any other unique identifiers, must be removed. This practice not only safeguards patient privacy but also aligns with the ethical obligations of the clinic and the broader standards set forth by regulatory bodies.

In addition to removing personal identifiers, trainee clinicians must ensure that the de-identified data is handled with care to prevent any potential re-identification. This includes being mindful of the context in which the data is shared and ensuring that it is not disclosed in a manner that could inadvertently lead to the identification of patients. For instance, when presenting data in reports or during discussions, clinicians should aggregate information to further obscure individual identities, focusing on trends and patterns rather than specific cases.

PSEC encourages the use of de-identified information for quality improvement initiatives, research projects, and training purposes. By analyzing de-identified data, trainee clinicians can contribute to the development of evidence-based practices and enhance the overall quality of care provided at the clinic. It is crucial that any use of de-identified information is conducted in accordance with established protocols, ensuring that all trainee clinicians are aware of their responsibilities in this regard.

Furthermore, PSEC maintains a commitment to transparency in its use of de-identified information. Patients are informed about how their data may be utilized in a de-identified format for research and educational purposes, empowering patients to understand the broader impact of their participation in clinical services.

CLINIC OPERATIONS

Appointment Scheduling

Trainee clinicians are responsible for guiding patients through the scheduling process. This begins with contacting the patient promptly to arrange an intake appointment, ideally within two business days of receiving notification of the new case assignment. Trainee clinicians must ensure that intake appointments are scheduled for a duration of two hours, allowing ample time for comprehensive assessment and discussion. To facilitate a smooth intake process, patients should be encouraged to arrive at least 45 minutes prior to their scheduled appointment. This allows sufficient time for them to complete the necessary paperwork and ensures that the appointment can begin promptly.

Trainee clinicians are also responsible for managing the appointment calendar effectively, ensuring that all scheduled appointments are accurately recorded in the clinic's system. This practice not only helps in tracking patient flow but also aids in optimizing the use of clinic resources. To ensure that trainee clinicians have appropriate time to center themselves and prepare for session, treatment rooms are booked out 15 minutes in advance of the patient's scheduled appointment time. If a patient needs to reschedule or cancel an appointment, trainee clinicians are trained to handle such requests with sensitivity and professionalism while adhering to the PSEC cancellation policies.

Cancellation and No-Show Policies

Circumstances can arise that may prevent patients from attending their scheduled appointments. However, where possible, the PSEC aims to minimize the impact of cancellations

and no-shows on both the clinic's operations and the patient's course of treatment. Ultimately, these policies are designed to enhance the overall efficiency of the clinic while ensuring that all patients receive the care they need in a timely manner.

To support this goal, the clinic has established clear policies regarding cancellations and no-shows. When a patient needs to cancel an appointment, it is important that they do so at least 24 hours in advance, except in cases of emergency.

The first time a patient does not notify the clinic in advance, they will be reminded of the cancellation policy. However, if a patient continues to miss appointments without proper notification, they may be charged for the missed session. This fee is intended to encourage accountability and ensure that all patients can receive timely care.

Trainee clinicians are encouraged to follow up with patients who miss appointments to understand any barriers that may have contributed to their absence. This follow-up not only demonstrates the clinic's commitment to patient care but also provides an opportunity to reschedule the appointment and continue the therapeutic process.

Emergency Procedures

In the event of a crisis, such as a medical emergency, a mental health crisis, or any situation that poses an immediate threat to the safety of patients or staff, prompt and appropriate responding is critical.

When an emergency arises during clinic hours, the PSEC Director will work with any involved trainee clinician and/or supervisor to assess the situation and determine the necessary course of action. This may involve contacting emergency services if the situation requires immediate medical attention or intervention. Trainee clinicians are encouraged to remain calm and provide reassurance to patients while ensuring that the environment remains safe and secure.

In cases where a patient is experiencing a mental health crisis, de-escalation techniques should be used to stabilize the situation. This includes actively listening to the patient, validating their feelings, and providing support while assessing the level of risk involved. If a patient poses a danger to themselves or others, the clinician must take appropriate steps to ensure safety, which may include involving additional clinic staff or contacting emergency services.

In the event of an emergency, trainee clinicians are expected to notify the supervisor immediately. The supervisor will provide guidance and support, ensuring that the situation is managed effectively and that all necessary protocols are followed. If the supervisor is not available, the PSEC Director should be contacted instead.

After an emergency has been resolved, it is essential for trainee clinicians to document the incident thoroughly. This documentation should include a detailed account of the events, the actions taken, and any follow-up measures that may be necessary. This process not only helps in

evaluating the response to the emergency but also contributes to ongoing training and improvement of clinic procedures.

COMMUNICATION AND SOCIAL MEDIA

Internal Communication Protocols

Effective communication among trainee clinicians, supervisors, and administrative staff is encouraged through regular meetings and check-ins. These gatherings provide an opportunity to discuss patient care, share insights, and address any challenges that may arise. Trainee clinicians are expected to actively participate in these discussions, bringing forth any relevant information regarding their experiences in the PSEC and seeking guidance when necessary. This open exchange of ideas and perspectives contributes to a culture of continuous improvement and professional growth within the clinic.

In addition to scheduled meetings, PSEC utilizes various communication tools. Email serves as a primary means of communication for sharing important updates, scheduling appointments, and disseminating PSEC-wide information. Trainee clinicians are encouraged to check their email regularly and respond promptly to messages from colleagues, staff, and supervisors. Secure messaging platforms may be employed for more immediate communication regarding patient care or urgent matters, ensuring that all team members can stay connected in real-time.

Confidentiality is a cornerstone of the communication protocols at PSEC. All staff members, including trainee clinicians and supervisors, are required to adhere to strict guidelines regarding the sharing of patient information. Discussions about patients should occur only in private settings and should involve only those individuals who are directly involved in the patient's care. This commitment to confidentiality not only protects the privacy of our patients but also reinforces the trust that is essential in the therapeutic relationship.

Social Media Guidelines for Staff

The use of social media by staff members, including trainee clinicians, is recognized as a powerful tool for communication and engagement. However, it is essential that all staff members approach social media with a clear understanding of the responsibilities and ethical considerations that accompany its use. The following guidelines are designed to ensure that social media interactions reflect the values and professionalism of PSEC while safeguarding the privacy and confidentiality of our patients.

When engaging on social media platforms, staff members must maintain a clear distinction between their personal and professional identities. It is crucial that trainee clinicians do not disclose any identifiable information about patients or their treatment experiences. This commitment to confidentiality is paramount, as it protects the privacy of our patients and upholds the ethical standards of the psychological profession. Even seemingly innocuous posts can inadvertently reveal sensitive information, and therefore, discretion is essential.

Trainee clinicians may use social media to promote the positive work being done in the PSEC, share relevant resources, and engage with the broader community in a manner that reflects the clinic's mission. However, any content shared should be carefully considered to ensure it aligns with the values of respect, inclusivity, and professionalism. When discussing topics related to mental health or the services offered at PSEC, provide accurate information and avoid making definitive statements that could be misinterpreted or taken out of context. It is also important to be aware of the potential impact of online behavior on one's professional reputation and the reputation of the PSEC. Engaging in heated debates, sharing controversial opinions, or posting inappropriate content can reflect poorly on both the individual posting and the PSEC. Therefore, all are expected to think critically about the content they share and the potential implications it may have on their professional image.

Direct communication with patients via social media platforms is to be avoided to maintain professional boundaries and protect patient confidentiality. Patients should be directed to appropriate channels for communication, such as the PSEC's website or designated contact methods.

Social Media Guidelines for Patients

Patients are encouraged to engage with PSEC's official social media channels to stay informed about updates, resources, and community events. These platforms serve as valuable tools for sharing information and fostering a sense of community among patients and staff. However, it is crucial for patients to remember that any interactions on social media should not replace direct communication with their trainee clinicians regarding their treatment or concerns. For personal matters related to treatment, patients should always reach out through secure and private channels provided by the PSEC.

The PSEC advises patients against sharing any identifiable information about themselves or others, including details about their treatment or interactions with trainee clinicians. Such disclosures can compromise not only their own privacy but also the confidentiality of fellow patients. It is important to remember that social media is a public space, and what is shared online can be seen by a wide audience.

While PSEC encourages open dialogue about mental health, patients should approach discussions on social media with caution. Engaging in conversations about personal experiences can be therapeutic, but it is essential to consider the potential impact of sharing sensitive information in a public forum. Patients are encouraged to seek support from their trainee clinicians or trusted individuals in private settings rather than relying solely on social media for guidance or validation.

PSEC is committed to creating a safe and respectful environment for all patients. Therefore, we ask that patients refrain from using social media to express grievances or concerns about their treatment or the clinic. Instead, we encourage open communication with trainee clinicians or clinic staff to address any issues directly. This approach not only fosters a constructive dialogue but also helps maintain the integrity of the therapeutic relationship.

TRAINING AND PROFESSIONAL DEVELOPMENT

Orientation

All staff, supervisors, and trainee clinicians are provided a comprehensive introduction to the clinic's mission, values, and operational procedures, ensuring that all team members are well-equipped to contribute to the high standards of care we uphold. During the orientation, the foundational principles that guide PSEC are introduced to emphasize our commitment to providing quality mental health services to our patients. This includes an overview of the ethical standards and professional conduct expected of all staff, as well as the importance of maintaining confidentiality and respecting the dignity of each patient. New staff will learn about the diverse range of services offered at PSEC and the specific roles and responsibilities they will assume within the clinic.

The orientation program also includes training on the clinic's policies and procedures, which are essential for ensuring a smooth workflow and effective communication among team members. New staff will receive detailed information about the operational aspects of PSEC, including intake processes, documentation requirements, and billing practices. In addition to administrative training, the orientation process emphasizes the importance of collaboration and teamwork within PSEC. New staff members will have the opportunity to meet with their colleagues, including experienced trainee clinicians and supervisors, to discuss their roles and establish supportive relationships. Various resources available for professional development and support within PSEC are also introduced during orientation. This includes access to mentorship programs, peer support networks, and opportunities for ongoing training.

Orientation additionally includes onboarding to the Process-Based Psychotherapy training platform (powered by COMETS). During this onboarding, staff and trainee clinicians complete account creation, a guided welcome tour of the platform, and acknowledgment of the Academic Integrity and Data Use Notice. A separate Research Consent Dialog addresses voluntary consent for archival research use of de-identified platform data. The platform serves as a required component of clinical training, providing structured curriculum modules, knowledge checks, AI-powered virtual patient practice coaches, and competency tracking tools. Trainee clinicians are expected to maintain regular engagement with the platform throughout their time in the PSEC.

Training Overview

aims to provide students with comprehensive, hands-on experiences that seamlessly integrate theoretical knowledge with practical skills via structured experiences that are sequential and graded in complexity. Students are expected to enter the concentration already exhibiting specific foundational competencies in professional integrity, deportment, and concern for the welfare of others with readiness to actively engage in clinical training.

Students begin their clinical training with a two-year practicum sequence in the PSEC, starting in their first semester. First-semester initially focuses on micro-skills development and observational skills under close supervision. Direct patient contact experiences are titrated slowly based on trainee readiness, as determined by their direct supervisor but can be generally described. A trainee may begin attending sessions in an already established course of evidence-based treatment with their participation in the therapeutic process focused on nonverbal communication, taking notes, and documenting treatment fidelity for subsequent discussion in supervision.

1. **Focus on key elements:** Pay attention to the therapist's techniques and the patient's responses. Note down the specific CBT strategies used, such as cognitive restructuring, exposure therapy, or behavioral activation.
2. **Identify cognitive distortions:** Observe if the patient exhibits any common cognitive distortions, such as catastrophizing, overgeneralization, or all-or-nothing thinking.
3. **Track progress:** Note the patient's progress in identifying and challenging negative thoughts, developing coping strategies, and implementing behavioral changes.
4. **Observe therapeutic alliance:** Pay attention to the quality of the therapeutic relationship and how it contributes to the patient's progress.
5. **Note any challenges or obstacles:** Identify any difficulties the patient may encounter in applying CBT techniques or achieving their goals.
6. **Ask clarifying questions:** If you're unsure about a particular technique or concept, feel free to ask the therapist for clarification after the session.
7. **Maintain confidentiality:** Remember to keep all patient information confidential, and avoid sharing any details outside of the training setting.

Figure 5. Example instructions for a beginning trainee on taking notes during a CBT session in the PSEC.

Within a few weeks, the trainee may be asked to take on discrete roles within the session for the same course of treatment, such as performing the start of session check-in on the patient's mood, helping the patient to format the session goal to be SMART (specific, measurable, achievable, relevant, and time-bound), or using their notes to provide a closing summary of the session at the end of the session.

By the end of the first semester, trainees typically display competency in micro-skills during role plays and/or simulations and have acquired the foundational knowledge necessary to participate as a co-assessor in intake interviews and provide patient psychoeducation regarding diagnosis and psychotherapy expectancies at the start of treatment. This gradual approach allows trainees to begin accruing patient contact and supervision hours as early as the first semester while building confidence and competence in their clinical skills to ensure they are well-prepared for more complex tasks in subsequent semesters.

In addition to in-person role-plays and supervised patient encounters, trainee clinicians are required to engage with AI-powered virtual patient practice coaches on the Process-Based Psychotherapy training platform. Three practice coaches (i.e., the MI Practice Coach, Intervention Practice Coach, and Case Practice Coach) offer simulated patient interactions that allow trainees to practice motivational interviewing techniques, intervention delivery, and case conceptualization in a low-stakes environment with immediate formative feedback. A "Practice My Case" feature adapts virtual patient behavior based on the trainee's active cases, including stage of change and chosen treatment modality. Virtual patient practice is a required supplement to, not a substitute for, supervised clinical work with real patients. Performance in virtual patient interactions is tracked for formative purposes and may inform supervisory

discussions, but it does not replace direct observation-based summative competency evaluation.

The PSEC setting provides a controlled environment for practicing evidence-based therapies with patient populations under close supervision. The concentration emphasizes the importance of discussions about individual differences and cultural competence from the outset of training. The concentration also emphasizes the importance of integrating scientific research with clinical practice, ensuring that students are well-versed in the latest methodologies and therapeutic techniques. For example, students are trained to utilize routine outcome monitoring in their clinical work, which is a key component of evidence-based practice. Supervisors encourage students to critically evaluate the effectiveness of their interventions, fostering a culture of continuous improvement and professional development. Expert supervisors (qualified by education, training, and experience that is grounded in evidence-based practice) will tailor experiences based on each student's readiness and demonstrated competencies, ensuring personalized training that aligns with their individual learning needs. As previously described, all students receive at least weekly formative feedback where supervisors provide insights into the students' clinical performance and areas for improvement. In addition, students received summative feedback for each patient's course of treatment (or assessment case) as well as each semester.

Continuing Education Opportunities

The field of psychology is constantly evolving, and ongoing professional development is expected regardless of one's career stage. To support our trainee clinicians, supervisors, and staff in their pursuit of knowledge and skill enhancement, the PSEC offers a variety of continuing education opportunities designed to foster both personal and professional growth.

The PSEC is committed to providing access to workshops, seminars, and training sessions that cover a wide range of topics relevant to clinical practice. These opportunities are tailored to address the latest advancements in psychological research, evidence-based practices, and emerging therapeutic techniques. By participating in these educational activities, trainee clinicians can stay informed about current trends and best practices, ensuring that they are well-prepared to meet the diverse needs of our patient population.

In addition to formal training sessions, PSEC encourages a culture of learning through collaborative discussions and peer-led learning initiatives. Trainee clinicians are invited to share their insights and experiences with one another, fostering an environment where knowledge is exchanged, and innovative ideas are cultivated. This collaborative approach not only enhances individual learning but also strengthens the overall clinical competencies of the team. PSEC also emphasizes the importance of integrating research into clinical practice. To this end, opportunities for involvement in ongoing research projects are made available to trainee clinicians. Engaging in research not only enriches their understanding of psychological principles but also allows them to contribute to the advancement of the field. This hands-on experience is invaluable in bridging the gap between theory and practice, equipping trainee clinicians with the skills necessary to apply research findings in their work with patients.

By prioritizing continuing education opportunities, PSEC is dedicated to ensuring that all staff members, particularly trainee clinicians, are equipped with the knowledge and skills necessary to provide exceptional care to our patients. This commitment to professional development not only enhances the quality of services offered at PSEC but also contributes to the personal fulfillment and career advancement of our team members. Ultimately, the ongoing education of our staff is a cornerstone of our mission to promote mental health and well-being within the community we serve.

Supervision and Feedback Mechanisms

Supervision plays a crucial role in the development of clinical competencies and the enhancement of patient care. It is particularly impactful during the early stages of a trainee's development, as it helps to build foundational knowledge and functional competencies. Tailoring supervision to meet the evolving learning needs of supervisees fosters a positive developmental process, which is essential for their growth and effectiveness as practitioners (Bernard & Goodyear, 2019; Inman et al., 2014; Watkins, Davis, & Callahan, 2018).

Over the course of 60 studies conducted over three decades, the supervisory alliance—defined by shared bonds, goals, and tasks—has emerged as a foundational element of contemporary supervision practice (Ladany, Friedlander, & Nelson, 2016; Watkins & Callahan, 2019). It stands out as the most robust and empirically validated common factor in supervision. Positive supervisory alliances are typically characterized by empathy, respect, warmth, facilitation, collaboration, flexibility, affirmation, encouragement, engagement, constructive feedback, and challenge (Watkins, 2011, 2016, 2018). These attributes are linked to several beneficial outcomes in supervision, including greater satisfaction, reduced burnout, enhanced self-efficacy and well-being among supervisees, increased willingness to self-disclose during supervision, improved access to coping resources, more supportive discussions of culture, and higher perceived effectiveness (Watkins, 2014).

In the last two decades, there has been a growing focus on investigating whether supervision is ultimately beneficial for patients (e.g., Ellis & Ladany, 1997; Simpson-Southward, Waller, & Hardy, 2017; Watkins, 2011). Within the field of psychology, specifically, high quality supervision has been found to exert a modest effect on patient outcomes (Callahan, Almstrom, Swift, Borja, & Heath, 2009; Rieck, Callahan, & Watkins, 2015; Wrape, Callahan, Ruggero, & Watkins, 2015). The PSEC therefore prioritizes the identification and use of exceptional supervisors (referred to as *super-supervisors* by Rousmaniere et al., 2016, p. 197).

Based on the extant research, faculty status (adjunct versus tenure system faculty) appears to be associated with super-supervisor status in the context of doctoral psychology training (Callahan et al., 2009). Years elapsed since the supervisor's completion of doctoral training may have some association (Callahan et al., 2009; Wrape et al., 2015) but appears to be nuanced. Rather than indicating a lasting advantage to those with fewer years in faculty ranks, the authors of those studies suggested that the supervisors in their samples who were more recent

graduates may have had more training in the theory and practice of supervision than those who had graduated more distally (i.e., as a function of evolving implementing regulations associated with accreditation requirements in effect for all doctoral psychology training programs in that era).

To address these nuances, the PSEC goes beyond review of qualifications and standard interviewing when evaluating prospective supervisors. The PSEC Director views incorporation of a supervision-specific competency-oriented approach to evaluating prospective supervisors as not only an educational necessity but also a patient care imperative. Prospective supervisor evaluation is guided by empirical findings in the extant literature. For example, in research by Milne and James (2002), the initial stages of competency development in supervision were marked by a high frequency of supervisor listening behaviors, accounting for 40% of supervisory interactions with clinical psychology trainees. As supervisors became more competent, their listening behaviors decreased progressively to 37%, 32%, and 24%. Concurrently, they began to incorporate more guided experiential learning techniques, such as role-plays, behavioral exercises, and modeling, which increased from 7% to 29% of their supervisory interactions with trainees.

In addition to an active stance and frequent use of experiential learning techniques, research indicates that a distinguishing trait of super supervisors is their habitual use of constructive criticism in a manner that does not diminish the supervisory bond and trainee relationship (Rieck, Callahan, & Watkins, 2015). In essence, super supervisors are critical, but their criticism is felt by trainees as constructive and supportive of their professional development.

Underscoring the importance of constructive criticism to patient care in the PSEC are findings by Reese and colleagues (2009). In their sample, super supervisors were those who reviewed patient scores on routine outcome measures during supervision and used the patient response to treatment to inform supervision. In other research, trainees' supervision satisfaction was found to improve when corrective feedback, based on routine client outcome data, was integrated into the supervisory process (Grossl, Reese, Norsworthy, & Hopkins, 2014).

The importance of this small, but growing body of literature to the PSEC to this manual is simple: Supervision in the PSEC looks quite different than found in many doctoral psychology training clinics. Trainees must come prepared and ready to engage with highly active supervisors who are both directive and constructive in promoting rapid advances in trainee competency while ensuring that patient care is not compromised by the relative inexperience of their treating provider. Being prepared for supervision involves both tangible preparation (i.e., review(s) of all digital recordings completed, all measures scored, and all progress notes and treatment plans drafted) and intangible preparation (i.e., openness to feedback and a growth mindset).

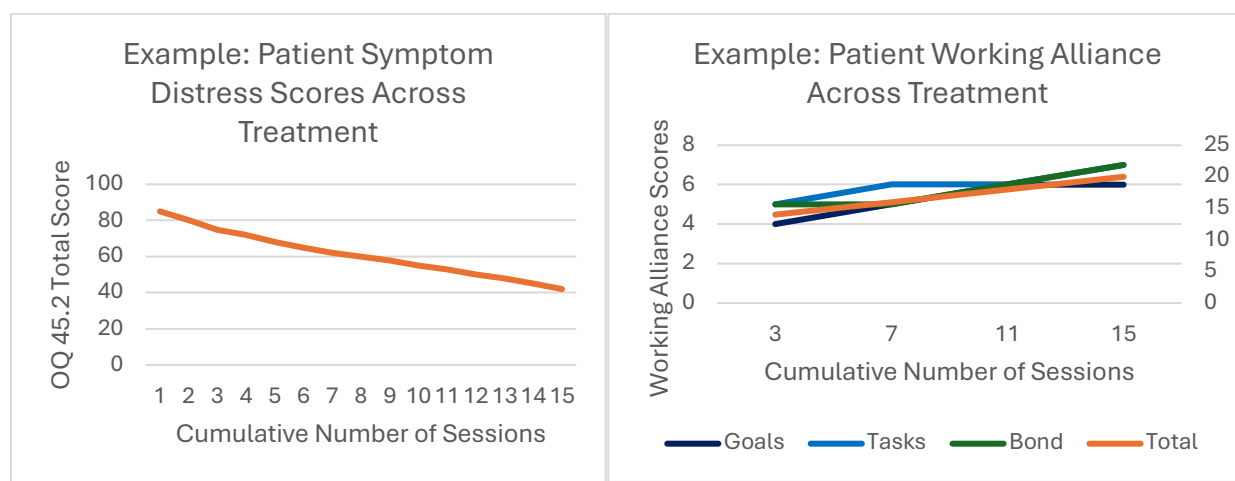
PSEC also promotes group supervision sessions, where trainee clinicians can learn from one another's experiences and perspectives. These settings foster collaboration, enabling them to share insights, discuss complex cases, and collectively problem-solve. This peer interaction

broadens their understanding of diverse clinical approaches and enhances everyone's ability to work effectively within a professional team.

In addition to the formative feedback provided in each supervision session (individually and in group format), summative feedback is provided at the end of each patient course of treatment (or assessment case) and at the end of each training semester. Summative feedback at the conclusion of patient care is focused on the specific case while summative feedback provided at the end of the training semester transcends specific patients. Both summative feedback types are used to identify training needs and guide future PSEC case assignments.

Supervisors are responsible for completing summative feedback sessions with trainees under their supervision. In addition to direct observation, supervisors are informed by patient scores on standardized measures of working alliance as well as their scores on routine outcome measures. Trainees are expected to be actively involved in the summative feedback process and provide figures to their supervisor that tracks patient scores on repeated measures over time (i.e., session-by-session routine outcome measures and periodic measures of working alliance). To guard against premature termination of the patient's services, summative figure must be provided to the supervisor prior to preparing the discharge summary. Provide salient score(s) for interpretation (e.g., clinical range cut score; reliable change index) in the figure caption. With respect to patient outcomes, the categorization should be one of the following: (1) reliably improved and no longer in clinically significant distress, a.k.a., "recovered," (2) reliably improved but still within the clinical range, a.k.a., "reliably improved," (3) no reliable change, or (4) reliable deterioration, a.k.a., "deteriorated". Alliance measures are typically given less frequently (e.g., monthly or every four sessions, whichever comes first) but should also be summarized across the course of treatment. Examples provided below are for illustration purposes and represent a prototypic patient response to treatment rather than real patient data.

Figure 6. Exemplars of figures that trainees create and submit to supervisors as part of the summative evaluation process.



Example figure caption: Reliable change on the OQ45.2 is indicated by a change of 14 or more raw score points. The cut score that demarks the clinical from non-clinical range is 63. This patient's total score on the OQ45.2 declined from 85 prior to the first session to 42 at termination. Based on this measure, the patient's scores are consistent with a treatment outcome classification of "recovered."

Example figure caption: For the Working Alliance Inventory – Short Form, a score of 4 or lower on a subscale (Goals, Tasks, Bond) is considered weak while a score of 6 or higher is considered strong. For the total score 10 or lower is indicative of a weak alliance, 11-13 indicates moderate alliance, 14-16 indicates good alliance, 17-19 indicates very good alliance, and 20 indicates excellent working alliance. Over the course of treatment, the alliance improved from moderate at session 3 to good (session 7), very good (session 11), and excellent (termination).

Empirical studies consistently show that patients receiving treatment from psychology practicum trainees often experience significantly attenuated rates of change and symptom reduction compared to other outpatient settings (e.g., Callahan, Almstrom, Swift, Borja, & Heath, 2009; Callahan, Gustafson, et al., 2014; Callahan & Hynan, 2005; Carr, Saules, Koch, & Waltz, 2017; Hansen, Lambert, & Forman, 2002; Kadera, Lambert, & Andrews, 1996; Nyman, Nafziger, & Smith, 2010; Okiishi et al., 2006; Rieck & Callahan, 2013; Shimokawa, Lambert, & Smart, 2010). The PSEC aims to differ from the status quo in psychology training clinics and demonstrate patient outcomes that are on par, or better, than observed in other settings.

Importantly, patients in practicum training clinics present with symptom severity levels like those reported in normative data and other psychotherapy studies using the same assessment measures. While the treatment gains may be less pronounced, the changes observed in patients during treatment — whether positive (Callahan, Swift, & Hynan, 2006) or negative (Swift, Callahan, Heath, Herbert, & Levine, 2010; Swift, Callahan, Herbert, & Heath, 2012) — reflect change processes that are consistent with those seen in other outpatient settings. In short, it appears neither patient severity nor change processes are explanatory of the poor outcomes associated with training clinics. Rather, there is some evidence to suggest that effective clinicians can be distinguished from ineffective clinicians very early in training, as early as the internal practicum stage of training (Callahan & Hynan, 2005).

While identification of trainee clinicians struggling to be effective is viewed through an educative lens, need for training must not interfere with the needs of patients. The PSEC holds an ethical responsibility not to prematurely approve trainee clinicians for external practicum. As such, readiness for external placement is carefully considered in the context of the trainee's effectiveness trajectory across patients treated within the PSEC. Semesterly summative evaluations are only one part of determining trainee readiness for external practicum placement, but they are important pieces of information for determining whether a given trainee is likely to reliably meet patient needs under conditions of greater independence and/or an increased patient caseload. While it is understood that small caseloads will inherently yield large effects, the PSEC takes this approach to summative evaluation due to a firm belief that the mental health of every patient is important.

To contribute to the end of semester summative feedback process, each trainee in the PSEC creates a figure that aggregates their patient's outcomes semester-by-semester. The frequency of patients in each outcome category should be on the primary y-axis. A secondary y-axis should be used to indicate the percent of patients. Semesters should be identified on the x-axis. Four lines should track the percentage of patients in each of the four outcome categories. The

example provided below is not associated with any specific trainee and is presented only for the purpose of illustrating expectations for figure preparation.

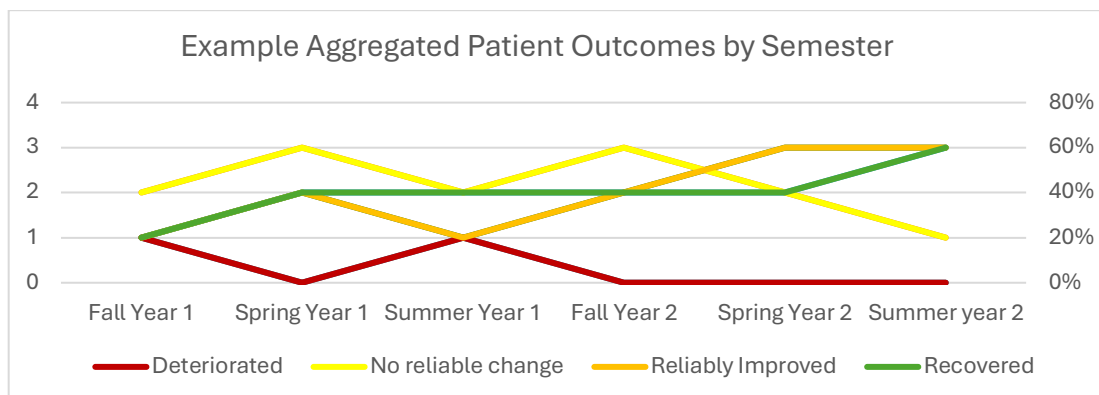


Figure 7. Exemplar of figure that trainees create and submit to supervisors as part of the summative evaluation process.

Based on direct observation (e.g., live supervision, delayed viewing of digital recordings, simulation and role-play activities) and the required trainee-provided figures, each trainee clinician is provided with summative feedback by their clinical supervisor(s) at the end of every semester in the PSEC.

The Profession-wide Competencies Form (Addendum B) is a standardized measure of trainee clinician competencies that uses a Visual Analog Scale (VAS) to capture supervisor ratings for specific competencies of focus in the PSEC (Price et al., 2017). The capture of VAS data is hosted on a secure, password-protected server compliant with HIPAA and FERPA regulations. It features a line with three labels: "Does Not Meet Expectations," "Readiness for Practicum," and "Well-Developed Competence." Supervisors use a computer mouse or touch device to slide a bar along the scale to indicate the trainee's competency level, with the final rating being auto coded on a continuous scale from 1 to 100. Supervisors may also indicate that a given competency is not being rated due to insufficient data during the period of evaluation.

Figure 8. Sample competency rating from the Profession-wide Competencies Form.

D. Concern for the welfare of others – Consistently acts to understand and safeguard the welfare of others:

☐ No information

Does not meet expectations
Competence
Readiness for Practicum
Well Developed

A competency codebook with operationalized behavioral anchors is used by supervisors as they make their ratings. Supervisor ratings are auto coded and categorized into six empirically derived bins, with a distribution reflecting varying levels of competency. Many-facets Rasch measurement, which is a form of IRT revealed the Profession-wide Competencies Form to have high reliability for use with practicum trainees (person reliability = .99) to measure specific

competencies (item reliability = .88) in the training clinic (Price et al., 2017). Further, the measure shows good sensitivity to competency changes across advancing semesters of practicum training in psychology clinics (Yanouri et al., 2023). This structured approach to tracking trainee competency development over time aims to mitigate bias in supervisor ratings and enhance the accuracy of trainee competency assessments.

In addition to the Profession-wide Competencies Form, the Process-Based Psychotherapy training platform generates AI-estimated competency scores based on trainee performance across knowledge checks, practice coach interactions, and curriculum module completion. These platform-derived estimates are clearly labeled as "AI-Estimated (Formative Only)" throughout the platform interface and in any associated reports. They are intended to supplement, not replace, the supervisor-administered summative competency evaluations described above. AI-estimated scores may be referenced during formative feedback discussions to identify areas for targeted development, but they do not carry weight in formal summative evaluation decisions, which remain the exclusive responsibility of clinical supervisors using the Profession-wide Competencies Form.

As we hope this manual makes clear, feedback is a cornerstone of the supervisory process in the PCES. Supervisors provide both formative and summative feedback, ensuring that trainee clinicians receive timely and constructive insights into their performance. We believe a continuous feedback loop enhances clinical competencies and instills a culture of self-reflection and accountability. Our multifaceted approach to feedback and supervision is intended to provide a well-rounded perspective on trainee development, highlighting strengths and areas for growth. The Clinical Science Steering Committee (CSSC) reviews these evaluations during annual student reviews and may offer additional guidance to help trainee clinicians navigate their clinical competency development.

ADDENDUMS

A. HIPAA Procedures and Practices

1. Purpose and Need

The UT Dallas Psychological Services and Education Center (PSEC) is committed to protecting the privacy and confidentiality of our clients' health information. This document outlines our policies and procedures in compliance with the Health Insurance Portability and Accountability Act of 1996 (HIPAA) and its subsequent amendments. It serves as a comprehensive guide for our workforce, clients, and stakeholders regarding the handling of Protected Health Information (PHI) and the rights of individuals under HIPAA.

The purpose of this addendum is to provide clear and concise information about the PSEC's HIPAA Privacy Procedures and Practices. It aims to ensure compliance with federal regulations regarding the privacy and security of health information while educating workforce members about their responsibilities in safeguarding PHI. Additionally, it informs clients of their rights

concerning their health information and the procedures in place to protect their privacy. This document also establishes a framework for addressing any potential breaches of confidentiality and outlines the steps for mitigation. By adhering to these practices, the PSEC strives to maintain the trust and confidence of our clients while delivering high-quality psychological services.

The Health Insurance Portability and Accountability Act (HIPAA) was enacted in 1996 to improve the efficiency and effectiveness of the healthcare system while safeguarding the privacy and security of individuals' health information. The key components of HIPAA include the Privacy Rule, which establishes national standards for the protection of certain health information, outlining how PHI can be used and disclosed while granting patient's rights over their health information. The Security Rule sets standards for safeguarding electronic PHI (ePHI) through administrative, physical, and technical safeguards to ensure the confidentiality, integrity, and availability of health information.

Additionally, HIPAA includes the Transaction and Code Sets Rule, which standardizes the electronic exchange of healthcare-related data to improve the efficiency of healthcare transactions. The Identifier Standards establish unique identifiers for healthcare providers, health plans, and employers to streamline administrative processes. Finally, the Enforcement Rule provides guidelines for the enforcement of HIPAA rules, including penalties for non-compliance. By implementing these regulations, the PSEC aims to enhance the rights of patients, ensure the confidentiality of their health information, and improve the overall quality of care provided.

2. Patient Rights

The PSEC recognizes and upholds the rights of patients regarding their health information. These rights are designed to empower patients and ensure their privacy and confidentiality are respected. Below are the specific rights patients have, along with the corresponding forms necessary to exercise these rights.

2.1 Right to Access

Patients have the right to access their protected health information (PHI) maintained by the PSEC. This right allows patients to review and obtain copies of their health records.

2.1.1 Access Request Form

To request access to their PHI, clients must complete the Access Request Form, which can be obtained from the PSEC office or the PSEC website. This form requires clients to provide identifying information and specify the records they wish to access.

2.2 Right to Request Amendment

Patients have the right to request amendments to their PHI if they believe the information is inaccurate or incomplete. This right ensures that patients can correct their health records to reflect accurate information.

2.2.1 Amendment Request Form

To initiate an amendment request, patients must complete the Amendment Request Form. This form requires patients to detail the specific information they wish to amend and provide a rationale for the requested change.

2.3 Right to Accounting of Disclosures

Patients have the right to receive an accounting of disclosures of their PHI made by the PSEC. This right allows patients to understand how their health information has been shared with others.

2.3.1 Accounting of Disclosures Request Form

To request an accounting of disclosures, clients must complete the Accounting of Disclosures Request Form. This form requires clients to provide identifying information and specify the time period for which they are requesting disclosures.

2.3.2 Accounting of Disclosures Form

The Accounting of Disclosures Form is provided to patients once their request is processed. This form details the disclosures made, including the date, recipient, and purpose of each disclosure.

2.4 Right to Request Restrictions on Uses and Disclosures

Patients have the right to request restrictions on the use and disclosure of their PHI. While the PSEC is not required to agree to all requested restrictions, it will consider each request carefully.

2.4.1 Restriction Request Form

Patients wishing to request restrictions must complete the Restriction Request Form. This form requires patients to specify the information they want to restrict and the reasons for the request.

2.5 Right to Confidential Communications

Patients have the right to request that communications regarding their PHI be conducted in a confidential manner. This right ensures that patients can receive information in a way that protects their privacy.

2.5.1 Confidential Communications Request Form

To request confidential communications, patients must complete the Confidential Communications Request Form. This form allows patients to specify their preferred method of communication and any necessary accommodations.

2.6 Right to File a Complaint

Patients have the right to file a complaint if they believe their privacy rights have been violated. The patients PSEC encourages patients to voice their concerns to ensure compliance with HIPAA regulations.

2.6.1 Privacy Complaint Form

Patients can file a complaint by completing the Privacy Complaint Form. This form requires clients to provide details about the alleged violation and any relevant information to support their complaint.

2.6.2 Privacy Complaint Resolution Form

Once a complaint is received, the PSEC will investigate the matter and provide a response. The Privacy Complaint Resolution Form will be used to communicate the outcome of the investigation to the patients.

3. Uses and Disclosures of PHI

The PSEC is committed to protecting the privacy of patients' protected health information (PHI). This section outlines the policies and procedures regarding the use and disclosure of PHI in compliance with HIPAA regulations.

3.1 Minimum Necessary Standard

The Minimum Necessary Standard requires that when using or disclosing PHI, or when requesting PHI from another entity, the PSEC will make reasonable efforts to limit the information disclosed to the minimum necessary to accomplish the intended purpose. This standard applies to all workforce members and ensures that PHI is only accessed and shared when necessary.

3.2 Uses and Disclosures without Authorization

Certain uses and disclosures of PHI may occur without the client's authorization. These include, but are not limited to:

- Treatment: PHI may be used and disclosed to provide, coordinate, or manage healthcare and related services.
- Payment: PHI may be used for billing and payment purposes, including obtaining payment from health plans.
- Healthcare Operations: PHI may be used for various administrative and operational activities, such as quality assessment, training, and compliance activities.
- Legal Requirements: PHI may be disclosed as required by law, including reporting to public health authorities or in response to court orders.

3.3 Uses and Disclosures with Authorization

In situations where the use or disclosure of PHI is not covered by the above categories, the PSEC will obtain the patient's written authorization before using or disclosing their PHI. This ensures that patients have control over their health information.

3.3.1 Authorization Form

Patients must complete the Authorization Form to grant permission for the use or disclosure of their PHI for purposes not otherwise permitted by HIPAA. This form specifies the information to be disclosed, the purpose of the disclosure, and the recipient of the information.

3.3.2 Revocation of Authorization Form

Patients have the right to revoke their authorization at any time, provided that the revocation is in writing. The Revocation of Authorization Form must be completed to formally withdraw consent for the use or disclosure of PHI. The revocation will be effective upon receipt, except to the extent that the PSEC has already acted based on the authorization.

3.4 Personal Representatives

Patients may designate a personal representative to act on their behalf regarding their PHI. This designation must be made in writing and can include parents, legal guardians, or individuals with power of attorney. The PSEC will recognize the authority of personal representatives to access and manage PHI in accordance with applicable laws.

3.5 Marketing and Fundraising Communications

The PSEC may use or disclose PHI for marketing and fundraising purposes only with the patient's prior written authorization. Patients will be informed of any marketing communications that involve the use of their PHI, and they will have the opportunity to opt-out of such communications. Fundraising efforts will also respect patient privacy and confidentiality, ensuring that any disclosures are compliant with HIPAA regulations.

4. Privacy Practices

The PSEC is dedicated to maintaining the privacy and security of protected health information (PHI). This section outlines the practices and procedures in place to safeguard PHI and ensure compliance with HIPAA regulations.

4.1 Safeguards for PHI

To protect the confidentiality, integrity, and availability of PHI, the PSEC implements a variety of safeguards. Administrative safeguards are established through policies and procedures that manage the selection, development, implementation, and maintenance of security measures to protect PHI. A designated Privacy Officer oversees compliance and safeguarding practices.

Physical safeguards are also in place, which include measures to protect the facilities and equipment that house PHI. Controlled access to areas where PHI is stored, secure disposal of documents containing PHI, and monitoring of physical access to sensitive areas are all part of these safeguards.

In addition, technical safeguards are employed to protect electronic PHI (ePHI). This involves the use of encryption, secure passwords, firewalls, and access controls to limit access to authorized personnel only. The PSEC conducts regular risk assessments to identify potential vulnerabilities in the handling of PHI and to implement necessary improvements to safeguarding practices.

4.2 Training and Awareness

All workforce members at the PSEC receive comprehensive training on HIPAA regulations and the importance of protecting PHI. New employees undergo mandatory training upon hire, which covers the basics of HIPAA, privacy practices, and the specific policies of the PSEC. To ensure that all staff remain informed about changes in regulations, policies, and best practices for safeguarding PHI, ongoing education is provided through regular refresher courses and updates.

The PSEC also promotes a culture of privacy awareness through ongoing communication, newsletters, and workshops that emphasize the importance of protecting patient information.

4.3 Incident Response and Reporting

In the event of a privacy incident or breach involving PHI, the PSEC has established procedures for response and reporting. Upon discovering a potential breach, staff are trained to take immediate action to contain the incident and mitigate any harm to affected individuals. All incidents must be reported to the Privacy Officer as soon as possible, and a formal incident report will be completed, detailing the nature of the incident, the PHI involved, and the actions taken in response.

The Privacy Officer will conduct a thorough investigation of the incident to determine the cause and assess the impact on PHI. This analysis will inform any necessary corrective actions to prevent future occurrences. If a breach of unsecured PHI occurs, the PSEC will notify affected individuals in accordance with HIPAA requirements, as well as any necessary regulatory bodies. Following an incident, the PSEC will review its policies and procedures to identify areas for improvement and enhance safeguards to protect PHI.

5. Administrative Responsibilities

The PSEC recognizes the importance of clearly defined roles and responsibilities in maintaining compliance with HIPAA regulations and ensuring the protection of protected health information (PHI). This section outlines the key administrative responsibilities within the PSEC.

5.1 Role of the Privacy Officer

The Privacy Officer plays a critical role in overseeing the development, implementation, and maintenance of policies and procedures related to the privacy of health information. This individual is responsible for ensuring compliance with HIPAA and other applicable federal and state laws. The Privacy Officer serves as the primary point of contact for privacy-related inquiries and concerns and is tasked with conducting regular assessments of the organization's privacy practices. Additionally, the Privacy Officer is responsible for providing training and guidance to staff regarding their obligations under HIPAA and the PSEC's privacy policies.

5.2 Staff Responsibilities

All staff members at the PSEC share the responsibility of protecting PHI and adhering to the organization's privacy policies. Each employee is expected to understand and comply with the established procedures for handling PHI, including maintaining confidentiality and reporting any suspected breaches or violations. Staff members are required to participate in training sessions and stay informed about updates to privacy practices. By fulfilling these responsibilities, employees contribute to the overall effectiveness of the organization's privacy program.

5.3 Documentation and Record Keeping

Proper documentation and record keeping are essential components of the PSEC's privacy practices. The organization maintains comprehensive records of all policies, procedures, training sessions, and incidents related to PHI. This documentation serves multiple purposes, including demonstrating compliance with HIPAA regulations, facilitating audits, and providing a basis for continuous improvement of privacy practices. The Privacy Officer is responsible for ensuring that all documentation is accurate, up-to-date, and retained for the required duration as mandated by HIPAA and other applicable laws.

6. Compliance and Enforcement

The PSEC is committed to ensuring compliance with HIPAA regulations and enforcing its privacy policies by monitoring compliance and addressing any violations.

6.1 Monitoring and Auditing

To ensure adherence to privacy policies and procedures, the PSEC conducts regular monitoring and auditing of its practices. This includes reviewing access logs, conducting risk assessments, and evaluating the effectiveness of training programs. The Privacy Officer is responsible for overseeing these monitoring activities and ensuring that any identified issues are addressed promptly. Audits may also involve external reviews to provide an objective assessment of the organization's compliance with HIPAA regulations.

6.2 Consequences of Non-Compliance

Non-compliance with HIPAA regulations and the PSEC's privacy policies can result in serious consequences. Employees found to be in violation of these policies may face disciplinary action, which can range from retraining to termination, depending on the severity of the violation. Additionally, the organization may be subject to legal penalties and fines imposed by regulatory authorities for breaches of HIPAA. The PSEC takes non-compliance seriously and is committed to enforcing its policies to protect the privacy and security of PHI.

7. Additional Information

7.1 Glossary of Terms

This glossary defines key terms used throughout the privacy policies and procedures to ensure clarity and understanding among staff and stakeholders.

- Protected Health Information (PHI): Any individually identifiable health information that is transmitted or maintained in any form or medium, including demographic information, medical history, and payment information.
- HIPAA: The Health Insurance Portability and Accountability Act of 1996, which establishes standards for the protection of health information.
- Privacy Officer: The individual responsible for overseeing compliance with HIPAA regulations and the organization's privacy policies.
- Workforce: All employees, volunteers, trainees, and other persons whose conduct, in the performance of work or patient care services for the PSEC, is under the direct control of the organization.

7.2 Relevant Laws and Regulations

The PSEC adheres to various laws and regulations that govern the privacy and security of health information. Key regulations include:

- Health Insurance Portability and Accountability Act (HIPAA): Establishes national standards for the protection of health information.
- Health Information Technology for Economic and Clinical Health (HITECH) Act: Promotes the adoption and meaningful use of health information technology and strengthens the privacy and security protections established under HIPAA.
- Texas Medical Privacy Act: Provides additional protections for the privacy of medical records and health information in the state of Texas.

7.3 Contact Information for Privacy Officer

For any questions or concerns regarding privacy practices, individuals may contact the Privacy Officer at the following:

Name: Dr. To Be Hired

Title: Privacy Officer / Clinic Director

Phone: [Insert Phone Number]
Email: [Insert Email Address]
Office Address: [Insert Office Address]

B. Profession-wide Competencies Rating Form

Note that evaluations are completed only after the trainee has completed all requirements for the semester under review. Evaluations are based on his/her/their current level of progress and competence and are completed electronically. For each item, the rater slides a bar across the rating line to provide feedback to the student regarding strengths and weaknesses. The comprehensive codebook for specific anchors in each area is maintained by, and available from, the PSEC Director and/or the Director of Clinical Science Training.

Research

A. Scientific Mindedness

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

B. Scientific Foundation of Psychology - Knowledge of core science

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

Ethical and Legal Standards

A. Knowledge of ethical, legal and professional standards and guidelines - Knowledge and understanding of the APA Ethical Principles and Code of Conduct and other relevant ethical/ professional codes, standards and guidelines; laws, statutes, rules, regulations

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

B. Awareness and Application of Ethical Decision Making - Knows and applies an ethical decision-making model and is able to apply relevant elements of ethical decision making to a dilemma

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

C. Ethical Conduct - Knowledge of own moral principles/ethical values integrated in professional conduct

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

Individual and Cultural Diversity

A. Self as shaped by individual and cultural diversity (e.g., cultural, individual, and role differences, including those based on age, gender, gender identity, race, ethnicity, culture, national origin, religion, sexual orientation, disability, language, and socioeconomic status) and context

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

B. Others as shaped by individual and cultural diversity (e.g., cultural, individual, and role differences, including those based on age, gender, gender identity, race, ethnicity, culture, national origin, religion, sexual orientation, disability, language, and socioeconomic status) and context.

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

C. Interaction of self and others as shaped by individual and cultural diversity (e.g., cultural, individual, and role differences, including those based on age, gender, gender identity, race, ethnicity, culture, national origin, religion, sexual orientation, disability, language, and socioeconomic status) and context

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

D. Applications based on individual and cultural context - Applies knowledge, sensitivity, and understanding regarding ICD issues to work effectively with diverse others in assessment, treatment, and consultation

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

E. Empowerment -- Uses awareness of the social, political, economic or cultural factors that may impact human development in the context of service provision

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

Professional Values, Attitudes, and Behaviors

A. Integrity – Honesty, personal responsibility and adherence to professional values

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

B. Deportment - Understands how to conduct oneself in a professional manner (appropriate communication and physical conduct, including attire, across different settings)

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

C. Professional Identity - Emerging professional identity as psychologist; uses resources (e.g., supervision, literature) for professional development

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

D. Accountability - Accountable and reliable (consistently reliable; consistently accepts responsibility for own actions)

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

E. Concern for the welfare of others - Consistently acts to understand and safeguard the welfare of others

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

F. Reflective Practice - Broadened self-awareness; self- monitoring; reflectivity regarding professional practice (reflection-on-action); use of resources to enhance reflectivity; elements of reflection-in-action

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

G. Self-Assessment - Broadly accurate self-assessment of competence; consistent monitoring and evaluation of practice activities

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

H. Self-Care – Attention to personal health and well-being to assure effective professional functioning

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

I. Skills Development -- Knowledge of the supervision literature and how clinicians develop to be skilled professionals:

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

Communication and Interpersonal Skills

A. Interpersonal Relationships

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

B. Expressive Skills – Ability to clearly and articulately express oneself

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

C. Affective Skills - Negotiates differences and handles conflict satisfactorily; provides effective feedback to others and receives feedback non-defensively

☐ No information

☐

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

D. Clinical Skills

☐ No information

☐

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

Assessment

A. Diagnosis

☐ No information

☐

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

B. Measurement and Psychometrics

☐ No information

☐

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

C. Evaluation Methods

☐ No information

☐

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

D. Application of Methods

☐ No information

☐

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

E. Conceptualization and Recommendations

☐ No information

☐

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

Intervention

A. Intervention Planning -- Formulates and conceptualizes cases and plan interventions utilizing at least one consistent theoretical orientation

☐ No information

☐

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

B. Intervention Implementation

☐ No information

☐

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

C. Knowledge of Interventions – Knowledge of scientific, theoretical, empirical and contextual bases of intervention, including theory, research, and practice

☐ No information

☐

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

D. Scientific Foundation of Professional Practice - Knowledge, understanding, and application of the concept of evidence-based practice

☐ No information

☐

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

E. Progress Evaluation

☐ No information

☐

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

Supervision

A. Expectations and Roles -- Knowledge of purpose for roles in supervision

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

B. Processes and Procedures -- Knowledge of procedures and processes of supervision

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

C. Awareness of Factors Affecting Quality -- Knowledge about the impact of diversity on all professional settings and supervision process including self as defined by APA policy

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

D. Participation in Supervision Process -- Observation of and participation in supervisory process (e.g., peer supervision):

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

E. Ethical and Legal Issues -- Knowledge of and compliance with ethical/professional codes, standards and guidelines; institutional policies; laws, statutes, rules, regulations, and case law relevant to the practice of psychology and its supervision

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

Consultation and Interprofessional / Interdisciplinary Skills

A. Knowledge of the shared and distinctive contributions of other professions

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

B. Functioning in multidisciplinary and interdisciplinary contexts

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

C. Understands how participation in interdisciplinary collaboration/ consultation enhances outcomes

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

D. Respectful and productive relationships with individuals from other professions

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

E. Role of Consultant

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

F. Addressing Referral Question

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

G. Application of Methods -- Consultation

☐ No information

Does not meet expectations
Competence

Readiness for Practicum

Well Developed

APPENDIX M

**Profession-wide
Competencies (PWC)
Codebook**

Profession-wide Competencies Codebook¹

1. Research - Understanding of research, research methodology, techniques of data collection and analysis, biological bases of behavior, cognitive-affective bases of behavior, and development across the lifespan. Respect for scientifically derived knowledge.

A. Scientific Mindedness-

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Passive acceptance of published literature as truth • Unable/Unwilling to present own research in multiple modalities (e.g., written, oral)	• Displays critical scientific thinking • Aware of need for evidence to support assertions • Questions assumptions of knowledge • Evaluates study methodology and scientific basis of findings • Presents own work for the scrutiny of others	• Values and applies scientific methods to professional practice. • Articulates, in supervision and case conference, support for issues derived from the literature • Formulates appropriate questions regarding case conceptualization • Generates hypotheses regarding own contribution to therapeutic process and outcome • Performs scientific critique of literature

B. Scientific Foundation of Psychology - Knowledge of core science.

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Unable to articulate the scientific method • Applies illogical steps in place of scientific reasoning and evaluation	• Understanding of psychology as a science. • Demonstrates understanding of core scientific conceptualizations of human behavior • Demonstrates understanding of psychology as a science, including basic knowledge of the breadth of scientific psychology. For example: able to cite scientific literature to support an argument. • Evaluates scholarly literature on a topic	• In-depth knowledge of core science • Displays intermediate level knowledge of and respect for scientific bases of behavior

¹ Although not all inclusive, examples of behaviors that correspond to each competency rating are listed below. Please note that “Well Developed Competence” category listed corresponds with the “Readiness for Internship” category from the “Competency Benchmark Document” (Fouad et al., 2009), which is the product of the Assessment of Competency Benchmarks Work Group convened by the APA Board of Educational Affairs (iterative versions are housed here: <https://www.apa.org/ed/graduate/benchmarks-evaluation-system>)

2. Ethical Legal Standards - Application of ethical concepts and awareness of legal issues regarding professional activities with individuals, groups, and organizations.

A. Knowledge of Ethical, Legal and Professional Standards and Guidelines - Knowledge and understanding of the APA Ethical Principles and Code of Conduct and other relevant ethical/ professional codes, standards and guidelines; laws, statutes, rules, regulations.

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Unable to locate the APA Ethical Code. • Unable to identify which code is currently in place (e.g., which year of publication represents the current code). • Unable to identify additional codes that are salient to professional practice (e.g, child custody, forensic, and so on)	• Basic knowledge of the principles of the APA Ethical Principles and Code of Conduct [ethical practice and basic skills in ethical decision making] • Beginning knowledge of legal and regulatory issues in the practice of psychology that apply to practice while placed at practicum setting. • Displays a basic understanding of this knowledge (e.g., APA, Ethics Code and principles, Ethical Decision Making Models) • Demonstrates knowledge of typical legal issues (e.g., child and elder abuse reporting, HIPAA, Confidentiality, Informed Consent)	• Intermediate level knowledge and understanding of the APA Ethical Principles and Code of Conduct and other relevant ethical/ professional codes, standards and guidelines; laws, statutes, rules, regulations • Identifies ethical dilemmas effectively • actively consults with supervisor to act upon ethical and legal aspects of practice • Addresses ethical and legal aspects within the case conceptualization • Discusses ethical implications of professional work • Recognizes and discusses limits of own ethical and legal knowledge

B. Awareness and Application of Ethical Decision Making - Knows and applies an ethical decision-making model and is able to apply relevant elements.

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Unable to identify, with the narrative code available, applicable sections to an ethical decision-making dilemma. • Failure to identify appropriate means and resources for obtaining consultation and/or supervision when facing an ethical decision-making dilemma	• Demonstrates the importance of an ethical decision model applied to practice. • Recognizes the importance of basic ethical concepts applicable in initial practice (e.g. child abuse reporting, Informed consent, confidentiality, multiple relationships, and competence) • Identifies potential conflicts between personal belief systems, APA ethics code and legal issues in practice.	• Knows and applies an ethical decision-making model and is able to apply relevant elements of ethical decision making to a dilemma. • Uses an ethical decision-making model when discussing cases in supervision. • Readily identifies ethical implications in cases and to understand the ethical elements in any present ethical dilemma or question • Discusses ethical dilemmas and decision-making in supervision, staffing, presentations, & practicum settings

C. Ethical Conduct - Knowledge of own moral principles/ethical values integrated in professional conduct.

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Failure to seek consultation before responding to an ethical decision-making dilemma • Enacts conduct that is inconsistent with supervisor mandates regarding ethical conduct, without first seeking consultation from Clinic Director	• Ethical attitudes and values evident in conduct • Evidences desire to help others • Demonstrates openness to new ideas • Shows honesty/integrity/values ethical behavior • Demonstrates personal courage consistent with ethical values of psychologists • Displays a capacity for appropriate boundary management • Implements ethical concepts into professional behavior	• Knowledge of own moral principles/ethical values integrated in professional conduct • Articulates knowledge of own moral principles and ethical values in discussions with supervisors and peers about ethical issues • Spontaneously discusses intersection of personal and professional ethical and moral issues

3. Individual and Cultural Diversity - Awareness, sensitivity and skills in working professionally with diverse individuals, groups and communities who represent various cultural and personal background and characteristics defined broadly and consistent with APA policy.

A. Self as Shaped by Individual and Cultural Diversity (e.g., cultural, individual, and role differences, including those based on age, gender, gender identity, race, ethnicity, culture, national origin, religion, sexual orientation, disability, language, and socioeconomic status) **and Context.**

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
Has difficulty recognizing prejudices	- Knowledge, awareness, and understanding of one's own dimensions of diversity and attitudes towards diverse others. - Demonstrates this self-knowledge, awareness, and understanding. For example: articulates how ethnic group values influence who one is and how one relates to other people.	- Monitors and applies knowledge of self as a cultural being in assessment, treatment, and consultation - Understands and monitors own cultural identities in relation to work with others - Uses knowledge of self to monitor effectiveness as a professional - Critically evaluates feedback and initiates supervision regularly about diversity issues

B. Others as Shaped by Individual and Cultural Diversity (e.g., cultural, individual, and role differences, including those based on age, gender, gender identity, race, ethnicity, culture, national origin, religion, sexual orientation, disability, language, and socioeconomic status) **and Context.**

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
Does not talk about inexperience or lack of exposure to individuals/groups from different backgrounds	- Knowledge, awareness, and understanding of others individuals as cultural beings. - Demonstrates knowledge, awareness and understanding of the way culture and context shape the behavior of other individuals. - Critically evaluates feedback and initiates supervision regularly about diversity issues with others.	

C. Interaction of Self and Others as Shaped by Individual and Cultural Diversity (e.g., cultural, individual, and role differences, including those based on age, gender, gender identity, race, ethnicity, culture, national origin, religion, sexual orientation, disability, language, and socioeconomic status) **and Context.**

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
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• Holds some beliefs which limit effectiveness with certain clients	• Knowledge, awareness, and understanding of interactions between self and diverse others • Demonstrates knowledge, awareness and understanding of the way culture and context shape interactions between and among individuals	• Applies knowledge of the role of culture in interactions in assessment, treatment, and consultation of diverse others • Understands the role of multiple cultural identities in interactions among individuals • Uses knowledge of the role of culture in interactions in work as a professional • Critically evaluates feedback and initiates supervision regularly about diversity issues with others
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D. Applications Based on Individual and Cultural Context - Applies knowledge, sensitivity, and understanding regarding ICD issues to work effectively with diverse others in assessment, treatment, and consultation.

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Lacks insight into impact of personal experiences on case conceptualization, choice of treatment, etc.	• Basic knowledge of and sensitivity to the scientific, theoretical, and contextual issues related to ICD (as defined by APA policy) as they apply to professional psychology. • Understanding of the need to consider ICD issues in all aspects of professional psychology work (e.g., assessment, treatment, research, relationships with colleagues). • Demonstrates basic knowledge of literatures on individual and cultural differences and engages in respectful interactions that reflects this knowledge • Demonstrates understanding of the need to consider ICD issues in all aspects of professional psychology work through respectful interactions	• Demonstrates knowledge of ICD literature and APA policies, including guidelines for practice with diverse individuals, groups and communities • Demonstrates ability to address ICD issues across professional settings and activities • Works effectively with diverse others in professional activities • Demonstrates awareness of effects of oppression and privilege on self and others

E. Empowerment - Uses awareness of the social, political, economic or cultural factors that may impact human development in the context of service provision.

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Lack of awareness that social, political, economic, or cultural factors may impact human development. • Lack of awareness that service provision must be sensitive to social, political, economic, or cultural factors.	• Awareness of social, political, economic and cultural factors that impact individuals, institutions and systems, in addition to other factors that may lead them to seek intervention. • Articulates social, political, economic or cultural factors that may impact on human development and functioning.	• Identifies specific barriers to client improvement, e.g., lack of access to resources. • Assists client in development of self-advocacy plans.

4. Professional Values, Attitudes, and Behaviors – Professional values and ethics as evidenced in behavior and comportment that reflects the values and ethics of psychology, integrity, and

A. Integrity - Honesty, personal responsibility and adherence to professional values.

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
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• Failure to complete assigned or routine tasks during clinic on call hours • Failure to complete required clinic trainings (e.g., HIPPA) and orientations	• Demonstrates honesty, even in difficult situations • Takes responsibility for own actions • Displays basic understanding of core professional values • Demonstrates ethical behavior & basic knowledge of APA Ethical Principles & Code of Conduct: see below: Foundational Competency: Ethical-legal standards-policy	• Demonstrates knowledge of professional values • Demonstrates adherence to professional values • Identifies situations that challenge professional values, and seeks faculty/supervisor guidance as needed. • Demonstrates ability to share, discuss and address failures and lapses in adherence to professional values with supervisors/faculty as appropriate
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B. Deportment - Understands how to conduct oneself in a professional manner (appropriate communication and physical conduct, including attire, across different settings).

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Not presenting for clinic on call hours or session observation appointments in professional attire and appropriate hygiene • Discussing client(s) information in public areas of the clinic or elsewhere • Discussion of client(s) in unprofessional terms or with disrespect for individual differences	• Demonstrates appropriate personal hygiene and attire • Distinguishes between appropriate and inappropriate language and demeanor in professional contexts	• Demonstrates awareness of the impact behavior has on client, public and profession • Utilizes appropriate language and demeanor in professional communications • Demonstrates appropriate physical conduct, including attire, consistent with context

C. Professional Identity - Emerging professional identity as psychologist; uses resources (e.g., supervision, literature) for professionals.

Does Not Meet Expectations	Does Not Meet Expectations	Does Not Meet Expectations
• Cannot conduct an appropriate literature search • Fails to seek needed supervision when appropriate • Does not integrate supervision into care of client(s) • Failure to attend schedule supervision (individually or on team)	• Cannot conduct an appropriate literature search • Fails to seek needed supervision when appropriate • Does not integrate supervision into care of client(s) • Failure to attend schedule supervision (individually or on team)	• Cannot conduct an appropriate literature search • Fails to seek needed supervision when appropriate • Does not integrate supervision into care of client(s) • Failure to attend schedule supervision (individually or on team)

D. Accountability - Accountable and reliable (consistently reliable; consistently accepts responsibility for own actions).

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Failure to complete required observation or client contact hours and notes or reports in a timely manner • Failure to maintain responsibility in scheduling rooms and/or resources both when initiating scheduling and when changes are needed	• Turns in assignments in accordance with established deadlines • Demonstrates personal organization skills • Plans and organizes own workload • Aware of and follows policies and procedures of institution	• Completes required case documentation promptly and accurately • Accepts responsibility for meeting deadlines • Available when "on-call" • Acknowledges errors • Utilizes supervision to strengthen effectiveness of practice

E. Concern for the Welfare of Others - Consistently acts to understand and safeguard the welfare of others.

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
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- Engaging in behaviors that compromise a safe environment emotionally or physically - Abandonment of client(s)	- Displays initiative to help others - Articulates importance of concepts of confidentiality, privacy, informed consent - Demonstrates compassion	- Regularly demonstrates compassion - Displays respect in interpersonal interactions with others including those from divergent perspectives or backgrounds - Determines when response to client needs takes precedence over personal needs
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F. Reflective Practice - Broadened self-awareness; self- monitoring; reflectivity regarding professional practice (reflection-on-action); use of resources to enhance reflectivity; elements of reflection-in-action.

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
- Does not articulate attitudes, values, and beliefs that guide behavior and decision-making - Fails to acknowledge impact of self on others - Does not use introspection to monitor the effect of personal issues on professional work and relationships - Failure to foster development of individual and cultural identities - Does not use supervision as an opportunity to facilitate self-awareness as related to professional performance, roles, and responsibilities	- Displays basic mindfulness and self-awareness; basic reflectivity regarding professional practice (reflection-on-action) - Problem solving skills (critical thinking) - organized reasoning - intellectual curiosity and flexibility Demonstrates openness to: - considering own personal concerns & issues - recognizing impact of self on others - articulating attitudes, values, and beliefs toward diverse others - self-identifying multiple individual and cultural identities - systematically reviewing own professional performance with supervisors/teachers	- Articulates attitudes, values, and beliefs toward diverse others - Recognizes impact of self on others - Self-identifies multiple individual and cultural identities - Describes how others experience him/her and identifies roles one might play within a group - Responsively utilizes supervision to enhance reflectivity - Systematically and effectively reviews own professional performance via videotape or other technology with supervisors - Initial indicators of monitoring and adjusting professional performance in action as situation requires

G. Self-Assessment - Broadly accurate self-assessment of competence; consistent monitoring and evaluation of practice activities.

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
- Fails to recognize professional limitations, 'blindspots' & mistakes - Fails to recognize strengths - Despite input from others, does not develop specific and realistic professional goals - Cannot describe an adequate subset of core competencies for scientist-practitioners - Does not devote adequate time to self-evaluation and review of professional performance	- Knowledge of core competencies; emerging self-assessment re: competencies - Demonstrates awareness of clinical competencies for professional training - Develops initial competency goals for early training (with input from faculty)	- Self-assessment comes close to congruence with assessment by peers and supervisors - Identifies areas requiring further professional growth - Writes a personal statement of professional goals - Identifies learning objectives for overall training plan - Systemically and effectively reviews own professional performance via videotape or other technology

H. Self-Care - Attention to personal health and well-being to assure effective professional functioning.

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
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• Lacks awareness of personal & professional values and priorities • Fails to balance personal & professional commitments in a manner consistent with ones values & priorities • Relies on ineffective &/or maladaptive coping (e.g., puts physical health or important relationships at risk) • Denies personal vulnerabilities • Fails to seek help/advice for personal difficulties	• Understanding of the importance of self-care in effective practice • Knowledge of self-care methods • Attention to self-care • Demonstrates basic awareness and attention to self-care	• Monitoring of issues related to self-care with supervisor. • Understanding of the central role of self-care to effective practice • Works with supervisor to monitor issues related to self-care • Takes action recommended by supervisor for self-care to ensure effective training
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I. Skills Development - Knowledge of the supervision literature and how clinicians develop to be skilled professionals.

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Cannot describe different models of supervision • Fails to complete self-assessment related to supervisory skills.	• Interpersonal skills of communication and openness to feedback. • Completes self-assessment (e.g., Hatcher & Lassiter, 2006). • Integrates faculty/supervisor feedback into self-assessment.	• Successfully completes coursework on supervision. • Demonstrates formation of supervisory relationship integrating theory and skills including knowledge of development, educational praxis.

5. Communication and Interpersonal Skills - Relates effectively and meaningfully with individuals, groups, and/or communities.
A. Interpersonal Relationships

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Limited or poor interpersonal relationships • Fails to listen/attend to others • Unable to see or dismisses alternative views & values • Unreliable, inconsistent • Blames others; does not take responsibility for own errors • Repeated boundary violations or excessive distance in therapeutic/supervisory relationship • Does not prepare for supervision or seek assistance	• Displays interpersonal skills • Listens and is empathic with others • Respects and shows interest in others' cultures, experiences, values, points of view, goals and desires, fears, etc. • Demonstrates skills verbally and non-verbally. • Receives feedback well	• Forms/maintains productive, respectful relationships with clients, peers, supervisors and professionals from other disciplines • Forms effective working alliance with clients • Engages with supervisors to work effectively • Works cooperatively with peers • Involved in departmental, institutional, or professional activities or governance • Demonstrates respectful and collegial interactions with those who have different professional models or perspectives

B. Expressive Skills – Ability to clearly and articulately express oneself.

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Uses language that is vague or contains too much jargon • Unable to articulate e.g. stumbles, incomplete sentences/ideas, rambles too much or overly brief	• Demonstrates coherent expressive skills • Communicates ideas, feelings and information verbally and non-verbally	• Clear and articulate expression • Communicates clearly using verbal, nonverbal, and written skills • Demonstrates understanding of professional language
C. Affective Skills - Negotiates differences and handles conflict satisfactorily; provides effective feedback to others and receives feedback non-defensively.		
Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Frequent conflict with others OR avoids conflict • Doesn't challenge or confront clients when appropriate • Immature, superficial or distant relationships • Poor working alliance with clients • Becomes angry or overly upset by negative feedback • Insensitive, over critical feedback or resistant to providing constructive criticism to peers • Avoids introspection; lacks awareness of own role in interactions	• Demonstrates affective skills • Demonstrates affect tolerance • Tolerates and understands interpersonal conflict • Tolerates ambiguity and uncertainty • Demonstrates awareness of inner emotional experience • Demonstrates emotional maturity • Listens to and acknowledges feedback from others	• Negotiates differences and handles conflict satisfactorily; provides effective feedback to others and receives feedback non-defensively. • Works collaboratively • Demonstrates active problem-solving • Makes appropriate disclosures regarding problematic interpersonal situations • Acknowledges own role in difficult interactions • Provides feedback to supervisor regarding supervisory process • Provides feedback to peers regarding peers' clinical work in context of group supervision or case conference • Accepts and implements supervisory feedback non-defensively.
D. Clinical Skills		
Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Has difficulty establishing rapport. • Frequently shows lack of confidence. • Misses or misperceives important information/themes presented by client. • Failure to identify any goals. • Lacks awareness of underlying problems. • Unable to demonstrate empathy and caring such that most clients become willing to trust and utilize therapeutic assistance.	• Basic helping skills. • Demonstrates helping skills, such as empathic listening, framing problems.	• Develops rapport with most clients. • Develops therapeutic relationships. • Demonstrates appropriate judgment about when to consult supervisor.

A. Diagnosis		
Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Lack of basic knowledge of range of normal and abnormal behavior. - Lack of or misunderstanding of DSM criteria and system.	• Basic knowledge regarding the range of normal and abnormal behavior in the context of stages of human development and diversity. • Identifies DSM criteria. • Describes normal development consistent with broad area of training.	• Applies concepts of normal/abnormal behavior to case formulation and diagnosis in the context of stages of human development and diversity. • Articulates relevant developmental features and clinical symptoms as applied to presenting question. • Demonstrates ability to identify problem areas and to use concepts of differential diagnosis.
B. Measurement and Psychometrics		
Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Lack of, or misunderstanding of, the scientific, theoretical, and contextual basis of test construction. • Lack of, or misunderstanding of, basic psychometric constructs. •	• Basic knowledge of the scientific, theoretical, and contextual basis of test construction and interviewing. • Demonstrates awareness of the benefits of standardized assessment. • Demonstrates knowledge of the construct(s) being assessed. • Evidences understanding of basic psychometric constructs such as validity, reliability, and test construction.	• Selects assessment measures with attention to issues of reliability and validity. • Identifies appropriate assessment measures for cases seen at practice site. • Routinely consults with supervisor regarding selection of assessment measures.
C. Evaluation Methods		
Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Lack of or misunderstanding of the scientific, theoretical, and contextual basis of evaluation methodologies. • Failure to check out successfully (with supervisor or their designee) on standardized assessment measures within two attempts.	• Basic knowledge of administration and scoring of traditional assessment measures, models and techniques, including clinical interviewing and mental status exam. • Accurately and consistently administers and scores various assessment measures. • Demonstrates knowledge of initial interviewing (both structured and semi-structured interviews, mini-mental status exam).	• Awareness of the strengths and limitations of administration, scoring and interpretation of traditional assessment measures as well as related technological advances. • Demonstrates intermediate level ability to accurately and consistently select, administer, score and interpret assessment tools with client populations. • Collects accurate and relevant data from structured and semi-structured interviews and mini-mental status exams.
D. Application of Methods -- Assessment		
Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Lack of, or misunderstanding of, the scientific, theoretical, and contextual basis of the correct application of evaluation methodologies. • Cannot articulate to supervisor the multi-trait, multi-method matrix approach to assessment.	• Knowledge of measurement across domains of functioning and practice settings. • Demonstrates awareness of need to base diagnosis and assessment on multiple sources of information.	• Selects appropriate assessment measures to answer diagnostic question. • Selects assessment tools that reflect awareness of patient population served at a given practice site. • Regularly selects and uses appropriate methods of evaluation.

Proposes to supervisor an assessment battery that attends only to confirming hypotheses (e.g., hypothesizes ADHD and proposes a battery that fails to consider other possible causes for problems with attention/concentration).	Demonstrates awareness of need for selection of assessment measures appropriate to population/problem.	Demonstrates ability to adapt environment and materials according to client needs (e.g., lighting, privacy, ambient noise).
E. Conceptualization and Recommendations		
Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
- Lack of or misunderstanding of diagnostic formulation/case conceptualization - Inability to prepare basic reports which articulate theoretical material applied to case conceptualization.	- Basic knowledge of formulating diagnosis and case conceptualization. - Demonstrates the ability to discuss diagnostic formulation and case conceptualization. - Prepares basic reports which articulate theoretical material applied to case conceptualization.	- Utilizes systematic approaches of gathering data to inform clinical decision-making. - Presents cases and reports demonstrating how diagnosis is based on case material.

7. Intervention- Interventions designed to alleviate suffering and to promote health and well-being of individuals, groups, and/or organizations.

A. Intervention Planning - Formulates and conceptualizes cases and plan interventions utilizing at least one consistent theoretical orientation.

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
- Responses to clients indicate significant inadequacies in their theoretical understanding and case formulation. - Chooses interventions without regard to their scientific status or their alignment. - Chooses interventions without regard to their acceptability within the client's values. - Chooses interventions without regard to own relevant preparation/expertise.	- Basic understanding of the relationship between assessment and intervention. - Articulates a basic understanding of how intervention choices are informed by assessment.	- Formulates and conceptualizes cases and plan interventions utilizing at least one consistent theoretical orientation. - Articulates a theory of change and identifies interventions to implement change; as consistent with the AAPI. - Writes understandable case conceptualization reports and collaborative treatment plans incorporating evidence-based practices.

B. Intervention Implementation

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
Failure to target interventions to client's level of understanding and motivation.	- Basic knowledge of intervention strategies. - Articulates awareness of theoretical basis of intervention and some general strategies.	Implements evidence-based interventions that take into account empirical support, clinical judgment,

• Unable to apply interventions that are technically consistent with supervisory and literature-based guidance. • Applies interventions without regard to their scientific status. • Applies interventions without regard to their acceptability within the client's values. • Applies interventions without regard to own relevant preparation/expertise.		and client diversity (e.g., client characteristics, values, and context). • Applies specific evidence-based interventions. • Presents case that documents application of evidence-based practice.
C. Knowledge of Interventions - Knowledge of scientific, theoretical, empirical and contextual bases of intervention, including theory, research, and practice.		
Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Lacks self-initiative to request readings or resources used to inform treatment. • Fails to complete assigned readings associated. • Unable to articulate basic knowledge of scientific, theoretical, and contextual bases of evidence-based practice.	• Basic knowledge of scientific, theoretical, and contextual bases of intervention and basic knowledge of the value of evidence-based practice and its role in scientific psychology. • Articulates the relationship of EBP to the science of psychology. • Identifies basic strengths and weaknesses of intervention approaches for different problems and populations.	• Demonstrates knowledge of interventions and explanations for their use based on EBP. • Demonstrates the ability to select interventions for different problems and populations related to the practice setting. • Investigates existing literature related to problems and client issues. • Writes a statement of one's own theoretical perspective regarding intervention strategies.
D. Scientific Foundation of Professional Practice - Knowledge, understanding, and application of the concept of evidence-based practice.		
Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Fails to learn the evidence base associated with interventions and assessments • Considers interventions and assessments without regard to their scientific status • Considers interventions and assessments without regard to their acceptability within the client's values • Considers interventions and assessments without regard to the clinician's preparation/expertise	• Understanding the scientific foundation of professional practice • Understands the development of evidence based practice in psychology (EBP) as defined by APA • Displays understanding of the scientific foundations of the functional competencies • Cites scientific literature to support an argument • Evaluates scholarly literature on a practice-related topic	• Knowledge, understanding, and application of the concept of evidence-based practice • Applies EBP concepts in case conceptualization, treatment planning, and interventions • Compares and contrasts EBP approaches with other theoretical perspectives and interventions in the context of case conceptualization and treatment planning.

E. Progress Evaluation

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
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• Failure to appreciate the need for ongoing evaluation of working alliance and treatment effectiveness. • Unaware of methods to examine intervention outcomes. • Unable/Unwilling to perform assessments of intervention outcomes. • Persistent application of interventions that appear to be having deleterious effects.	• Basic knowledge of the assessment of intervention progress and outcome. • Demonstrates basic knowledge of methods to examine intervention outcomes.	• Evaluate treatment progress and modify treatment planning as indicated, utilizing established outcome measures. • Assesses and documents treatment progress and outcomes. • Alters treatment plan accordingly. • Describes instances of lack progress and actions taken in response.
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8. Supervision- Supervision and training in the professional knowledge base and of evaluation of the effectiveness of various professional activities.

A. Expectations and Roles – Knowledge of purpose for roles in supervision.

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• As a supervisee, is routinely unprepared for supervision (e.g., unsure what to discuss, fails to bring clinical materials for review). • As a supervisee, fails to disclose information that supervisor needs to understand clinical situations and/or accurately assess supervisee's training needs and performance.	• Basic knowledge of expectations for supervision. • Demonstrates knowledge of the process of supervision.	• Identifies roles and responsibilities of the supervisor and supervisee in the supervision process.

B. Processes and Procedures - Knowledge of procedures and processes of supervision.

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Does not demonstrate a basic understanding of supervision models and practice.	• Demonstrates basic knowledge of supervision models and practice.	• Identifies goals and tasks of supervision related to developmental progression. • Tracks progress achieving goals and setting new goals.

C. Awareness of Factors Affecting Quality – Knowledge about the impact of diversity on all professional settings and supervision Participants including self as defined by APA policy.

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Cannot differentiate appropriate and inappropriate responses to individual and cultural differences in supervision. • Lacks insight into how one's cultural identity may differentially affect supervisees.	• Basic knowledge of and sensitivity to issues related to individual and cultural differences (i.e., the APA definition) as they apply to the supervision process and relationships. • Demonstrates basic knowledge of literature on individual and cultural differences and engages in respectful interactions that reflect that knowledge.	• Beginning knowledge of personal contribution to therapy and to supervision. • Demonstrates knowledge of Individual and Cultural Differences (I&CD) literature and APA guidelines in supervision practice. • Demonstrates awareness of role of oppression and privilege on supervision process.

D. Participation in Supervision Process - Observation of and participation in supervisory process (e.g., peer supervision).

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Fails to provide candid and sensitive critical feedback to supervisee when needed. • Is a passive observer rather than actively engaged in group supervision • Discounts supervisee's perspective or opinion. • Does not seek supervision of supervision &/or supervisory consultation when needed. • Fails to adopt a genuine openness to feedback from supervisees as evident from developmental stagnation as a supervisor.	• Awareness of need for straightforward, truthful, and respectful communication in supervisory relationship. • Demonstrates willingness to admit errors, accept feedback.	• Reflects on supervision process, areas of strength and those needing improvement. • Seeks supervision to improve performance, presenting work for feedback, and integrating feedback into performance.
E. Ethical and Legal Issues in Supervision - Knowledge of and compliance with ethical/professional codes, standards and guidelines; institutional policies; laws, statutes, rules, regulations, and case law relevant to the practice of psychology and its supervision.		
Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Fails to recognize ethical issues in supervision. • Fails to recognize legal issues in supervision. • Cannot articulate institutional &/or APA principles as related to supervision. • Behaves unethically toward supervisee (e.g., verbally abusive, sexually objectifying).	• Knowledge of principles of ethical practice and basic skills in supervisory ethical decision making, knowledge of legal and regulatory issues in supervision. • Demonstrates understanding of this knowledge (e.g., APA 2002 ethical principles).	• Behaves ethically. • Recognizes ethical and legal issues in clinical practice and supervision.

9. Consultation and Intraprofessional / Interdisciplinary Skills- The ability to provide expert guidance or professional assistance in response to a client's needs or goals. Effectively relates to dialogue with other professionals. Knowledge of key issues and concepts in related disciplines. Identify and interact with professionals in multiple disciplines

A. Knowledge of the Shared and Distinctive Contributions of Other Professions.

Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
• Lacks awareness or understanding of basic contributions of allied professions such as social work, psychiatry	• Beginning, basic knowledge of the viewpoints and contributions of other professions/ professionals • Demonstrates knowledge, respect, and valuing of roles, functions and service delivery systems of other professions	• Awareness of multiple and differing worldviews, roles, professional standards, and contributions across contexts and systems • Intermediate level knowledge of common and distinctive roles of other professionals • Reports observations of commonality and differences among professional roles, values, and standards

B. Functioning in Multidisciplinary and Interdisciplinary Contexts.		
Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
Displays defensiveness when interacting with others	- Demonstrates cooperation - Demonstrates ability to cooperate with others in task completion	- Beginning, knowledge of strategies that promote interdisciplinary collaboration vs. multidisciplinary functioning - Demonstrates knowledge of the nature of interdisciplinary vs. multidisciplinary function and the skills that support interdisciplinary process
C. Understands How Participation in Interdisciplinary Collaboration/ Consultation Enhances Outcomes.		
Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
Does not practice reflective listening skills or dominates discussion with own ideas	- Knowledge of how participating in interdisciplinary collaboration/ consultation can be directed toward shared goals. - Demonstrates understanding of concept	- Participates in and initiates interdisciplinary collaboration/ consultation directed toward shared goals. - Consults with and cooperates with other disciplines in service of clients
D. Respectful and Productive Relationships with Individuals from Other Professions.		
Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
Makes dismissive or disparaging comments of other disciplines	- Awareness of the benefits of forming collaborative relationships with other professionals - Expresses interest in developing collaborative relationships and respect for other professionals	- Develops and maintains collaborative relationships and respect for other professionals - Communicates effectively with individuals from other profession
E. Role of Consultant		
Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
- Does not recognize multiple professional roles such as consultant. - Cannot distinguish among consultant roles - Does not discuss with or inform consultee of clinician's role in consultation.		- Knowledge of the consultant's role and its unique features as distinguished from other professional roles (such as therapist, supervisor, teacher). - Articulates common and distinctive roles of consultant. - Compares and contrast consultation, clinical and supervision roles.
F. Addressing Referral Question		
Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
Inappropriate or incorrect selection of assessment tools.		- Knowledge of and ability to select appropriate means of assessment to answer referral questions - Implements systematic approach to data collection in a consultative role. - Identifies sources and types of assessment tools.

G. Application of Methods -- Consultation		
Does Not Meet Expectations	Readiness for Practicum	Well Developed Competence
		• Identifies and acquires literature relevant to unique consultation methods (assessment & intervention) within systems, clients or settings. • Identifies appropriate interventions based on consultation assessment findings.

APPENDIX Q

Internship Level Competency Codebook

Competencies Codebook (Interns and Emerging Professionals)

I. PROFESSIONALISM

1. Professional Values and Attitudes: as evidenced in behavior and comportsment that reflect the values and attitudes of psychology.			
Does Not Meet Expectations	Pre-internship Level	Internship Level	Readiness For Entry To Practice
1A. Integrity - Honesty, personal responsibility and adherence to professional values			
Examples: - Failure to complete assigned or routine tasks during clinic on call hours - Failure to complete required trainings (e.g., HIPPA) and orientations	Understands professional values; honest, responsible Examples: - Demonstrates honesty, even in difficult situations - Takes responsibility for own actions - Demonstrates ethical behavior and basic knowledge of APA Ethical Principles and Code of Conduct	Adherence to professional values infuses work as psychologist-in-training; recognizes situations that challenge adherence to professional values Examples: - Identifies situations that challenge professional values, and seeks faculty/supervisor guidance as needed - Demonstrates ability to discuss failures and lapses in adherence to professional values with supervisors/faculty as appropriate	Monitors and independently resolves situations that challenge professional values and integrity Examples: - Takes independent action to correct situations that are in conflict with professional values - Addresses situations that challenge professional values
1B. Deportment			
Examples: - Not presenting for on call hours or other professional duties in professional attire with appropriate hygiene - Discussing client(s) information in public areas - Discussion of client(s) in unprofessional terms or with disrespect for individual differences	Understands how to conduct oneself in a professional manner Examples: - Demonstrates appropriate personal hygiene and attire - Distinguishes between appropriate and inappropriate language and demeanor in professional contexts	Communication and physical conduct (including attire) is professionally appropriate, across different settings Examples: - Demonstrates awareness of the impact behavior has on client, public and profession - Utilizes appropriate language and demeanor in professional communications	Conducts self in a professional manner across settings and situations Examples: - Verbal and nonverbal communications are appropriate to the professional context, including in challenging interactions - Flexibly shifts demeanor to effectively meet requirements of professional situation and enhance outcomes

Professional Values and Attitudes continued			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
1C. Accountability			
Examples: - Failure to complete required observation or client contact hours and notes or reports in a timely manner - Failure to maintain responsibility in scheduling rooms and/or resources both when initiating scheduling and when changes are needed	Accountable and reliable Examples: - Turns in assignments in accordance with established deadlines - Demonstrates personal organization skills - Plans and organizes own workload - Follows policies and procedures of institution - Follows through on commitments	Accepts responsibility for own actions Examples: - Completes required case documentation promptly and accurately - Accepts responsibility for meeting deadlines - Available when “on-call” - Acknowledges errors - Utilizes supervision to strengthen effectiveness of practice	Independently accepts personal responsibility across settings and contexts Examples: - Enhances own professional productivity - Holds self accountable for and submits to external review of quality service provision
1D. Concern for the Welfare of Others			
Examples: - Engaging in behaviors that compromise a safe environment emotionally or physically - Abandonment of client(s)	Demonstrates awareness of the need to uphold and protect the welfare of others Examples: - Displays initiative to help others - Articulates importance of concepts of confidentiality, privacy, and informed consent - Demonstrates compassion (awareness of suffering and the wish to relieve it) for others	Acts to understand and safeguard the welfare of others Examples: - Displays respect in interpersonal interactions with others including those from divergent perspectives or backgrounds - Determines when response to client needs takes precedence over personal needs	Independently acts to safeguard the welfare of others Examples: - Communications and actions convey sensitivity to individual experience and needs while retaining professional demeanor and deportment - Respectful of the beliefs and values of colleagues even when inconsistent with personal beliefs and values - Demonstrates compassion for others who are dissimilar from oneself, who express negative affect (e.g., hostility), and/or who seek care for proscribed behavior, such as violence, predation, or dangerousness
1E. Professional Identity			
Examples: - Cannot conduct an appropriate literature search - Fails to seek needed supervision when appropriate - Does not integrate supervision into care of client(s) - Failure to attend scheduled supervision (individually or on team)	Demonstrates beginning understanding of self as professional: “thinking like a psychologist” Examples: - Demonstrates knowledge of the program and profession (training model, core competencies) - Demonstrates knowledge about practicing within one’s competence	Displays emerging professional identity as psychologist; uses resources (e.g., supervision, literature) for professional development Examples: - Has membership in professional organizations - Attends colloquia, workshops, conferences - Consults literature relevant to client care	Displays consolidation of professional identity as a psychologist; demonstrates knowledge about issues central to the field; integrates science and practice Examples: Keeps up with advances in profession

2. Individual and Cultural Diversity: Awareness, sensitivity and skills in working professionally with diverse individuals, groups and communities who represent various cultural and personal background and characteristics defined broadly and consistent with APA policy.			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
2A. Self as Shaped by Individual and Cultural Diversity (e.g., cultural, individual, and role differences, including those based on age, gender, gender identity, race, ethnicity, culture, national origin, religion, sexual orientation, disability, language, and socioeconomic status) and Context			
Has difficulty recognizing prejudices	Demonstrates knowledge, awareness, and understanding of one's own dimensions of diversity and attitudes towards diverse others Examples: - Articulates how ethnic group values influence who one is and how one relates to other people - Articulates dimensions of diversity (e.g., race, gender, sexual orientation)	Monitors and applies knowledge of self as a cultural being in assessment, treatment, and consultation Examples: - Uses knowledge of self to monitor effectiveness as a professional - Initiates supervision about diversity issues	Independently monitors and applies knowledge of self as a cultural being in assessment, treatment, and consultation Examples: - Uses knowledge of self to monitor and improve effectiveness as a professional - Seeks consultation or supervision when uncertain about diversity issues
2B. Others as Shaped by Individual and Cultural Diversity and Context			
Does not talk about inexperience or lack of exposure to individuals/groups from different backgrounds	Demonstrates knowledge, awareness, and understanding of other individuals as cultural beings Examples: - Demonstrates knowledge, awareness and understanding of the way culture and context shape the behavior of other individuals - Articulates beginning understanding of the way culture and context are a consideration in working with clients	Applies knowledge of others as cultural beings in assessment, treatment, and consultation Examples: - Demonstrates understanding that others may have multiple cultural identities - Initiates supervision about diversity issues with others	Independently monitors and applies knowledge of others as cultural beings in assessment, treatment, and consultation Examples: - Uses knowledge of others to monitor and improve effectiveness as a professional - Seeks consultation or supervision when uncertain about diversity issues with others
2C. Interaction of Self and Others as Shaped by Individual and Cultural Diversity and Context			
Holds some beliefs which limit effectiveness with certain clients	Demonstrates knowledge, awareness, and understanding of interactions between self and diverse others Examples: - Demonstrates knowledge, awareness and understanding of the way culture and context shape interactions between and among individuals - Articulates beginning understanding of the way culture and context are a consideration in the therapeutic relationship	Applies knowledge of the role of culture in interactions in assessment, treatment, and consultation of diverse others Examples: - Understands the role that diversity may play in interactions with others - Initiates supervision about diversity issues in interactions with others	Independently monitors and applies knowledge of diversity in others as cultural beings in assessment, treatment, and consultation Examples: - Uses knowledge the role of culture in interactions to monitor and improve effectiveness as a professional - Seeks consultation or supervision when uncertain about diversity issues in interactions with others

Individual and Cultural Diversity continued			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
2D. Applications based on Individual and Cultural Context			
Lacks insight into impact of personal experiences on case conceptualization, choice of treatment, etc.	Demonstrates basic knowledge of and sensitivity to the scientific, theoretical, and contextual issues related to ICD (as defined by APA policy) as they apply to professional psychology. Understands the need to consider ICD issues in all aspects of professional psychology work (e.g., assessment, treatment, research, relationships with colleagues) Examples: - Demonstrates basic knowledge of literatures on individual and cultural differences and engages in respectful interactions that reflect this knowledge - Seeks out literature on individual and cultural differences to inform interactions with diverse others	Applies knowledge, sensitivity, and understanding regarding ICD issues to work effectively with diverse others in assessment, treatment, and consultation Examples: - Demonstrates knowledge of ICD literature and APA policies, including guidelines for practice with diverse individuals, groups and communities - Works effectively with diverse others in professional activities - Demonstrates awareness of effects of oppression and privilege on self and others	Applies knowledge, skills, and attitudes regarding dimensions of diversity to professional work Examples: - Adapts professional behavior in a manner that is sensitive and appropriate to the needs of diverse others - Articulates and uses alternative and culturally appropriate repertoire of skills and techniques and behaviors - Seeks consultation regarding addressing individual and cultural diversity as needed - Uses culturally relevant best practices

3. Ethical Legal Standards and Policy: Application of ethical concepts and awareness of legal issues regarding professional activities with individuals, groups, and organizations.			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
3A. Knowledge of Ethical, Legal and Professional Standards and Guidelines			
• Unable to locate the APA Ethical Code. • Unable to identify which code is currently in place (e.g., which year of publication represents the current code). • Unable to identify additional codes that are salient to professional practice (e.g, child custody, forensic, and so on)	Demonstrates basic knowledge of the principles of the APA Ethical Principles and Code of Conduct [ethical practice and basic skills in ethical decision making]; demonstrates beginning level knowledge of legal and regulatory issues in the practice of psychology that apply to practice while placed at practicum setting Examples: • Demonstrates beginning knowledge of typical legal issues, including child and elder abuse reporting, confidentiality, and informed consent • Identifies key documents/policies that guide the practice of psychology (e.g., APA Ethical Principles and Code of Conduct)	Demonstrates intermediate level knowledge and understanding of the APA Ethical Principles and Code of Conduct and other relevant ethical/professional codes, standards and guidelines, laws, statutes, rules, and regulations Examples: • Identifies ethical dilemmas effectively • Actively consults with supervisor to act upon ethical and legal aspects of practice • Addresses ethical and legal aspects within the case conceptualization • Discusses ethical implications of professional work • Recognizes and discusses limits of own ethical and legal knowledge • Demonstrates intermediate knowledge of typical legal issues, including child and elder abuse reporting, confidentiality, and informed consent	Demonstrates advanced knowledge and application of the APA Ethical Principles and Code of Conduct and other relevant ethical, legal and professional standards and guidelines Examples: • Addresses complex ethical and legal issues • Articulates potential conflicts in complex ethical and legal issues. • Seeks to prevent problems and unprofessional conduct • Demonstrates advanced knowledge of typical legal issues, including child and elder abuse reporting, HIPAA,, confidentiality, and informed consent
3B. Awareness and Application of Ethical Decision Making			
• Unable to identify, with the narrative code available, applicable sections to an ethical decision making dilemma. • Failure to identify appropriate means and resources for obtaining consultation and/or supervision when facing an ethical decision making dilemma	Demonstrates awareness of the importance of applying an ethical decision model to practice Examples: • Recognizes the importance of basic ethical concepts applicable in initial practice (e.g. child abuse reporting, Informed consent, confidentiality, multiple relationships, and competence) • Demonstrates awareness of an ethical decision making model applied to case vignettes	Demonstrates knowledge and application of an ethical decision-making model; applies relevant elements of ethical decision making to a dilemma Examples: • Uses an ethical decision-making model when discussing cases in supervision • Identifies ethical implications in cases and understands the ethical elements present in ethical dilemma or question • Discusses ethical dilemmas and decision making in supervision, staff meetings, presentations, practicum settings	Independently utilizes an ethical decision-making model in professional work Examples: • Applies applicable ethical principles and standards in professional writings and presentations • Applies applicable ethics concepts in research design and subject treatment • Applies ethics and professional concepts in teaching and training activities • Develops strategies to seek consultation regarding complex ethical and legal dilemmas • Takes appropriate steps when others behave unprofessionally • Identifies potential conflicts between personal belief systems, APA Ethics Code and legal issues in practice

Ethical Legal Standards and Policy continued			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
3C. Ethical Conduct			
- Failure to seek consultation before responding to an ethical decision making dilemma - Enacts conduct that is inconsistent with supervisor mandates regarding ethical conduct, without first seeking consultation from Clinic Director	Displays ethical attitudes and values Examples: - Evidences desire to help others - Shows honesty and integrity; values ethical behavior - Demonstrates personal courage consistent with ethical values of psychologists - Displays appropriate boundary management	Integrates own moral principles/ethical values in professional conduct Examples: - Is able to articulate knowledge of own moral principles and ethical values in discussions with supervisors and peers about ethical issues - Is able to spontaneously discusses intersection of personal and professional ethical and moral issues	Independently integrates ethical and legal standards with all competencies Examples: - Demonstrates adherence to ethical and legal standards in professional activities - Takes responsibility for continuing professional development
4. Reflective Practice/Self-Assessment/Self-Care: Practice conducted with personal and professional self-awareness and reflection; with awareness of competencies; with appropriate self-care.			
4A. Reflective Practice			
- Does not articulate attitudes, values, and beliefs that guide behavior and decision-making - Fails to acknowledge impact of self on others - Does not use introspection to monitor the effect of personal issues on professional work and relationships - Failure to foster development of individual and cultural identities - Does not use supervision as an opportunity to facilitate self-awareness as related to professional performance, roles, and responsibilities	Displays basic mindfulness and self-awareness; engages in reflection regarding professional practice Examples: Demonstrates openness to: - Considering own personal concerns and issues - Recognizing impact of self on others - Articulating attitudes, values, and beliefs toward diverse others - Self-identifying multiple individual and cultural identities - Systematically reviewing own professional performance with supervisors/teachers	Displays broadened self-awareness; utilizes self-monitoring; engages in reflection regarding professional practice; uses resources to enhance reflectivity Examples: - Is able to articulate attitudes, values, and beliefs toward diverse others - Recognizes impact of self on others - Self-identifies multiple individual and cultural identities - Is able to describe how others experience him/her and identifies roles one might play within a group - Responsively utilizes supervision to enhance reflectivity - Reviews own professional performance via video or audiotape with supervisors - Displays ability to adjust professional performance as situation requires.	Demonstrates reflectivity both during and after professional activity; acts upon reflection; uses self as a therapeutic tool Examples: - Monitors and evaluates attitudes, values and beliefs towards diverse others - Systematically and effectively monitors and adjusts professional performance in action as situation requires - Recognizes and addresses own problems, minimizing interference with competent professional functioning

Reflective Practice/Self-Assessment/Self-Care continued			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
4B. Self-Assessment			
• Fails to recognize professional limitations, 'blindspots' & mistakes • Fails to recognize strengths • Despite input from others, does not develop specific and realistic professional goals • Cannot describe an adequate subset of core competencies for scientist-practitioners • Does not devote adequate time to self-evaluation and review of professional performance	Demonstrates knowledge of core competencies; engages in initial self-assessment re: competencies Examples: • Demonstrates awareness of competencies for professional training • Develops initial competency goals for early training (with input from faculty)	Demonstrates broad, accurate self-assessment of competence; consistently monitors and evaluates practice activities; works to recognize limits of knowledge/skills, and to seek means to enhance knowledge/skills Examples: • Self-assessment comes close to congruence with assessment by peers and supervisors • Identifies areas requiring further professional growth • Writes a personal statement of professional goals • Identifies learning objectives for overall training plan • Systemically and effectively reviews own professional performance via videotape or other technology	Accurately self-assesses competence in all competency domains; integrates self-assessment in practice; recognizes limits of knowledge/skills and acts to address them; has extended plan to enhance knowledge/skills Examples: • Accurately identifies level of competence across all competency domains • Accurately assesses own strengths and weaknesses and seeks to prevent or ameliorate impact on professional functioning • Recognizes when new/improved competencies are required for effective practice • Writes a personal statement of professional goals identifying areas for further professional growth, with extended plans to achieving the goals
4C. Self-Care (attention to personal health and well-being to assure effective professional functioning)			
• Lacks awareness of personal & professional values and priorities • Fails to balance personal & professional commitments in a manner consistent with ones values & priorities • Relies on ineffective &/or maladaptive coping (e.g., puts physical health or important relationships at risk) • Denies personal vulnerabilities • Fails to seek help/advice for personal difficulties with the potential to impair professional functioning	Understands the importance of self-care in effective practice; demonstrates knowledge of self-care methods; attends to self-care Examples: • Articulates benefits of engaging in self-care • Makes use of opportunities to engage in self-care	Monitors issues related to self-care with supervisor; understands the central role of self-care to effective practice Examples: • Takes action recommended by supervisor for self-care to ensure effective training • Maintains/alters weekly schedule to allow for self care activities	Self-monitors issues related to self-care and promptly intervenes when disruptions occur Examples: • Anticipates and self-identifies disruptions in functioning and intervenes at an early stage/with minimal support from supervisors

Reflective Practice/Self-Assessment/Self-Care continued			
Does Not Meet Expectations	Baseline Competence	Readiness For Internship	Readiness For Entry To Practice
4D. Participation in Supervision Process			
• Fails to provide candid and sensitive critical feedback to supervisee when needed. • Is a passive observer rather than actively engaged in group supervision • Discounts supervisee's perspective or opinion. • Does not seek supervision of supervision &/or supervisory consultation when needed. • Fails to adopt a genuine openness to feedback from supervisees as evident from developmental stagnation as a supervisor.	Demonstrates straightforward, truthful, and respectful communication in supervisory relationship Examples: • Demonstrates willingness to admit errors and accept feedback • Acknowledges supervisor's differing viewpoints in supervision	Effectively participates in supervision Examples: • Seeks supervision to improve performance; presents work for feedback, and integrates feedback into performance • Initiates discussion with supervisor of own reaction to client/patients in session • Seeks supervisor's perspective on client progress	Independently seeks supervision when needed Examples: • Seeks supervision when personal problems may interfere with professional activities • Seeks supervision when working with client problems for which he/she has had limited experience to ensure competence of services

II. RELATIONAL

5. Relationships: Relate effectively and meaningfully with individuals, groups, and/or communities.			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
5A. Interpersonal Relationships			
• Limited or poor interpersonal relationships • Fails to listen/attend to others • Unable to see or dismisses alternative views & values • Unreliable, inconsistent • Blames others; does not take responsibility for own errors • Repeated boundary violations or excessive distance in therapeutic/supervisory relationship • Does not prepare for supervision or seek assistance	Displays interpersonal skills Examples: • Listens and is empathic with others • Respects and shows interest in others' cultures, experiences, values, points of view, goals and desires, fears, etc. • Demonstrates interpersonal skills verbally and non-verbally • Receives feedback • Works cooperatively and collaboratively with peers	Forms and maintains productive and respectful relationships with clients, peers/colleagues, supervisors and professionals from other disciplines Examples: • Forms effective working alliances with most clients • Engages with supervisors to work effectively • Involved in departmental, institutional, or professional activities or governance • Demonstrates respectful and collegial interactions with those who have different professional models or perspectives	Develops and maintains effective relationships with a wide range of clients, colleagues, organizations and communities Examples: • Effectively negotiates conflictual, difficult and complex relationships including those with individuals and groups that differ significantly from oneself • Maintains satisfactory interpersonal relationships with clients, peers, faculty, allied professionals, and the public

Relationships continued			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
5B. Affective Skills			
• Frequent conflict with others OR avoids conflict • Doesn't challenge or confront clients when appropriate • Immature, superficial or distant relationships • Poor working alliance with clients • Becomes angry or overly upset by negative feedback • Insensitive, over critical feedback or resistant to providing constructive criticism to peers • Avoids introspection; lacks awareness of own role in interactions	Displays affective skills Examples: • Demonstrates affect tolerance • Tolerates interpersonal conflict • Demonstrates awareness of inner emotional experience • Demonstrates emotional maturity • Listens to and acknowledges feedback from others • Notices and expresses feelings • Demonstrates comfort with a range of emotions • Affect does not overwhelm judgment • Is flexible when things don't go according to plan	Negotiates differences and handles conflict satisfactorily; provides effective feedback to others and receives feedback non-defensively Examples: • Demonstrates active problem-solving • Makes appropriate disclosures regarding problematic interpersonal situations • Acknowledges own role in difficult interactions • Initiates discussion regarding disagreements with colleagues or supervisors • Efforts to resolve disagreements do not escalate negative affect among the parties involved • Seeks clarification in challenging interpersonal communications • Demonstrates understanding of diverse viewpoints in challenging interactions • Provides feedback to supervisor regarding supervisory process • Provides feedback to peers regarding peers' clinical work in context of group supervision or case conference • Accepts and implements supervisory feedback nondefensively • Maintains affective equilibrium and focus on therapeutic task in face of client distress • Tolerates ambiguity and uncertainty	Manages difficult communication; possesses advanced interpersonal skills Examples: • Accepts, evaluates and implements feedback from others • Uses affective reactions in the service of resolving disagreements or fostering growth in others • Tolerates patient's feelings, attitudes, and wishes, particularly as they are expressed toward the therapist, so as to maintain and/or promote therapeutic dialogue • Allows, enables, and facilitates the patient's exploration and expression of affectively difficult issues • Works flexibly with patients' intense affects which could destabilize the therapeutic relationship

Relationships continued			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
5C. Expressive Skills			
• Uses language that is vague or contains too much jargon • Unable to articulate e.g. stumbles, incomplete sentences/ideas, rambles too much or overly brief	Communicates ideas, feelings, and information clearly using verbal, nonverbal, and written skills Examples: • Written work is organized, easy to understand, and conveys the main points • Shares opinions with others using language that others can understand • Non-verbal behavior is consistent with verbal communications	Communicates clearly using verbal, nonverbal, and written skills in a professional context; demonstrates clear understanding and use of professional language Examples: • Uses professional terms and concepts appropriately and clearly in discussions, case reports, etc. • Understands terms and concepts used in professional texts and in others' case reports • Communication is understandable, consistent across expressive modalities • Prepares clearly written assessment reports • Presents clinical process to supervisor in a succinct, organized, well-summarized way • Provides verbal feedback to client regarding assessment and diagnosis using language the client can understand • Presents clear, appropriately detailed clinical material	Verbal, nonverbal, and written communications are informative, articulate, succinct, sophisticated, and well-integrated; demonstrate thorough grasp of professional language and concepts Examples: • Demonstrates descriptive, understandable command of language, both written and verbal • Communicates clearly and effectively with clients • Uses appropriate professional language when dialoguing with other healthcare providers • Prepares sophisticated and compelling case reports • Treatment summaries are concise, yet comprehensive

III. SCIENCE

6. Scientific Knowledge and Methods: Understanding of research, research methodology, techniques of data collection and analysis, biological bases of behavior, cognitive-affective bases of behavior, and development across the lifespan. Respect for scientifically derived knowledge.			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
6A. Scientific Mindedness			
- Passive acceptance of published literature as truth - Unable/Unwilling to present own research in multiple modalities (e.g., written, oral)	Displays critical scientific thinking Examples: - Questions assumptions of knowledge - Evaluates study methodology and scientific basis of findings - Presents own work for the scrutiny of others	Values and applies scientific methods to professional practice Examples: - Uses literature to support ideas in case conferences and supervision - Formulates appropriate questions regarding case conceptualization - Generates hypotheses regarding own contribution to therapeutic process and outcome	Independently applies scientific methods to practice Examples: - Independently accesses and applies scientific knowledge and skills appropriately to the solution of problems - Implements appropriate methodology to address research questions
6B. Scientific Foundation of Psychology			
- Unable to articulate the scientific method - Applies illogical steps in place of scientific reasoning and evaluation	Demonstrates understanding of psychology as a science Examples: - Demonstrates understanding of core scientific conceptualizations of human behavior - Demonstrates basic knowledge of the breadth of scientific psychology - Cites scientific literature to support an argument when appropriate - Evaluates scholarly literature on a topic as needed	Demonstrates intermediate level knowledge of core science (i.e., scientific bases of behavior) Examples: - Critically evaluates scientific literature - Demonstrates understanding of intersections across core areas of psychological science	Demonstrates advanced level knowledge of core science (i.e., scientific bases of behavior) Examples: - Accurately evaluates scientific literature regarding clinical issues - Identifies multiple factors and interactions of those factors that underlie pathological behavior

Scientific Knowledge and Methods continued			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
6C. Scientific Foundation of Professional Practice			
• Fails to learn the evidence base associated with interventions and assessments • Considers interventions and assessments without regard to their scientific status • Considers interventions and assessments without regard to their acceptability within the client's values • Considers interventions and assessments without regard to the clinician's preparation/expertise	Understands the scientific foundation of professional practice Examples: • Understands the development of evidence based practice in psychology (EBP) as defined by APA • Displays understanding of the scientific foundations of the competencies • Cites scientific literature to support an argument when appropriate • Evaluates scholarly literature on a practice-related topic as needed	Demonstrates knowledge, understanding, and application of the concept of evidence-based practice Examples: • Applies EBP concepts in case conceptualization, treatment planning, and interventions in consultation with supervisor • Works with supervisor to compare and contrast EBP approaches with other theoretical perspectives and interventions in the context of case conceptualization and treatment	Independently applies knowledge and understanding of scientific foundations independently applied to practice Examples: • Reviews scholarly literature related to clinical work and applies knowledge to case conceptualization • Independently applies EBP concepts in practice • Independently compares and contrasts EBP approaches with other theoretical perspectives and interventions in the context of case conceptualization and treatment planning
7. Research/Evaluation: Generating research that contributes to the professional knowledge base and/or evaluates the effectiveness of various professional activities			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
7A. Scientific Approach to Knowledge Generation			
	Participates effectively in scientific endeavors when available Examples: • Demonstrates understanding that psychologists evaluate the effectiveness of their professional activities • Open to scrutiny of one's work by peers and faculty • Writes literature review as part of course requirement • Assists faculty with research projects	Demonstrates development of skills and habits in seeking, applying, and evaluating theoretical and research knowledge relevant to the practice of psychology Examples: • Demonstrates understanding of research methods and techniques of data analysis • Demonstrates research and scholarly activity, which may include presentations at conferences; participation in research teams; submission of manuscripts for publication • Demonstrates being a critical consumer of research	Generates knowledge Examples: • Engages in systematic efforts to increase the knowledge base of psychology through implementing and reviewing research • Uses methods appropriate to the research question, setting and/or community • Consults and partners with community stakeholders when conducting research in diverse communities

Research/Evaluation continued			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
7B. Application of Scientific Method to Practice			
	No expectation at this level	Demonstrates knowledge of application of scientific methods to evaluating practices, interventions, and programs Examples: • Describes how outcomes are measured in each practice activity • Demonstrates knowledge of program evaluation	Applies scientific methods of evaluating practices, interventions, and programs Examples: • Evaluates practice activities using accepted techniques • Compiles and analyzes data on own clients (outcome measurement) • Uses findings from outcome evaluation to alter intervention strategies as indicated • Participates in program evaluation

IV. APPLICATION

8. Evidence-Based Practice: Integration of research and clinical expertise in the context of patient factors.			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
8A. Knowledge and Application of Evidence-Based Practice			
• Lacks self-initiative to request readings or resources used to inform treatment. • Fails to complete assigned readings associated. • Unable to articulate basic knowledge of scientific, theoretical, and contextual bases of evidence-based practice.	Demonstrates basic knowledge of scientific, theoretical, and contextual bases of assessment, intervention and other psychological applications; demonstrates basic knowledge of the value of evidence-based practice and its role in scientific psychology Examples: • Articulates the relationship of EBP to the science of psychology • Identifies basic strengths and weaknesses of different assessment and intervention approaches	Applies knowledge of evidence-based practice, including empirical bases of assessment, intervention, and other psychological applications, clinical expertise, and client preferences Examples: • Demonstrates knowledge of interventions and explanations for their use based on EBP • Demonstrates the ability to select interventions, assessment tools, and consultation methods for different problems and populations related to the practice setting • Investigates existing literature related to problems and client issues • Writes a statement of own theoretical perspective regarding intervention strategies • Creates a treatment plan that reflects successful integration of empirical findings, clinical judgment, and client preferences in consultation with supervisor	Independently applies knowledge of evidence-based practice, including empirical bases of assessment, intervention, and other psychological applications, clinical expertise, and client preferences Examples: • Writes a case summary incorporating evidence-based practice • Presents rationale for intervention strategy that includes empirical support • Independently creates a treatment plan that reflects successful integration of empirical findings, clinical judgment, and client preferences

9. Assessment: Assessment and diagnosis of problems, capabilities and issues associated with individuals, groups, and/or organizations.			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
9A. Knowledge of Measurement and Psychometrics			
• Lack of, or misunderstanding of, the scientific, theoretical, and contextual basis of test construction. • Lack of, or misunderstanding of, basic psychometric constructs.	Demonstrates basic knowledge of the scientific, theoretical, and contextual basis of test construction and interviewing Examples: • Demonstrates awareness of the benefits and limitations of standardized assessment • Demonstrates knowledge of the construct(s) being assessed • Evidences understanding of basic psychometric constructs such as validity, reliability, and test construction	Selects assessment measures with attention to issues of reliability and validity Examples: • Identifies appropriate assessment measures for cases seen at practice site • Consults with supervisor regarding selection of assessment measures	Independently selects and implements multiple methods and means of evaluation in ways that are responsive to and respectful of diverse individuals, couples, families, and groups and context Examples: • Demonstrates awareness and competent use of culturally sensitive instruments, norms • Seeks consultation as needed to guide assessment • Describes limitations of assessment data reflected in assessment reports
9B. Knowledge of Assessment Methods			
• Lack of or misunderstanding of the scientific, theoretical, and contextual basis of evaluation methodologies. • Failure to check out successfully (with supervisor or their designee) on standardized assessment measures within two attempts.	Demonstrates basic knowledge of administration and scoring of traditional assessment measures, models and techniques, including clinical interviewing and mental status exam Examples: • Accurately administers and scores various assessment tools in non-clinical (e.g. course) contexts • Demonstrates knowledge of initial interviewing methods (both structured and semi-structured interviews, mini-mental status exam)	Demonstrates awareness of the strengths and limitations of administration, scoring and interpretation of traditional assessment measures as well as related technological advances Examples: • Demonstrates intermediate level ability to accurately select, administer, score and interpret assessment tools with client populations • Collects accurate and relevant data from structured and semi-structured interviews and mini-mental status exams	Independently understands the strengths and limitations of diagnostic approaches and interpretation of results from multiple measures for diagnosis and treatment planning Examples: • Independently and accurately selects, administers, and scores and interprets assessment tools with clinical populations • Selection of assessment tools reflects a flexible approach to answering the diagnostic questions • Comprehensive reports include discussion of strengths and limitations of assessment measures as appropriate • Interview and report lead to formulation of a diagnosis and the development of appropriate treatment plan

Assessment continued			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
9C. Application of Assessment Methods			
- Lack of, or misunderstanding of, the scientific, theoretical, and contextual basis of the correct application of evaluation methodologies. - Cannot articulate to supervisor the multi-trait, multi-method matrix approach to assessment. - Proposes to supervisor an assessment battery that attends only to confirming hypotheses (e.g., hypothesizes ADHD and proposes a battery that fails to consider other possible causes for problems with attention/concentration).	Demonstrates knowledge of measurement across domains of functioning and practice settings Examples: - Demonstrates awareness of need to base diagnosis and assessment on multiple sources of information - Demonstrates awareness of need for selection of assessment measures appropriate to population/problem	Selects appropriate assessment measures to answer diagnostic question Examples: - Selects assessment tools that reflect awareness of patient population served at a given practice site - Demonstrates ability to adapt environment and materials according to client needs (e.g., lighting, privacy, ambient noise)	Independently selects and administers a variety of assessment tools and integrates results to accurately evaluate presenting question appropriate to the practice site and broad area of practice Examples: - Independently selects assessment tools that reflect awareness of client population served at practice site - Interprets assessment results accurately taking into account limitations of the evaluation method - Provides meaningful, understandable and useful feedback that is responsive to client need
9D. Diagnosis			
- Lack of basic knowledge of range of normal and abnormal behavior. - Lack of or misunderstanding of DSM criteria and system.	Demonstrates basic knowledge regarding the range of normal and abnormal behavior in the context of stages of human development and diversity Examples: - Identifies DSM criteria - Describes normal development consistent with broad area of training	Applies concepts of normal/abnormal behavior to case formulation and diagnosis in the context of stages of human development and diversity Examples: - Articulates relevant developmental features and clinical symptoms as applied to presenting question - Demonstrates ability to identify problem areas and to use concepts of differential diagnosis	Utilizes case formulation and diagnosis for intervention planning in the context of stages of human development and diversity Examples: - Treatment plans incorporate relevant developmental features and clinical symptoms as applied to presenting problem - Demonstrates awareness of DSM and relation to ICD codes - Independently identifies problem areas and makes a diagnosis

Assessment continued			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
9E. Conceptualization and Recommendations			
- Lack of or misunderstanding of diagnostic formulation/case conceptualization - Inability to prepare basic reports which articulate theoretical material applied to case conceptualization.	Demonstrates basic knowledge of formulating diagnosis and case conceptualization Examples: - Discusses diagnostic formulation and case conceptualization in courses - Prepares basic reports which articulate theoretical material	Utilizes systematic approaches of gathering data to inform clinical decision-making Examples: - Presents cases and reports demonstrating how diagnosis is based on case material - Makes clinical decisions based on connections between diagnoses, hypotheses and recommendations	Independently and accurately conceptualizes the multiple dimensions of the case based on the results of assessment Examples: - Independently prepares reports based on case material - Accurately administers, scores and interprets test results - Formulates case conceptualizations incorporating theory and case material
9F. Communication of Assessment Findings			
- Does not understand models of report writing and progress notes - Is not able to accomplish intake report within specified time constraints - Does not complete written assessment evaluation within time specified	Demonstrates awareness of models of report writing and progress notes Examples: - Demonstrates knowledge of content of test reports and progress notes - Demonstrates knowledge of organization of test reports and progress notes	Writes assessment reports and progress notes and communicates assessment findings verbally to client Examples: - Writes complete psychological reports - Works with supervisor to prepare and provide feedback regarding findings - Reports reflect data that has been collected via interview	Communicates results in written and verbal form clearly, constructively, and accurately in a conceptually appropriate manner Examples: - Writes an effective, comprehensive report - Effectively communicates assessment results verbally to clients - Reports reflect data that has been collected via interview and its limitations
10. Intervention: Interventions designed to alleviate suffering and to promote health and well-being of individuals, groups, and/or organizations.			
10A. Intervention planning			
- Responses to clients indicate significant inadequacies in their theoretical understanding and case formulation. - Chooses interventions without regard to their scientific status or their alignment. - Chooses interventions without regard to their acceptability within the client's values. - Chooses interventions without regard to own relevant preparation/expertise.	Displays basic understanding of the relationship between assessment and intervention Examples: - Articulates a basic understanding of how intervention choices are informed by assessment (e.g., clinical intake, testing) - Articulates a basic understanding of how assessment guides the process of intervention	Formulates and conceptualizes cases and plans interventions utilizing at least one consistent theoretical orientation Examples: - Articulates a theory of change and identifies interventions to implement change, as consistent with the AAPI - Writes case conceptualization reports and collaborative treatment plans incorporating evidence-based practices	Independently plans interventions; case conceptualizations and intervention plans are specific to case and context Examples: - Accurately assesses presenting issues taking in to account the larger life context, including diversity issues - Conceptualizes cases independently and accurately - Independently selects intervention(s) appropriate for the presenting issue(s)

Intervention continued			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
10B. Skills			
• Has difficulty establishing rapport. • Frequently shows lack of confidence. • Misses or misperceives important information/themes presented by client. • Failure to identify any goals. • Lacks awareness of underlying problems. • Unable to demonstrate empathy and caring such that most clients become willing to trust and utilize therapeutic assistance.	Displays basic helping skills Examples: • Demonstrates helping skills, such as empathic listening, framing problems • Uses non-verbal communication such as eye-contact and body positioning with clients to convey interest and concern	Displays clinical skills Examples: • Develops rapport with clients • Develops therapeutic relationships • Demonstrates appropriate judgment about when to consult supervisor	Displays clinical skills with a wide variety of clients and uses good judgment even in unexpected or difficult situations Examples: • Develops rapport and relationships with wide variety of clients • Uses good judgment about unexpected issues, such as crises, use of supervision, confrontation • Effectively delivers intervention
10C. Intervention Implementation			
• Failure to target interventions to client's level of understanding and motivation. • Unable to apply interventions that are technically consistent with supervisory and literature-based guidance. • Applies interventions without regard to their scientific status. • Applies interventions without regard to their acceptability within the client's values. • Applies interventions without regard to own relevant preparation/expertise.	Demonstrates basic knowledge of intervention strategies Examples: • Is able to articulate awareness of theoretical basis of intervention and some general strategies • Is able to articulate awareness of the concept of evidence-based practice	Implements evidence-based interventions Examples: • Case presentations demonstrate application of evidence-based practice • Discusses evidence based practices during supervision	Implements interventions with fidelity to empirical models and flexibility to adapt where appropriate Examples: • Independently and effectively implements a typical range of intervention strategies appropriate to practice setting • Independently recognizes and manages special circumstances • Terminates treatment successfully • Collaborates effectively with other providers or systems of care
10D. Progress Evaluation			
• Failure to appreciate the need for ongoing evaluation of working alliance and treatment effectiveness. • Unaware of methods to examine intervention outcomes. • Unable/Unwilling to perform assessments of intervention outcomes. • Persistent application of interventions that appear to be having deleterious effects.	Demonstrates basic knowledge of the assessment of intervention progress and outcome Examples: • Identifies measures of treatment progress and outcome by name • Is able to articulate an understanding of the use of repeated assessment to guide treatment • Appropriately administers and scores treatment progress and outcome measures	Evaluates treatment progress and modifies treatment planning as indicated, utilizing established outcome measures Examples: • Describes instances of lack of progress and actions taken in response • Demonstrates ability to evaluate treatment progress in context of evidence based interventions	Independently evaluates treatment progress and modifies planning as indicated, even in the absence of established outcome measures Examples: • Critically evaluates own performance in the treatment role • Seeks consultation when necessary

11. Consultation: The ability to provide expert guidance or professional assistance in response to a client's needs or goals.			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
11A. Role of Consultant			
- Does not recognize multiple professional roles such as consultant. - Cannot distinguish among consultant roles - Does not discuss with or inform consultee of clinician's role in consultation.	No expectation at this level	Demonstrates knowledge of the consultant's role and its unique features as distinguished from other professional roles (such as therapist, supervisor, teacher) Examples: - Is able to compare and contrast consultation, clinical, and supervision roles - Is able to describe a consultant's role in a hypothetical professional activity	Determines situations that require different role functions and shifts roles accordingly to meet referral needs Examples: - Is able to articulate different forms of consultation (e.g., mental health, educational, systems, advocacy) - Accurately matches professional role function to situation
11B. Addressing Referral Question			
Inappropriate or incorrect selection of assessment tools.	No expectation at this level	Demonstrates knowledge of and ability to select appropriate means of assessment to answer referral questions Examples: - Implements systematic approach to data collection in a consultative role - Identifies sources and types of assessment tools	Demonstrates knowledge of and ability to select appropriate and contextually sensitive means of assessment/data gathering that answers consultation referral question Examples: - Demonstrates ability to gather information necessary to answer referral question - Clarifies and refines referral question based on analysis/assessment of question
11C. Communication of Consultation Findings			
Poor written or verbal feedback to consultees.	No expectation at this level	Identifies literature and knowledge about process of informing consultee of assessment findings Examples: - Identifies appropriate approaches and processes for providing written and verbal feedback and recommendations to consultee - Carries out a mock presentation of findings	Applies knowledge to provide effective assessment feedback and to articulate appropriate recommendations Examples: - Prepares clear, useful consultation reports and recommendations to all appropriate parties - Provides verbal feedback to consultee of results and offers appropriate recommendations

Consultation continued			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
11D. Application of Consultation Methods			
	No expectation at this level	Identifies literature relevant to consultation methods (assessment and intervention) within systems, clients, or settings Examples: • Identifies appropriate interventions based on consultation assessment findings • Demonstrates ability to identify collaborative methods across systems, clients, or settings	Applies literature to provide effective consultative services (assessment and intervention) in most routine and some complex cases Examples: • Identifies and implements consultation interventions based on assessment findings • Identifies and implements consultation interventions that meet consultee goals

V. EDUCATION

12. Teaching: Providing instruction, disseminating knowledge, and evaluating acquisition of knowledge and skill in professional psychology.			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
12A. Knowledge			
	No expectation at this level	Demonstrates awareness of theories of learning and how they impact teaching Examples: • Observes differences in teaching styles and need for response to different learning skills • Is able to articulate awareness of body of knowledge to inform teaching and learning	Demonstrates knowledge of didactic learning strategies and how to accommodate developmental and individual differences Examples: • Demonstrates knowledge of one learning strategy • Demonstrates clear communication skills
12B. Skills			
	No expectation at this level	Demonstrates knowledge of application of teaching methods Examples: • Demonstrates example of application of teaching method • Organizes and presents information related to a topic	Applies teaching methods in multiple settings Examples: • Identifies and differentiates factors for implementing particular teaching methods • Demonstrates accommodation to diverse others (e.g., cultural, individual, and role differences, including those based on age, gender, gender identity, race, ethnicity, culture, national origin, religion, sexual orientation, disability, language, and socioeconomic status) and context Introduces innovation/creativity into application of teaching method

13. Supervision: Supervision and training in the professional knowledge base of enhancing and monitoring the professional functioning of others.			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
13A. Expectations and Roles			
- As a supervisee, is routinely unprepared for supervision (e.g., unsure what to discuss, fails to bring clinical materials for review). - As a supervisee, fails to disclose information that supervisor needs to understand clinical situations and/or accurately assess supervisee's training needs and performance.	Demonstrates basic knowledge of expectations for supervision Examples: - Demonstrates knowledge of the process of supervision - Articulates components of effective supervision such as the working alliance	Demonstrates knowledge of, purpose for, and roles in supervision Examples: - Identifies roles and responsibilities of the supervisor and supervisee in the supervision process - Demonstrates understanding of supervisor and supervisee roles in relation to client - Demonstrates understanding of vicarious liability of the supervisor	Understands the ethical, legal, and contextual issues of the supervisor role Examples: - Articulates a model of supervision and reflects on how this model is applied in practice, - Integrates contextual, legal, and ethical perspectives in supervision vignettes - Writes supervisory contract that accurately reflects roles and expectations of supervisor and supervisee
13B. Processes and Procedures			
Does not demonstrate a basic understanding of supervision models and practice.	Demonstrates basic knowledge of supervision models and practice	Identifies and tracks progress achieving the goals and tasks of supervision; demonstrates basic knowledge of supervision models and practices Examples: - Presents goals and related tasks of supervisee's growth and development - Demonstrates ability to monitor and communicate progress on goals	Demonstrates knowledge of supervision models and practices; demonstrates knowledge of and effectively addresses limits of competency to supervise Examples: - Prepares supervision contract - Assesses supervision competency - Constructs plans to deal with areas of limited competency - Articulates range of supervision methods available and the utility of such methods - Demonstrates knowledge of the scholarly literature on supervision - Identifies the basic tenets of specific model of supervision

Supervision continued			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
13C. Skills Development			
• Cannot describe different models of supervision • Fails to complete self-assessment related to supervisory skills.	Displays interpersonal skills of communication and openness to feedback Examples: • Completes self-assessment (e.g., Hatcher & Lassiter, 2006) • Integrates faculty/supervisor feedback into self-assessment	Demonstrates knowledge of the supervision literature and how clinicians develop to be skilled professionals Examples: • Successfully completes coursework on supervision • Demonstrates formation of supervisory relationship integrating theory and skills including knowledge of development, educational practice	Engages in professional reflection about one's clinical relationships with supervisees, as well as supervisees' relationships with their clients Examples: • Articulates how supervisory relationships may enhance the development of supervisees and their clients • Elicits evaluation from supervisee about supervisory relationship and uses feedback to improve quality of supervision
13D. Supervisory Practices			
• Fails to provide candid and sensitive critical feedback to supervisee when needed. • Is a passive observer rather than actively engaged in group supervision • Discounts supervisee's perspective or opinion. • Does not seek supervision of supervision &/or supervisory consultation when needed. • Fails to adopt a genuine openness to feedback from supervisees as evident from developmental stagnation as a supervisor.	• Awareness of need for straightforward, truthful, and respectful communication in supervisory relationship. • Demonstrates willingness to admit errors, accept feedback.	Provides helpful supervisory input in peer and group supervision Examples: • Identifies core skills on which to provide feedback to peers • Demonstrates ability to provide constructive criticism to peers	Provides effective supervised supervision to less advanced students, peers, or other service providers in typical cases appropriate to the service setting Examples: • Helps supervisee develop evidence based treatment plans • Directs supervisee to literature that may inform case • Provides supervision input according to developmental level of supervisee • Encourages supervisee to discuss reactions and helps supervisee develop strategies to use reactions in service of clients • Presents supervisor of supervision with accurate account of case material and supervisory relationship, seeks input, and utilizes feedback to improve outcomes

VI. SYSTEMS

14. Interdisciplinary Systems: Knowledge of key issues and concepts in related disciplines. Identify and interact with professionals in multiple disciplines.			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
14A. Knowledge of the Shared and Distinctive Contributions of Other Professions			
Lacks awareness or understanding of basic contributions of allied professions such as social work, psychiatry	Demonstrates knowledge, respect, and valuing of roles, functions and service delivery systems of other professions	Demonstrates beginning, basic knowledge of the viewpoints and contributions of other professions/professionals Examples: - Articulates the roles of other professions in patient care - Awareness of various levels of education and training required for other professions involved in patient care	Demonstrates awareness of multiple and differing worldviews, roles, professional standards, and contributions across contexts and systems; demonstrates intermediate level knowledge of common and distinctive roles of other professionals Examples: - Reports observations of commonality and differences among professional roles, values, and standards - Demonstrates respect for and value of contributions from related professions
14B. Functioning in Multidisciplinary and Interdisciplinary Contexts			
Displays defensiveness when interacting with others	Cooperates with others Examples: - Cooperates with others in task completion - Demonstrates willingness to listen to others	Demonstrates beginning knowledge of strategies that promote interdisciplinary collaboration vs. multidisciplinary functioning Examples: - Compares and contrast multidisciplinary functioning and interdisciplinary collaboration - Describes a hypothetical case involving both interdisciplinary collaboration and multidisciplinary functioning	Demonstrates beginning, basic knowledge of and ability to display the skills that support effective interdisciplinary team functioning Examples: - Demonstrates skill in interdisciplinary clinical settings in working with other professionals to incorporate psychological information into overall team planning and implementation - Communicates without jargon - Effectively resolves disagreements about diagnosis or treatment goals - Maintains own position when appropriate while acknowledging the value of others' positions and initiates mutually accepting resolutions - Supports and utilizes the perspectives of other team members

Interdisciplinary Systems continued			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
14C. Understands how Participation in Interdisciplinary Collaboration/Consultation Enhances Outcomes			
Does not practice reflective listening skills or dominates discussion with own ideas	Demonstrates understanding of collaboration/consultation concept	Demonstrates knowledge of how participating in interdisciplinary collaboration/consultation can be directed toward shared goals Examples: - Identifies common challenges in delivering collaborative care - Articulates examples from the literature or direct experience on benefits of delivering collaborative care	Participates in and initiates interdisciplinary collaboration/consultation directed toward shared goals Examples: - Engages in consultation with allied professionals in service of clients - Demonstrates ability to communicate shared goals
14D. Respectful and Productive Relationships with Individuals from Other Professions			
Makes dismissive or disparaging comments of other disciplines	Demonstrates awareness of the benefits of forming collaborative relationships with other professionals Examples: - Expresses interest in developing collaborative relationships and respect for other professionals - Articulates the advantages in patient care afforded by working collaboratively with other disciplines	Develops and maintains collaborative relationships and respect for other professionals Examples: - Communicates effectively with individuals from other professions - Demonstrates knowledge of mechanisms necessary to maintain collaborative relationships	Develops and maintains collaborative relationships over time despite differences Examples: - Appreciates and integrates perspectives from multiple professions - Effectively relates to other professionals in accordance with their unique patient care roles
15. Management-Administration: Manage the direct delivery of services (DDS) and/or the administration of organizations, programs, or agencies (OPA).			
15A. Appraisal of Management and Leadership			
- Passively accepts management and leadership without recognition of personal responsibility to engage and improve the institution. - Hostile denigration of management and leadership without genuine attempt to engage and constructively work to improve the institution.	No expectation at this level	Forms autonomous judgment of organization's management and leadership Examples: - Applies theories of effective management and leadership to form an evaluation of organization - Identifies specific behaviors by management and leadership that promote or detract from organizational effectiveness	Develops and offers constructive criticism and suggestions regarding management and leadership of organization Examples: - Identifies strengths and weaknesses of management and leadership or organization - Provides input appropriately; participates in organizational assessment

Management-Administration continued			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
15B. Management			
• Naïve understanding of functional role as independent of the institutional structures.	No expectation at this level	Demonstrates awareness of roles of management in organizations Examples: • Articulates understanding of management role in own organization(s) • Responds appropriately to direction provided by managers	Participates in management of direct delivery of professional services; responds appropriately in management hierarchy Examples: • Responds appropriately to managers and subordinates • Manages DDS under supervision, e.g., scheduling, billing, maintenance of records • Identifies responsibilities, challenges, and processes of management
15C. Administration			
• Attempts to solve problems without engaging the administrative structures consistent with institutional policies and procedural guidelines. • Operates in ways that are counter to the institution's policies and procedural guidelines.	Complies with regulations Examples: • Completes assignments by due dates • Complies with relevant regulations; follows established procedures • Responds appropriately to direction provided by managers • Participates in trainings mandated by organization	Demonstrates knowledge of and ability to effectively function within professional settings and organizations, including compliance with policies and procedures Examples: • Articulates approved organizational policies and procedures • Completes reports and other assignments promptly • Complies with record-keeping guidelines	Demonstrates emerging ability to participate in administration of service delivery programs Examples: • Demonstrates emerging leadership in clinical situations or clinical teams • Participates in institutional committees or workgroups • Develops new program offerings or clinical services
15D. Leadership			
• Unwilling/Unable to contribute to the administrative functioning of the institution. • Behaves in ways that are hostile to the administrative policies and procedural guidelines of the institution.	No expectation at this level	No expectation at this level	Participates in system change and management structure Examples: • Provides others with face to face and written direction • Communicates appropriately to parties at all levels in the system • Participates in organizational committees • Participates in institutional planning

16. Advocacy: Actions targeting the impact of social, political, economic or cultural factors to promote change at the individual (client), institutional, and/or systems level.			
Does Not Meet Expectations	Pre-internship Level	Readiness For Internship	Readiness For Entry To Practice
16A. Empowerment			
• Lack of awareness that social, political, economic, or cultural factors may impact human development. • Lack of awareness that service provision must be sensitive to social, political, economic, or cultural factors.	Demonstrates awareness of social, political, economic and cultural factors that impact individuals, institutions and systems, in addition to other factors that may lead them to seek intervention Examples: • Articulates social, political, economic or cultural factors that may impact on human development and functioning • Demonstrates the recognition of the importance of consideration of these factors as part of the therapeutic process	Uses awareness of the social, political, economic or cultural factors that may impact human development in the context of service provision Examples: • Identifies specific barriers to client improvement, e.g., lack of access to resources • Assists client in development of self-advocacy plans	Intervenes with client to promote action on factors impacting development and functioning Examples: • Promotes client self-advocacy • Assesses implementation and outcome of client's self-advocacy plans
16B. Systems Change			
• Unable to identify means by which therapists may promote systems change(s) important to the individuals the therapist serves. • Unwillingness to engage in advocacy at systems level.	Understands the differences between individual and institutional level interventions and system's level change Examples: • Articulates role of therapist as change agent outside of direct patient contact • Demonstrates awareness of institutional and systems level barriers to change	Promotes change to enhance the functioning of individuals Examples: • Identifies target issues/agencies most relevant to specific issue • Formulates and engages in plan for action • Demonstrates understanding of appropriate boundaries and times to advocate on behalf of client	Promotes change at the level of institutions, community, or society Examples: • Develops alliances with relevant individuals and groups • Engages with groups with differing viewpoints around issue to promote change

APPENDIX S

Alumni Survey Draft



Alum Survey
(administered via Qualtrics)

Name:

Graduation Year (use YYYY format):

Your Email Address (if you'd like to be sent a copy of this evaluation):

Across the last academic year were you in a formal postdoctoral training program OR employed professionally (not formal postdoctoral training)?

- ☐ Yes, formal postdoctoral training
- ☐ Yes, employed professionally
- ☐ No

Display This Question:

If Across the last academic year were you in a formal postdoctoral training program OR employed prof... = Yes, formal postdoctoral training

Select the emphasis of the postdoctoral training program

- ☐ Primarily clinical
- ☐ Primarily research
- ☐ Equally clinical and research
- ☐ Other

Display This Question:

If Across the last academic year were you in a formal postdoctoral training program OR employed prof... = Yes, formal postdoctoral training

Select all setting types that apply to this position

- ☐ Academic Teaching
- ☐ Community Mental Health Center
- ☐ Consortium

- ☐ Correctional Facility
- ☐ Health Maintenance Organization
- ☐ Hospital / Medical Center
- ☐ Independent Practice
- ☐ Psychiatric Facility
- ☐ School District or System
- ☐ University Counseling Center
- ☐ Other

Display This Question:

If Across the last academic year were you in a formal postdoctoral training program OR employed prof... = Yes, formal postdoctoral training

Select all activities that apply to this position

- ☐ Administration
- ☐ Assessment
- ☐ Consultation
- ☐ Psychotherapy
- ☐ Research
- ☐ Supervision
- ☐ Teaching
- ☐ Other

Display This Question:

If Across the last academic year were you in a formal postdoctoral training program OR employed prof... = Yes, employed professionally

Select all setting types that apply to this position

- ☐ Academic Teaching
- ☐ Community Mental Health Center
- ☐ Consortium
- ☐ Correctional Facility
- ☐ Health Maintenance Organization
- ☐ Hospital/Medical Center
- ☐ Independent Practice
- ☐ Psychiatric Facility
- ☐ School District or System
- ☐ University Counseling Center
- ☐ Other

Display This Question:

If Across the last academic year were you in a formal postdoctoral training program OR employed prof... = Yes, employed professionally

Select all activities that apply to this position

- ☐ Administration
- ☐ Assessment
- ☐ Consultation
- ☐ Psychotherapy
- ☐ Research
- ☐ Supervision
- ☐ Teaching
- ☐ Other

Display This Question:

If Across the last academic year were you in a formal postdoctoral training program OR employed prof... = Yes, employed professionally

Enter the job title of this position

Have you obtained a license as a psychologist?

- ☐ Yes
- ☐ No
- ☐ Not yet eligible for licensing

Next you will be rating how well your training prepared you across a range of profession-wide competencies. For each of the competencies below, please slide the bar along the scale provided to show how far you felt your training supported your preparation in that area. If you cannot make an informed rating for some reason (e.g., you do not engage in activities that involve a given competency), select "Not Applicable" and leave the slider bar alone. You can hover your mouse over the categorical descriptors under each rating scale to activate a pop-up box with specific behavioral anchors that may help you with your ratings (pop-ups need to be

enabled in your browser).

NOTE TO REVIEWERS: Each of the ratings requested below are responded to via a visual analog scale that looks as below. For efficiency, the scale is not reproduced below each item.

☐ No information



Training did not prepare

Training resulted in
competency sufficient
for independent practice

I. PROFESSIONALISM

1. Professional Values and Attitudes - as evidenced in behavior and comportment that reflect the values and attitudes of psychology.

- 1.A. **Integrity**- Honesty, personal responsibility and adherence to professional values.
- 1.B. **Deportment**
- 1.C. **Accountability**
- 1.D. **Concern for the Welfare of Others**
- 1.E. **Professional Identity**

2. Individual and Cultural Diversity - Awareness, sensitivity and skills in working professionally with diverse individuals, groups and communities who represent various cultural and personal background and characteristics defined broadly and consistent with APA policy.

- 2.A. **Self as Shaped by Individual and Cultural Diversity** (e.g., cultural, individual, and role differences, including those based on age, gender, gender identity, race, ethnicity, culture, national origin, religion, sexual orientation, disability, language, and socioeconomic status) **and Context.**
- 2.B. **Others as Shaped by Individual and Cultural Diversity and Context.**
- 2.C. **Interaction of Self and Others as Shaped by Individual and Cultural Diversity and Context.**
- 2.D. **Applications based on Individual and Cultural Context**

3. Ethical Legal Standards and Policy - Application of ethical concepts and awareness of legal issues regarding professional activities with individuals, groups, and organizations.

- 3.A. **Knowledge of Ethical, Legal, and Professional Standards and Guidelines**
- 3.B. **Awareness and Application of Ethical Decision Making.**
- 3.C. **Ethical Conduct**

4. Reflective Practice/Self-Assessment/Self-Care: Practice conducted with personal and professional self-awareness and reflection; with awareness of competencies; with appropriate self-care.

4.A. **Reflective Practice**

4.B. **Self- Assessment**

4.C. **Self-Care** (attention to personal health and well-being to assure effective professional functioning)

4.D. **Participation in Supervision Process**

II. RELATIONAL

5. Relationships

5.A. **Interpersonal Relationships**

5.B. **Affective Skills**

5.C. **Expressive Skills**

III. SCIENCE

6. Scientific Knowledge and Methods - Understanding of research, research methodology, techniques of data collection and analysis, biological bases of behavior, cognitive-affective bases of behavior, and development across the lifespan. Respect for scientifically derived knowledge.

6.A. **Scientific Mindedness**

6.B. **Scientific Foundation of Psychology**

6.C. **Scientific Foundation of Professional Practice**

7. Research/ Evaluation - Generating research that contributes to the professional knowledge base and/or evaluates the effectiveness of various professional activities.

7.A. **Scientific Approach to Knowledge Generation**

7.B. **Application of Scientific Method to Practice**

IV. APPLICATION

8. Evidence-Based Practice - Integration of research and clinical expertise in the context of patient factors.

8.A. **Knowledge and Application of Evidence-Based Practice**

9. Assessment - Assessment and diagnosis of problems, capabilities, and issues associated with individuals, groups, and/or organizations.

9.A. **Knowledge of Measurement and Psychometrics**

9.B. **Knowledge of Assessment Methods**

9.C. **Application of Assessment Methods**

9.D. **Diagnosis**

9.E. **Conceptualization and Recommendations**

9.F. Communication of Assessment Findings

10. Intervention - Interventions designed to alleviate suffering and to promote health and well-being of individuals, groups, and/or organizations.

- 10.A. Intervention Planning**
- 10.B. Skills**
- 10.C. Intervention Implementation**
- 10.D. Progress Evaluation**

11. Consultation - The ability to provide expert guidance or professional assistance in response to a client's needs or goals.

- 11.A. Role of Consultant**
- 11.B. Addressing Referral Question**
- 11.C. Communication of Consultation Findings**
- 11.D. Application of Consultation Methods**

V. EDUCATION

12. Teaching - Providing instruction, disseminating knowledge, and evaluating acquisition of knowledge and skill in professional psychology.

- 12.A. Knowledge**
- 12.B. Skills**

13. Supervision - Supervision and training in the professional knowledge base of enhancing and monitoring the professional functioning of others.

- 13.A. Expectations and Roles**
- 13.B. Processes and Procedures**
- 13.C. Skills Development**
- 13.D. Supervisory Practices**

VI. SYSTEMS

14. Interdisciplinary Systems - Knowledge of key issues and concepts in related disciplines. Identify and interact with professionals in multiple disciplines.

- 14.A. Knowledge of the Shared and Distinctive Contributions of Other Professions**
- 14.B. Functioning in Multidisciplinary and Interdisciplinary Contexts.**
- 14.C. Understands How Participation in Interdisciplinary Collaboration/ Consultation Enhances Outcomes.**
- 14.D. Respectful and Productive Relationships with Individuals from Other Professions.**

15. Management-administration - manage the direct delivery of services (DDS) and/or the administration of organizations, programs, or agencies (OPA).

- 15.A. Appraisal of Management and Leadership**

- 15.B Management
- 15.C. Administration
- 15.D. Leadership

16. Advocacy - Actions targeting the impact of social, political, economic, or cultural factors to promote change at the individual (client), institutional, and/or systems level.

- 16.A. Empowerment
- 16.B. Systems Change

Start of Block: 17. Professional Activities

The following questions are designed to help us understand the nature and extent to which our graduates integrate science and practice in their work activities.

Since graduation, have you conducted (or assisted with) original research relevant to clinical psychology?

- ☐ Yes
- ☐ No

Since graduation, have you engaged in direct clinical service?

- ☐ Yes
- ☐ No

Skip To: QID231 If Since graduation, have you engaged in direct clinical service? = No

Have you engaged in evidence-based assessment or therapy approaches?

- ☐ Yes
- ☐ No

Since graduation, have you engaged in supervision of clinical work?

- ☐ Yes
- ☐ No

Skip To: QID237 If Since graduation, have you engaged in supervision of clinical work? = No

If providing supervision of direct clinical work, have you taught evidence based assessment or therapy approaches?

☐ Yes

☐ No

Since graduation, have you engaged in consultation, in-services, peer-supervision?

☐ Yes

☐ No

Skip To: QID232 If Since graduation, have you engaged in consultation, in-services, peer-supervision? = No

Have you advised others to use evidence-based approaches or to approach their clinical work scientifically?

☐ Yes

☐ No

Since graduation, have you engaged in dissemination activities?

☐ Yes

☐ No

Skip To: QID233 If Since graduation, have you engaged in dissemination activities? = No

Have you helped others to engage in evidence-based practice?

☐ Yes

☐ No

Since graduation, have you taught/mentored undergraduates, graduate students, postdoctoral fellows, or allied health professionals?

☐ Yes

☐ No

Skip To: QID234 If Since graduation, have you taught/mentored undergraduates, graduate students, postdoctoral fellow... = No

Have you also taught others how to engage in scientific research, how to consider the scientific bases of clinical psychology, or how to engage in evidence-based practice?

☐ Yes

☐ No

Since graduation, have you engaged in volunteer or paid service to the profession to help advocate for clinical psychology research, scientifically-oriented training, and/or evidence-based practice?

☐ Yes

☐ No

Please enter your current job title(s). If you have more than one official job title, please separate via punctuation (*for example, my entry would list: Professor. Director of Clinical Training. Licensed Psychologist. Journal Editor.*)

If you would like to provide additional feedback regarding the program's preparation for your professional activities, please enter it below.

End of Block: Additional Feedback and Captcha
